DBT: Deb's Board Training Domain Packet

Area II: Assessment and Planning

357 original practice questions, grouped by sub-area. Questions come first; answers and rationales for every option are in a separate section at the back, so no answer appears on the same page as its question.

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All questions are original practice items written for this study set; they are not ASWB questions. The ID beside each answer lets you report a problem with a question in the app.

II.A Assessment Concepts

1. Two days after a tornado destroyed several homes in a small town, a social worker staffs a community shelter. Many residents report poor sleep, jumpiness at loud sounds and repeated images of the storm. Most are still caring for their families and helping with cleanup. What **MOST** likely explains these responses?

A. Posttraumatic stress disorder

B. Expected acute stress reactions

C. Acute stress disorder

D. Adjustment disorder with anxiety

2. A client's partner died 5 months ago. The client kept working, cared for two children, and handled the funeral and finances. Last week the client's car was towed, and since then the client cannot sleep, cries at work, and says, 'I can't handle one more thing.' In Golan's crisis model, the towing is **MOST** likely which element?

A. Hazardous event

B. Vulnerable state

C. Precipitating factor

D. Active crisis state

3. A teacher asks the school social worker to begin an ADHD referral for an 8-year-old. Over the past month, the student has stopped finishing work, jumps at loud sounds, and acts out hiding under a desk during play. The parent mentions a shooting on their street 6 weeks ago. What should the social worker do **FIRST**?

A. Send the ADHD rating scales the teacher requested to the parent

B. Assess the student's exposure to violence and reactions

C. Refer the student for trauma-focused therapy

4. In a nursing facility, a resident with major neurocognitive disorder strikes staff during evening showers but calmly accepts morning sponge baths. An adult child shares that the resident was held in a crowded detention camp as a teenager, where guards forced group showers. Staff ask the social worker to request a sedative before showers. What is the **BEST** response?

A. Ask the prescriber to consider a sedative before showers

B. Help the resident talk through the camp memories

C. Record the hitting as aggression from the dementia

D. Adjust the shower routine to remove the camp reminder

5. At a veterans clinic, a client describes following an order during deployment that led to civilian deaths. The client reports deep shame, says, 'I'm not a good person anymore,' and has pulled away from a faith community they once valued. Which concept **MOST** likely describes this reaction?

A. Survivor guilt

B. Moral injury

C. Secondary traumatic stress

D. Prolonged grief disorder

6. At a disaster recovery center 7 weeks after a wildfire destroyed their home, a client reports nightmares about the flames most nights, avoids driving past the burned area, and has missed work for 3 weeks. A brief screen at the center confirms these symptoms, and the client reports no suicidal thoughts. What should the social worker do **NEXT**?

A. Refer the client for a trauma-focused mental health evaluation

B. Reassure the client that these reactions are expected after disasters

C. Rescreen the client in a month to see if symptoms fade

7. In a residential treatment program, a 14-year-old with years of abuse before foster placement has sudden outbursts, freezes and stares blankly at times, and sleeps poorly. Staff notice that outbursts follow loud voices and room checks at night. The team proposes treating the youth for bipolar disorder. What is the social worker's **BEST** contribution to the team?

A. Support the plan to treat the youth for bipolar disorder

B. Recommend a new placement to reduce the outbursts

C. Point out that outbursts track reminders of earlier trauma

8. Eight months after a hurricane damaged their home, a family that had been coping well becomes overwhelmed. Since the storm, one parent lost a job, insurance disputes dragged on, and a child's asthma has worsened. In the Double ABCX model, these later events are part of which factor?

A. Pile-up (aA factor)

B. Family resources (bB factor)

C. Family perception (cC factor)

D. Adaptation (xX factor)

9. Two neighboring families lose their homes in the same river flood. One family starts rebuilding within weeks. The other family received the larger insurance payout, and relatives nearby offer it rooms, yet its daily routines collapse. The parents tell a disaster social worker, 'This proves we were always going to lose everything.' In Hill's ABC-X model, which factor **MOST** likely explains the difference?

A. The family's existing resources

B. The family's definition of the event

C. The pile-up of later stressors

10. At an outpatient intake, an American Indian client describes a heavy grief that began before any loss in their own life. The client's grandparents were sent to boarding schools that punished them for speaking their language, and the client's parent rarely spoke about that time. The client says the family 'carries a sadness nobody names.' Which concept **MOST** likely fits this presentation?

A. Complex trauma

B. Historical trauma

C. Acculturative stress

D. Vicarious trauma

11. In an integrated primary care clinic, a social worker completes a routine intake screening with a new adult client. When the social worker reads a question about past physical or sexual violence, the client's hands begin to shake, and the client says quietly, 'I don't really talk about that.' What is the **BEST** response?

A. Reflect the distress and offer a choice to pause

B. Gently ask what happened so the screening is complete

C. Move on to the next question to finish the screening

12. A refugee resettlement social worker completes an assessment with an adult client who fled armed conflict 8 months ago. The client reports poor sleep, irritability and occasional nightmares about the war. The client says that what keeps them awake most nights is an eviction notice, a job search with no results, and no word from a sibling still in a camp. What should the social worker do **NEXT**?

A. Refer the client for trauma-focused therapy for war memories

B. Teach sleep hygiene and relaxation skills for the insomnia

C. Help the client address housing, work and family tracing

13. A 14-year-old tells a school social worker they have felt 'huge' since joining a dance team where teammates trade diet tips. At home, a parent often criticizes their own weight, and the student follows several fitness accounts online. Which framework **BEST** accounts for these sources of the student's body dissatisfaction?

A. Objectification theory

B. Social comparison theory

C. Tripartite influence model

D. Social learning theory

14. At a pediatric clinic, a parent reports that their 11-year-old, whose weight is in the healthy range, recently began calling themselves 'fat' and comparing their stomach to a sibling's. The parent recently put the family on a strict diet and weighs the child each morning 'to keep everyone on track.' What is the social worker's **MOST** appropriate response?

A. Teach the child to challenge negative thoughts about their body

B. Have the parent arrange an eating disorder evaluation

C. Help the parent swap dieting for regular shared meals

15. A school social worker notices that a 9-year-old has stopped going outside at recess and now wears long sleeves on hot days. When asked, the student says, 'I don't want to get darker. Lighter skin is prettier.' What should the social worker do **FIRST**?

A. Reassure the student that their skin color is beautiful

B. Ask where the student heard ideas about skin color

C. Plan a class lesson on diverse beauty standards

16. Five days after surgery that created a permanent colostomy for rectal cancer, a 62-year-old hospital patient will not look at the stoma and asks nurses to handle all of its care. On the social worker's first visit, the patient says, 'I'm disgusting now. Who would want to be near me?' What is the **MOST** appropriate response?

A. Screen the patient for depression with a standard tool

B. Offer a referral to an ostomy peer visitor program

C. Invite the patient to talk about what the change means to them

17. A week after a parent's death, a client spends the session describing research on the stages of grief and how bereavement affects the immune system. When the social worker asks how the client is feeling, the client returns to discussing studies. Which defense mechanism is the client **MOST** likely using?

A. Intellectualization

B. Denial

C. Rationalization

D. Reaction formation

18. A client who was passed over for a promotion says they feel fine about it. Over the next few sessions, the client often describes the coworker who got the job as bitter, envious and out to undermine the client. Other staff describe the coworker as friendly and supportive. Which defense mechanism **MOST** likely explains the client's view of the coworker?

A. Displacement

B. Reaction formation

C. Rationalization

D. Projection

19. In a residential treatment program, a roommate earns the early weekend pass that a resident had been working toward. Over the next week, the resident praises the roommate loudly in every group, buys the roommate snacks and calls the roommate 'the kindest person here.' Staff notice the praise sounds forced, and the resident tenses whenever the pass comes up. Which defense mechanism is the resident **MOST** likely using?

A. Reaction formation

B. Idealization

C. Altruism

20. On an inpatient psychiatric unit, a patient tells the day nurse and the social worker that they are the only staff who truly understand. The patient describes the evening staff as heartless and incompetent. At the next team meeting, day and evening staff argue sharply over whether the patient's privileges should be reduced. Which defense is the patient **MOST** likely using?

A. Projection

B. Displacement

C. Splitting

21. Two days after a home invasion, a client meets with a social worker in a prosecutor's victim services office. The client gives a precise, minute-by-minute account of being tied up and threatened with a gun. Throughout, the client speaks in a flat, even tone, shows no visible emotion and asks what happens next in the case. Which defense mechanism is the client **MOST** likely using?

A. Dissociation

B. Intellectualization

C. Denial

D. Isolation of affect

22. A person chooses on purpose to set a painful worry aside until a better time. Which defense mechanism is this?

A. Suppression

B. Repression

C. Denial

23. In a fourth session, a client repeatedly accuses the social worker of being cold and judgmental. The client mocks each suggestion and cancels the next appointment at the last minute. As the session goes on, the social worker begins giving short, clipped replies and later tells a supervisor about feeling unusually irritated. Which defense is the client **MOST** likely using?

A. Projection

B. Projective identification

C. Splitting

D. Passive aggression

24. At a family service agency, the caregiver of a 6-year-old reports that the child's parent left on a military deployment 3 weeks ago. Since then, the child, who was fully toilet trained by age 3, has begun wetting the bed, asks to be fed with a spoon and talks in a baby voice. Which defense mechanism is the child **MOST** likely using?

A. Regression

B. Fixation

C. Displacement

25. A hospital patient awaiting surgery describes an argument with an adult sibling. In anger, the patient told the sibling, 'I hope you get what's coming to you.' Since then, whenever the sibling is mentioned, the patient knocks three times on the bed rail, repeats a set phrase and asks staff to send the sibling a small gift. The patient says, 'I keep worrying my words could make something bad happen.' Which defense mechanism is the patient **MOST** likely using?

A. Reaction formation

B. Undoing

C. Isolation of affect

D. Displacement

26. A 10-year-old in a residential treatment program was removed from a home where a caregiver often screamed threats and insults. Staff report that the child now corners younger residents and shouts at them using the caregiver's exact words, tone and gestures. When staff step in, the child laughs and says, 'Nobody can push me around now.' Which defense mechanism is the child **MOST** likely using?

A. Identification with the aggressor

B. Displacement

C. Acting out

D. Projection

27. A 20-year-old college student lost a soccer scholarship after a knee injury ended the student's playing career. Within a semester, the student joined the debate team and now prepares 30 hours a week. In counseling, the student says, 'I'm not the strong one anymore, so I have to be the smartest one in every room.' Which defense mechanism does this **MOST** likely reflect?

A. Sublimation

B. Compensation

C. Rationalization

28. In an intensive outpatient program, a client describes drinking at a coworker's party after 40 days without alcohol. The client says, 'Everyone was toasting, and refusing would have been rude. Besides, after the week I had, anyone would have had a drink.' Which defense mechanism is the client **MOST** likely using?

A. Denial
B. Rationalization
C. Intellectualization

29. According to U.S. law enforcement data, which statement describes most people who sexually assault children?

A. They are strangers to the child
B. They first met the child online
C. They know the child or family
D. They are younger than 18

30. At intake for a court-ordered partner violence program, a participant says, 'I lose my temper and can't stop myself.' The participant describes staying calm when a manager yells at them in front of coworkers. Twice, the participant smashed the partner's phone after the partner went out with friends. Which explanation is **MOST** likely supported by this pattern?

A. Purposeful control of the partner
B. Poor impulse control across settings
C. Reaction to the partner's provocations
D. Job stress displaced onto home

31. A 3-year-old has had 8 hospital stays for breathing pauses that only the parent has witnessed. The parent, a former nursing assistant, is warm with staff and presses for more invasive tests. During one monitored stay, the parent was away for 4 days and no pauses occurred. Soon after the parent returned, a pause requiring resuscitation happened while the parent was alone with the child. Which explanation is **MOST** likely?

A. An undiagnosed breathing disorder
B. Parental anxiety causing overreporting
C. Breath-holding spells triggered by distress
D. Illness induced by the caregiver

32. A parent asks a youth program's social worker about a volunteer coach who has grown close to the parent's 12-year-old. The coach praises the child in front of the team, offers rides home after practice, and recently gave the child a phone, saying the gift should stay 'just between us.' Which behavior is the **MOST** concerning sign of grooming?

A. Asking that the gift be kept from others

B. Praising the child in front of the team

C. Offering the child rides home after practice

33. A 2-year-old has been in a children's hospital for 3 weeks, and the parents can visit only on weekends. For the first few days, the child screamed and searched the doorway for the parents. Now the child lies quietly, plays little, eats poorly and rarely cries. Nurses describe the child as 'settled.' Which phase of separation response is the child **MOST** likely in?

A. Protest

B. Detachment

C. Despair

D. Rapprochement

34. A 7-year-old in foster care has weekly visits with a birth parent, and reunification is the case goal. After each visit, the child cries, wets the bed and pushes the foster parents away for a day or two. The foster parents ask to reduce the visits. Visit notes from the past 2 months describe a warm, appropriate parent and no safety concerns. What should the social worker do **NEXT**?

A. Help the foster parents plan supportive routines around visits

B. Request fewer visits until the foster child's behavior settles

C. Ask the birth parent to avoid talk of reunification at visits

35. A 51-year-old client was released 3 weeks ago after 14 years in prison and now lives with a sibling. The sibling says the client waits to be told when to eat or shower, freezes in the grocery store when faced with choices, and sits facing the door in every room. The client says they are glad to be home. At last week's visit, both learned that these are common adaptations to long incarceration. What should the social worker do **NEXT**?

A. Ask the sibling to set the daily schedule and make choices for now

B. Help the client practice making small daily choices with sibling support

C. Refer the client for an evaluation for major depressive disorder

36. A 9-year-old has lived with a foster family for 4 months and has weekly visits with their birth parent, who is working toward reunification. The foster parent reports that the child refuses outings and recently said, 'If I have fun here, it means I don't love my real family.' The birth parent has been cooperative with the agency. What would be **MOST** helpful?

A. Help the birth parent tell the child it is fine to have fun there

B. Tell the child the birth parent would want them to have fun

C. Refer the child for an evaluation of a possible attachment disorder

37. A 15-year-old has spent 10 months in a residential treatment program and has met most treatment goals. Discharge home is set for 3 weeks from now, and the youth has said little about it. In the past week, the youth has broken curfew twice and picked fights with peers, behaviors not seen in months. Staff ask the social worker whether discharge should be delayed. What is the **BEST** course of action?

A. Postpone discharge until the behavior settles for several weeks

B. Help the youth talk about leaving while keeping the planned date

C. Keep the planned date and review program rules with the youth

38. At a memory clinic, the adult child who cares full-time for a parent with major neurocognitive disorder meets with the social worker. The caregiver quit a job last month, sleeps about 4 hours a night because the parent wanders, and has begun snapping at their own teenagers. Siblings out of state send money but no hands-on help. Which response is **BEST**?

A. Give the caregiver a standardized caregiver burden questionnaire

B. Connect the caregiver with respite care and a day program

C. Refer the caregiver to individual psychotherapy for stress

39. The spouse of a nursing home resident with advanced major neurocognitive disorder visits every day. The resident no longer recognizes the spouse. The spouse tells the social worker, 'I don't know whether to think of myself as married anymore.' The spouse has put off decisions about selling their house and feels guilty about accepting a friend's dinner invitation. Which concept **MOST** likely describes the spouse's experience?

A. Anticipatory grief

B. Disenfranchised grief

C. Caregiver burnout

D. Ambiguous loss

40. A school social worker meets with a 14-year-old who often falls asleep in class and has fallen behind on assignments. The student explains that a parent with multiple sclerosis needs help getting out of bed, bathing and eating, and the other parent works nights. The student says, 'It's my job. I don't mind.' Which action is **BEST**?

A. Arrange in-home personal care with the parents

B. Arrange a school plan with the parents for extended deadlines

C. Refer the student to counseling for caregiver stress

41. Parents aged 79 and 81 have cared at home for decades for their 50-year-old adult child, who has schizophrenia. At a community mental health visit, the parents say they are slowing down and worry about who will help their child after they die. The adult child is stable, takes medication and attends a day program. Which response is **BEST**?

A. Help the parents draft a plan to share with their adult child later

B. Start planning meetings with the adult child as a core participant

C. Teach the parents to recognize early warning signs of relapse

42. A home health social worker meets with an adult child who cares full time for a 78-year-old parent with advanced Parkinson's disease. The caregiver reports back pain, poor sleep and missed work. When the social worker mentions an adult day program, the caregiver shakes their head and says, 'That's not how our family does things.' What should the social worker do **NEXT**?

A. Ask what kinds of help would feel acceptable to the caregiver

B. Arrange in-home aide visits in place of the day program

C. Urge the caregiver to try the day program for a week

43. Parents of a 6-year-old in treatment for leukemia ask the hospital social worker for help with their 10-year-old, who has started skipping homework and slamming doors. Since the diagnosis, the parents have spent most nights at the hospital, and relatives have cared for the 10-year-old. The parents want a reward chart for school behavior. What should the social worker do **FIRST**?

A. Help the parents set up a reward chart for finishing homework

B. Refer the 10-year-old for an evaluation for ADHD

C. Hear from the 10-year-old how home life has changed

44. At a community health clinic, a client who works at a nail salon seeks care for back pain. Privately, the client says the owner keeps the client's passport, deducts a growing 'travel debt' from each paycheck, and warns that complaining will bring immigration agents. The client wants to keep working for now but asks what help exists. What is the social worker's **BEST** response?

A. Report the salon owner's conduct to local police today

B. With consent, link the client to anti-trafficking services

C. Explain how the client can apply for a T visa

45. In a video session, a 52-year-old client mentions plans to wire $6,000 to an online partner they have never met, who needs money for a 'customs fee' abroad. The client has already sent $9,000 in gift cards for earlier emergencies. The client otherwise manages money well and says, 'It's my choice, and I trust them.' What response is **MOST** helpful?

A. Describe common scam signs and offer to check the story together

B. Accept the client's choice and return to the session agenda

C. Urge the client to end contact with the online partner

46. A 15-year-old in a group home returns from a weekend away with new clothes, a second phone and cash. The youth says a 24-year-old 'partner' pays for hotel rooms and sometimes asks the youth to spend time with the partner's friends, who then give the youth money. The youth insists no one forces anything. What should the social worker do **FIRST**?

A. Ask the youth whether threats or force were ever used

B. Make a safety plan with the youth for the next outing

C. Make a child abuse report for suspected sex trafficking

47. A client at a community health center paid a smuggler to bring them across the border. On arrival, the smuggler said the client now owed twice the agreed fee, took the client's identity papers, and placed the client in a restaurant kitchen that pays nothing until the debt is cleared. The client says, 'I chose to come, so this is my problem.' Which term **MOST** likely describes the client's current situation?

A. Human smuggling

B. Wage theft

C. Labor trafficking

48. A 76-year-old client in a senior center counseling program pays part of an adult grandchild's rent each month. The client manages their own bills, which are current, and explains, 'I want to help while I'm alive to see it.' The client describes no pressure from the grandchild. The client's adult child calls the social worker, angry, insisting the payments are elder abuse that must be reported. What is the **BEST** response?

A. File an adult protective services report as the child asks

B. Ask the client to pause the payments until the family agrees

C. Share the client's finances with the adult child to ease worry

D. Support the client's choice and decline to file a report

49. A parent tells the social worker that their 20-month-old, who used to explore the playground confidently, now clings to the parent and protests at separations, then pushes the parent away moments later. The child is otherwise healthy, and there have been no recent changes at home. What **MOST** likely explains the behavior?

A. Separation anxiety disorder

B. Insecure-avoidant attachment

C. Regression after a stressful event

D. Rapprochement subphase

50. A 48-year-old client says work and home life feel empty. The client recently quit mentoring new employees and dropped out of a community youth program, saying, 'Nothing I do will matter to anyone after me.' The client spends most free time alone watching TV. According to Erikson, which psychosocial crisis is the client **MOST** likely facing?

A. Intimacy versus isolation

B. Generativity versus stagnation

C. Integrity versus despair

D. Identity versus role confusion

51. Which change in a 74-year-old fits normal cognitive aging rather than a neurocognitive disorder?

A. Getting lost while driving a familiar route

B. Needing help to pay monthly bills

C. Asking the same question minutes after an answer

D. Recalling a name slowly but remembering it later

52. During a home visit, a parent worries that their 5-year-old talks and plays with an invisible friend. The child sets a place for the friend at meals but says, 'My friend is just pretend.' The child attends preschool, plays well with peers and sleeps normally. What is the **MOST** helpful response to the parent?

A. Recommend an evaluation for possible hallucinations

B. Suggest the parent gently discourage the pretend friend

C. Explain that imaginary companions are common at this age

53. A hospital social worker meets with a 5-year-old whose grandparent had a stroke this morning. The child says the stroke happened because the child refused to share toys at breakfast. Later, the child pats the hospital bed and says the bed feels sad. According to Piaget, this thinking is **MOST** likely typical of which stage?

A. Sensorimotor

B. Preoperational

C. Concrete operational

54. During a home visit, the parent of a 20-month-old tells an early childhood social worker that the toddler used to say 'ball,' 'milk' and 'up' but stopped using any words about 3 months ago. The toddler does not point to show objects and seldom turns when called by name. Relatives say the child will grow out of it. What should the social worker do **FIRST**?

A. Refer the child for a prompt developmental evaluation

B. Plan to recheck the child's language at the next visit

C. Ask the parent how the family feels about the changes

D. Coach the parent in daily language-building play

55. A child welfare social worker observes a supervised visit between a 15-month-old in foster care and the birth parent. After a brief separation, the parent returns and holds out their arms. The toddler starts toward the parent, then stops and freezes with a blank stare. Next, the toddler backs toward the parent with face turned away. Which attachment pattern does this behavior **MOST** likely reflect?

A. Secure

B. Insecure-avoidant

C. Insecure-ambivalent

D. Disorganized

56. In a school-based substance use group, a 16-year-old says two classmates crashed after drinking because 'they're bad drivers.' The teen adds that driving after a few beers is fine for them because 'nothing like that is going to happen to me.' Which developmental concept **MOST** likely explains this belief?

A. Imaginary audience

B. Identity diffusion

C. Personal fable

57. A parent at a pediatric clinic tells the integrated behavioral health social worker that their 2-year-old has turned 'defiant.' The toddler insists on putting on shoes alone, says 'no' to many requests and has brief tantrums when stopped. The toddler sleeps well, speaks in short phrases and plays happily at child care. What is the **MOST** appropriate response?

A. Explain that these behaviors fit typical toddler development

B. Refer the toddler for a disruptive behavior disorder evaluation

C. Ask the parent to track every tantrum for 2 weeks

58. A 19-year-old at a university counseling center is failing pre-nursing courses. Both parents are nurses, and the client says the plan to become a nurse was set at age 12. Asked what other fields have interested them, the client looks puzzled and says they have never looked into anything else. Which identity status does this **MOST** likely reflect?

A. Identity moratorium

B. Identity achievement

C. Identity foreclosure

D. Identity diffusion

59. A school social worker meets with a 9-year-old who has stopped turning in math assignments. The child compares their test scores with classmates' scores, says 'Everyone is smarter than me,' and refuses to try new tasks in case they fail. Which Erikson stage is the child **MOST** likely working through?

A. Initiative versus guilt

B. Industry versus inferiority

C. Autonomy versus shame and doubt

60. In a court-ordered program, a client says they follow every rule, even ones they think are unfair, because 'if everyone picked which laws to obey, society would fall apart.' The client adds that changing a bad law is up to lawmakers, not individuals. In Kohlberg's theory, this reasoning **MOST** likely reflects which level?

A. Preconventional

B. Conventional

C. Postconventional

61. A school social worker meets with the parent of a 14-year-old. Over the past 2 months, the teen has stopped seeing friends, quit the swim team, sleeps most afternoons and has dropped from B's to failing grades. The parent says, 'It's just a moody teenage phase.' What should the social worker do **FIRST**?

A. Agree that these changes are common in adolescence

B. Refer the teen to a psychiatrist for medication

C. Meet with the teen to screen for depression

D. Set up a daily homework check with teachers

62. During a home visit, a parent describes their 13-month-old, who began walking last week. The toddler roams happily from room to room, shrugs off small tumbles and often seems to forget the parent is nearby. Every few minutes the child toddles back, pats the parent's knee and heads off again. Which of Mahler's subphases is this child **MOST** likely in?

A. Differentiation

B. Rapprochement

C. Object constancy

D. Practicing

63. A child welfare social worker visits a kinship home where a grandparent cares for a 15-month-old born 8 weeks preterm. The social worker has compared the child's skills with milestones for the corrected age of 13 months. The child is not yet walking alone and does not babble, wave or look up when called by name. The grandparent says a nurse told them preemies catch up. What should the social worker do **NEXT**?

A. Help the grandparent arrange an early intervention evaluation

B. Plan to recheck milestones at a corrected age of 18 months

C. Reassure the grandparent that preemies often walk later

D. Ask the grandparent to describe the child's daily routine

64. A hospital social worker is helping an 8-year-old patient get ready for a scheduled surgery. The child asks many questions and wants to know exactly what will happen. Which approach is **BEST** suited to the child's stage of thinking?

A. Show the real mask and gown and walk through each step

B. Discuss possible risks and what happens if complications occur

C. Tell a story about a brave animal cured by magic medicine

65. Parents of a 23-year-old who lives at home ask for a family session. In 2 years, the young adult has changed jobs three times, switched college majors and moved in and out of a shared apartment. The young adult works part time, has close friends and says, 'I'm still figuring out what fits.' The parents worry something is wrong. Which response by the social worker is **BEST**?

A. Recommend vocational testing to settle on a career path

B. Suggest the parents set a firm date for the young adult to move out

C. Explain that such exploration is typical in the early twenties

66. One view holds that barriers in buildings, policies and attitudes limit people more than their impairments do. Which model of disability is this?

A. Social model

B. Medical model

C. Moral model

67. A 47-year-old client with Down syndrome has lived in a supported apartment for 10 years. Over the past 6 months, staff report that the client gets lost on familiar bus routes, skips steps when cooking and has lost interest in a weekly bowling league. Staff call the client 'stubborn' and want a behavior plan. What should the social worker do **FIRST**?

A. Seek a medical and cognitive assessment of the decline

B. Create a reward plan for completing daily tasks

C. Ask the client how they feel about the recent changes

D. Reteach cooking steps using picture-based cues and prompts

68. Staff at a senior center ask the social worker about a 76-year-old member who has stopped joining group activities. In the busy dining room, the member often answers questions that were not asked and asks others to repeat themselves. A relative wonders if dementia is starting. The social worker meets with the member in a quiet office, where they talk easily about recent events. What should the social worker do **NEXT**?

A. Move the member to smaller, quieter activity groups

B. Refer the member for a neurocognitive evaluation

C. Recommend that the member get a hearing evaluation

69. At an aging services agency, a social worker meets with a 76-year-old client with macular degeneration who has had only routine eye exams. The client can no longer read pill labels or recipes and has stopped cooking and going to a weekly card game. The client's adult children want the client to move to assisted living. The client says, 'If I could just see well enough to manage, I'd stay home.' What should the social worker do **FIRST**?

A. Help the family compare nearby assisted living options

B. Arrange home-delivered meals to replace cooking

C. Refer the client for low vision rehabilitation

D. Hold a family meeting to weigh the proposed move

70. In the family life-cycle framework, what is the central emotional task for families in later life?

A. Accepting many exits from and entries into the family

B. Accepting the shift in generational roles

C. Committing to a newly formed family system

D. Loosening boundaries to allow children's independence

71. The adult children of an 81-year-old who lives alone ask a senior center social worker for help. In 4 months, the parent has had two minor crashes, and new scrapes keep appearing on the car. The parent says both crashes were other drivers' fault and refuses to stop driving. One adult child wants to hide the car keys. Which recommendation is **BEST**?

A. Arrange a professional evaluation of the parent's driving ability

B. Suggest the parent limit driving to daylight hours near home

C. Support the adult children in removing access to the car

D. Ask the parent's physician to report the parent to the licensing agency

72. An employee assistance client tells the social worker that their widowed 78-year-old parent will move into the client's home next month. The parent drives, manages their own medications and volunteers weekly. The client's two teenagers will have to share a bedroom and are openly resentful, and the client's partner worries about privacy. What should the social worker recommend **FIRST**?

A. An in-home assessment of the parent's daily functioning

B. Individual counseling for the teenagers about their resentment

C. A meeting of the three generations to plan roles and space

73. A 74-year-old widowed client plans to remarry. The client's adult children request a family session. They say the new partner is replacing their late parent, and they worry about the family home and their inheritance. The client listens, then says the decision is made but that the client wants the children's blessing. Which response by the social worker is **BEST**?

A. Suggest the client delay the wedding until the children adjust

B. Help the children tell the client their worries directly

C. Request an evaluation of the client's decision-making capacity

74. Which theory of aging holds that as people see their remaining time as limited, they narrow their social circle to the relationships that matter most to them?

A. Socioemotional selectivity theory

B. Disengagement theory

C. Activity theory

75. A 20-year-old Mexican American college student meets with a campus social worker. Through high school, the student tried to fit in with white classmates and avoided speaking Spanish. After a Chicano history course, the student now spends time only with Latino peers and distrusts white faculty. The student feels angry about years of 'trying to be someone else.' In the racial/cultural identity development model, which stage does this **MOST** likely reflect?

A. Conformity

B. Dissonance

C. Resistance and immersion

D. Introspection

76. In a telehealth session, a 22-year-old who moved away for graduate school says the faith absorbed from family and congregation no longer feels like their own. The client is reading widely, questioning teachings once accepted without thought and working out which beliefs to keep by personal choice. According to Fowler, which stage of faith does this **MOST** likely describe?

A. Synthetic-conventional

B. Conjunctive

C. Mythic-literal

D. Individuative-reflective

77. A post-adoption social worker meets with parents whose 14-year-old was adopted from South Korea as an infant. The teen recently asked to take Korean language classes and join a Korean American youth group. The parents, who have spent most of the session voicing their fear that the teen is rejecting them, now ask the social worker to help talk the teen out of it. What is the **BEST** response?

A. Ask the parents to say more about their fear of rejection

B. Explain that exploring heritage is a normal part of identity development

C. Refer the family to therapy for adoption attachment issues

D. Suggest that the teen postpone these activities until the parents feel ready

78. Helms described a series of statuses in White racial identity. Which status brings guilt, shame and confusion as a person first faces racism and White advantage?

A. Contact
B. Reintegration
C. Pseudo-independence
D. Disintegration

79. During a home visit, a parent says their 4-year-old pointed at a person with darker skin in a store and asked loudly why the person's skin was 'dirty.' The embarrassed parent hushed the child. The parent says that race was never discussed at home growing up and now plans to avoid the topic so the child will not 'notice differences.' What is the **MOST** appropriate response by the social worker?

A. Agree to wait on race talk until the child is older
B. Say that preschoolers register race and gain from open talk
C. Explore further how the parent's family handled talk about race

80. Parents in a high-conflict marriage each press their 14-year-old to agree that the other parent is at fault. Each parent also asks the teen to carry messages they refuse to say to each other. The teen agrees with whichever parent is talking and now has frequent stomachaches. Which family dynamic is **MOST** likely present?

A. Enmeshment
B. Cross-generational coalition
C. Emotional cutoff
D. Triangulation

81. When one family member's symptoms improve, another member sometimes develops problems. In family systems theory, what term describes the family's pull to keep its usual patterns?

A. Equifinality
B. Morphogenesis
C. Homeostasis
D. Family projection process

82. Research on families of adults with schizophrenia links which family pattern to higher relapse rates?

A. High expressed emotion
B. Double-bind communication
C. Disengaged boundaries
D. Emotional cutoff

83. A 38-year-old client moved across the country at age 19 and has had no contact with parents or siblings since. The client says, 'They have no hold on me anymore.' The client becomes furious and cannot sleep after the partner's parents offer any advice, and has ended two long friendships over minor disagreements. Which concept **MOST** likely describes this pattern?

A. Differentiation of self
B. Disengaged boundaries
C. Family projection process
D. Emotional cutoff

84. A pediatrician refers an 8-year-old with recurring stomachaches whose medical workup was normal. The child has missed 9 school days this term. During the first meeting, the parents mention that the stomachaches seem worst on days when they argue. What should the social worker do **FIRST**?

A. Recommend couples counseling to reduce the parents' arguing
B. Assess how each family member reacts when the pain begins
C. Teach the child relaxation skills to use when pain starts

85. Four months after a parent lost a factory job, a family of five meets with a social worker at a community agency to assess its strengths. Which observation **BEST** indicates a key family resilience process?

A. The parents keep the job loss from the children to protect them
B. Each member handles worries privately to avoid burdening others
C. Members describe the job loss as a challenge they will face together
D. The family keeps every routine exactly as it was before the loss

86. A 24-year-old client with obsessive-compulsive disorder lives with a parent. In a joint session, the parent says they rewash the client's clothes on request, answer the same reassurance questions for up to 2 hours each night, and no longer invite guests to the home. The client's rituals have increased over the past year. Which response is **MOST** appropriate?

A. Plan with both how the parent will reduce these accommodations

B. Begin exposure exercises with the client in individual sessions

C. Refer the parent to a caregiver support group for stress

87. In a family session about a parent's alcohol use, an 11-year-old cracks jokes and makes faces whenever the adults' voices rise. The family laughs, and the tension drops for a moment. Teachers call the child the class comedian, but the child tells the social worker privately, 'I'm always scared something bad will happen.' In Wegscheider-Cruse's model, which family role is this child **MOST** likely filling?

A. Hero

B. Scapegoat

C. Mascot

D. Lost child

88. At a family intake, parents say their 15-year-old's silence and closed bedroom door force them to keep checking in and asking questions. The teen says the constant questions are the reason for staying in the room. Each side asks the social worker to say who is right. What should the social worker do **FIRST** to understand how the family functions?

A. Determine which family member's behavior started the conflict

B. Coach the parents to give the teen more privacy

C. Map how each person's response sets off the other's

89. Parents who have decided to divorce after years of frequent shouting matches ask the social worker how to protect their children, ages 7 and 11. Which step would be **MOST** helpful to the children's long-term adjustment?

A. Help the parents keep the children out of their disputes

B. Help the parents keep the children in one home and school

C. Arrange individual therapy for each child right away

90. A school social worker meets with a parent whose 10-year-old has missed 6 days this month. The family lost their apartment and is staying with relatives across town, about an hour away by bus. The parent wants the child to stay at the current school but assumes the move requires a transfer. The social worker has confirmed the family has no other housing. What should the social worker do **NEXT**?

A. Help the parent enroll the child at the school near the relatives

B. Connect the family with the district's homeless education liaison

C. Refer the family to a housing agency before addressing school

D. Start a truancy referral as the attendance rules require

91. At a family resource center, a parent asks for help applying for food assistance for their 4-year-old, a U.S. citizen. Partway through the form, the parent stops and says, 'I don't have papers. If I apply, could they use it against me or deport me?' The parent wants the child to eat well but is afraid to continue. What is the **BEST** action by the social worker?

A. Assure the parent that a child's application cannot affect them

B. Encourage finishing the application since the child is eligible

C. Connect the parent with a nonprofit immigration legal service

D. Explore the parent's past experiences with government agencies

92. At a refugee resettlement youth program, a 16-year-old who arrived with family 4 years ago now refuses to speak the family's language or attend community celebrations, calling them 'embarrassing.' Classmates mock the teen's accent, and the teen has stopped trying to join school activities. The teen says they do not fit in anywhere. Which acculturation strategy is the teen **MOST** likely showing?

A. Marginalization

B. Assimilation

C. Separation

D. Integration

93. A 56-year-old client has lived in emergency shelters for 8 years. Shelter staff tell the social worker that the client has fallen twice this month, struggles to climb to an upper bunk, and recently wet the bed. Staff add that the client is 'too young' for the agency's senior services. What is the **MOST** appropriate action?

A. Arrange a geriatric assessment of function, falls and continence

B. Move the client to a lower bunk to prevent further falls

C. Plan to apply for senior services once the client reaches age 60

94. A 15-year-old tells the school social worker that an online contact who seemed to be a peer received a private image from the teen and now demands money. The contact threatens to send the image to the teen's followers by tonight. Shaking, the teen says they want to delete everything and pretend it never happened. What should the social worker do **FIRST**?

A. Help the teen delete the chat and close the account

B. Ask the teen what led them to share the image

C. Help the teen save the messages and file a report

95. What is the status of internet gaming disorder in the DSM-5-TR?

A. An official diagnosis among the addictive disorders

B. A condition for further study in Section III

C. A specifier used with gambling disorder

96. At a health center visit, a client says their partner always knows where they have been and quotes their private texts back to them. The client found an unfamiliar app on their phone and wants the social worker's help deleting it right away. The client adds that the partner 'explodes' when not in control. What should the social worker do **FIRST**?

A. Help the client remove the app during the visit

B. Urge the client to report the tracking to police now

C. Explain that the partner's monitoring is a form of abuse

D. Plan for safety, using a device the partner cannot see

97. A 15-year-old who uses a wheelchair has few accessible activities nearby and spends most evenings in an online group for teens with the same condition. The teen calls these group members their closest friends. After a news story about online predators, the parent asks the social worker to support removing the teen's internet access. What is the **MOST** appropriate response?

A. Support the parent's plan until the teen is older

B. Review with the teen and parent the group's benefits and risks

C. Tell the parent the group is a strength that should continue

D. Suggest an in-person activity to replace the online group

98. A social worker on a mobile outreach team serves a client with schizophrenia who takes an antipsychotic and lives alone in a top-floor apartment without air conditioning. A week of extreme heat is forecast. The client declines the city cooling center, saying crowds make the voices worse, but agrees the apartment 'gets like an oven.' What is the **BEST** plan?

A. Arrange a quieter cool place and daily check-ins during the heat

B. Ask the prescriber to hold the antipsychotic during the heat wave

C. Respect the refusal and review heat safety tips with the client

99. During a home visit, a social worker sees a 2-year-old pick paint chips from a peeling windowsill and put them in their mouth. The family rents a house built in 1950. The parent says the child has been more irritable lately, sleeps poorly, and often has stomachaches. What should the social worker do **FIRST**?

A. Teach the parent calming routines for the irritability

B. Ask the landlord to repaint the windowsill

C. Help the parent arrange a blood lead test with the pediatrician

100. A 20-year-old tells a social worker at the campus counseling center that worry about climate change has grown since a summer of wildfires near home. The student keeps up with classes, sleeps a little less after reading the news, and recently joined a campus sustainability group. The student asks, 'Is something wrong with me?' What is the **MOST** appropriate response?

A. Challenge the catastrophic thoughts with cognitive restructuring

B. Frame the concern as understandable and build meaning-focused coping

C. Refer the student for a medication evaluation for anxiety

101. Two days after an ice storm knocked out power, a social worker makes a scheduled home visit. A gasoline generator is running in the attached garage. The parent and two children report headaches and nausea, and the infant seems unusually sleepy. The parent says everyone feels better after time outside. What should the social worker do **FIRST**?

A. Ask each family member about fever, cough and other flu signs

B. Urge the parent to take the infant to the pediatrician today

C. Have the parent move the generator outdoors, away from windows

D. Get the family out of the house and call 911

102. A hospital social worker meets with an 81-year-old patient who lives alone and was admitted after a fall. The team recommends assisted living, but the patient insists on going home. A screening last week showed some memory problems, and the patient cannot explain how they would manage their medications. The patient's adult child asks the social worker to arrange placement. What should the social worker do **FIRST**?

A. Arrange home services so the patient can return home

B. Assess the patient's ability to make this specific decision

C. Begin the assisted living placement the adult child requested

103. After a stroke, a patient with left-side weakness wants to go home alone. The patient correctly repeats the team's explanation that the weakness makes walking unsafe without help. The patient then says, 'That's true for other stroke patients, but my leg is fine,' yet needs two staff to stand. Which decision-making ability is **MOST** likely impaired?

A. Understanding

B. Appreciation

C. Reasoning

104. On a medical unit, a 78-year-old patient treated for a urinary tract infection is confused each evening and refuses the planned move to a rehabilitation facility. Each morning the patient is alert, oriented and asking about home. The transfer is planned for 4 days from now. What should the social worker do **FIRST**?

A. Ask the adult child to make the transfer decision

B. Record the evening refusal as the patient's decision

C. Revisit the decision with the patient early in the day

105. A client with schizophrenia served by an assertive community treatment team asks to reduce home visits from daily to weekly. For 8 months, the client has kept a daily symptom log, twice called the team when sleep loss and suspicious thoughts began, and followed the agreed steps each time. A teammate wants to keep daily visits because of a hospitalization 2 years ago. What is the **BEST** response?

A. Keep daily visits until the client has a full year of stability

B. Plan a gradual step-down built on the client's warning-sign log

C. Transfer the client from the team to routine clinic care

106. A hospital patient with advanced major neurocognitive disorder needs a decision about a feeding tube. The team's assessment finds the patient cannot describe the choice or its risks after repeated simple explanations. The patient's health care agent, named in an advance directive, reports that the patient often said they never wanted 'to be kept alive by machines.' A nurse suggests seeking a court-appointed guardian. What should the social worker do **NEXT**?

A. Help the agent decide based on the patient's stated wishes

B. Support seeking a court guardian to replace the agent

C. Request an ethics consultation to settle the patient's care

107. A 16-year-old has lived with the same foster parents for 4 years, and they ask to adopt. The youth tells the child welfare social worker, 'I love them, but I won't give up my name or act like my birth family is gone.' The youth wants to stay in the home. Agency staff prefer adoption as the most permanent option. What is the **MOST** appropriate next step?

A. Ask the court to waive the youth's consent to adoption

B. Review adoption and guardianship choices with the youth

C. Change the goal to independent living without a legal parent

108. An adult client with bipolar I disorder has kept a daily mood chart for a year. Reviewing it with the social worker, the client sees that the chart rated their mood as 'normal' during the 2 weeks before last spring's manic episode, while the client's partner noticed clear changes then. The client wants to use the chart alone to decide when to call the prescriber. What is the **BEST** response?

A. Plan with the client to add the partner's observations

B. Support the client's plan to rely on the mood chart

C. Have the partner report mood changes to the prescriber directly

D. Switch the client to a more detailed daily mood scale

109. A parent remarried 4 months ago. The new spouse, who has no children, began enforcing strict curfews and chore rules with the parent's 13-year-old. The teen now slams doors and refuses to speak to the stepparent. The couple asks the social worker how to restore order at home. Which guidance is **BEST**?

A. Have the parent and stepparent agree on rules and enforce them equally

B. Have the stepparent stay out of the teen's daily life for now

C. Have the parent handle discipline while the stepparent builds a bond

110. In the eighth session of a weekly group for 15-year-olds at a community center, members jokingly treat the social worker as a parent and compete for the worker's attention like siblings. They now share personal struggles openly and compare the group to their own households. Early sessions were marked by testing rules and challenging the worker. Which stage of group development is the group **MOST** likely in?

A. Power and control

B. Intimacy

C. Differentiation

111. At intake, a client seeks help for anxiety after losing a job. The client mentions living with two partners in a committed polyamorous relationship. Both partners have offered to help with bills and job leads. What should the social worker do **FIRST**?

A. Include both partners, as the client defines them, in assessing supports

B. Assess whether the arrangement with both partners is causing the anxiety

C. Refer the client to a provider who specializes in polyamory

112. A social worker leads a closed 12-week bereavement group for adults whose partners have died. In week 10, members are reviewing what they have learned and planning how to say goodbye. The intake coordinator asks the social worker to add a newly bereaved client who is struggling and on the waitlist. Which action is **BEST**?

A. Add the client now so they can gain support from the group

B. Ask the members to vote on whether to admit the client

C. Offer the client individual support until the next group starts

113. Four months after their infant died, a couple meets with a hospital bereavement social worker for the first time. Within minutes, each partner says the other 'doesn't care' about the baby. One partner is tearful; the other sits quietly and mentions working long hours since the death. What should the social worker do **FIRST**?

A. Encourage the quieter partner to express feelings openly in sessions

B. Invite each partner to describe how they have been carrying the loss

C. Screen both partners for prolonged grief disorder

D. Help both partners see that each grieves in a different, valid way

114. A couple with a 4-month-old, their first child, comes for a first counseling session. In the opening minutes, they say they argue about chores and sleep and fear they are 'falling out of love.' Then both turn to the social worker, waiting for a verdict. What should the social worker do **FIRST**?

A. Help the couple make a plan for sharing chores and night feedings

B. Ask each partner how life together has changed since the birth

C. Explain that satisfaction often dips with a new baby

D. Refer the couple to a specialized couples therapy program

115. In the fourth session of a community agency's early-recovery group, a member mentions taking buprenorphine. Two longtime members reply that people on 'replacement drugs' are not truly sober, and several others nod. The member goes quiet. Over the past two sessions, members have made similar remarks about anyone who uses medication. What is the social worker's **BEST** response?

A. Talk with the member one-on-one once the group has ended

B. Let the debate continue so members reach their own view

C. Name the pattern and open discussion of different recovery paths

D. Suggest the member move to a recovery group for people on medication

116. Which bruise location is **MOST** concerning for physical abuse in an older adult?

A. Front of the lower leg

B. Upper arm

C. Back of the hand

D. Forearm below the elbow

117. In an emergency department, a social worker is asked to see a patient with a broken wrist. The patient says the injury came from a fall, but the physician notes bruises of different ages on both upper arms. The patient's partner stays in the room and answers most questions. What should the social worker do **FIRST**?

A. Arrange to speak with the patient alone

B. Ask about safety at home with the partner present

C. Give the patient domestic violence hotline information

118. When a caregiver says a young child's injury came from a fall at home, which bruise **MOST** warrants an evaluation for physical abuse?

A. Bruised shins on an active 4-year-old

B. A forehead bruise on a toddler who just began walking

C. Bruising on the ear of a 2-year-old

119. At a health clinic, a client says their partner 'grabbed my throat until I almost blacked out' last month during an argument about the client moving out. The client laughs it off: 'It only happened once, and I'm fine now.' The client still plans to leave in a few weeks. What is the social worker's **BEST** response?

A. Accept the client's view that it was a one-time event

B. Explain that strangling sharply raises the risk of homicide

C. Explore why the client sees the incident as minor

120. A 10-year-old client in a child advocacy center program disclosed last month that a parent's partner sexually abused them. The partner then moved out, and the family lost much of its income. Today the client says, 'I made it up,' while looking at the floor, and adds that the parent cries every night. What is the **MOST** appropriate action for the social worker?

A. Tell the investigating caseworker about the recantation

B. Accept the recantation and note the report was likely false

C. Question the client in detail to learn which account is true

121. A home-delivered meals worker refers a 78-year-old client with mild memory problems. A new acquaintance moved into the client's home 2 months ago and now collects the mail. The electricity is about to be shut off despite a steady pension, and the bank has called about large ATM withdrawals. The client says the acquaintance 'handles everything.' What form of mistreatment is **MOST** likely occurring?

A. Self-neglect

B. Financial exploitation

C. Caregiver neglect

D. Psychological abuse

122. At a family clinic, the parent of a 4-year-old says the child touches their own genitals at nap time and recently asked why a baby sibling 'looks different.' The touching stops when the parent offers a toy. The parent's voice shakes: 'I just have a bad feeling something happened.' What should the social worker do **FIRST**?

A. Make a report of possible sexual abuse

B. Ask the parent what has made them worried

C. Ask the child directly whether anyone has touched them

D. Explain to the parent that this is typical curiosity for the age

123. A 12-year-old with type 1 diabetes is admitted with diabetic ketoacidosis for the second time in 3 months. The parent missed 5 of 6 clinic visits despite transportation vouchers and home nurse visits, and insulin is supplied free. The parent can describe the insulin plan but says the child 'is old enough to handle it.' After meeting with the parent, what should the social worker do **NEXT**?

A. Report suspected medical neglect to child welfare authorities

B. Arrange another round of diabetes teaching for the parent and child

C. Coach the child to take over the daily insulin doses

124. A 15-year-old in foster care was sexually abused for years by a stepparent while the client's other parent knew and did nothing. In sessions, the client says adults 'only pretend to care,' breaks rules to test each new foster parent, and refuses to lean on any staff member. In Finkelhor and Browne's model, which traumagenic dynamic do these reactions **MOST** likely reflect?

A. Betrayal

B. Powerlessness

C. Stigmatization

D. Traumatic sexualization

125. A hospital social worker is asked to meet with the parent of a 6-year-old whose lab results show gonorrhea. The physician confirms the infection could not have been passed at birth. The parent says the child must have caught it from shared towels at a relative's home. What should the social worker do **FIRST**?

A. Record the towel explanation and arrange the child's treatment

B. Arrange a forensic interview at a children's advocacy center

C. Notify the child abuse hotline of the possible sexual abuse

126. At a community health center, a 24-year-old client tells the social worker that their partner has flushed the client's birth control pills twice and says a baby would 'keep us together.' In answer to safety questions, the client describes no physical violence. The client does not want to become pregnant now and does not want to end the relationship. What is the **MOST** appropriate response?

A. Encourage the client to leave the relationship for their safety

B. Connect the client with the provider about less detectable methods

C. Suggest couples counseling so both partners can discuss family plans

127. A social worker in a rehabilitation facility meets with an 82-year-old patient recovering from hip surgery. Staff report that the patient, who was alert and oriented last week, has become confused over the past 2 days. The patient's attention drifts, and the patient says insects are crawling on the wall. What should the social worker do **FIRST**?

A. Administer a cognitive screening to document the decline

B. Meet with family to discuss memory care placement

C. Notify the medical team of the sudden change in mental status

D. Request a psychiatric consultation for new-onset psychosis

128. A primary care provider refers a 74-year-old client for memory concerns that began about 2 months ago. The client says, 'My mind is gone,' and answers 'I don't know' to many screening questions. The client also reports poor sleep, weight loss, lost interest in gardening and feeling like a burden. The client still manages bills and medications, alertness is steady and medical tests are normal. What is the **MOST** likely explanation?

A. Major neurocognitive disorder

B. Delirium

C. Major depressive disorder

D. Mild neurocognitive disorder

129. Which mood can count toward a depressive episode in children and adolescents in place of sad or depressed mood?

A. Anxious mood

B. Labile mood

C. Irritable mood

D. Detached mood

130. During a home visit 7 weeks after delivery, a parent tearfully says they keep getting sudden images of the baby falling down the stairs. The images horrify them, so they now avoid carrying the baby near the staircase. The parent also reports feeling worthless and sleeping poorly even when the baby sleeps. What should the social worker do **FIRST**?

A. Assess the thoughts, mood, safety and any intent to act

B. Refer the parent to a perinatal mental health specialist

C. Explain that such unwanted thoughts are common after birth

131. A high school social worker meets a 15-year-old cross-country runner referred by a coach. The student has lost a lot of weight over 3 months, skips lunch and runs extra miles. Today the student fainted at practice, and the athletic trainer recorded a very slow resting heart rate. The student says they feel fine. What should the social worker do **FIRST**?

A. Explore the student's thoughts about weight and body shape

B. Have the parents take the student to a doctor right away

C. Refer the student to an eating disorder therapy program

132. Parents bring their 16-year-old to an outpatient family clinic after 4 months of falling grades and pulling away from friends. The teen says people at school seem to send hidden signals and sometimes a faint voice calls their name, but adds that it might be their mind playing tricks. The parents believe it is a phase. What is the social worker's **BEST** recommendation?

A. Monitor the symptoms and reassess at monthly visits

B. Begin family sessions to support the teen at school

C. Refer the teen for a specialized early psychosis evaluation

133. A client with bipolar I disorder, stable for 2 years, meets the social worker by video. The client reports sleeping 4 hours a night for 3 nights, feeling 'better than ever' and planning a new business. The relapse plan the client wrote while well lists short sleep as a warning sign that means calling the prescriber. The client now wants to wait for next month's appointment. What should the social worker do **FIRST**?

A. Help the client contact their medication provider today

B. Explore the client's concerns about medication changes

C. Agree to track sleep together until the next appointment

134. At a community mental health center, the parents of a 23-year-old with schizophrenia say their adult child now showers rarely, speaks little and sits for hours without starting anything. The voices have faded with treatment. The parents call it laziness and have started criticizing the client every day. What is the social worker's **BEST** response?

A. Help the parents set firm rewards and consequences for chores

B. Explain that these behaviors are common symptoms of the illness

C. Link the client to a vocational rehabilitation program

135. A primary care physician refers a 76-year-old client after a full workup found no cause for months of fatigue, aches and poor appetite. The client has stopped gardening and no longer meets friends for cards but says, 'I'm not sad, just worn out.' What should the social worker do **NEXT**?

A. Encourage the client to rejoin a gardening group

B. Accept the denial of sadness and focus on coping with pain

C. Assess for depression, including lost interest and hopelessness

136. A 34-year-old group home resident with moderate intellectual developmental disorder communicates with a few words and gestures. Over the past 3 weeks, staff have seen the resident strike the side of their own jaw, wake often at night, and push meals away. The house manager asks the social worker to write a behavior plan. What should the social worker do **FIRST**?

A. Complete a functional assessment of the jaw striking

B. Arrange a medical and dental exam for possible pain

C. Refer the resident to psychiatry to evaluate depression

D. Draft a plan that rewards calm mealtimes and sleep

137. A client with schizophrenia has worked part-time stocking shelves for 4 months through a supported employment program. The supervisor reports that the client arrives early, chats easily with coworkers, and calls the job the best part of the week. However, the client cannot hold multistep instructions in mind and gets lost when a task changes midway. Which problem is **MOST** likely affecting job performance?

A. Negative symptoms

B. Disorganized thinking

C. Depressive symptoms

D. Cognitive impairment

138. A school nurse refers a 9-year-old who comes to the health office with stomachaches on most test days and many Monday mornings. A pediatrician found no medical cause. The parent asks the school social worker to support a plan that sends the child home whenever the pain starts. What should the social worker do **FIRST**?

A. Assess the child's worries and avoidance with child and parent

B. Support the parent's go-home plan to ease distress

C. Teach the child slow breathing to use before each test

139. At a community mental health agency, a client has had major depressive disorder since adolescence and began drinking heavily 2 years ago. The client now meets criteria for moderate alcohol use disorder and denies suicidal thoughts. An intake staff member suggests the client get sober before starting mental health care. What is the **BEST** treatment approach?

A. Address the drinking first, then reassess the depression

B. Refer to separate agencies for each disorder at once

C. Treat both disorders together in one coordinated plan

140. A client entered residential treatment for severe alcohol use disorder 6 days ago after years of heavy daily drinking. The client reports low mood, poor sleep and guilt, and says the low mood began after the drinking became heavy 3 years ago. The client answers no to direct questions about suicide. The staff psychiatrist asks for the social worker's diagnostic input. What is the social worker's **BEST** recommendation?

A. Recommend diagnosing major depressive disorder now

B. Reassess the mood after about a month of abstinence

C. Treat the low mood as alcohol-related and stop assessing it

D. Suggest starting an antidepressant during detox

141. An adult with schizophrenia in supportive housing has said for 2 days that 'the devil is twisting my stomach.' The client has voiced similar beliefs before. Today the client is hunched over, has vomited twice, refuses meals and feels warm to the touch. Staff ask the social worker to treat it as a familiar delusion. What should the social worker do **FIRST**?

A. Ask the psychiatrist to review the antipsychotic dose

B. Arrange a same-day medical evaluation of the stomach symptoms

C. Explore what the stomach image means to the client

142. An adult in an intensive outpatient program for moderate alcohol use disorder has cut drinking from daily to 2 evenings a week over 4 weeks. The client also has PTSD from an assault 2 years ago, with nightmares and avoidance that the client links to drinking. The client has stable housing, no withdrawal signs and no suicidal thoughts, and asks to start trauma treatment now. What is the social worker's **BEST** recommendation?

A. Defer trauma therapy until 6 months of sobriety

B. Refer to a separate trauma clinic after the program ends

C. Begin trauma-focused therapy alongside current care

143. A client in private practice has major depressive disorder. After 12 weeks of treatment, the client's mood, energy and interest have largely returned. However, the client still takes over an hour to fall asleep most nights, as for the past 5 months, and feels exhausted at work. The prescriber expects sleep to recover as the depression lifts. What is the **MOST** appropriate recommendation?

A. Wait for sleep to improve as the depression resolves

B. Ask the prescriber about adding a sleep medication

C. Give the client a handout on sleep hygiene

D. Add CBT for insomnia to the client's care plan

144. An adult with bipolar I disorder comes to a community mental health center for help with worsening depression. The client drinks 2 or 3 beers most evenings and has never had withdrawal symptoms. The intake coordinator says center policy sends anyone who drinks to an addiction program before mental health care. What is the social worker's **BEST** response?

A. Assess both the mood episode and alcohol use today

B. Refer the client to the addiction program first

C. Help the client find an integrated program elsewhere

145. An ACT team social worker visits a 23-year-old client 6 months after a first hospitalization for schizophrenia, during which the voices stopped. The client now speaks of having 'ruined everything,' feels guilty about burdening family, sleeps poorly and no longer enjoys music. The client is tearful and expressive while talking. A team member calls this negative symptoms. What is the **MOST** likely explanation?

A. Negative symptoms of schizophrenia

B. Co-occurring depression

C. Antipsychotic-induced parkinsonism

D. Schizoaffective disorder

146. A client with schizophrenia who lives in supportive housing smokes two packs of cigarettes a day. The client tells the social worker they want to quit because cigarettes use up most of their spending money. A staff member says quitting will worsen the client's symptoms and that smoking is the client's only pleasure. The client's symptoms have been stable for a year. What is the social worker's **BEST** response?

A. Suggest waiting until the client's stability is more established

B. Encourage the client to cut down gradually on their own

C. Help the client get cessation treatment through the prescriber

147. A client with schizoaffective disorder served by an assertive community treatment team has gained weight over the past year. At the client's apartment, the client mentions being thirsty all the time, getting up several times a night to urinate, and having blurry vision. The client distrusts doctors and plans to bring it up at a psychiatric appointment in 5 weeks. What is the **MOST** appropriate action?

A. Agree to the client's plan and check on the symptoms next month

B. Offer to go with the client to a primary care visit this week

C. Encourage the client to switch to water and sugar-free drinks

148. Six weeks after a heart attack, a patient in a hospital cardiac rehabilitation program has missed several exercise sessions. The patient tells the social worker they feel down most days, have lost interest in hobbies, and sleep poorly. The patient adds, 'I don't need a shrink. I just need my strength back.' A nurse calls this a normal adjustment. What is the **MOST** appropriate action?

A. Screen for depression as a routine part of rehab care

B. Focus on exercise goals as the patient prefers

C. Arrange a psychiatric consult to consider antidepressants

D. Reassure the patient that low mood after a heart attack passes

149. Engel's biopsychosocial model was proposed as a challenge to which approach to illness?

A. The biomedical model

B. The psychoanalytic model

C. The ecological model

150. During a biopsychosocial assessment in a hospital dialysis unit, a patient mentions that prayer has helped them cope, but adds that they have not attended services since starting treatment 3 months ago. The intake form already lists the patient's religious affiliation. What should the social worker do **NEXT**?

A. Refer the patient to the hospital chaplain for support

B. Move on to the social history section of the assessment

C. Ask what faith means to the patient and how it affects care

151. A court refers a client for assessment and describes the problem as 'anger and aggression.' At intake, the client says the real problem is losing visits with their children since a breakup and not knowing how to get them back. The social worker is writing the presenting problem section of the biopsychosocial assessment. What is the **BEST** way to record it?

A. Record the court's description as the official presenting problem

B. Record the client's own words along with the referral reason

C. Record the social worker's view of the underlying problem

152. A social worker is writing a biopsychosocial formulation for a 71-year-old with depression. The client's parent had depression, and symptoms began after a hip fracture last spring. Since then the client rarely leaves the house, and mood dips further with each week spent alone indoors. A neighbor visits daily. In a 4P formulation, how is the client's staying home classified?

A. Precipitating factor

B. Perpetuating factor

C. Predisposing factor

D. Protective factor

153. In a primary care clinic, a patient with 2 years of low back pain is referred to the social worker after normal imaging. The patient lost a warehouse job, sleeps poorly, and avoids walking for fear of reinjury. The patient asks, 'So the doctor thinks the pain is all in my head?' Which response is **MOST** helpful?

A. Explain that real pain is affected by mood, stress and activity

B. Reassure the patient that the pain has a purely physical cause

C. Explain that emotional stress is the likely cause of the pain

154. A social worker in private practice has seen a client for 8 sessions for work stress, based on a biopsychosocial assessment completed at intake. Today the client mentions, for the first time, an eating disorder in their teens and says they have started skipping meals again during a busy project. What should the social worker do **FIRST**?

A. Refer the client to an eating disorder treatment program

B. Continue the stress plan and monitor meals in later sessions

C. Ask about current eating patterns and physical health

155. During a biopsychosocial intake at a primary care clinic, a 47-year-old client reports 2 weeks of low mood, irritability, tearfulness and sleeping about 3 hours a night. The medical history shows the client began high-dose prednisone for a lupus flare 3 weeks ago. The client has no past mood episodes and wants help 'getting back to normal.' What should the social worker do **FIRST**?

A. Ask consent to tell the prescriber about the mood changes

B. Begin weekly CBT sessions targeting the low mood and sleep

C. Use a PHQ-9 to confirm and record a depressive disorder

156. An emergency department social worker is asked to see a 68-year-old patient who was treated for a minor fall and is medically ready to leave. Transport home is arranged for 1 hour from now. The nurse notes that the patient seemed unsure how they would manage at home. What is the **BEST** way to approach the assessment?

A. Complete a full biopsychosocial history before the ride home arrives

B. Leave the assessment to the patient's primary care provider

C. Focus on home safety, supports and daily functioning now

157. A hospital social worker begins a biopsychosocial assessment with a 74-year-old patient admitted overnight after a fall. While describing their living situation, the patient becomes anxious and says their spouse, who has major neurocognitive disorder and needs daily help, has been home alone since the ambulance came yesterday afternoon. The patient cannot name anyone who has checked on the spouse. What should the social worker do **FIRST**?

A. Finish the assessment, then plan for the spouse's care

B. Arrange for someone to see the spouse at home right away

C. Report the spouse's situation to adult protective services

158. A client in an outpatient program arrives restless and irritable after missing 2 days of treatment. The social worker notices yawning, a runny nose, watery eyes, dilated pupils and goosebumps. The client reports muscle aches and diarrhea. Which condition is the client **MOST** likely experiencing?

A. Opioid withdrawal

B. Opioid intoxication

C. Stimulant intoxication

D. Alcohol withdrawal

159. A school social worker meets a quiet 10-year-old whose parent drinks heavily. Teachers describe the student as 'no trouble at all.' The student spends recess alone drawing, rarely asks for help and is often forgotten when groups are formed. At home, the student stays in their room for hours. Which family role in alcohol-affected families does this pattern **MOST** likely reflect?

A. Family hero
B. Scapegoat
C. Lost child
D. Mascot

160. Staff at a youth drop-in center ask a social worker to see a 13-year-old who arrived dizzy and unsteady. The youth's speech is slurred, the eyes make jerky side-to-side movements, and there is silver paint around the nose and mouth. The youth's clothes smell strongly of chemicals. Which condition is **MOST** likely?

A. Inhalant intoxication
B. Alcohol intoxication
C. Sedative-hypnotic intoxication

161. An adult walks into an outpatient clinic asking for help staying sober. The client drank about a fifth of liquor daily for years and stopped on their own 30 hours ago. The client's hands shake, they are sweating and restless, and they briefly saw insects on the wall this morning. What should the social worker do **FIRST**?

A. Help the client enroll in outpatient withdrawal care tomorrow
B. Book an intake for the intensive outpatient program
C. Arrange same-day medical care for alcohol withdrawal

162. A child protective services social worker receives a report that the parent of a 3-year-old is 'on methadone.' With the parent's consent, the opioid treatment program confirms 8 months of steady attendance, negative drug screens and newly approved take-home doses. The child appears healthy at the first home visit. What is the social worker's **BEST** approach?

A. Close the report, since the parent's methadone is lawful care
B. Assess parenting and safe storage of the take-home doses
C. Require tapering the doses before the case can close

163. On a home visit, a 77-year-old client who was widowed last year has had 2 falls in 3 months and forgets recent conversations. An adult child asks the social worker to arrange memory testing. The social worker notices many empty wine bottles in the recycling bin, and the client says a few glasses help with sleep. What should the social worker do **FIRST**?

A. Ask about the amount and timing of the client's drinking

B. Refer the client for the memory testing the family requested

C. Arrange a home safety evaluation to prevent further falls

164. A 7-year-old has lived with a relative caregiver since infancy. Child welfare records show the birth parent drank heavily throughout the pregnancy. The 7-year-old repeats the same mistakes despite consequences, forgets rules learned the day before and has a small head size noted by the pediatrician. The school asks the social worker to arrange an ADHD evaluation. What is the social worker's **MOST** appropriate recommendation?

A. Arrange the ADHD evaluation that the school requested

B. Start a home reward chart to teach rule following

C. Refer for trauma-focused therapy about early separation

D. Request a fetal alcohol spectrum disorder evaluation

165. Teachers ask a high school social worker about a 16-year-old with strong past grades who, over the last 3 weeks, has become restless, irritable, anxious, and unable to focus in morning classes. The changes began when the school installed vape sensors in the restrooms. A classmate says the student used to vape between every class. The student now asks to leave campus at lunch and comes back calmer. Which explanation is **MOST** likely?

A. Attention-deficit/hyperactivity disorder

B. Generalized anxiety disorder

C. Tobacco withdrawal

166. In the lobby of a community mental health center, the social worker finds a client who has opioid use disorder slumped in a chair. The client does not respond to a loud voice or a firm knuckle rub on the breastbone. The client's lips look bluish, and they are making slow snoring sounds. Naloxone is kept at the front desk. What should the social worker do **FIRST**?

A. Call 911 and give the naloxone

B. Place the client on their side and hold naloxone in reserve

C. Ask the front desk whether the client took anything

167. An emergency department asks the social worker to see a young adult patient on a fourth visit this year for days of repeated vomiting. Staff wonder about an eating disorder. Labs and imaging have been normal. The patient describes smoking cannabis several times a day for years, taking long hot showers that ease the nausea, and smoking more to settle the stomach. What should the social worker do **NEXT**?

A. Tell the patient that cannabis is causing the vomiting

B. Share the use and bathing history with the physician

C. Refer the patient for an eating disorder evaluation

168. A school social worker learns that a 7-year-old often arrives hungry and wears the same clothes several days in a row. The parent, who works two part-time jobs, explains that the family's food benefits stopped 2 months ago after a missed renewal appointment. The child is clean and up to date on checkups, and the parent attends every school conference. What should the social worker do **FIRST**?

A. Assist with restoring the lapsed aid and arranging free meals

B. Notify child protective services of possible neglect

C. Interview the child privately about conditions at home

169. A 52-year-old patient is readmitted to the hospital with high blood sugar for the third time in 4 months. A team note calls the patient 'noncompliant with insulin.' The patient works two part-time jobs without health benefits and takes two buses to reach the nearest pharmacy. What should the social worker do **FIRST**?

A. Teach the patient about complications of skipping insulin

B. Ask the patient what makes getting and using insulin difficult

C. Refer the patient to a diabetes self-management education class

170. A client with bipolar disorder attends a supported employment program and receives Social Security disability benefits and Medicaid. The employer offers the client more weekly hours. The client says, 'I want the hours, but if I earn more I could lose my check and my medication coverage.' Unsure how earnings affect either benefit, the client asks the social worker what to do. What is the **BEST** response?

A. Encourage the client to accept the hours as a recovery step

B. Explain that the added earnings will end the client's benefits

C. Explore what the extra hours would mean to the client

D. Refer the client to a work incentives benefits planner

171. A community mental health client with depression has missed 3 of 5 appointments, and clinic policy allows closure after 3 missed visits. When reached, the client explains that their hourly job posts shifts only 2 days ahead and their phone is often shut off for unpaid bills, so reminder calls fail. The client says they want to keep coming. What is the **MOST** appropriate response?

A. Offer short-notice scheduling and a workable contact method

B. Explore the client's ambivalence about continuing treatment

C. Close the case and invite the client to reapply later

172. A teacher refers a 9-year-old to the school social worker for an ADHD evaluation, noting that the child cannot focus after mid-morning and puts their head down by lunch. In a phone call, the parent explains that their work hours were cut and food often runs out before payday. The child usually skips breakfast. What should the social worker do **FIRST**?

A. Collect parent and teacher ratings of the child's attention

B. Suggest the parent ask the pediatrician about ADHD medication

C. Connect the family to free meal programs and food aid

173. A 34-year-old client with schizophrenia grew up in a middle-income family and was in college when symptoms began at age 20. Since then, the client has lost a series of jobs and now lives on disability benefits in low-cost housing. A new staff member remarks that the client's poverty probably caused the illness. Which account of the link between low income and mental illness does this history **MOST** likely support?

A. Social causation

B. Culture of poverty

C. Intergenerational transmission

D. Social selection

II.B Assessment Methods and Techniques

174. During a routine session, a 70-year-old client says, 'Lately I've been thinking about ending my life.' The social worker thanks the client for sharing and stays calm. What should the social worker do **NEXT**?

A. Begin a written safety plan with coping strategies

B. Ask about a plan, intent and access to lethal means

C. Arrange an emergency psychiatric evaluation later today

175. In an integrated primary care clinic, a client who lost their job 3 months ago meets with the social worker about stress. The client says, 'My family would be better off without me,' and mentions giving a prized guitar to a friend last week. The client then changes the subject to money worries. What should the social worker do **FIRST**?

A. Ask the client directly about thoughts of suicide

B. Arrange an emergency psychiatric evaluation today

C. Reflect the client's feeling of being a burden

176. A client with borderline personality disorder has had near-daily passive thoughts of death for years and attends weekly DBT. After a breakup this week, the client says the thoughts feel different. In the risk assessment that follows, the client says they bought two large bottles of acetaminophen yesterday and will 'probably' take them this weekend. The client cannot name anything that would keep them safe. What should the social worker do **NEXT**?

A. Continue DBT skills, noting the thoughts are long-standing

B. Complete a safety plan and add a midweek session

C. Have the client evaluated for hospitalization today

D. Explore what the breakup means to the client

177. At a home visit 2 weeks after delivery, a parent has barely slept for 4 days and speaks rapidly about 'signs' that the baby is evil. When asked directly about harm, the parent calmly says the baby 'needs to be sent back to God' and mentions the bathtub. The parent's partner is in the next room. What should the social worker do **NEXT**?

A. Get the parent to a psychiatric emergency service now

B. Book a perinatal psychiatry appointment for this week

C. Explore the parent's spiritual beliefs about the baby

D. Ask the partner to stay with the baby until follow-up

178. In a youth residential program, staff find fresh, shallow cuts on a 15-year-old resident's thigh. The resident tells the social worker, 'It's not like that. It just takes the pressure off when I get overwhelmed.' The unit manager wants to start one-to-one suicide watch and remove the resident's privileges for a week. What should the social worker do **FIRST**?

A. Start one-to-one observation until the psychiatrist visits

B. Teach replacement coping skills for overwhelming feelings

C. Assess the self-injury's function and any wish to die

179. A rural client in their 60s, in treatment for depression after losing the family farm, reports passive thoughts of death most days. The client keeps several hunting rifles at home and refuses to sell them, saying hunting is part of who they are. The client's sibling lives nearby and owns a gun safe. What is the social worker's **BEST** response?

A. Accept the refusal and note the rifles in the chart

B. Discuss temporary storage at a relative's home

C. Arrange a short voluntary hospital stay for safety

180. Police bring an adult to the emergency department after a neighbor heard the person shouting that they would 'end it tonight.' The patient smells of alcohol, slurs words and walks unsteadily. Now the patient laughs it off, says, 'I was just drunk,' and asks to leave. What should the social worker do **FIRST**?

A. Observe until sober, then do a full assessment

B. Complete the full suicide risk assessment right now

C. Accept the retraction and discharge with a crisis line

181. In a nursing facility, a 79-year-old resident with heart failure has refused most meals and several medications since their spouse died 6 weeks ago. The resident says, 'I'm just done. Let me be.' The resident has no advance directive about refusing care. Staff want to honor the refusal as the resident's right. What is the **MOST** appropriate action for the social worker?

A. Support the refusal and document it as an informed choice

B. Request a hospice referral to focus on comfort care

C. Evaluate the refusal for depression and a wish to die

182. A psychiatric inpatient team is planning discharge for a patient with schizophrenia. The record shows the patient attacked neighbors twice in the past 3 years, a current belief that a neighbor is poisoning the water supply, and heavy drinking on weekends. Which factor **MOST** strongly predicts future violence?

A. The persecutory belief about the neighbor

B. Heavy weekend alcohol use

C. The diagnosis of schizophrenia

D. Prior violent behavior

183. Which finding is a warning sign of near-term suicide risk rather than a long-term risk factor?

A. Searching online for ways to die

B. A family history of death by suicide

C. Childhood physical abuse

D. A chronic pain condition

184. A client with schizoaffective disorder tells a social worker at a supportive housing program that for 3 days the voices have been telling them to step in front of a bus. The client looks tense but is talking openly and agrees to keep meeting today. What should the social worker do **FIRST**?

A. Call for an emergency evaluation for hospitalization

B. Ask whether the client intends to obey the voices

C. Notify the prescriber that the voices have increased

185. A 66-year-old hospital patient with advanced kidney disease tells the social worker, 'I've turned into nothing but a cost to my kids. They'd manage fine without me.' Which construct from the interpersonal theory of suicide does this statement **MOST** likely reflect?

A. Perceived burdensomeness

B. Thwarted belongingness

C. Acquired capability

186. On a night of subzero temperatures, an outreach social worker finds a client with schizophrenia sleeping outside in a light jacket. The client's fingers are white and numb. The client insists that shelters 'pump poison through the vents,' cannot describe any other way to stay warm, and refuses a blanket, a ride and the warming center. What should the social worker do **NEXT**?

A. Leave blankets nearby and return early tomorrow

B. Explore what would make a shelter feel safe

C. Call emergency services for medical and psychiatric care

187. An adult child calls a geriatric care program about a parent with moderate Alzheimer's disease who keeps a loaded handgun by the bed. Last week the parent woke at night, did not recognize the adult child, and pointed the gun at them. The parent insists the gun is needed for protection. What is the social worker's **BEST** response?

A. Suggest unloading the gun and hiding the ammunition

B. Help the family remove the gun from the home now

C. Wait for a capacity evaluation before changing anything

188. A middle school social worker learns from a teacher that a 13-year-old student wrote a list titled 'people who will pay' with five classmates' names. Classmates say the student recently talked about a relative's guns. In a brief meeting, the student shrugs and calls the list a joke. What should the social worker do **FIRST**?

A. Refer the concern to the threat assessment team today

B. Call the parents to ask whether the relative's guns are locked

C. Call the police to search the student's home for weapons

189. A 79-year-old client has cared for a spouse with advanced Parkinson's disease for 6 years. In a caregiver support session, the client says, 'Some nights I think it would be kinder if neither of us woke up.' The client mentions keeping a handgun in the bedroom. What should the social worker do **FIRST**?

A. Ask whether the thoughts include ending both their lives

B. Arrange respite care to ease the caregiving load

C. Counsel the client to store the handgun elsewhere

D. Explore the grief of watching the spouse decline

190. In a pediatric emergency department, a 15-year-old being treated for an asthma attack completes the hospital's suicide risk screen with a nurse. The teen answers yes when asked about having thoughts of killing themselves right now. The parent is in the waiting room. What should the social worker do **NEXT**?

A. Schedule an outpatient mental health visit for this week

B. Ask the parent whether the teen has talked about suicide before

C. Ensure the teen is watched until an urgent safety evaluation

191. Two days after a 10th grader died by suicide, a high school social worker reviews the students staff are worried about. One was the student's closest friend, was hospitalized last year after an overdose, and has not come to school yet today. Another sat beside the student in chemistry and keeps crying in class. A third posted a tribute video that friends shared widely. Whom should the social worker contact **FIRST**?

A. The classmate crying in chemistry class

B. The friend who is absent from class

C. The student whose tribute video friends shared

192. In a violence risk assessment, which of these risk factors is dynamic and therefore a direct target for a risk-management plan?

A. Two prior assault convictions

B. First arrest at age 15

C. Witnessing violence between parents

D. Current heavy weekend drinking

193. In an afternoon session, a 58-year-old client with depression tells the social worker they have no thoughts of suicide. That evening, the client's partner, who is named on a signed release, calls to say they found a large supply of sleeping pills hidden in a garage toolbox. The partner adds that the client has been unusually quiet since losing a job last month. What should the social worker do **NEXT**?

A. Contact the client promptly about the partner's call and reassess

B. Ask police to bring the client in for an emergency evaluation

C. Add a session later this week to discuss the partner's call

194. For 2 years, a veteran with chronic back pain has described a passive wish not to wake up at most sessions. Today the social worker asks the standard risk questions, and the client again reports no plan and no intent to act. Firearms remain stored with a sibling under an earlier agreement, and the client used the safety plan during a hard night last week. A new team member urges an emergency evaluation. What is the **BEST** response?

A. Arrange an emergency evaluation for the client's safety

B. Review the safety plan together and continue current care

C. Ask the client to sign a no-suicide agreement

D. Reduce sessions to monthly check-ins

195. Which part of the mental status examination records answers that wander off and never reach the point?

A. Thought content

B. Perception

C. Thought process

196. Which mental status exam term describes laughing while recounting a sad event, such as a recent death?

A. Inappropriate affect

B. Labile affect

C. Restricted range of affect

D. Euphoric mood

197. What does the mental status exam contribute to a clinical assessment?

A. A diagnosis that meets DSM-5-TR criteria

B. A chronological history of the presenting problem

C. A snapshot of present thought, mood and behavior

D. A norm-referenced score of symptom severity

198. On a mental status exam, a client misperceives a real object, such as seeing a coat on a hook as an intruder. Which finding is this?

A. Visual hallucination

B. Illusion

C. Delusion

D. Derealization

199. An inpatient is asked how the morning went and answers rapidly: 'Grey morning, grey like the sea, I sailed once, sailors steer by the stars, I'm writing a book on astronomy.' The answer continues in the same way for several minutes. Which MSE finding does this answer **MOST** likely show?

A. Flight of ideas

B. Loose associations

C. Circumstantiality

D. Clang associations

200. During a telehealth intake, an adult client smiles steadily while describing a recent job loss and says, 'Honestly, I feel hollow most days.' The social worker is completing the mood section of the mental status exam. What is the **BEST** way to document the client's mood?

A. Record mood as euthymic, consistent with the steady smile

B. Record mood as depressed, based on the worker's impression

C. Record mood using the client's own description, in quotes

201. A hospital social worker completes a mental status exam with a patient admitted after a fall. The patient looks down at the floor throughout, answers fully and warmly, and describes a good mood. The patient explains that in their family, looking an elder or professional in the eye is disrespectful. What is the **MOST** appropriate way to record the patient's eye contact?

A. Note the patient's poor eye contact as a sign of low mood

B. Describe the downward gaze and the patient's stated reason

C. Omit eye contact from the record to avoid cultural bias

202. During a mental status exam, the social worker asks a client to spell the word 'world' backward. Which area is the social worker assessing?

A. Remote memory

B. Abstract reasoning

C. Attention and concentration

203. At an outpatient intake, a client describes recurring urges to shout insults during religious services. The client calls them 'my own brain being cruel,' finds them upsetting, and tries to push them away with silent prayer. The client has never acted on them. Which mental status finding is this **MOST** likely to represent?

A. Delusion

B. Thought insertion

C. Obsession

204. An inpatient with bipolar I disorder who lives alone is being considered for discharge. The team asks the social worker to describe the client's judgment for the mental status exam. Which question serves this purpose?

A. 'What would you do if you ran out of medication at home?'

B. 'Do you believe you have an illness that needs treatment?'

C. 'What does it mean that the early bird catches the worm?'

205. A parent seeks help for a 9-year-old's constant defiance and says the school complains about the child every day. With the parent's written consent to exchange information with the school, the social worker obtains school records. The records show good grades, no discipline reports and positive comments from staff. What should the social worker do **NEXT**?

A. Diagnose oppositional defiant disorder from the parent's report

B. Refer the parent to a behavioral parent training program

C. Discuss the conflicting accounts with the parent and teacher

206. A social worker at a community brain injury program receives a neuropsychological evaluation for a 34-year-old client injured in a fall 8 months ago. The client has signed releases for the program and the evaluator. After reading the report, the social worker understands its summary but not how the processing-speed and memory scores should shape the client's job-training plan. What should the social worker do **NEXT**?

A. Ask the psychologist who did the testing about practical implications

B. Explain to the client what each test score means for work

C. Revise the job-training plan based on the report's summary

D. Request a second neuropsychological evaluation from another provider

207. A 15-year-old enters a residential program after five foster placements. Records from prior providers list ADHD at age 8, oppositional defiant disorder at age 11 and bipolar disorder at age 13. School and child welfare records show a placement change or a new school shortly before each new diagnosis. Staff observe irritability and poor sleep. What is the **MOST** appropriate way for the social worker to use these records?

A. Adopt the most recent diagnosis as the working one

B. Build a timeline linking symptoms, moves and diagnoses

C. Set the records aside and rely on current observation

208. During intake at a community mental health clinic, a 46-year-old reports 4 months of low mood, tiredness and weight gain. With the client's written consent, the social worker obtains primary care records. They show a thyroid test marked abnormal 5 months ago with a plan to repeat it in 6 weeks, but no repeat test appears. What should the social worker do **FIRST**?

A. Start a treatment plan aimed at the client's low mood

B. Ask the client's physician to follow up on the lab result

C. Tell the client that the thyroid result explains the symptoms

209. A school social worker receives a referral for a 14-year-old whom teachers describe as defiant because the student 'ignores directions.' With a parent's consent, the worker reviews school records. A third-grade evaluation identified a language disorder, and speech-language services stopped after a school transfer in sixth grade, with no reevaluation on file. Teacher notes say most problems occur with multistep spoken instructions. What is the **BEST** action?

A. Interview the student about trouble following directions

B. Start a school behavior plan rewarding prompt compliance

C. Ask the student's school team to revisit the earlier evaluation

D. Refer the student for a psychiatric evaluation for defiance

210. A social worker at a family services agency holds an intake with a client who has limited English proficiency. A trained interpreter is present in the room. The client is seeking help with a child's attendance problems at school. How should the social worker **BEST** conduct the interview?

A. Ask the interpreter to summarize the client's answers

B. Pose questions to the interpreter to relay to the client

C. Speak directly to the client in the first person

211. A client with limited English arrives for intake at an outpatient substance use program. The scheduled interpreter is trained and certified. While the interpreter steps out, the client tells the social worker in basic English, 'That interpreter knows my family. Our community is small.' When the interview resumes, the client gives short answers whenever drinking comes up. What is the **BEST** action?

A. Switch to a remote interpreter with no local ties

B. Remind the client that interpreters keep information private

C. Continue the intake in English using simpler questions

D. Ask the interpreter privately whether they know the client

212. On an inpatient psychiatric unit, a client tells the social worker during an assessment interview, 'Sometimes I just lose it, and then it's over.' The client offers nothing more and looks at the social worker, waiting. What should the social worker do **FIRST**?

A. Reflect the client's frustration about losing control

B. Ask whether the client hurts anyone when this happens

C. Ask the client to describe a recent time they lost it

D. Suggest the anger management group on the unit

213. A child welfare social worker has met monthly for 4 months with a parent working toward reunification, and they have a trusting working relationship. Today the parent says, 'I'm doing everything the plan asks.' Records show the parent missed 4 of the last 6 parenting classes, and a court review is scheduled next month. Earlier in this visit, the parent vented about how long the process is and said, 'At least you understand.' What should the social worker do **NEXT**?

A. Affirm the parent's commitment to getting the children back

B. Point out the gap between the parent's statement and attendance

C. Explore the parent's feelings about the case plan

214. In a hospice family meeting, an adult child who has cared for a dying parent for 8 months says, 'I feel like a terrible person for wishing this were over.' The social worker replies, 'After months of caring day and night, wanting the strain to end makes sense.' Which interviewing skill is the social worker **MOST** likely using?

A. Reflection of feeling

B. Validation

C. Reassurance

D. Interpretation

215. A client joins a video session 20 minutes late for the third week in a row. The client apologizes and says, 'I know, I know.' The social worker wants to understand what is getting in the way without sounding critical. Which question is **BEST**?

A. Why do you keep joining late?

B. Is it your work schedule, or are you unsure about coming?

C. What has been making it hard to join on time?

216. At a case management intake in a homeless shelter, a client sighs, pushes the forms aside and says, 'The shelter staff, the benefits office, now you. Everybody wants paperwork from me.' The social worker wants to build engagement early in the meeting. Which response is **BEST**?

A. 'So several offices need forms from you right now.'

B. 'It sounds like you're worn out by everyone's demands.'

C. 'I'll keep my part of the paperwork as short as I can.'

217. During a hospital family meeting, the social worker asks how the patient manages the stairs at home. A trained interpreter and the patient then talk back and forth in the patient's language for about a minute. The interpreter turns to the social worker and says, 'Basically, they're fine with the stairs.' What is the **MOST** appropriate response?

A. Ask the interpreter to interpret everything both people said

B. Accept the summary and move on to the next question

C. Request a different interpreter for the rest of the meeting

218. At an outpatient intake, a client referred for poor sleep spends the first 20 minutes describing a long dispute with a neighbor over parking. Each time the social worker reflects the frustration, the client agrees the dispute has been 'eating at' them and adds another detail about the neighbor. About 30 minutes remain. What should the social worker do **NEXT**?

A. Continue reflecting the client's anger at the neighbor

B. Ask a quick series of yes-or-no questions about sleep

C. Link the neighbor stress to sleep and shift the focus there

219. In a second session, a client is describing a pregnancy loss that happened 2 months ago. Mid-sentence, the client stops, looks down and begins to cry softly. The client has spoken openly so far and has not asked for anything. What response would be **MOST** helpful at this moment?

A. Ask what thoughts are coming up for the client right now

B. Stay quietly present and let the client resume when ready

C. Reassure the client that this kind of grief reaction is normal

220. On an inpatient psychiatric unit, the treatment team drafts a discharge plan before meeting with a patient admitted after a manic episode. At the planning meeting, the patient, now calm and organized, says, 'You all already decided everything.' What should the social worker do **FIRST**?

A. Walk the patient through each part of the team's draft plan

B. Explain why the team chose each recommendation in the plan

C. Invite the patient to name the problems that matter most to them

221. At an annual planning meeting in a day program, a 40-year-old client with intellectual developmental disorder sits beside a parent who is the client's legal guardian. Each time the social worker asks about goals for the coming year, the parent answers for the client. The client speaks in short sentences and understands pictures well. Which action is **MOST** appropriate?

A. Record the parent's answers as the client's goals for the year

B. Ask the client directly, using plain words and picture choices

C. Ask the parent to leave so the client can speak more freely

222. A child welfare agency holds a family group decision-making meeting for a 7-year-old in foster care. The coordinator has made introductions, and professionals have shared their safety concerns and the resources available. Relatives have asked their questions and seem ready to talk among themselves. What should the social worker do **NEXT**?

A. Step out with the agency staff so the family can plan

B. Present the agency's preferred plan for the family's feedback

C. Stay with the family to guide them through the discussion

223. A parent whose two children entered foster care meets with a child welfare social worker to build a case plan. The court has ordered substance use treatment and parenting classes. The parent says the top priority is finding a job, because without income they will lose their apartment, and the court also requires stable housing for reunification. What is the **MOST** appropriate way to proceed?

A. Include a job goal in the plan alongside the court-ordered services

B. Focus the plan on the court orders and add employment later

C. Make employment the first goal and begin services once the parent works

224. In a second video session, a client with moderate depression agrees that setting goals would help but says, 'I honestly don't know what I want. I just want to feel less stuck.' The client is engaged and willing to keep talking about it. What is the **BEST** way for the social worker to proceed?

A. Offer a list of common goals for people with depression

B. Wait to set goals until the client's mood improves

C. Ask how the client's days would differ if the problem lifted

D. Set a goal of lowering the client's PHQ-9 score

225. A client tells the social worker, 'I know my drinking is hurting my relationship, and part of me wants to cut back. But I'm not sure I'm ready to give up my evening drinks.' Which stage of change is the client **MOST** likely in?

A. Precontemplation

B. Preparation

C. Contemplation

226. At a follow-up visit, a client says, 'I stopped drinking 2 months ago. I go to support meetings twice a week and keep no alcohol at home. Weekends are still hard.' Which stage of change **MOST** likely describes this client?

A. Preparation

B. Action

C. Maintenance

227. In a primary care clinic, a client with a slow-healing foot ulcer smokes a pack a day. Asked to rate how important quitting is on a 0-10 scale, the client says 9. Asked how confident they are that they could quit, the client says 2. What should the social worker do **NEXT**?

A. Ask why the client's importance rating is 9 rather than 5

B. Explain how smoking slows the healing of foot wounds

C. Ask what would raise the confidence rating from 2 to 4

228. On a smoke-free inpatient psychiatric unit, a patient with schizophrenia who smoked heavily before admission tells the social worker, 'I don't want to start again when I leave. I'm picking a quit date with my doctor.' Discharge is in 5 days. What should the social worker do **FIRST**?

A. Review the pros and cons of smoking with the patient

B. Arrange a quitline referral to begin after discharge

C. Praise the decision and check in after discharge

D. Ask what could make it hard to avoid smoking at home

229. A client referred by drug court after a third arrest for driving under the influence arrives for an outpatient intake. The client sits with arms crossed and says, 'I'm only here because the judge made me. My drinking is nobody's business.' What should the social worker do **FIRST**?

A. Ask the client to rate readiness to change on a 0-10 scale

B. Acknowledge the client's anger about being sent to treatment

C. Point out that three arrests suggest drinking is a problem

230. At an opioid treatment program, a client on methadone for 4 months has had no opioid use and attends every counseling session. The client also uses cannabis daily and says, 'Weed isn't a problem. It's the only thing that helps me sleep.' Program staff want the treatment plan to require stopping cannabis now. What is the **BEST** way to proceed?

A. Make stopping cannabis a required goal in the plan

B. Explain to the client the health risks of using cannabis every day

C. Explore the client's own view of what cannabis gives and costs

D. Leave cannabis out of the plan while opioid goals come first

231. A cardiac rehabilitation team describes a 74-year-old patient as 'unmotivated' because the patient rarely does the home exercises. The patient says they want to get stronger. The social worker notices that the patient asks the same questions several times per visit and got lost driving to the clinic last week. What should the social worker do **FIRST**?

A. Use motivational interviewing to build commitment to exercising

B. Give the patient a simplified written home exercise schedule

C. Ask the physician to evaluate the patient's memory problems

D. Ask a family member to supervise the daily exercises

232. Which instrument is designed to measure how ready a client is to change their drinking?

A. SOCRATES

B. AUDIT

C. CAGE

D. CIWA-Ar

233. A client in outpatient treatment for gambling disorder had not placed a bet for 4 months. After a sports app sent a promotion, the client bet throughout a weekend and lost several hundred dollars. In session, the client says, 'Maybe I don't need to quit completely. I could just bet small amounts.' What should the social worker do **FIRST**?

A. Revise the relapse-prevention plan to block betting app promotions

B. Ask how the client now sees their recovery goals

C. Reassure the client that slips are common and progress remains

234. In Lazarus and Folkman's stress and coping model, what is a person's judgment about whether they have the resources to handle a stressor called?

A. Primary appraisal

B. Secondary appraisal

C. Emotion-focused coping

235. A hospital patient had a below-knee amputation after a work injury. The patient jokes with staff about 'saving on socks,' asks detailed questions about prostheses and calls coworkers daily. The social worker's assessment finds that the patient sleeps well, eats meals, works hard in physical therapy and talks about returning to work in a new role. A nurse requests a psychiatric consult because the patient 'must be in denial.' What should the social worker do **NEXT**?

A. Request the psychiatric consult the nurse suggested

B. Encourage the patient to set humor aside and grieve

C. Share the coping findings with the treatment team

D. Explore what the patient's jokes may be covering up

236. A 14-year-old in kinship care has lived through a parent's incarceration and three school moves. The teen earns steady grades, plays in the school band and talks about future plans. Since age 8, the same basketball coach has checked in weekly and attended the teen's court hearings. Which factor **MOST** likely accounts for the teen's resilience?

A. A committed relationship with a caring adult

B. Above-average intelligence and school ability

C. Taking part in structured after-school activities

237. A neurologist asks a hospital social worker to assess how a 34-year-old patient newly diagnosed with multiple sclerosis will manage after discharge. The patient is tearful but calm and plans to return home to a demanding job. Which question is **MOST** useful for this assessment?

A. Over the past 2 weeks, how often have you felt hopeless?

B. Would you like information about local MS support groups?

C. How have you gotten through hard times before, and who helped?

238. Which assessment tool **BEST** maps a family's current connections to schools, agencies, work and other outside systems?

A. Ecomap

B. Genogram

C. Culturagram

239. At a senior center, a 71-year-old client reports forgetting appointments over the past 2 months. The social worker gives a brief cognitive screen, and the score falls in the impaired range. The client mentions starting a new bladder medication about 10 weeks ago and has not seen a doctor since. What should the social worker do **NEXT**?

A. Tell the client the score points to early dementia

B. Arrange a prompt medical evaluation of the memory change

C. Repeat the screen in 6 months to track any change

D. Give the client a second cognitive screen to confirm

240. Which tool measures how well an older adult can handle money, medicines, shopping and meals?

A. Lawton Instrumental Activities of Daily Living Scale

B. Katz Index of Independence in Activities of Daily Living

C. Mini-Cog brief cognitive screen

241. At intake in a community clinic, a client whose preferred language is Tagalog reports low mood and poor sleep. The clinic routinely gives a standardized depression questionnaire, and its publisher offers versions in many languages, including Tagalog. A trained interpreter is on the call. Which approach is **BEST**?

A. Skip the questionnaire and rely on the clinical interview

B. Have the interpreter read the English version aloud in Tagalog

C. Give the client the published Tagalog version of the questionnaire

242. A hospital social worker meets with a 72-year-old spouse who has cared at home for 3 years for a partner with major neurocognitive disorder. The spouse describes losing sleep, giving up friendships and feeling trapped. The social worker wants a standardized measure of how the caregiving role itself is affecting the spouse. Which instrument is the **BEST** choice?

A. Geriatric Depression Scale

B. Lawton IADL Scale

C. Zarit Burden Interview

D. Ecomap

243. Which tool has a client list the people in their life and rate each one on concrete help, emotional support, closeness, and how often they are in contact?

A. Ecomap

B. Genogram

C. Culturagram

D. Social network map

244. At a primary care visit, a parent of two completes an adverse childhood experiences (ACE) questionnaire and scores 7. The parent describes fears that the score means they will become seriously ill or harm their children, then asks the social worker what the number means. What is the **MOST** appropriate response?

A. Explain that the score estimates risk for groups rather than individuals

B. Recommend trauma-focused therapy because of the high score

C. Encourage the parent to keep talking about these parenting fears

245. In a cardiac rehabilitation program, a 68-year-old client mentions having 'a few glasses of wine' most evenings. The social worker needs an accurate picture of the client's drinking to share with the care team. Which question is **BEST**?

A. 'You don't drink much more than that, do you?'

B. 'Would you say your drinking is a problem?'

C. 'On a typical drinking day, how much do you have?'

246. A 15-year-old comes to a pediatric clinic with a parent for a sports physical. The social worker plans a brief psychosocial screen covering substance use, sexual activity and mood. The parent sits beside the teen and starts answering the social worker's first questions. What should the social worker do **FIRST**?

A. Explain confidentiality and its limits, then meet with the teen alone

B. Ask the teen the screening questions in a calm, matter-of-fact way

C. Assure the teen that everything said today will stay private

247. During a video intake for depression, the social worker reaches the trauma history section, which asks about sexual abuse and assault. The client is at home. When the client's partner comes up, the client glances off-screen, lowers their voice and begins answering in single words. What should the social worker do **FIRST**?

A. Ask about unwanted sexual experiences in specific terms

B. Skip the trauma section unless the client raises it later

C. Email the trauma questionnaire for the client to finish later

D. Ask whether the client is alone and able to talk freely

248. At intake to a residential substance use program, a social worker needs to ask a 29-year-old client about any history of sexual violence. The client has heard the limits of confidentiality and agreed to continue with the intake questions. Which question is **MOST** effective for this purpose?

A. 'Has anyone ever forced or pressured you into sexual contact?'

B. 'Have you ever been sexually abused, molested or assaulted?'

C. 'Is there anything about your past that you would like to share?'

249. A school-based health center social worker meets alone with a 15-year-old for a first visit about headaches and missed classes. The visit opened with a review of what stays private and when information must be shared, which the teen restated accurately. The social worker needs to ask about substance use and sexual activity during this visit. What should the social worker do **NEXT**?

A. Ask about substance use and sexual activity right away

B. Wait to cover those topics until the teen raises them

C. Begin with home life and daily activities, then risk topics

250. A 74-year-old hospital patient had a stroke 5 days ago and now speaks in short, halting phrases. The team plans discharge in 2 days and asks the social worker whether the patient can take part in choosing between home and a rehabilitation facility. The patient's yes and no answers to the social worker's questions have been inconsistent. Which action is **BEST**?

A. Ask the patient simple yes-or-no questions about the options

B. Request a speech-language evaluation of the patient's communication

C. Ask the patient's adult child to choose the discharge setting

D. Note in the record that the patient cannot state a preference

251. On a home visit, the social worker explains how a client can book rides to dialysis through a new transportation service. The client nods throughout and says, 'Yes, I've got it.' Two ride requests the client tried to make last month were never completed. Which response is **MOST** appropriate?

A. Ask the client to show how they would book the next ride

B. Ask the client whether they have any questions about it

C. Leave a written copy of the booking steps with the client

252. A school social worker receives a referral for a 5-year-old who arrived 2 months ago from a country where English is not spoken. At school the child says almost nothing, yet watches classmates closely, points to request items and joins games. The parent reports that the child chats easily at home in the family's first language. Which explanation is **MOST** likely?

A. Selective mutism linked to school anxiety

B. Language disorder affecting expression

C. Normal silent phase of learning a new language

D. Autism spectrum disorder with language delay

253. At a community clinic, a client returns intake forms with most sections blank. When the social worker points to the consent page, the client says they left their reading glasses at home and asks to take everything home to finish. The client missed two earlier appointments that were scheduled by letter. What should the social worker do **FIRST**?

A. Ask the client to read the first paragraph aloud

B. Offer to review the forms together aloud

C. Let the client take the forms home to finish

D. Refer the client to an adult literacy program

254. A 24-year-old client is brought to a community mental health agency by a sibling. For the past 3 months, the client has heard voices commenting on their actions and has spoken in a disorganized way. There is no history of mood episodes, substance use or medical illness. What is the **MOST** likely diagnosis?

A. Brief psychotic disorder
B. Schizophreniform disorder
C. Schizophrenia
D. Schizoaffective disorder

255. On an inpatient psychiatric unit, a social worker gathers history for a 22-year-old patient. The patient has heard voices and believed neighbors were spying for the past 4 months. A parent reports that for about 5 months before the voices began, the patient withdrew from friends, stopped attending classes and talked about strangers sending hidden signals. There is no substance use, mood episode or medical cause. What is the **MOST** likely diagnosis?

A. Brief psychotic disorder
B. Schizophreniform disorder
C. Schizophrenia
D. Delusional disorder

256. A 54-year-old client with no psychiatric history comes to a community mental health agency after 2 weeks of hearing voices and accusing neighbors of stealing. The client's partner reports new morning headaches. The client also complains of a burning-rubber smell in the apartment that no one else notices. What should the social worker do **FIRST**?

A. Obtain a detailed family psychiatric history
B. Arrange a prompt medical and neurological workup
C. Refer the client to a first-episode psychosis program
D. Begin psychoeducation about psychotic disorders

257. A 45-year-old client in private practice says their partner of 20 years is having an affair. For 8 months the client has checked the partner's phone, followed the partner to work and read meaning into a moved car seat. No proof has turned up, and the client's certainty has not changed. The client still runs a small business well and speaks clearly. What is the **MOST** likely diagnosis?

A. Delusional disorder

B. Paranoid personality disorder

C. Schizophrenia

258. During a telehealth session, a 31-year-old client describes 5 days last month of sleeping 3 hours a night without feeling tired. The client talked rapidly, started several work projects and felt 'brilliant.' A coworker commented on the change, but the client kept working and met deadlines. Last year the client was treated for a major depressive episode. What is the **MOST** likely diagnosis?

A. Bipolar I disorder

B. Cyclothymic disorder

C. Major depressive disorder

D. Bipolar II disorder

259. At a college counseling center, a 20-year-old client seeks help for 6 weeks of depression. The client mentions that last spring they barely slept for 5 days, felt unstoppable and started several ventures, and roommates said they seemed like a different person. The campus prescriber plans to start an antidepressant at an appointment this afternoon. What should the social worker do **FIRST**?

A. Explore how the energetic period affected the client's friendships

B. Advise the client to delay the prescriber's antidepressant plan

C. Tell the prescriber about the earlier high-energy period

260. In a hospice bereavement program, a client meets with the social worker 18 months after an adult child died. Nearly every day the client longs intensely for the child and says, 'Part of me died too.' The client avoids the child's neighborhood, has stopped seeing friends and feels life has no purpose. The distress far exceeds what the client's faith community expects. What is the **MOST** likely diagnosis?

A. Major depressive disorder

B. Prolonged grief disorder

C. Uncomplicated bereavement

261. On an inpatient unit, a social worker reviews the 4-year history of a 33-year-old patient. The patient has had four manic episodes, each with grandiose delusions and voices, and these episodes have filled most of the illness. Twice, after the mood returned to normal, the patient kept hearing voices that criticized them for about 5 weeks. What is the **MOST** likely diagnosis?

A. Schizoaffective disorder, bipolar type

B. Bipolar I disorder with psychotic features

C. Schizophrenia

D. Delusional disorder

262. A patient is admitted to an inpatient psychiatric unit during a first manic episode. On the unit, the patient demands special privileges, insults staff and boasts of being destined for fame. A psychiatry resident suggests adding narcissistic personality disorder to the discharge diagnoses. What is the **BEST** response by the social worker?

A. Support adding it as a provisional diagnosis for follow-up

B. Administer a personality inventory before discharge to confirm it

C. Recommend deferring personality diagnosis until mood stabilizes

263. At an integrated primary care clinic, a 42-year-old client reports feeling low more days than not for the past 4 years, with little energy and a sense of being a failure. The client has never felt well for more than a few weeks at a time and says, 'This is just who I am.' The low mood began during a divorce that was finalized 4 years ago. The client works full time and cares for two children. What is the **MOST** likely diagnosis?

A. Persistent depressive disorder

B. Major depressive disorder

C. Adjustment disorder with depressed mood

264. A 76-year-old client is evaluated at a memory clinic. Over the past year, the client's alertness has varied widely from day to day, and the client often sees small children in the living room who are not there. Months after the memory changes began, the client developed a shuffling walk and stiff arms. The client's partner says the client now needs help with bills and medications. What is the **MOST** likely diagnosis?

A. Major neurocognitive disorder due to Alzheimer's disease

B. Major neurocognitive disorder with Lewy bodies

C. Delirium

D. Major neurocognitive disorder due to Parkinson's disease

265. A client is referred to a community agency after an emergency department visit labeled 'panic attack.' The social worker has taken a history: during a heated family argument, the client screamed, shook, felt heat rising to the head and fell to the floor. The client calls it an ataque de nervios and says such episodes run in the family after major conflicts. What should the social worker do **NEXT**?

A. Use the Cultural Formulation Interview to learn the client's view

B. Record panic disorder and refer for exposure-based treatment

C. Transfer the client to a practitioner from the client's culture

266. An assertive community treatment team visits a client with schizophrenia who has stopped taking medication. The client calmly explains that the voices are real broadcasts from neighbors and that nothing is wrong with their mind. When the team reviews two past hospitalizations, the client agrees they happened but still sees no illness. Which concept **MOST** likely explains the client's view?

A. Denial

B. Anosognosia

C. Avolition

267. A social worker in private practice completes an intake with a 38-year-old client who reports 2 months of low mood, fatigue and poor concentration. The client also mentions a 15-pound weight gain, feeling cold all the time and thinning hair. The client has not had a physical exam in 5 years. What should the social worker do **NEXT**?

A. Begin cognitive behavioral therapy for the depression

B. Administer a standardized depression scale to rate severity

C. Refer the client to a primary care provider for a medical evaluation

268. Which term describes the combination of mutism, holding odd postures for long periods, resisting being moved and repeating other people's words?

A. Catatonia

B. Acute dystonia

C. Negative symptoms

269. In a telehealth intake, a 34-year-old client asks for an ADHD diagnosis to get medication. The client describes poor focus and missed details at work that began 18 months ago, soon after starting rotating night shifts. The client earned good grades in school. Which action is **BEST**?

A. Administer an adult ADHD self-report rating scale

B. Assess childhood symptoms and sleep disruption

C. Tell the client the timeline rules out ADHD

270. A hospital social worker meets with a 40-year-old patient admitted with sudden weakness in the left leg after a car accident that caused no injury. The medical team's exam findings do not match any known nerve or brain disease, and imaging is normal. The patient is upset about missing work. What is the **MOST** likely diagnosis?

A. Malingering

B. Factitious disorder imposed on self

C. Somatic symptom disorder

D. Functional neurological symptom disorder

271. In the fourth session at a rural agency, a client reports losing hours at a time, finding receipts for purchases the client does not remember and being greeted by strangers using another name. The social worker has no training in dissociative disorders, and no one at the agency does. The clinical director can connect the worker with a regional expert in these conditions. What is the **MOST** appropriate action?

A. Record dissociative amnesia based on the reported gaps

B. Ask the client to describe early trauma in detail

C. Consult a specialist about how to assess the client

272. A client is brought to an emergency department with hallucinations and disorganized speech. The client cannot give a history, no records or collateral contacts are available, and the duration of symptoms is unknown. Which diagnosis does DSM-5-TR provide for this situation?

A. Other specified schizophrenia spectrum and other psychotic disorder

B. Unspecified schizophrenia spectrum and other psychotic disorder

C. Schizophreniform disorder, provisional

273. A probation officer refers a 27-year-old client with a letter listing bipolar disorder from a jail intake screening. The client describes days of feeling invincible with almost no sleep, but only during cocaine binges, followed by several days of crashing. The client has had negative drug tests for the past 6 weeks of court-ordered treatment. What is the social worker's **MOST** appropriate next step?

A. Assess the client's mood since substance use stopped

B. Accept the listed diagnosis and refer for mood stabilizers

C. Request the jail's full screening record before going further

274. A 26-year-old client at a community agency describes mood swings that shift within hours, often after a partner is slow to answer texts. During these times the client feels intense rage, cuts their arms and fears being left. Between swings the client describes a constant inner emptiness. This pattern has continued since the client's late teens. What is the **MOST** likely diagnosis?

A. Bipolar II disorder

B. Cyclothymic disorder

C. Borderline personality disorder

D. Histrionic personality disorder

275. A school social worker completes an assessment of a 15-year-old student who identifies as nonbinary and uses they/them pronouns. The student has close friends, steady grades and says they feel comfortable with who they are. The student's parent asks the social worker for a diagnosis 'so we know what is wrong.' What is the **MOST** appropriate response?

A. Diagnose gender dysphoria based on the gender incongruence

B. Explain that a diagnosis requires distress or impairment

C. Refer the student to a gender clinic for a diagnosis

276. A child welfare social worker reviews the case of a 9-year-old in kinship foster care. Since age 7, the child has been angry and irritable most of every day. Several times a week, the child has severe outbursts at home and at school, such as screaming and throwing chairs, far beyond what peers do. What is the **MOST** likely diagnosis?

A. Disruptive mood dysregulation disorder

B. Oppositional defiant disorder

C. Bipolar I disorder with rapid cycling

D. Intermittent explosive disorder

277. A social worker at an assisted living facility meets with a 78-year-old resident who has no psychiatric history. Over 6 weeks the resident has stopped eating, sleeps poorly and says, 'I am worthless.' The resident now insists their organs are rotting as punishment for old sins. Staff report that this belief appeared only after the low mood began. What is the **MOST** likely diagnosis?

A. Delusional disorder, somatic type

B. Schizoaffective disorder, depressive type

C. Major depressive disorder with psychotic features

D. Major neurocognitive disorder due to Alzheimer's disease

278. A 34-year-old client spends about 3 hours a day washing after touching doorknobs or mail. The client is completely sure that skipping the routine will give their children a fatal illness and cannot be talked out of it. Work and family life have narrowed around the washing. Outside of this topic, the client's conversation is organized and goal-directed. Which diagnosis is **MOST** likely?

A. Delusional disorder

B. Obsessive-compulsive disorder

C. Schizophrenia

D. Illness anxiety disorder

279. At an overnight shelter, staff ask the social worker to see a resident with years of heavy drinking who has not had alcohol for 2 days. Since this morning, the resident has been confused about where they are, walks with a wide, staggering gait and has jerky eye movements. Staff think the resident just needs to sleep it off. What should the social worker do **FIRST**?

A. Complete a brief cognitive screen such as the MoCA

B. Arrange admission to a withdrawal management program

C. Let the resident rest and check on them hourly

D. Send the resident to the hospital right away

280. In private practice, a client seeks help after receiving an eviction notice. The client feels worried and sad, keeps working full time, sleeps well and is actively searching for housing. The client says the worry 'makes sense given the notice.' After a full assessment, the social worker must record the findings, and the client's insurance requires a diagnosis for payment. How should the social worker record the assessment?

A. Record a Z-code condition and no mental disorder

B. Record adjustment disorder because the stressor is identifiable

C. Record unspecified anxiety disorder pending further sessions

281. During a telehealth intake, a 27-year-old client describes 3 years of shifting moods. Every few weeks the client has 2 or 3 days of extra energy, talkativeness and less need for sleep, followed by a week or so of low mood, tiredness and poor concentration. The client has kept the same job throughout and has rarely felt steady for more than a few weeks at a time. Which diagnosis is **MOST** likely?

A. Bipolar II disorder

B. Persistent depressive disorder

C. Cyclothymic disorder

D. Borderline personality disorder

282. At a community mental health intake, a client completes the DSM-5-TR Level 1 Cross-Cutting Symptom Measure. Most domains are rated low, but the client rates the items about sleeping less than usual and starting many more projects than usual as moderate. The social worker reviews these answers with the client. What should the social worker do **NEXT**?

A. Record a provisional bipolar disorder diagnosis

B. Give the matching Level 2 measure for follow-up

C. Repeat the Level 1 measure at the next visit

283. A jail mental health team is assessing a 31-year-old detainee for frequent rage outbursts, impulsive spending and unstable relationships. A staff member proposes borderline personality disorder. Records show a serious head injury in a motorcycle crash 18 months ago, and family members describe the detainee as easygoing and steady before the crash. What is the social worker's **BEST** recommendation?

A. Request a neuropsychological evaluation of brain functioning

B. Begin dialectical behavior therapy skills for emotion regulation

C. Record borderline personality disorder as a provisional diagnosis

D. Gather more history on personality traits in adolescence

284. A client meets criteria for a major depressive episode and, on most days of the episode, also has racing thoughts, pressured speech and increased goal-directed activity. Which DSM-5-TR specifier applies?

A. With anxious distress

B. With rapid cycling

C. With atypical features

D. With mixed features

285. A hospice patient, 81, with advanced pancreatic cancer has low mood, poor appetite, weight loss, fatigue and broken sleep. The hospice team asks the social worker whether the patient also has major depression. How can the social worker **BEST** judge whether a depressive disorder is present?

A. Count the appetite, sleep and energy changes as depressive symptoms

B. View the low mood as an expected reaction to dying

C. Focus on hopelessness, worthlessness and loss of pleasure

286. A partner brings a 57-year-old client to a hospital neurology clinic. Over the past year the client has made crude remarks to strangers, spent savings on impulse purchases, eaten sweets compulsively and shown little concern for family. The changes have come on gradually and steadily. The client remembers recent events and appointments well. Which diagnosis is **MOST** likely?

A. Bipolar I disorder, manic episode

B. Frontotemporal neurocognitive disorder

C. Alzheimer's neurocognitive disorder

D. Antisocial personality disorder

287. In a hospital pain clinic, a patient has taken prescribed oxycodone exactly as directed for 8 months after spinal surgery. The patient now needs a higher dose than at first and feels achy and sweaty when a dose is late. A nurse asks the social worker to note opioid use disorder in the chart. The social worker confirms the dosing history with the prescriber. What should the social worker do **NEXT**?

A. Record mild opioid use disorder in the chart

B. Refer the patient to an opioid treatment program

C. Check for criteria beyond tolerance and withdrawal

288. At a university diabetes clinic, a 20-year-old client with type 1 diabetes is at a normal weight but has rising blood sugar readings. The client describes eating large amounts of food in a short time about twice a week for 4 months, feeling unable to stop. Afterward, the client skips insulin doses 'to cancel out the calories.' The client says their weight decides how they feel about themselves. Which diagnosis is **MOST** likely?

A. Bulimia nervosa

B. Binge-eating disorder

C. Anorexia nervosa, binge-eating/purging type

D. Other specified feeding or eating disorder

289. A school social worker meets with a 17-year-old referred for 'seeing things.' The student describes vivid dreamlike images while drifting off to sleep, falls asleep in class almost daily despite 9 hours of sleep at night, and has had knees buckle suddenly while laughing with friends. These problems began 6 months ago, and grades are dropping. What is the social worker's **BEST** recommendation?

A. Refer the student for a psychiatric evaluation of possible psychosis

B. Refer the student for evaluation by a sleep medicine specialist

C. Screen the student for depression as a cause of sleep problems

290. On an inpatient unit, a 24-year-old patient with autism spectrum disorder diagnosed in early childhood is admitted after a family conflict. The team notes flat facial expression, few words, rigid routines and little interest in peers, and a resident suggests adding schizophrenia. The patient's parent says these traits have always been present. What is the **MOST** appropriate input from the social worker?

A. Support adding schizophrenia based on the negative symptoms

B. Gather a detailed developmental history from the parent

C. Explain that prominent delusions or hallucinations are also required

291. A 58-year-old client in an employee assistance program has been meeting with a social worker about work stress. Midway through a session, the client becomes sweaty and short of breath and rubs their left arm, saying the chest pressure is 'just my nerves.' The client has never had a panic attack before. What should the social worker do **FIRST**?

A. Guide the client through slow, paced breathing

B. Ask what thoughts came just before the pressure began

C. Get an ambulance to the office immediately

292. A 27-year-old is admitted to an inpatient unit after 3 weeks of near-daily methamphetamine use. On admission, the patient believes neighbors are pumping gas through the vents and hears them plotting. With no drug use on the unit, these beliefs fade by day 5, and the patient returns to their usual self. Which diagnosis is **MOST** likely?

A. Brief psychotic disorder

B. Schizophreniform disorder

C. Psychotic disorder due to another medical condition

D. Stimulant-induced psychotic disorder

293. On an inpatient unit, the team plans to diagnose schizophrenia in a Black adult who is guarded, distrusts staff and declines to share personal details. The social worker's interview finds 5 weeks of early waking, slowed movement, poor appetite and intense guilt. The client says past providers 'never listened.' What is the **MOST** appropriate action for the social worker?

A. Press the team for a full evaluation of a depressive episode

B. Explore the client's earlier experiences with mental health providers

C. Support the working diagnosis given the persistent suspicion

294. Which DSM-5-TR tool rates how well a person gets around, cares for self, gets along with others and takes part in community life?

A. Global Assessment of Functioning (GAF)

B. WHODAS 2.0

C. Level 1 Cross-Cutting Symptom Measure

D. Cultural Formulation Interview

295. Group home staff call the social worker about a resident with schizophrenia. For 2 days, the resident has taken only sips of water and almost no food. The resident stands in one spot for hours, holds odd postures, does not answer questions and stiffens when staff try to guide them. What should the social worker do **FIRST**?

A. Get the resident to an emergency department today

B. Rate the resident's negative symptoms with a standard scale

C. Ask the psychiatrist to see the resident at the next clinic visit

D. Have staff push fluids and track intake at the home

296. A 31-year-old comes to a community agency at a parent's urging. The client has no close friends besides a sibling, dresses in mismatched layers and believes they can sense coworkers' moods through 'vibrations.' The client often thinks strangers' conversations are about them but agrees this may not be true. Unease around coworkers has not faded after 6 years at the same job. Which diagnosis is **MOST** likely?

A. Schizoid personality disorder

B. Schizophrenia

C. Schizotypal personality disorder

D. Avoidant personality disorder

297. A client in private practice was laid off 2 months ago and since then has had poor sleep, tearfulness and trouble concentrating. The client is suing the former employer and asks the social worker to record PTSD because 'that will help the case.' The social worker's assessment supports adjustment disorder with mixed anxiety and depressed mood. What is the **MOST** appropriate response?

A. Record PTSD as a provisional diagnosis while gathering more history

B. Explain why the findings point to adjustment disorder instead

C. Record adjustment disorder and direct legal questions to the client's attorney

298. Police bring a 23-year-old to the emergency department after finding them directing traffic and announcing a plan to buy the city's stadium. A roommate says the client has barely slept for 3 days, has talked nonstop and has spent their savings online. The client is admitted to the psychiatric unit that night. Which type of mood episode is **MOST** likely?

A. Manic episode

B. Hypomanic episode

C. Major depressive episode with mixed features

299. In a hospice bereavement program, a 79-year-old client whose spouse died 4 months ago mentions hearing the spouse say 'good night' most evenings. The client then looks away and asks quietly, 'Does that mean I'm losing my mind?' The client has not told anyone about this before. What should the social worker do **FIRST**?

A. Refer the client for a psychiatric evaluation of hallucinations

B. Tell the client that hearing a late spouse is common in grief

C. Ask what the experience is like and how the client makes sense of it

300. A hospital social worker meets with a 45-year-old patient who has rheumatoid arthritis. The patient's joint pain is persistent and distressing. For the past year, the patient has spent hours each day reading about joint damage, has gone to the emergency room most weeks fearing disability and has stopped working. Reassurance from the rheumatologist brings relief for only a day. Which diagnosis is **MOST** likely?

A. Illness anxiety disorder

B. Psychological factors affecting other medical conditions

C. Generalized anxiety disorder

D. Somatic symptom disorder

301. A jail social worker interviews a detainee awaiting sentencing who reports hearing voices 'all day, every day' and asks to move to the mental health unit. The detainee describes the voices vaguely and denies thoughts of harming self or others. Officers' logs describe the detainee sleeping well and playing cards calmly with others. After completing the interview, what should the social worker do **NEXT**?

A. Recommend transfer to the mental health unit for observation

B. Request prior treatment records and reassess over several contacts

C. Document malingering based on the inconsistencies found

302. A mobile crisis team brings an adult to a crisis stabilization unit. The adult is agitated, irritable and tearful, with rapid speech, and cannot give a history. No records or family are available. The symptoms clearly involve mood, but it is unclear whether they belong to a bipolar or a depressive disorder. Which DSM-5-TR diagnosis fits this situation?

A. Unspecified bipolar and related disorder

B. Unspecified mood disorder

C. Unspecified mental disorder

D. Unspecified depressive disorder

303. On a home visit, an adult child reports that their 74-year-old parent with Parkinson's disease has begun seeing small animals in the corners of rooms. The parent knows the animals are not real and is not frightened. A Parkinson's medication dose was raised 3 weeks ago. The adult child wants the parent admitted to a psychiatric hospital. What is the social worker's **BEST** action?

A. Help the family tell the neurologist about the new symptoms

B. Support a voluntary psychiatric admission for a full evaluation

C. Reassure the family that hallucinations are expected with the disease

304. A 34-year-old began sertraline for depression 5 weeks ago. Two weeks later, the client stopped sleeping, spent recklessly and announced a plan to run for governor. The prescriber stopped the sertraline at once. Three weeks after it was stopped, the client remains on an inpatient unit with grandiosity, pressured speech and little need for sleep. Which diagnosis is **MOST** likely?

A. Substance/medication-induced bipolar and related disorder

B. Bipolar II disorder

C. Bipolar I disorder

305. At a pediatric clinic, a parent asks the social worker to start an ADHD diagnosis for their 7-year-old. For 8 months, the child has struggled to finish chores, loses things and interrupts siblings. So far, no information about the child's behavior at school has been gathered. What should the social worker do **FIRST**?

A. Refer the child to the pediatrician to discuss medication

B. Obtain rating scales from the parent and the teacher

C. Start behavioral parent training for the problems at home

306. At an area agency on aging, an adult child asks for help placing their 71-year-old parent in memory care. Over the past 5 months, the parent has developed a shuffling, unsteady walk, new bladder accidents and growing forgetfulness. The adult child, the main caregiver, says they are exhausted. The parent has had no checkup since the changes began. What should the social worker do **FIRST**?

A. Help the family compare memory care facilities

B. Set up in-home respite care for the tired caregiver

C. Arrange a prompt visit with the parent's physician

D. Screen the parent's memory with a brief cognitive test

307. At a crisis residential program, a client with borderline personality disorder becomes convinced for an afternoon that staff are plotting to discharge them, after a close peer leaves the program. By evening, the client laughs about the idea. A psychiatry resident proposes adding delusional disorder. Which response by the social worker is **MOST** helpful to the team?

A. Note that brief, stress-linked suspicion fits the existing diagnosis

B. Agree to add the diagnosis and monitor the beliefs closely

C. Suggest psychological testing to clarify the diagnosis first

308. A housing inspector refers a 52-year-old tenant to a community agency. Stacks of newspapers, bags and boxes fill every room, and the tenant sleeps in a chair because the bed is buried. The tenant becomes very upset at the idea of discarding anything, saying each item 'might be needed someday.' Which diagnosis is **MOST** likely?

A. Hoarding disorder

B. Obsessive-compulsive personality disorder

C. Obsessive-compulsive disorder

II.C Assessment Practices

309. At a pediatric clinic, a social worker meets the parent of a child who turned 3 last month. The child uses only a few words, and the pediatrician recommends a developmental evaluation and possible services. The family has never received any early childhood services. Which resource is the **BEST** fit for the parent to contact?

A. The local school district's special education office

B. The state's Part C early intervention program

C. A local Head Start program

310. A primary care social worker gave a client with panic disorder a list of three low-cost counseling clinics. At follow-up 2 weeks later, the client has not called any of them and explains that phone calls with strangers trigger panic and that the clinic websites were confusing. The client still wants counseling. What should the social worker do **NEXT**?

A. Ask the client what made calling the clinics difficult

B. Call one of the clinics together with the client now

C. Give the client a shorter list with clearer directions

311. A social worker at a rural community agency meets a client who wants medication treatment for opioid use disorder. The nearest opioid treatment program is 80 miles away and requires near-daily visits at first. The client works days and has no car. The local primary care clinic offers in-person and telehealth visits. Which resource is **MOST** appropriate to pursue?

A. Daily methadone dosing at the distant opioid treatment program

B. A nearby prescriber of buprenorphine

C. A weekly peer support group in the county

D. A residential treatment program in the nearest city

312. A 31-year-old veteran who completed two combat deployments tells a community agency social worker about nightmares and growing distance from their partner. The veteran refuses to go to a VA hospital, saying it 'feels like being back on base,' and cannot afford private fees. Which referral is **MOST** appropriate?

A. A Vet Center offering individual and couples readjustment counseling

B. The outpatient mental health clinic at the VA medical center

C. A trauma specialist in private practice near the client's home

313. At intake, a parent reports being evicted last week and sleeping in a car with 2 young children during freezing nights. The parent also describes low mood and a custody hearing in 6 weeks, and asks first about the hearing. The parent denies suicidal thoughts. What should the social worker address **FIRST**?

A. The parent's depressive symptoms and treatment options

B. Emergency shelter for the family for tonight

C. A legal aid referral for the custody hearing

314. On a busy afternoon, a social worker is the only staff member covering walk-in triage at a community mental health center. Four people have signed in at about the same time and are waiting in the lobby. Which person should the social worker see **FIRST**?

A. A client with long-standing, unchanged passive death wishes, here for routine care

B. A client who ran out of an antidepressant 2 days ago and feels fine

C. A parent who needs a letter for a housing appointment tomorrow morning

D. An older adult whose confusion and fever began this morning

315. A client with bipolar I disorder has spent 12 days on an inpatient unit for mania. The client now sleeps through the night and voices no thoughts of harming anyone, but still has racing thoughts at times while a new medication is adjusted. The client lives with a supportive partner and wants to go home. Which aftercare plan is **BEST**?

A. Continue inpatient care until every symptom resolves

B. Step down to a partial hospitalization program

C. Discharge home with weekly outpatient therapy

D. Transfer to a residential treatment program

316. A child welfare social worker meets with a parent whose 4-year-old was placed in foster care after being left alone overnight. The court-approved case plan lists nine tasks, including parenting classes, a substance use assessment, housing applications and a job search. The parent says, 'I can't do all of this at once, and I don't know where to start.' The review hearing is in 5 months. What should the social worker do **FIRST**?

A. Rank the tasks with the parent by their link to safety

B. Help the parent enroll in the nine services this week

C. Ask the court to reduce the number of required tasks

D. Refer the parent to counseling for feeling overwhelmed

317. A social worker providing case management at an HIV services agency works with a client who also sees a primary care physician, a psychiatrist and an outpatient substance use program. The client arrives upset after receiving three different instructions about sleep medication timing and appointment days and says, 'Everyone tells me something different.' The client has signed releases for all providers. Which action is **MOST** helpful?

A. Help the client decide which provider's advice to follow

B. Write every instruction on one calendar for the client

C. Convene the providers to agree on one coordinated approach

318. A 16-year-old and a parent come to a family service agency for help. The parent wants the plan to focus on raising the teen's grades. The teen wants the parent to 'stop yelling.' Both say that evenings at home have become tense and that they want more peace. What is the **MOST** appropriate way to begin setting goals?

A. Start with the goal they share: calmer evenings

B. Focus on grades first, since the parent consents to care

C. Start with the teen's goal of less yelling in the evenings

319. The ASAM Criteria (4th edition) rate a person's needs on six dimensions. Which dimension covers how safe and supportive the person's home is, such as roommates who use?

A. Substance use-related risks

B. Person-centered considerations

C. Intoxication, withdrawal, and addiction medications

D. Recovery environment interactions

320. A parent calls an outpatient clinic about a 16-year-old on its 3-week waitlist. The teen has posted goodbye messages online and given away a prized gaming system. When the parent asked directly, the teen said they have been saving pills to take this weekend. The teen is at home now. What should the intake social worker do **FIRST**?

A. Have the parent call emergency services now and stay with the teen

B. Book the teen into the clinic's first urgent opening tomorrow morning

C. Complete a full suicide risk screening with the parent over the phone

321. At a first treatment-planning meeting at a community mental health center, a 20-year-old discharged from a psychiatric unit 2 weeks ago says the only goal is to return to college full time next month. The client adds that this 'matters more than anything.' The discharge summary lists steady sleep and medication routines as early priorities. What should the social worker do **FIRST**?

A. Explain that full-time enrollment next month is too soon

B. Record the college goal in the plan exactly as the client stated it

C. Break the goal into steps, such as one course next term

D. Ask what attending college full time would mean to the client

322. An emergency department social worker assesses a 38-year-old whose partner brought them in after they said, 'Everyone would be better off without me,' following a job loss. When asked directly, the patient describes passive wishes to be dead but no intent or plan, has no firearms at home, and has never attempted suicide. The partner will stay home this week, and the patient has an established outpatient mental health provider. What is the **BEST** plan?

A. Admit the patient to the inpatient psychiatric unit for observation

B. Discharge home with a written safety plan and a therapy visit within 2 days

C. Keep the patient in the emergency department overnight for reassessment

D. Refer the patient to a partial hospitalization program next week

323. A 4-year-old is referred for frequent tantrums and hitting at home and preschool. The caregiver, who benefited from cognitive behavioral therapy for anxiety, asks whether the preschooler could receive the same kind of individual talk therapy. Which modality is **MOST** appropriate for this preschooler?

A. Parent-child interaction therapy

B. Individual cognitive therapy with thought records

C. Weekly social skills group with peers

324. A school social worker meets with the parents of a 14-year-old recently diagnosed with anorexia nervosa. The pediatrician has found the teen medically stable for outpatient care. The teen lives at home, and the parents are close to the teen and eager to help. The social worker has no training in eating disorder treatment. Which plan is **BEST**?

A. Begin weekly individual counseling with the teen at school

B. Refer the teen to a family-based treatment provider

C. Recommend a residential eating disorder treatment program

325. A 28-year-old client with mild intellectual developmental disorder lives in a group home and feels anxious about riding the bus to a new job. The client reads at about a second-grade level, learns best by practicing tasks with staff and asks for help 'feeling less scared.' Which treatment approach is **MOST** appropriate?

A. Standard CBT using the agency's written workbook

B. Insight-oriented therapy exploring the fear's origins

C. CBT adapted with pictures, repetition and a support person

326. A hospice social worker visits an 86-year-old patient with advanced heart failure. The patient's mood is low, and the patient says, 'I keep going over the past. I want to know it all counted for something before I go.' The patient is alert and has a few months to live. Which approach is **MOST** helpful?

A. Guide a structured life review

B. Begin exposure therapy for death-related anxiety

C. Schedule pleasant activities to lift daily mood

327. A juvenile court asks a social worker to start a weekly group for eight 14- and 15-year-olds on probation for fighting and theft. The court says a group will serve more youth at lower cost. Most of the youth already spend time together, and several have parents who are willing to take part in services. What is the **BEST** recommendation?

A. Run the group with a structured anger-management curriculum

B. Offer family-based work instead of a peer group

C. Hold individual sessions focused on insight into behavior

328. A 20-month-old was present during several violent incidents between caregivers before the nonoffending caregiver moved with the toddler to a relative's home. Since then, the toddler startles at loud voices, wakes screaming most nights, and clings constantly. The caregiver is also on edge and struggles to soothe the toddler. Which treatment model is the **BEST** fit?

A. Child-parent psychotherapy

B. Parent-child interaction therapy

C. Trauma-focused cognitive behavioral therapy

D. Nondirective individual play therapy

329. Staff at a nursing facility ask the social worker to begin weekly talk therapy for a resident with moderate Alzheimer's disease. The resident yells at and pushes away aides several times a week, usually during morning bathing. The resident cannot recall conversations from earlier in the day. What is the **MOST** appropriate approach?

A. Hold weekly individual sessions exploring the resident's frustrations

B. Ask the physician to consider medication to calm the resident

C. Help caregivers adjust the routine that sets off the distress

330. A 24-year-old client is in treatment for depression at a college counseling center. The client lives with parents and grandparents, as the family's culture expects until marriage, and describes the arrangement as supportive. While reviewing a draft plan, the social worker notes that the center's template lists 'move to independent housing' as a standard goal for clients this age. How should the social worker handle this goal?

A. Keep the goal, since leaving home fits this life stage

B. Ask the client privately whether the family applies pressure

C. Replace it with goals the client defines within the family

331. A client is referred to a mixed-gender trauma recovery group after a sexual assault. She tells the social worker that her faith forbids discussing sexual matters in the presence of men, and she wants trauma treatment. The agency also runs a trauma group for women that has openings. How should the social worker adjust the plan?

A. Encourage her to try the mixed group for a few weeks

B. Switch her referral to the female survivors' group

C. Ask the mixed group to agree to avoid sexual details

332. A client raised in a rural Latin American community is in outpatient treatment for depression. The client believes the illness began with susto, a loss of spirit caused by a frightening event. The client plans to visit a traditional healer for a cleansing and asks the social worker whether that is 'allowed' alongside counseling. What should the social worker do **FIRST**?

A. Refer the client to a provider who shares the client's culture

B. Contact the healer to coordinate care with counseling

C. Ask how the client hopes the healer's work will help

D. Explain that depression is a treatable medical condition

333. An agency is planning a caregiver stress group for adult children caring for parents with dementia. Most of the caregivers who have enrolled speak Cantonese and have limited English. The agency's standard curriculum is written and taught in English. Which planning choice is **MOST** likely to make the group effective?

A. Run the group in English with a trained interpreter present

B. Have a Cantonese-speaking facilitator lead the group in Cantonese

C. Translate handouts for Cantonese-speaking members and teach in English

334. A family that immigrated 4 years ago seeks help because a 15-year-old and the parents argue daily. The teen, now fluent in English, wants to spend evenings with friends and attend parties. The parents see this as rejecting family values and say the teen is 'becoming a stranger.' The teen's grades remain steady. Which intervention plan is **BEST**?

A. Individual sessions to help the teen manage anger with family

B. Parent training in setting consistent limits and consequences

C. Family sessions helping members value skills from both cultures

D. Guidance urging the parents to allow freedoms typical for local teens

335. An adult client with bipolar disorder takes lithium. At a home visit, the client reports 2 days of vomiting and diarrhea from a stomach virus. The social worker notices a coarse hand tremor and an unsteady walk that were absent last week. What should the social worker do **FIRST**?

A. Help the client get emergency medical care today

B. Advise the client to skip lithium until the illness passes

C. Recheck the client's symptoms at next week's home visit

336. An adult began aripiprazole 10 days ago for psychotic symptoms. At an outpatient visit, the client cannot stay seated, shifts from foot to foot and says, 'I feel like I have to keep moving inside.' The client is distressed by the feeling. Clinic staff call it anxiety about the new treatment. Which explanation is **MOST** likely?

A. Generalized anxiety disorder

B. Tardive dyskinesia

C. Worsening psychotic agitation

D. Akathisia

337. A client who has taken sertraline for a year begins tramadol after dental surgery. The next day, at a scheduled clinic visit, the client is agitated and sweating heavily, with diarrhea, a racing heart and jerking muscles in both legs. The client thinks it is a stomach bug. What should the social worker do **FIRST**?

A. Have the client seen in an emergency department now

B. Call the prescriber's office to report the new medicine

C. Advise the client to stop the tramadol and rest at home

338. A school social worker meets with the parent of a 15-year-old who has taken fluoxetine for 3 months. The teen's mood and attendance have improved, and the teen reports no suicidal thoughts at regular check-ins. The parent just read about the antidepressant warning for youth and plans to stop the medicine tonight. What is the social worker's **BEST** response?

A. Support the parent's decision to stop the medicine tonight

B. Encourage the parent to talk with the prescriber before stopping

C. Assure the parent that the risk is limited to the first weeks

339. An adult with schizophrenia who has been stable on clozapine moves into a smoke-free group residence and stops smoking 1.5 packs a day all at once. Five days later, staff report that the client is much sleepier, drooling more and walking unsteadily. What is the **MOST** important action for the social worker?

A. Tell the prescriber today that the client quit tobacco

B. Arrange nicotine replacement to ease the client's withdrawal

C. Assess for withdrawal symptoms since the client quit smoking

340. A client with depression has taken sertraline for 6 months with partial benefit and also takes an oral contraceptive. In session, the client shows a bottle of St. John's wort bought that morning on a friend's advice and plans to start it tonight. The next prescriber visit is in 3 weeks. What is the social worker's **BEST** response?

A. Support the plan, since herbal remedies are the client's choice

B. Explore the appeal of herbs and revisit at the prescriber visit

C. Urge a call to the prescriber or pharmacist before the first dose

341. In an integrated primary care clinic, an adult taking escitalopram for depression reports feeling much better after 2 months. Pharmacy records show doses are often skipped on weekends. When the social worker asks about the missed doses, the client hesitates and says, 'It's kind of personal.' What should the social worker do **NEXT**?

A. Ask plainly whether sexual side effects play a part

B. Notify the prescriber that the client skips doses

C. Remind the client that daily doses keep the mood stable

342. In an integrated primary care clinic, a client with depression and chronic back pain reported passive thoughts of death 3 weeks ago. At intake, the client agreed to the clinic's team-based care, in which providers share clinical information. The physician has just started amitriptyline and, to save on copays, prescribed a 90-day supply. The client asks the social worker not to 'make a big deal' of the earlier thoughts. What is the social worker's **BEST** response?

A. Coach the client to keep the pills locked away at home

B. Respect the request and track mood at follow-up

C. Alert the client's doctor to the overdose danger

343. An 81-year-old client with mild memory problems lives with an adult child. During a home visit, the adult child reports that over the past 4 days the client has become confused at night, has trouble passing urine and complains of a dry mouth. Last week the client began taking an over-the-counter nighttime pain reliever that contains diphenhydramine. What is the **MOST** likely cause of the change?

A. Progression of a neurocognitive disorder

B. Anticholinergic effects of the new product

C. Onset of a late-life major depressive episode

D. Normal age-related cognitive decline

344. A 29-year-old client with bipolar I disorder has been stable for 2 years on valproate. At a community clinic visit, the client happily shares a plan with their partner to start trying for a baby next month. The client's next psychiatry appointment is in 4 months. What is the social worker's **BEST** response?

A. Encourage seeing the prescriber before trying to conceive

B. Suggest tapering off valproate before trying to conceive

C. Explore the client's hopes and worries about parenting

345. A psychiatric inpatient with schizophrenia had a haloperidol dose increase 5 days ago. Today the patient is confused and sweating heavily, with a temperature of 103°F. Nursing staff report that the patient's arms are stiff and hard to bend and that the blood pressure keeps swinging up and down. What is the **MOST** likely explanation?

A. Serotonin syndrome

B. Acute dystonic reaction

C. Worsening psychotic symptoms

D. Neuroleptic malignant syndrome

346. Ten days after starting sertraline for depression, a client on a telehealth visit says the pills 'are not doing anything' and plans to stop them this weekend. The client mentions mild nausea that is fading. The client has not talked with the prescriber about stopping. What is the **MOST** appropriate response from the social worker?

A. Suggest asking the prescriber for a stronger dose

B. Explain the usual delay in benefit and urge a prescriber call

C. Explore the client's general beliefs about taking medication

347. Long-term antipsychotic use can lead to involuntary lip smacking, chewing motions, and tongue movements. Which movement disorder does this describe?

A. Acute dystonia

B. Tardive dyskinesia

C. Akathisia

D. Parkinsonism

348. During a telehealth session, a client who takes phenelzine for depression says they ate aged cheese and dry salami at a party an hour ago. The client now has a pounding headache at the back of the head, a stiff neck, and a racing heart. What should the social worker do **FIRST**?

A. Contact the prescriber and wait for a call back

B. Have the client lie down and recheck the symptoms in an hour

C. Arrange immediate emergency treatment for the client

349. A client stopped taking paroxetine 2 days ago after losing insurance coverage. The client now has dizziness, nausea, irritability, and brief 'electric zaps' in the head, and fears the depression is coming back. What is the social worker's **BEST** response?

A. Help the client reach the prescriber today about the stop and cost

B. Explain that these symptoms are harmless and will fade soon

C. Arrange an emergency evaluation for possible serotonin syndrome

350. An 82-year-old client has taken lorazepam every night for 10 years. After two falls this month, the client's adult child tells the home-visit social worker they plan to throw the pills away tonight. The client says they would like to stop the pills but fear they will not sleep. What is the **BEST** action?

A. Support removing the pills tonight to prevent another fall

B. Help the family make the home safer against falls

C. Arrange a prescriber review to plan a gradual taper

D. Respect the client's fear and keep the routine as is

351. In a nursing facility, a resident with major neurocognitive disorder due to Alzheimer's disease has become agitated in the evenings. The physician proposes starting risperidone. The resident's adult child, who is the health care proxy, has read that such drugs can be risky in dementia and asks the social worker what to do. What is the **MOST** helpful response?

A. Advise the proxy to refuse the drug because of its known risks

B. Help the proxy ask the physician about risks and alternatives

C. Reassure the proxy that the physician has weighed the risks

352. A client treated for panic disorder has had no panic attacks for 2 months and uses breathing and exposure skills without prompting. At the planned next-to-last session, the client says they feel nervous about ending and asks to keep meeting weekly 'just in case.' What is the social worker's **BEST** response?

A. Continue weekly sessions until the client feels ready to stop

B. Explore the client's worries and review the gains and relapse plan

C. Resume exposure work because the anxiety signals a relapse

353. A client began services 5 months ago for social anxiety, with goals of joining a weekly activity and speaking up in work meetings. At a progress review, the social worker considers several findings. Which finding **MOST** clearly indicates that the client is ready to end services?

A. Says sessions have become the best part of the week

B. Has reached both targets and copes with setbacks on their own

C. Has attended every session and completed each assignment

354. A school social worker has seen an 8-year-old for separation anxiety since the fall. The child now attends school every day, the teacher reports no morning distress for 6 weeks, and the parent handles drop-offs with the plan they practiced. The child says, 'I like coming here. Can we keep playing?' What is the **BEST** way to proceed?

A. Continue weekly sessions because the child enjoys them

B. End services at the next session to prevent dependence

C. Plan a gradual ending with fewer sessions over several weeks

355. An adult client in outpatient treatment for PTSD after a car crash agreed in the fifth session to begin processing the crash memory. The next week, the client says, 'I think I'm better now. Let's make this our last session.' Nightmares still occur several times a week, and the client still avoids driving. What should the social worker do **FIRST**?

A. Ask what has prompted the wish to stop now

B. Agree to end and explain how to return to services

C. Pause trauma processing and shift to coping skills

D. Point out that ongoing nightmares mean the goals are unmet

356. After 10 weeks of in-home family services, a parent has met the plan's goals during visits by using a reward chart and calm time-outs with a 6-year-old. The parent's daily log shows the chart is used only on visit days, and the parent calls the social worker during most conflicts between visits. The supervisor suggests closing the case. What is the **BEST** response?

A. Close services, since the plan's goals were met during visits

B. Continue services while fading support to build independent use

C. Refer the parent to a community parenting class and close the case

357. A couple began counseling 5 months ago for frequent arguments. For the past 2 months, they report using time-outs and calm check-ins on their own, and they recently settled a disagreement about moving without help. They canceled the last two sessions. When the social worker asks about the cancellations, both say things are going well and they feel less need to come. What should the social worker do **NEXT**?

A. Explore whether the cancellations reflect unspoken dissatisfaction

B. Continue biweekly sessions to guard against slipping back

C. Propose planning an ending with a final review session

Answers and rationales

II.A Assessment Concepts

1. ASM-0100 (rev 2) · Best answer: B

Two days after a tornado destroyed several homes in a small town, a social worker staffs a community shelter. Many residents report poor sleep, jumpiness at loud sounds and repeated images of the storm. Most are still caring for their families and helping with cleanup. What **MOST** likely explains these responses?

A. Posttraumatic stress disorder
B. Expected acute stress reactions
C. Acute stress disorder
D. Adjustment disorder with anxiety

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). PTSD requires symptoms lasting more than 1 month after the event, along with distress or impairment. At 2 days, no one can meet the duration requirement. It becomes a consideration if symptoms persist and impair functioning beyond a month.

B **Correct.** Most people exposed to a disaster have short-term stress reactions such as poor sleep, startle and intrusive images, and most recover without developing a disorder (Brymer et al., 2006). These residents are 2 days out and still functioning, caring for family and helping with cleanup. It beats C because acute stress disorder cannot be diagnosed until symptoms have lasted at least 3 days, and it requires significant distress or impairment (DSM-5-TR). The fitting response now is psychological first aid and monitoring, not a diagnosis.

C **Incorrect** (trap: Near-miss diagnosis). Acute stress disorder applies from 3 days to 1 month after trauma, when many symptoms cause significant distress or impairment. These residents are only 2 days out and functioning. It is worth screening for if symptoms continue and worsen.

D **Incorrect** (trap: Near-miss diagnosis). Adjustment disorder fits a reaction to a stressor that is out of proportion or impairs functioning and does not meet criteria for another disorder. Common, short-lived responses in people who are coping well do not reach that bar.

Takeaway: Early stress reactions after a disaster are expected; diagnose only when duration and impairment thresholds are met, and offer psychological first aid meanwhile.

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Acute Stress Disorder; Brymer, M., Jacobs, A., Layne, C., Pynoos, R., Ruzek, J., Steinberg, A., Vernberg, E., & Watson, P. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, introduction and overview

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2. ASM-1108 (rev 1) · Best answer: C

A client's partner died 5 months ago. The client kept working, cared for two children, and handled the funeral and finances. Last week the client's car was towed, and since then the client cannot sleep, cries at work, and says, 'I can't handle one more thing.' In Golan's crisis model, the towing is **MOST** likely which element?

A. Hazardous event
B. Vulnerable state
C. Precipitating factor
D. Active crisis state

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The hazardous event is the initial blow that upsets a person's balance, here the partner's death 5 months ago. The towing came much later and was minor on its own.

B **Incorrect** (trap: Wrong model or concept). The vulnerable state is the person's strained reaction after the hazardous event, such as the months of coping with grief. It describes the client's condition, not the towing.

C **Correct.** Golan (1978) describes a crisis as unfolding from a hazardous event, through a vulnerable state, to a precipitating factor that tips the person into active crisis. The towing is minor by itself, but it arrives while the client is still strained by the loss and turns that strain into acute disorganization. It beats A because the hazardous event is the original blow, the partner's death, which the client coped with for months. Spotting the precipitating factor explains why a small event produces a large reaction.

D **Incorrect** (trap: Wrong model or concept). Active crisis is the acute disorganization itself, such as the sleeplessness and crying at work. The towing set off that state; it is not the state.

Takeaway: In Golan's model, a minor precipitating factor can tip a person already in a vulnerable state into active crisis.

Sources: Golan, N. (1978). Treatment in Crisis Situations. Free Press, Components of the crisis situation
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3. ASM-1109 (rev 1) · Best answer: B

A teacher asks the school social worker to begin an ADHD referral for an 8-year-old. Over the past month, the student has stopped finishing work, jumps at loud sounds, and acts out hiding under a desk during play. The parent mentions a shooting on their street 6 weeks ago. What should the social worker do **FIRST**?

A. Send the ADHD rating scales the teacher requested to the parent
B. Assess the student's exposure to violence and reactions
C. Refer the student for trauma-focused therapy

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). ADHD symptoms begin before age 12, appear in more than one setting and last at least 6 months. Problems that started this month, right after a shooting, call for trauma assessment before an ADHD workup.

B **Correct.** Assessment comes before referral, and attention problems that begin after a frightening event may be trauma reactions. DSM-5-TR lists trouble concentrating and an exaggerated startle among PTSD symptoms and notes that children may replay trauma themes in play. The timing after the shooting, the startle and the hiding play point to trauma exposure, so the social worker first assesses what the student experienced and how they are reacting. It beats C because trauma-focused therapy may be the right outcome, but assessment must establish the need and guide the plan.

C **Incorrect** (trap: Right action, wrong time). Trauma-focused therapy is likely appropriate if assessment confirms trauma-related symptoms. Referring first skips the step that shows what the child needs and how severe the reactions are.

Takeaway: When new attention or behavior problems follow a frightening event, assess trauma exposure and reactions before pursuing other explanations.

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder; DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder
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4. ASM-1110 (rev 2) · Best answer: D

In a nursing facility, a resident with major neurocognitive disorder strikes staff during evening showers but calmly accepts morning sponge baths. An adult child shares that the resident was held in a crowded detention camp as a teenager, where guards forced group showers. Staff ask the social worker to request a sedative before showers. What is the **BEST** response?

A. Ask the prescriber to consider a sedative before showers
B. Help the resident talk through the camp memories
C. Record the hitting as aggression from the dementia
D. Adjust the shower routine to remove the camp reminder

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Medication may be considered if distress continues after the routine changes. Requesting it first treats a trauma reaction as a symptom to suppress and adds sedation and fall risk for an older adult.

B **Incorrect** (trap: Ignores risk). Talking through trauma can help in some therapies, but asking a resident with major neurocognitive disorder to revisit camp memories can deepen distress without removing the trigger. The immediate need is a safer bathing routine.

C **Incorrect** (trap: Near-miss diagnosis). Neurocognitive disorder can cause agitation, but this pattern is tied to evening showers and matches a known trauma history. Labeling it dementia-related aggression misses the cause and invites the wrong response.

D **Correct.** Trauma-informed care means recognizing reminders of past trauma and changing practices to avoid re-traumatizing (SAMHSA, 2014), and federal nursing facility rules require trauma-informed care that reduces triggers (42 CFR 483.25(m)). The resident fights only during showers that echo forced camp showers and accepts sponge baths, so the behavior points to a trigger, not random aggression. Changing the

routine treats the cause. It beats A because a sedative would mask distress, add sedation and fall risk, and skip a nondrug change the pattern suggests will work.

Takeaway: When an older adult's distress tracks a situation linked to past trauma, change the care routine to remove the trigger.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Four Rs: realize, recognize, respond, resist re-traumatization; Requirements for Long-Term Care Facilities (CMS), 42 CFR 483.25(m)

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5. ASM-1111 (rev 1) · Best answer: B

At a veterans clinic, a client describes following an order during deployment that led to civilian deaths. The client reports deep shame, says, 'I'm not a good person anymore,' and has pulled away from a faith community they once valued. Which concept **MOST** likely describes this reaction?

A. Survivor guilt
B. Moral injury
C. Secondary traumatic stress
D. Prolonged grief disorder

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Survivor guilt centers on having lived when comrades or others died. This client's shame is about their own action that caused deaths, which points elsewhere.

B **Correct.** Moral injury is the lasting shame, guilt and loss of meaning that can follow taking part in, failing to prevent, or witnessing acts that violate deeply held moral beliefs (Litz et al., 2009). The client's distress centers on their own role in civilian deaths, a damaged sense of being a good person, and withdrawal from a faith community. It beats A because survivor guilt concerns living when others died, while this client's anguish concerns what they did. Moral injury is not a DSM-5-TR diagnosis and can occur with or without PTSD.

C **Incorrect** (trap: Wrong model or concept). Secondary traumatic stress arises from hearing about or caring for others' trauma, as in helping professionals. The client was directly involved in the events.

D **Incorrect** (trap: Near-miss diagnosis). Prolonged grief disorder involves intense longing for, or preoccupation with, a person who died, lasting at least a year in adults. The client's distress is shame over a moral violation, not yearning for a loved one.

Takeaway: Shame and loss of meaning after acts that violate one's moral code suggest moral injury, distinct from fear-based trauma reactions.

Sources: Litz, B. T., Stein, N., Delaney, E., Lebowitz, L., Nash, W. P., Silva, C., & Maguen, S. (2009). Moral injury and moral repair in war veterans: A preliminary model and intervention strategy. Clinical Psychology Review, 29(8), 695-706, Definition of moral injury

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6. ASM-1112 (rev 2) · Best answer: A

At a disaster recovery center 7 weeks after a wildfire destroyed their home, a client reports nightmares about the flames most nights, avoids driving past the burned area, and has missed work for 3 weeks. A brief screen at the center confirms these symptoms, and the client reports no suicidal thoughts. What should the social worker do **NEXT**?

A. Refer the client for a trauma-focused mental health evaluation
B. Reassure the client that these reactions are expected after disasters
C. Rescreen the client in a month to see if symptoms fade

Why each option is right or wrong

A **Correct.** Most early stress reactions after a disaster fade, which is why psychological first aid, not therapy, is the standard first response (Brymer et al., 2006). Symptoms that last more than a month and impair functioning, though, meet the duration DSM-5-TR uses for PTSD and call for full evaluation and evidence-based treatment. At 7 weeks, with frequent nightmares, avoidance and 3 weeks of missed work, this client needs that referral. It beats B because reassurance fits the first days after a disaster, not persistent symptoms that disrupt work.

B **Incorrect** (trap: Right action, wrong time). Normalizing reactions helps in the first days after a disaster, when most distress is expected and fades. At 7 weeks, with frequent symptoms and missed work, reassurance alone leaves a likely treatable condition unaddressed.

C **Incorrect** (trap: Good but not best). Watchful waiting suits mild reactions soon after an event. These symptoms have already lasted beyond a month and disrupt work, so another month of waiting delays effective care.

Takeaway: Early disaster reactions are expected, but symptoms lasting over a month with impaired functioning warrant referral for trauma evaluation.

Sources: Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Introduction and overview; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder
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7. ASM-1113 (rev 1) · Best answer: C

In a residential treatment program, a 14-year-old with years of abuse before foster placement has sudden outbursts, freezes and stares blankly at times, and sleeps poorly. Staff notice that outbursts follow loud voices and room checks at night. The team proposes treating the youth for bipolar disorder. What is the social worker's **BEST** contribution to the team?

A. Support the plan to treat the youth for bipolar disorder
B. Recommend a new placement to reduce the outbursts
C. Point out that outbursts track reminders of earlier trauma

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Mood disorders do occur in youth with trauma histories and deserve evaluation. Agreeing now, though, sets aside the trigger-linked pattern; bipolar disorder is defined by distinct mood episodes, which reactions to specific reminders do not show.

B **Incorrect** (trap: Good but not best). Another move would add a fresh loss for a youth whose history is already marked by disruption. Placement changes tend to worsen trauma reactions; adjusting triggers in the current setting is the better course.

C **Correct.** Complex trauma from repeated childhood abuse often appears as emotional dysregulation, dissociation and reactions to reminders, which can be mistaken for a mood disorder. Here the outbursts follow loud voices and night room checks, plausible reminders of past abuse, and the blank staring suggests dissociation. Bringing this pattern to the team supports a trauma-informed plan and changes to night routines (SAMHSA, 2014). It beats A because bipolar disorder requires distinct mood episodes, not reactions to specific triggers, and a mislabel could lead to unneeded medication (DSM-5-TR).

Takeaway: In youth with abuse histories, trace outbursts to trauma reminders before accepting a mood-disorder explanation.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Four Rs: realize, recognize, respond, resist re-traumatization; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder

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8. ASM-1114 (rev 2) · Best answer: A

Eight months after a hurricane damaged their home, a family that had been coping well becomes overwhelmed. Since the storm, one parent lost a job, insurance disputes dragged on, and a child's asthma has worsened. In the Double ABCX model, these later events are part of which factor?

A. Pile-up (aA factor)
B. Family resources (bB factor)
C. Family perception (cC factor)
D. Adaptation (xX factor)

Why each option is right or wrong

A **Correct.** McCubbin and Patterson (1983) extended Hill's (1949) ABC-X model to describe what happens after the first crisis. The aA factor, pile-up, is the build-up of the original stressor's hardships, new stressors and normal transitions over time. The job loss, insurance disputes and worsening asthma came after the hurricane and stacked on top of it, which explains why a family that coped at first is now overwhelmed. It beats D because adaptation (xX) is the outcome the family reaches, not the demands acting on it. Assessing pile-up keeps attention beyond the original event.

B **Incorrect** (trap: Wrong model or concept). The bB factor covers the family's existing and newly developed resources, such as savings, support and coping skills. The listed events drain resources; they are demands, not resources.

C **Incorrect** (trap: Wrong model or concept). The cC factor is the meaning the family gives to the original stressor, the later demands and its resources. The events themselves are not perceptions.

D **Incorrect** (trap: Wrong model or concept). The xX factor is the family's adaptation over time, ranging from good adjustment to maladaptation. Being overwhelmed reflects the outcome so far; the job loss, disputes and illness are inputs that shape it.

Takeaway: The Double ABCX pile-up factor captures stressors that accumulate after a crisis, explaining delayed family breakdown.

Sources: McCubbin, H. I., & Patterson, J. M. (1983). The family stress process: The Double ABCX model of adjustment and adaptation. Marriage & Family Review, 6(1-2), 7-37, aA factor: pile-up; Hill, R. (1949). Families Under Stress. Harper & Brothers, ABC-X model

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9. ASM-1186 (rev 2) · Best answer: B

Two neighboring families lose their homes in the same river flood. One family starts rebuilding within weeks. The other family received the larger insurance payout, and relatives nearby offer it rooms, yet its daily routines collapse. The parents tell a disaster social worker, 'This proves we were always going to lose everything.' In Hill's ABC-X model, which factor **MOST** likely explains the difference?

A. The family's existing resources

B. The family's definition of the event

C. The pile-up of later stressors

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Resources (the B factor) often explain different outcomes, and they are the strongest alternative here. But the family in crisis has the larger insurance payout and relatives offering rooms, so a lack of resources cannot explain why it is the one that falls apart.

B **Correct.** Hill's (1949) ABC-X model holds that a crisis (X) results from the stressor (A), the family's resources (B) and the family's definition of the event (C). With the same flood and ample resources, the family that reads the loss as proof of lifelong failure is the one in crisis, so the C factor explains the difference. It beats A because the family in crisis has the larger payout and relatives' help, so resources do not explain its collapse.

C **Incorrect** (trap: Wrong model or concept). Pile-up of later stressors is the double A factor added in McCubbin and Patterson's (1983) Double ABCX model. It describes demands that build up over time after a crisis, and no later stressors are described here.

Takeaway: In the ABC-X model, how a family defines a stressor helps decide whether the same event becomes a crisis.

Sources: Hill, R. (1949). Families Under Stress. Harper & Brothers, ABC-X model; McCubbin, H. I., & Patterson, J. M. (1983). The family stress process: The Double ABCX model of adjustment and adaptation. Marriage & Family Review, 6(1-2), 7-37, whole article

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10. ASM-1187 (rev 1) · Best answer: B

At an outpatient intake, an American Indian client describes a heavy grief that began before any loss in their own life. The client's grandparents were sent to boarding schools that punished them for speaking their language, and the client's parent rarely spoke about that time. The client says the family 'carries a sadness nobody names.' Which concept **MOST** likely fits this presentation?

A. Complex trauma
B. Historical trauma
C. Acculturative stress
D. Vicarious trauma

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Complex trauma refers to prolonged, repeated interpersonal trauma, usually in the person's own childhood, such as ongoing abuse. The client describes no such personal history; the wound began with earlier generations.

B **Correct.** Historical trauma is cumulative emotional and psychological wounding across generations that comes from massive group trauma, such as forced removal of children to boarding schools (Brave Heart et al., 2011). Unresolved grief passed down without being spoken about, and felt before any personal loss, is a classic sign of it. It beats C because acculturative stress comes from the person's own adjustment to a new or dominant culture, not from grief inherited from a group's past.

C **Incorrect** (trap: Wrong model or concept). Acculturative stress is strain from a person's own contact with and adjustment to a dominant culture, such as language barriers or conflicting norms. The loss of language is part of this family's history, but the client describes inherited grief, not current adjustment strain.

D **Incorrect** (trap: Wrong model or concept). Vicarious trauma is the change in a helper's beliefs and emotions from repeated exposure to clients' trauma stories. It applies to professionals, not to a family's experience across generations.

Takeaway: Grief and trauma responses passed down from a group's collective trauma, such as boarding schools, describe historical trauma.

Sources: Brave Heart, M. Y. H., Chase, J., Elkins, J., & Altschul, D. B. (2011). Historical trauma among Indigenous peoples of the Americas: Concepts, research, and clinical considerations. Journal of Psychoactive Drugs, 43(4), 282-290, whole article

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11. ASM-1188 (rev 1) · Best answer: A

In an integrated primary care clinic, a social worker completes a routine intake screening with a new adult client. When the social worker reads a question about past physical or sexual violence, the client's hands begin to shake, and the client says quietly, 'I don't really talk about that.' What is the **BEST** response?

A. Reflect the distress and offer a choice to pause
B. Gently ask what happened so the screening is complete
C. Move on to the next question to finish the screening

Why each option is right or wrong

A **Correct.** Trauma-informed practice puts safety and the client's voice and choice first (SAMHSA, 2014), and screening should not push for details a client is not ready to share (TIP 57). Naming the visible distress and letting the client decide whether to pause or go on returns control at the moment it feels lost. It beats C because skipping ahead without comment avoids pressing for details but leaves the client alone with a visible reaction and gives no choice.

B **Incorrect** (trap: Ignores risk). Asking for details during a screening can retraumatize a client who has just signaled they are not ready. Screening aims to identify a need for later assessment, not to gather the trauma story.

C **Incorrect** (trap: Good but not best). Moving on avoids pressing for details, which is better than probing. But ignoring a visible stress reaction misses a chance to restore the client's sense of control and can feel dismissive. It is reasonable after the client has chosen to continue.

Takeaway: When trauma questions trigger visible distress, acknowledge it and give the client control over pacing; don't probe for details during screening.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: safety; empowerment, voice and choice; SAMHSA TIP 57 (2014), Ch. 4: Screening and Assessment
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12. ASM-1189 (rev 1) · Best answer: C

A refugee resettlement social worker completes an assessment with an adult client who fled armed conflict 8 months ago. The client reports poor sleep, irritability and occasional nightmares about the war. The client says that what keeps them awake most nights is an eviction notice, a job search with no results, and no word from a sibling still in a camp. What should the social worker do **NEXT**?

A. Refer the client for trauma-focused therapy for war memories
B. Teach sleep hygiene and relaxation skills for the insomnia
C. Help the client address housing, work and family tracing

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Trauma-focused therapy is appropriate if war-related symptoms persist or dominate once current stressors ease. Here the client links most distress to present problems, so a trauma referral first would leave the main drivers untouched.

B **Incorrect** (trap: Good but not best). Sleep skills may ease some symptoms and can be added. They treat the result rather than the cause, though; the client says worry about eviction, work and a missing sibling is what keeps them awake.

C **Correct.** Among people displaced by war, ongoing daily stressors, such as housing insecurity, unemployment and separation from family, explain much of their current distress and partly carry the effect of war exposure (Miller & Rasmussen, 2010). This client names those stressors as the main cause of sleeplessness, so addressing them comes next. It beats A because trauma-focused therapy can follow if war-related symptoms persist once the daily stressors are reduced.

Takeaway: For displaced clients, address current daily stressors such as housing, income and family separation before assuming distress is mainly past trauma.

Sources: Miller, K. E., & Rasmussen, A. (2010). War exposure, daily stressors, and mental health in conflict and post-conflict settings. Social Science & Medicine, 70(1), 7-16, whole article
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13. ASM-1119 (rev 1) · Best answer: C

A 14-year-old tells a school social worker they have felt 'huge' since joining a dance team where teammates trade diet tips. At home, a parent often criticizes their own weight, and the student follows several fitness accounts online. Which framework **BEST** accounts for these sources of the student's body dissatisfaction?

A. Objectification theory
B. Social comparison theory
C. Tripartite influence model
D. Social learning theory

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Objectification theory explains how being viewed as a body to be looked at leads people to watch their own appearance. It does not frame the combined roles of a parent, peers and media named here.

B **Incorrect** (trap: Wrong model or concept). Social comparison is part of the picture, since comparing oneself with dancers or online figures can raise dissatisfaction. It describes a mechanism, not the three sources the student names.

C **Correct.** The tripartite influence model holds that three sources, parents, peers and media, shape body image, working through internalized appearance ideals and appearance comparison (van den Berg et al., 2002). The student names all three: a parent's weight criticism, teammates' diet talk and fitness accounts online. It beats B because social comparison describes one way these sources act but does not organize the three sources. Assessing each source helps target family guidance, peer context and media use.

D **Incorrect** (trap: Wrong model or concept). Social learning explains how the student might copy a parent's dieting or weight talk. It fits the family piece but does not account for peer and media pressures together.

Takeaway: The tripartite influence model frames body image as shaped by parents, peers and media, through internalization and appearance comparison.

Sources: van den Berg, P., Thompson, J. K., Obremski-Brandon, K., & Coovert, M. (2002). The Tripartite Influence model of body image and eating disturbance: A covariance structure modeling investigation testing the mediational role of appearance comparison. Journal of Psychosomatic Research, 53(5), 1007-1020, whole article
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14. ASM-1120 (rev 1) · Best answer: C

At a pediatric clinic, a parent reports that their 11-year-old, whose weight is in the healthy range, recently began calling themselves 'fat' and comparing their stomach to a sibling's. The parent recently put the family on a strict diet and weighs the child each morning 'to keep everyone on track.' What is the social worker's **MOST** appropriate response?

A. Teach the child to challenge negative thoughts about their body
B. Have the parent arrange an eating disorder evaluation
C. Help the parent swap dieting for regular shared meals

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Cognitive skills can help a child with body dissatisfaction. Used alone, they leave the dieting and weigh-ins in place, so the child is asked to challenge messages the household repeats every morning.

B **Incorrect** (trap: Skips assessment). An eating disorder evaluation fits when there are signs such as restriction, weight loss, purging or compulsive exercise. Body talk that follows a family diet calls first for changing the home environment while watching for those signs.

C **Correct.** Family dieting and weight talk are linked to body dissatisfaction and disordered eating in youth, and pediatric guidance advises families to avoid dieting and weight talk and to focus on healthy habits and family meals (Golden et al., 2016). The child's new 'fat' talk began after the strict diet and morning weigh-ins, and the child's weight is healthy. Helping the parent shift to health-focused habits targets the main influence. It beats A because coping skills for the child leave in place the daily routine that keeps feeding the dissatisfaction.

Takeaway: When a child's body dissatisfaction follows family dieting or weight talk, guide parents toward health-focused habits rather than weight.

Sources: Golden, N. H., Schneider, M., Wood, C., & AAP Committee on Nutrition, Committee on Adolescence, Section on Obesity (2016). Preventing obesity and eating disorders in adolescents. Pediatrics, 138(3), e20161649, Recommendations for pediatricians

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15. ASM-1121 (rev 1) · Best answer: B

A school social worker notices that a 9-year-old has stopped going outside at recess and now wears long sleeves on hot days. When asked, the student says, 'I don't want to get darker. Lighter skin is prettier.' What should the social worker do **FIRST**?

A. Reassure the student that their skin color is beautiful
B. Ask where the student heard ideas about skin color
C. Plan a class lesson on diverse beauty standards

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Warm affirmation has a place, but offering it first skips learning what the child has been told and by whom. Reassurance alone may also signal that the topic is closed.

B **Correct.** Assessment comes before intervention, and self-image forms through messages from family, peers, media and community (Hepworth et al.). Colorism, the

favoring of lighter skin, is spread partly through media images and the beauty industry (Hunter, 2007). The student's avoidance of the sun and new long sleeves show the belief already shapes behavior, so the first step is learning where the message comes from: relatives, classmates, shows or social media. It beats A because reassurance before understanding the source can feel dismissive and leaves the message unaddressed.

C **Incorrect** (trap: Right action, wrong time). A lesson on diverse beauty may help later, especially if the message comes from peers at school. Planning it first assumes the source before the student has named it.

Takeaway: When a child voices negative beliefs about their appearance, first assess where the messages come from before reassuring or intervening.

Sources: Hunter, M. L. (2007). The persistent problem of colorism: Skin tone, status, and inequality. Sociology Compass, 1(1), 237-254, whole article; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: exploring the problem and its sources

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16. ASM-1192 (rev 1) · Best answer: C

Five days after surgery that created a permanent colostomy for rectal cancer, a 62-year-old hospital patient will not look at the stoma and asks nurses to handle all of its care. On the social worker's first visit, the patient says, 'I'm disgusting now. Who would want to be near me?' What is the **MOST** appropriate response?

A. Screen the patient for depression with a standard tool

B. Offer a referral to an ostomy peer visitor program

C. Invite the patient to talk about what the change means to them

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Depression screening is reasonable and may follow, especially if hopelessness persists. Opening a first visit with a questionnaire, right after a raw statement of shame, skips past the specific concern the patient just raised.

B **Incorrect** (trap: Right action, wrong time). Peer visitors who live with an ostomy can ease shame and teach practical coping. The referral fits later in the stay, once the social worker understands the patient's concerns; offered now, it can feel like a hand-off.

C **Correct.** Cancer surgery that changes appearance or body function often disrupts body image, and assessment starts with how the change affects the person (White, 2000). At a first visit, empathic exploration of the patient's words builds the relationship and shows what the stoma means for intimacy, identity and closeness (Hepworth et al.). It beats A because the patient has named a specific body-image fear; screening can follow once that meaning is understood.

Takeaway: When illness changes the body, explore what the change means to the person before screening or referring.

Sources: White, C. A. (2000). Body image dimensions and cancer: A heuristic cognitive behavioural model. Psycho-Oncology, 9(3), 183-192, whole article; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Empathic communication

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17. ASM-0003 (rev 1) · Best answer: A

A week after a parent's death, a client spends the session describing research on the stages of grief and how bereavement affects the immune system. When the social worker asks how the client is feeling, the client returns to discussing studies. Which defense mechanism is the client **MOST** likely using?

A. Intellectualization
B. Denial
C. Rationalization
D. Reaction formation

Why each option is right or wrong

A **Correct.** Intellectualization uses abstract thinking, facts and analysis to keep painful emotions at a distance (Vaillant, 1992). This client acknowledges the loss but talks about research instead of feelings, and returns to it when asked about emotions. It beats B because denial refuses to accept the reality of an event, and this client openly discusses the parent's death. Intellectualization is not pathological in itself. Recognizing it helps the social worker gently invite feelings at the client's pace.

B **Incorrect** (trap: Wrong model or concept). Denial means refusing to acknowledge an external reality, such as insisting the parent is still alive. This client accepts the death and talks about bereavement, which rules denial out.

C **Incorrect** (trap: Wrong model or concept). Rationalization offers plausible-sounding excuses to justify a behavior or decision. The client is not explaining away an action; they are using facts to avoid feelings.

D **Incorrect** (trap: Wrong model or concept). Reaction formation is acting the opposite of an unacceptable feeling, such as being overly cheerful about a resented event. Academic discussion of grief is avoidance through thinking, not opposite behavior.

Takeaway: Intellectualization is retreating into facts and analysis to avoid feelings; denial is refusing to accept that the event happened.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, intellectualization

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18. ASM-0106 (rev 1) · Best answer: D

A client who was passed over for a promotion says they feel fine about it. Over the next few sessions, the client often describes the coworker who got the job as bitter, envious and out to undermine the client. Other staff describe the coworker as friendly and supportive. Which defense mechanism **MOST** likely explains the client's view of the coworker?

A. Displacement
B. Reaction formation
C. Rationalization
D. Projection

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Displacement shifts a feeling from its real source to a safer target, such as snapping at a family member after a work setback. The client is not redirecting their own anger; they are seeing their feelings in the coworker.

B **Incorrect** (trap: Wrong model or concept). Reaction formation is acting the opposite of an unacceptable feeling, such as lavishly praising the coworker. The client criticizes the coworker instead, so the behavior is not the opposite of the hidden feeling.

C **Incorrect** (trap: Wrong model or concept). Rationalization offers a reasonable-sounding excuse, such as 'I never wanted that job anyway.' The client is not explaining away the outcome but assigning their own envy to someone else.

D **Correct.** Projection is attributing one's own unacceptable feelings to someone else (Vaillant, 1992). The client denies any hurt about the promotion yet sees the coworker as bitter and envious, the very feelings the client would be expected to have, and others see no such behavior. It beats A because displacement redirects a feeling toward a safer target, whereas projection changes who seems to own the feeling. Recognizing the pattern helps the social worker gently explore the client's disappointment.

Takeaway: Projection assigns one's own unacceptable feelings to another person; displacement redirects the feeling to a safer target.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, projection

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19. ASM-1509 (rev 2) · Best answer: A

In a residential treatment program, a roommate earns the early weekend pass that a resident had been working toward. Over the next week, the resident praises the roommate loudly in every group, buys the roommate snacks and calls the roommate 'the kindest person here.' Staff notice the praise sounds forced, and the resident tenses whenever the pass comes up. Which defense mechanism is the resident **MOST** likely using?

A. Reaction formation
B. Idealization
C. Altruism

Why each option is right or wrong

A **Correct.** Reaction formation keeps an unacceptable feeling out of awareness by displaying exaggerated feelings and behavior of the reverse kind (Vaillant, 1992). The resident lost a hoped-for pass to the roommate, then shifted to loud, forced praise and gifts and tenses when the pass comes up, signs of defended envy rather than genuine warmth. It beats B because idealization assigns exaggerated positive qualities to someone admired or needed, without the hidden resentment and overdone, forced quality that drive this display.

B **Incorrect** (trap: Wrong model or concept). Idealization attributes exaggerated perfection to another person, often someone the client depends on. The praise fits partly, but its timing after the lost pass, the tension when the pass comes up and its forced, overdone quality point to a defense against hidden resentment.

C **Incorrect** (trap: Wrong model or concept). Altruism is a mature defense that meets one's own needs through genuine, constructive service to others. The gifts look generous, but they follow a painful loss to the roommate and feel forced, which marks a defensive reversal rather than mature helping.

Takeaway: Reaction formation shows exaggerated, often forced behavior that reverses a hidden unacceptable feeling; altruism and idealization lack that hidden reversal.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, reaction formation; altruism; idealization

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20. ASM-1510 (rev 1) · Best answer: C

On an inpatient psychiatric unit, a patient tells the day nurse and the social worker that they are the only staff who truly understand. The patient describes the evening staff as heartless and incompetent. At the next team meeting, day and evening staff argue sharply over whether the patient's privileges should be reduced. Which defense is the patient **MOST** likely using?

A. Projection

B. Displacement

C. Splitting

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Projection places one's own unacceptable feelings onto someone else, such as accusing others of a hostility one feels. Calling the evening staff heartless fits partly, but the defining feature here is all-good versus all-bad sorting of staff.

B **Incorrect** (trap: Wrong model or concept). Displacement shifts feelings from a threatening target to a safer one, such as anger at family vented at staff. Nothing suggests the feelings belong to another target; the pattern is idealizing some staff and devaluing others.

C **Correct.** Splitting divides people into all good or all bad, because the person cannot hold positive and negative views of the same person at once (Kernberg, 1975). The patient casts day staff as uniquely understanding and evening staff as heartless, and

the team itself has divided along those lines, a classic inpatient sign of splitting. It beats A because projection attributes one's own unacceptable feelings to others, while this pattern is extreme good-versus-bad sorting of people. Recognizing splitting points the team toward a unified approach.

Takeaway: Splitting sorts people into all-good and all-bad camps and can divide treatment teams; it is common in borderline personality organization.

Sources: Kernberg, O. F. (1975). Borderline Conditions and Pathological Narcissism. Jason Aronson, splitting
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21. ASM-1511 (rev 2) · Best answer: D

Two days after a home invasion, a client meets with a social worker in a prosecutor's victim services office. The client gives a precise, minute-by-minute account of being tied up and threatened with a gun. Throughout, the client speaks in a flat, even tone, shows no visible emotion and asks what happens next in the case. Which defense mechanism is the client **MOST** likely using?

A. Dissociation
B. Intellectualization
C. Denial
D. Isolation of affect

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Dissociation disrupts consciousness, memory or identity, such as gaps in recall or feeling unreal. It is the closest neighbor here, but this client's detailed, continuous memory of the event points to emotion split off from an intact memory.

B **Incorrect** (trap: Wrong model or concept). Intellectualization keeps feelings at a distance through general, logical or abstract framing (Di Giuseppe & Perry, 2021). This client recounts concrete personal events in vivid detail, and asking about next steps in the case is a practical question, not abstract analysis. What stands out is the missing feeling, not theorizing.

C **Incorrect** (trap: Wrong model or concept). Denial refuses to accept that an event happened or matters. The client fully acknowledges the home invasion and asks about the case, so the reality is accepted while the feelings are kept away.

D **Correct.** Isolation of affect separates an idea or memory from the feeling attached to it, so a distressing event is recalled clearly but without emotion (Vaillant, 1992). This client gives a precise, detailed account of a terrifying assault in a flat tone, with memory intact and feeling absent. It beats A because dissociation disrupts memory, awareness or sense of self, and this client's minute-by-minute recall shows continuous memory. Isolation of affect can protect a client early after trauma, so the social worker paces emotional exploration.

Takeaway: Isolation of affect recalls distressing events clearly but without feeling; dissociation disrupts memory or awareness; intellectualization retreats into abstract analysis.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, isolation of affect; dissociation; Di Giuseppe, M., & Perry, J. C. (2021). The hierarchy of defense mechanisms: Assessing defensive functioning with the Defense Mechanisms Rating Scales Q-Sort. Frontiers in Psychology, 12, 718440, obsessional level: isolation of affects vs. intellectualization
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22. ASM-1512 (rev 1) · Best answer: A

A person chooses on purpose to set a painful worry aside until a better time. Which defense mechanism is this?

A. Suppression
B. Repression
C. Denial

Why each option is right or wrong

A **Correct.** Suppression is the conscious choice to postpone attention to a distressing thought or feeling, and Vaillant classes it among the mature defenses (Vaillant, 1992). Choosing on purpose decides it, because most defenses operate outside awareness. It beats B because repression pushes distressing material out of awareness unconsciously, so the person does not choose it and often cannot recall what was pushed away. Suppression is adaptive, for example setting worries aside to finish an exam.

B **Incorrect** (trap: Wrong model or concept). Repression excludes distressing thoughts or memories from awareness unconsciously. It is the classic contrast to suppression: the same goal, but without the deliberate choice the question describes.

C **Incorrect** (trap: Wrong model or concept). Denial refuses to acknowledge an external reality, such as a diagnosis or a loss, and operates largely outside awareness. It does not involve consciously setting a concern aside for later.

Takeaway: Suppression is conscious, mature postponement of a distressing concern; repression is unconscious exclusion from awareness.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, suppression; repression
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23. ASM-1513 (rev 2) · Best answer: B

In a fourth session, a client repeatedly accuses the social worker of being cold and judgmental. The client mocks each suggestion and cancels the next appointment at the last minute. As the session goes on, the social worker begins giving short, clipped replies and later tells a supervisor about feeling unusually irritated. Which defense is the client **MOST** likely using?

A. Projection
B. Projective identification
C. Splitting
D. Passive aggression

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Projection attributes one's own unacceptable feelings to another person. The accusations of coldness fit, which makes it the strongest competitor, but projection does not include inducing the other person to feel and act out the projected state.

B **Correct.** Projective identification attributes one's own disowned feelings to another person and then pressures that person, through the interaction, to feel or act them out (Ogden, 1979). The client accuses the social worker of coldness and provokes through mocking and late cancellation, and the worker starts responding coldly, taking on the projected role. It beats A because projection stops at attributing the feeling; the added step here is that the worker begins to experience and enact it. Supervision helps the worker understand and contain the induced feeling.

C **Incorrect** (trap: Wrong model or concept). Splitting sorts people into all-good or all-bad, and the client's picture of the worker as cold and judgmental fits partly. It does not explain the defining feature here: the worker starts to feel and act out the coldness the client attributes to them.

D **Incorrect** (trap: Wrong model or concept). Passive aggression expresses hostility indirectly, such as through lateness or quiet sabotage. The last-minute cancellation fits partly, but the open accusations and the worker's induced irritation point to a different defense.

Takeaway: Projective identification goes beyond projection: the client induces the other person to feel and act out the disowned feeling.

Sources: Ogden, T. H. (1979). On projective identification. International Journal of Psycho-Analysis, 60, 357-373, whole article

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24. ASM-1514 (rev 1) · Best answer: A

At a family service agency, the caregiver of a 6-year-old reports that the child's parent left on a military deployment 3 weeks ago. Since then, the child, who was fully toilet trained by age 3, has begun wetting the bed, asks to be fed with a spoon and talks in a baby voice. Which defense mechanism is the child **MOST** likely using?

A. Regression
B. Fixation
C. Displacement

Why each option is right or wrong

A **Correct.** Regression is a return to behavior from an earlier developmental level under stress, after the person had already moved beyond it (Freud, A., 1966). This child was toilet trained by age 3 and began bedwetting, spoon-feeding requests and a baby voice within weeks of a parent's deployment, a stress-linked return to earlier behavior. It beats B because fixation means development stalled at a stage and the behavior never stopped, while this child had clearly progressed and then went back. A medical check for new bedwetting is still sensible.

B **Incorrect** (trap: Wrong model or concept). Fixation describes getting stuck at an earlier stage so that its behaviors persist without a break. This child had mastered toileting years earlier and lost the skill under stress, which is a return rather than a stall.

C **Incorrect** (trap: Wrong model or concept). Displacement shifts feelings from their true target to a safer one, such as anger at the absent parent aimed at a sibling. The child's behavior is a return to earlier habits, not redirected feelings.

Takeaway: Regression is a stress-driven return to earlier behavior after progress; fixation is development that stalled and never moved on.

Sources: Freud, A. (1966). The Ego and the Mechanisms of Defense (rev. ed.). International Universities Press, regression

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25. ASM-1626 (rev 2) · Best answer: B

A hospital patient awaiting surgery describes an argument with an adult sibling. In anger, the patient told the sibling, 'I hope you get what's coming to you.' Since then, whenever the sibling is mentioned, the patient knocks three times on the bed rail, repeats a set phrase and asks staff to send the sibling a small gift. The patient says, 'I keep worrying my words could make something bad happen.' Which defense mechanism is the patient **MOST** likely using?

A. Reaction formation
B. Undoing
C. Isolation of affect
D. Displacement

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Reaction formation appears as a lasting attitude opposite to the hidden impulse, such as excessive sweetness toward someone the person resents. The gifts might suggest it, but the ritual knocking and phrase target one remark, which points to cancelling an act rather than reversing a feeling.

B **Correct.** Undoing is an attempt to cancel, or symbolically reverse, an unacceptable thought or act through a later act. The patient's ritual knocking, set phrase and gifts all follow the hostile remark and aim to neutralize it, as the patient's fear that the words could cause harm shows. It beats A because reaction formation is a lasting stance opposite to a hidden feeling, such as constant warmth toward a resented person; here the behavior is tied to cancelling one remark and repeats as a ritual (Freud, A., 1936).

C **Incorrect** (trap: Wrong model or concept). Isolation of affect separates a thought from its feeling, so a person can describe something painful without emotion. This patient is plainly troubled by the remark and acts to neutralize it, rather than discussing it in a detached way.

D **Incorrect** (trap: Wrong model or concept). Displacement shifts a feeling onto a safer target, such as snapping at a nurse after a fight with the sibling. The patient's attention stays on the sibling, and the behavior aims to cancel the hostility, not redirect it.

Takeaway: Undoing uses a later act or ritual to symbolically cancel a hostile thought or deed; reaction formation is a lasting opposite attitude.

Sources: Freud, A. (1936/1966). The Ego and the Mechanisms of Defense (rev. ed.). International Universities Press, Ch. 4, The mechanisms of defense (undoing, isolation, reaction formation)
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26. ASM-1632 (rev 2) · Best answer: A

A 10-year-old in a residential treatment program was removed from a home where a caregiver often screamed threats and insults. Staff report that the child now corners younger residents and shouts at them using the caregiver's exact words, tone and gestures. When staff step in, the child laughs and says, 'Nobody can push me around now.' Which defense mechanism is the child **MOST** likely using?

A. Identification with the aggressor
B. Displacement
C. Acting out
D. Projection

Why each option is right or wrong

A **Correct.** Identification with the aggressor, described by Anna Freud, is coping with fear of a threatening person by taking on that person's traits and behavior, turning from passive victim into active aggressor. The child copies the caregiver's words, tone and gestures and says no one can push them around now, which shows the role reversal. It beats B because displacement only redirects feelings to a safer target; it does not explain why the child reproduces the caregiver's own behavior so exactly (Freud, A., 1936).

B **Incorrect** (trap: Wrong model or concept). Displacement shifts feelings from a dangerous target to a safer one, and younger residents are safer targets. But displacement does not explain the child copying the caregiver's exact words and manner, which shows taking on the aggressor's role.

C **Incorrect** (trap: Wrong model or concept). Acting out expresses an impulse through behavior instead of words, and the child's aggression is behavioral. The term names the outlet, not the defining feature here: the child has adopted the caregiver's own role and style.

D **Incorrect** (trap: Wrong model or concept). Projection attributes one's own unacceptable feelings to others, such as accusing younger residents of being hostile. The child is not assigning feelings to them but openly enacting the caregiver's aggression.

Takeaway: In identification with the aggressor, a frightened person adopts the threatening person's behavior, turning helplessness into power over others.

Sources: Freud, A. (1936/1966). The Ego and the Mechanisms of Defense (rev. ed.). International Universities Press, Ch. 9, Identification with the aggressor

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27. ASM-1636 (rev 2) · Best answer: B

A 20-year-old college student lost a soccer scholarship after a knee injury ended the student's playing career. Within a semester, the student joined the debate team and now prepares 30 hours a week. In counseling, the student says, 'I'm not the strong one anymore, so I have to be the smartest one in every room.' Which defense mechanism does this **MOST** likely reflect?

A. Sublimation
B. Compensation
C. Rationalization

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Sublimation redirects an unacceptable impulse, such as aggression or sexual energy, into a valued outlet, like turning rage into competitive boxing. The student's drive comes from feeling diminished after the injury, not from an impulse that needs a safer outlet.

B **Correct.** Compensation is making up for a real or perceived weakness by striving to excel in another area, an idea rooted in Adler's work on feelings of inferiority. The student's own words tie the intense debate effort to no longer being 'the strong one,' which shows an effort to offset a felt deficit. It beats A because sublimation channels an unacceptable impulse, such as aggression, into a socially valued activity; nothing here suggests a forbidden impulse being redirected, only a loss being made up for (Adler, 1927).

C **Incorrect** (trap: Wrong model or concept). Rationalization offers a plausible excuse for behavior or a disappointment, such as claiming soccer never mattered. The student openly names the loss and its effect rather than explaining it away.

Takeaway: Compensation offsets a felt weakness by excelling elsewhere; sublimation channels an unacceptable impulse into a socially valued activity.

Sources: Adler, A. (1927). Understanding Human Nature. Greenberg, feelings of inferiority and compensation
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28. ASM-1642 (rev 1) · Best answer: B

In an intensive outpatient program, a client describes drinking at a coworker's party after 40 days without alcohol. The client says, 'Everyone was toasting, and refusing would have been rude. Besides, after the week I had, anyone would have had a drink.' Which defense mechanism is the client **MOST** likely using?

A. Denial
B. Rationalization
C. Intellectualization

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Denial is refusing to acknowledge a painful reality, such as insisting no drinking happened or that alcohol is not a problem at all. This client openly reports the drink and is explaining it, not denying it.

B **Correct.** Rationalization offers plausible, socially acceptable reasons for a behavior so the person can avoid facing its real cause or meaning. The client admits the drink but explains it as politeness and an understandable reaction to a hard week. It beats A because denial would mean refusing to acknowledge the drinking or its reality, while this client reports it openly and only excuses it (Kaplan & Sadock's Synopsis, 12th ed.).

C **Incorrect** (trap: Wrong model or concept). Intellectualization uses abstract, detached analysis to avoid feelings, such as discussing relapse statistics instead of personal regret. The client gives everyday excuses tied to the situation, not abstract analysis.

Takeaway: Rationalization gives plausible excuses for a behavior the person admits; denial refuses to acknowledge the reality itself.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Theories of personality and psychopathology: defense mechanisms
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29. ASM-1102 (rev 1) · Best answer: C

According to U.S. law enforcement data, which statement describes most people who sexually assault children?

A. They are strangers to the child
B. They first met the child online
C. They know the child or family
D. They are younger than 18

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Stranger abuse draws the most public attention, but strangers made up only about 7% of offenders against juveniles in law enforcement data. Focusing on strangers can lead adults to miss warning signs in people the child trusts.

B **Incorrect** (trap: Wrong model or concept). Online contact is a real route to exploitation, but it describes a minority of cases. Most offenders already have offline access to the child through family, school, sports or neighborhood ties.

C **Correct.** Most people who sexually abuse children are relatives or acquaintances, which is one reason grooming of the child and family often goes unnoticed. In Snyder's (2000) analysis of law enforcement data, about a third of offenders against juveniles were family members, nearly 6 in 10 were acquaintances, and 7% were strangers. It beats D because juvenile offenders are a sizable minority, about a third of offenders against juvenile victims, not most. Assessment should attend to trusted adults, not only strangers.

D **Incorrect** (trap: Wrong model or concept). Juveniles do commit a meaningful share of sexual offenses against children, about a third of offenders against juvenile victims in Snyder's data. That is a substantial minority, not most.

Takeaway: Most child sexual abuse is committed by someone the child or family knows and trusts, not by a stranger.

Sources: Snyder, H. N. (2000). Sexual Assault of Young Children as Reported to Law Enforcement: Victim, Incident, and Offender Characteristics (NCJ 182990). Bureau of Justice Statistics, Victim-offender relationship
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30. ASM-1103 (rev 1) · Best answer: A

At intake for a court-ordered partner violence program, a participant says, 'I lose my temper and can't stop myself.' The participant describes staying calm when a manager yells at them in front of coworkers. Twice, the participant smashed the partner's phone after the partner went out with friends. Which explanation is **MOST** likely supported by this pattern?

A. Purposeful control of the partner
B. Poor impulse control across settings
C. Reaction to the partner's provocations
D. Job stress displaced onto home

Why each option is right or wrong

A **Correct.** The Duluth model holds that partner violence is mainly a tactic of power and control, not a loss of control (Pence & Paymar, 1993). The decisive detail is selectivity: the participant stays calm when a manager humiliates them in public but destroys the partner's phone after the partner sees friends, an act that limits the partner's contacts. It beats B because a true impulse-control problem would show up across settings, including under workplace provocation. Claims of 'losing it' are a common way people who abuse partners minimize choice.

B **Incorrect** (trap: Wrong model or concept). Some people do have broad anger problems. This participant, though, holds back with a manager who yells in public and targets only the partner, which points away from a general impulse-control deficit.

C **Incorrect** (trap: Wrong model or concept). Blaming the partner's behavior is a common account given by people who abuse, not an explanation the facts support. Going out with friends is ordinary; smashing the phone in response is an effort to restrict the partner.

D **Incorrect** (trap: Wrong model or concept). Displacement would predict anger spilling onto whoever is near after hard days at work. Here the violence follows the partner's independence, which fits control of the partner rather than displaced job stress.

Takeaway: Violence aimed selectively at a partner, with restraint elsewhere, signals purposeful control rather than an anger or impulse problem.

Sources: Pence, E., & Paymar, M. (1993). Education Groups for Men Who Batter: The Duluth Model. Springer, Power and control wheel

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31. ASM-1104 (rev 2) · Best answer: D

A 3-year-old has had 8 hospital stays for breathing pauses that only the parent has witnessed. The parent, a former nursing assistant, is warm with staff and presses for more invasive tests. During one monitored stay, the parent was away for 4 days and no pauses occurred. Soon after the parent returned, a pause requiring resuscitation happened while the parent was alone with the child. Which explanation is **MOST** likely?

A. An undiagnosed breathing disorder
B. Parental anxiety causing overreporting
C. Breath-holding spells triggered by distress
D. Illness induced by the caregiver

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). A medical cause is reasonable to pursue early in a workup. It would not explain why events stopped during 4 monitored days without the parent and returned when the parent was alone with the child.

B **Incorrect** (trap: Near-miss diagnosis). An anxious parent may overreport or misread normal breathing. That explanation fails here because a monitored event required resuscitation, which overreporting cannot create.

C **Incorrect** (trap: Near-miss diagnosis). Breath-holding spells in toddlers follow crying, pain or frustration and are seen by many different caregivers. They would not cluster around one caregiver or vanish when that caregiver is away.

D **Correct.** Caregiver-fabricated illness is maltreatment in which a caregiver falsifies or causes a child's symptoms; it should be considered when illness is recurrent, unexplained and inconsistent with testing (Flaherty et al., 2013). The decisive details are timing and observation: no events across 4 monitored days without the parent, then a severe event while the parent was alone with the child. A medically knowledgeable, highly engaged caregiver who seeks more procedures fits the pattern. It beats B

because overreporting cannot produce a confirmed event needing resuscitation. In DSM-5-TR, the caregiver receives the factitious disorder diagnosis.

Takeaway: Recurrent unexplained illness that occurs with one caregiver and stops in their absence suggests caregiver-fabricated or induced illness.

Sources: Flaherty, E. G., MacMillan, H. L., & AAP Committee on Child Abuse and Neglect (2013). Caregiver-fabricated illness in a child: A manifestation of child maltreatment. Pediatrics, 132(3), 590-597, Recognition and assessment; DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Factitious Disorder
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32. ASM-1183 (rev 2) · Best answer: A

A parent asks a youth program's social worker about a volunteer coach who has grown close to the parent's 12-year-old. The coach praises the child in front of the team, offers rides home after practice, and recently gave the child a phone, saying the gift should stay 'just between us.' Which behavior is the **MOST** concerning sign of grooming?

A. Asking that the gift be kept from others
B. Praising the child in front of the team
C. Offering the child rides home after practice

Why each option is right or wrong

A **Correct.** Grooming builds trust and special status with a child while creating secrecy and isolation that make later abuse less likely to be disclosed. Craven et al. (2006) name keeping the child silent to avoid disclosure as a core goal of grooming, and note that many grooming behaviors look like ordinary caring. An adult asking a child to hide a gift, especially a private phone, has no innocent coaching purpose. It beats C because rides home can be ordinary help and become concerning mainly when combined with secrecy.

B **Incorrect** (trap: Near-miss diagnosis). Public praise is part of normal coaching and happens in front of others. It can feed a sense of being special, but on its own it is the least specific of these behaviors.

C **Incorrect** (trap: Near-miss diagnosis). Rides home can create time alone with a child, and offenders do seek access this way. Many coaches give rides for innocent reasons, though, so this is less specific than an explicit request for secrecy.

Takeaway: Grooming often looks like ordinary kindness; an adult's request that a child keep contact or gifts secret is a strong warning sign.

Sources: Craven, S., Brown, S., & Gilchrist, E. (2006). Sexual grooming of children: Review of literature and theoretical considerations. Journal of Sexual Aggression, 12(3), 287-299, whole article
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33. ASM-1542 (rev 1) · Best answer: C

A 2-year-old has been in a children's hospital for 3 weeks, and the parents can visit only on weekends. For the first few days, the child screamed and searched the doorway for the parents. Now the child lies quietly, plays little, eats poorly and rarely cries. Nurses describe the child as 'settled.' Which phase of separation response is the child **MOST** likely in?

A. Protest
B. Detachment
C. Despair
D. Rapprochement

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Protest is the first phase: crying, screaming and searching for the absent parent. The child showed this in the first days but has since become quiet and withdrawn, a sign of the next phase.

B **Incorrect** (trap: Wrong model or concept). Detachment is the later phase, when the child seems to recover, engages with staff and may treat the parents with indifference when they return. This child is withdrawn and apathetic rather than outwardly sociable.

C **Correct.** Robertson and Bowlby described young children's responses to prolonged separation from caregivers, as in hospitals, in three phases: protest, despair and detachment (Bowlby, 1973). In despair, the child becomes withdrawn, inactive and apathetic, a quiet state that staff often mistake for settling in. This child has moved from screaming and searching to lying quietly and eating poorly. It beats B because detachment comes later, when the child grows more sociable with staff and may greet returning parents with indifference.

D **Incorrect** (trap: Wrong model or concept). Rapprochement is a Mahler subphase of typical development, around 16 to 24 months, in which toddlers alternate between seeking and pushing away the caregiver. It describes normal separation-individuation, not a reaction to prolonged separation.

Takeaway: Young children's responses to prolonged separation run from protest to despair to detachment; quiet despair is easily mistaken for adjustment.

Sources: Bowlby, J. (1973). Attachment and Loss, Vol. 2: Separation: Anxiety and Anger. Basic Books, Ch. 1: protest, despair and detachment

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34. ASM-1543 (rev 2) · Best answer: A

A 7-year-old in foster care has weekly visits with a birth parent, and reunification is the case goal. After each visit, the child cries, wets the bed and pushes the foster parents away for a day or two. The foster parents ask to reduce the visits. Visit notes from the past 2 months describe a warm, appropriate parent and no safety concerns. What should the social worker do **NEXT**?

A. Help the foster parents plan supportive routines around visits
B. Request fewer visits until the foster child's behavior settles
C. Ask the birth parent to avoid talk of reunification at visits

Why each option is right or wrong

A **Correct.** Distress before or after family time is a common response to separation and loss, not by itself a sign that contact is harmful, and federal guidance warns that cutting visits because of a child's negative reactions can deepen trauma (Children's Bureau, 2020). These visits are safe, and regular contact supports reunification. Coaching the foster parents to expect these reactions and build calming routines around visits helps the child manage grief and divided loyalties. It beats B because fewer visits would ease the foster home's strain while weakening the child's bond with the parent and the reunification plan.

B **Incorrect** (trap: Skips assessment). Fewer visits would ease the foster family's strain, but the child's reactions reflect grief and divided loyalties rather than unsafe contact. Reducing family time on this basis can intensify the child's sense of loss and slow reunification.

C **Incorrect** (trap: Good but not best). Coaching the birth parent could help if visit content were upsetting the child, but the stem describes appropriate parent behavior. Limiting talk of reunification addresses an assumed cause instead of helping the child cope with repeated separations.

Takeaway: A child's distress around safe family visits usually reflects grief and loyalty conflict; support the child and caregivers rather than reducing contact.

Sources: Children's Bureau, U.S. Department of Health and Human Services (2020). Family Time and Visitation for Children and Youth in Out-of-Home Care, ACYF-CB-IM-20-02: children's reactions to family time
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35. ASM-1544 (rev 2) · Best answer: B

A 51-year-old client was released 3 weeks ago after 14 years in prison and now lives with a sibling. The sibling says the client waits to be told when to eat or shower, freezes in the grocery store when faced with choices, and sits facing the door in every room. The client says they are glad to be home. At last week's visit, both learned that these are common adaptations to long incarceration. What should the social worker do **NEXT**?

A. Ask the sibling to set the daily schedule and make choices for now
B. Help the client practice making small daily choices with sibling support
C. Refer the client for an evaluation for major depressive disorder

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). A predictable routine can ease the first weeks home, but having the sibling make the client's choices recreates prison-style control. It maintains the dependence on outside structure that the client needs to unlearn.

B **Correct.** Long incarceration can produce institutionalization: dependence on outside structure, a weakened capacity to make decisions and start activities, and hypervigilance (Haney, 2001). These are learned adaptations, and Haney calls for transitional help to readapt to life outside. After normalizing the reactions, the next step is graded practice in everyday decisions, such as choosing a meal, with the sibling encouraging rather than deciding. It beats A because taking over the client's choices would recreate the external control that maintains the dependence.

C **Incorrect** (trap: Skips assessment). Withdrawal after release can resemble depression and should be assessed if mood symptoms appear. Here the client reports being glad to be home, and the specific pattern of waiting for permission and freezing at choices points to institutional adaptation.

Takeaway: Institutionalization after long incarceration involves dependence on external structure; rebuild decision-making through graded everyday choices, not continued outside control.

Sources: Haney, C. (2001). The psychological impact of incarceration: Implications for post-prison adjustment. U.S. Department of Health and Human Services, ASPE, From Prison to Home conference, institutionalization effects; transitional services

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36. ASM-1651 (rev 1) · Best answer: A

A 9-year-old has lived with a foster family for 4 months and has weekly visits with their birth parent, who is working toward reunification. The foster parent reports that the child refuses outings and recently said, 'If I have fun here, it means I don't love my real family.' The birth parent has been cooperative with the agency. What would be **MOST** helpful?

A. Help the birth parent tell the child it is fine to have fun there

B. Tell the child the birth parent would want them to have fun

C. Refer the child for an evaluation of a possible attachment disorder

Why each option is right or wrong

A **Correct.** Children in care often feel that enjoying a new home betrays their parents. This loyalty conflict can block adjustment, and the message that frees the child carries most weight when it comes from the person the child fears hurting. Because this birth parent is cooperative, helping them tell the child directly that having fun with the foster family is fine can release the bind. Fahlberg notes that attachments transfer more readily with the cooperation of earlier parenting figures. It beats B because a secondhand message from the social worker cannot answer the child's fear about the parent's real feelings.

B **Incorrect** (trap: Good but not best). Passing along what the parent would probably want is well meant, but it puts words in the parent's mouth without checking. The

child's worry is about the parent's real feelings, so a secondhand message from the social worker carries little weight.

C **Incorrect** (trap: Skips assessment). The child is describing a common loyalty bind, not a pattern such as indiscriminate friendliness or failure to seek comfort. A referral would pathologize an expected reaction to placement; evaluation fits if broader attachment problems appear.

Takeaway: Loyalty conflicts in placement ease most when birth parents themselves tell the child it is all right to enjoy the new home.

Sources: Fahlberg, V. I. (1991). A Child's Journey Through Placement. Perspectives Press, loyalty conflicts; transferring attachments with the cooperation of prior caregivers
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37. ASM-1652 (rev 1) · Best answer: B

A 15-year-old has spent 10 months in a residential treatment program and has met most treatment goals. Discharge home is set for 3 weeks from now, and the youth has said little about it. In the past week, the youth has broken curfew twice and picked fights with peers, behaviors not seen in months. Staff ask the social worker whether discharge should be delayed. What is the **BEST** course of action?

A. Postpone discharge until the behavior settles for several weeks
B. Help the youth talk about leaving while keeping the planned date
C. Keep the planned date and review program rules with the youth

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Delaying discharge can fit when new behavior shows real danger or lost skills. Here the timing points to a reaction to the coming separation. Postponing may reward the behavior and prolong a restrictive placement the youth no longer needs.

B **Correct.** Old problems often resurface as an important ending approaches; Hepworth et al. describe clients reintroducing earlier difficulties as termination nears. A youth who has met most goals, says little about leaving, and acts out only as discharge nears is most likely reacting to losing the program's relationships and structure. Opening that conversation, and using the remaining weeks to prepare for the move, protects the youth's gains. It beats A because delaying discharge treats an expected transition reaction as treatment failure and can teach the youth that acting out postpones hard endings.

C **Incorrect** (trap: Good but not best). Holding to the date is reasonable, but answering the behavior with a rules review treats it as simple misconduct. It misses the likely meaning, the youth's unspoken feelings about leaving, so the stress behind the behavior goes unaddressed.

Takeaway: Regression as discharge nears often reflects anxiety about the ending; address the transition directly before treating it as treatment failure.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Termination: clients' reactions, including return of earlier problems
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38. ASM-1521 (rev 2) · Best answer: B

At a memory clinic, the adult child who cares full-time for a parent with major neurocognitive disorder meets with the social worker. The caregiver quit a job last month, sleeps about 4 hours a night because the parent wanders, and has begun snapping at their own teenagers. Siblings out of state send money but no hands-on help. Which response is **BEST**?

A. Give the caregiver a standardized caregiver burden questionnaire
B. Connect the caregiver with respite care and a day program
C. Refer the caregiver to individual psychotherapy for stress

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A standardized burden measure is useful when the level of strain is unclear or to track change over time. Here the job loss, sleep deprivation and family conflict already show heavy burden, so measuring it first delays relief.

B **Correct.** Caregiving for a parent with neurocognitive disorder strains the whole family system, and practical relief such as respite care is a core caregiver support (42 U.S.C. 3030s-1). This caregiver has lost a job and sleep and is straining relationships with their own children, signs of severe burden already evident (Zarit et al., 1980). It beats A because a burden questionnaire would mainly confirm what the stem shows, while respite care and a day program for the parent reduce the overload directly. Engaging siblings and planning for wandering safety can follow.

C **Incorrect** (trap: Hands off too soon). Counseling can help caregivers cope and may be added later. Referring out first treats the strain as an individual mental health problem when the main driver is an unrelieved caregiving load that respite can reduce.

Takeaway: When caregiver burden is already evident, prioritize concrete relief such as respite care and day programs over further measurement or referral out.

Sources: Zarit, S. H., Reever, K. E., & Bach-Peterson, J. (1980). Relatives of the impaired elderly: Correlates of feelings of burden. The Gerontologist, 20(6), 649-655, whole article; Older Americans Act, National Family Caregiver Support Program, 42 U.S.C. 3030s-1(b)
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39. ASM-1528 (rev 1) · Best answer: D

The spouse of a nursing home resident with advanced major neurocognitive disorder visits every day. The resident no longer recognizes the spouse. The spouse tells the social worker, 'I don't know whether to think of myself as married anymore.' The spouse has put off decisions about selling their house and feels guilty about accepting a friend's dinner invitation. Which concept **MOST** likely describes the spouse's experience?

A. Anticipatory grief
B. Disenfranchised grief
C. Caregiver burnout
D. Ambiguous loss

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Anticipatory grief is mourning before an expected death, and dementia caregivers often feel it. It fits part of this picture, but the spouse's central struggle is not knowing whether they are still married, a sign of a loss that has no clear ending.

B **Incorrect** (trap: Wrong model or concept). Disenfranchised grief is grief that others do not recognize or support, such as grief for an ex-partner. Nothing here shows the spouse's loss being dismissed by others; the difficulty comes from the nature of the loss itself.

C **Incorrect** (trap: Wrong model or concept). Burnout is exhaustion from prolonged caregiving demands. This spouse visits daily but describes confusion about their marital status and frozen decisions, not depletion from care tasks, which nursing home staff now provide.

D **Correct.** Ambiguous loss is a loss without clarity or closure. In one type, a person is physically present but psychologically absent, as in advanced dementia (Boss, 1999). The resident is alive but no longer knows the spouse, so the spouse cannot tell whether the marriage continues, puts off decisions about the house and feels guilty about living a normal life. It beats A because anticipatory grief is mourning a death that is expected; this spouse's distress centers on the unclear status of a person who is still here.

Takeaway: Ambiguous loss occurs when a loved one is physically present but psychologically gone, as in dementia, leaving no clear ending to grieve.

Sources: Boss, P. (1999). Ambiguous Loss: Learning to Live with Unresolved Grief. Harvard University Press, loss when a person is physically present but psychologically absent
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40. ASM-1529 (rev 2) · Best answer: A

A school social worker meets with a 14-year-old who often falls asleep in class and has fallen behind on assignments. The student explains that a parent with multiple sclerosis needs help getting out of bed, bathing and eating, and the other parent works nights. The student says, 'It's my job. I don't mind.' Which action is **BEST**?

A. Arrange in-home personal care with the parents
B. Arrange a school plan with the parents for extended deadlines
C. Refer the student to counseling for caregiver stress

Why each option is right or wrong

A **Correct.** Young caregivers are common, as many as 1.4 million U.S. children ages 8 to 18 in one national survey, and are often overlooked (National Alliance for Caregiving & United Hospital Fund, 2005). This student's sleep loss and missed work come from a care load the family cannot meet alone, so the most effective step is helping the parents arrange in-home care for the parent with MS. It beats B because school accommodations ease the symptoms but leave a 14-year-old providing bathing and personal care that adult services should cover.

B **Incorrect** (trap: Good but not best). Extended deadlines and similar school supports may help and can be part of the plan. Alone, they adjust school around an excessive care load instead of reducing it, so the student's sleep and schoolwork keep suffering.

C **Incorrect** (trap: Hands off too soon). Counseling may help the student cope, but it treats the strain as the student's problem to manage. The source is an unmet care need at home that in-home services can address; counseling can be added if stress persists.

Takeaway: When a child carries heavy caregiving duties, reduce the load by arranging adult care services for the ill family member, not only supporting the child.

Sources: National Alliance for Caregiving & United Hospital Fund (2005). Young Caregivers in the U.S.: Findings from a National Survey, prevalence of caregivers ages 8 to 18
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41. ASM-1530 (rev 1) · Best answer: B

Parents aged 79 and 81 have cared at home for decades for their 50-year-old adult child, who has schizophrenia. At a community mental health visit, the parents say they are slowing down and worry about who will help their child after they die. The adult child is stable, takes medication and attends a day program. Which response is **BEST**?

A. Help the parents draft a plan to share with their adult child later
B. Start planning meetings with the adult child as a core participant
C. Teach the parents to recognize early warning signs of relapse

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). The parents' initiative is welcome, but drafting the plan without the adult child decides major questions about housing and support for a stable adult who can take part. Plans made this way may not fit the person's preferences and are harder to carry out.

B **Correct.** Future care planning should start while aging parents are still able to help, and the person with serious mental illness should be a core participant whose preferences shape the plan (Sivakumar et al., 2021). Including the adult child also honors self-determination (NASW Code 1.02). It beats A because a plan drafted by the parents alone may not match the adult child's wishes or abilities, and parents often underestimate what their child can learn. Housing, finances, legal arrangements and the roles of siblings or friends can then be worked out together.

C **Incorrect** (trap: Good but not best). Relapse education helps families, but after decades of caregiving these parents are raising a different need: who will provide support when they no longer can. Teaching warning signs does not answer their stated concern.

Takeaway: When aging parents care for an adult child with serious mental illness, begin future care planning early and include the adult child as a core participant.

Sources: Sivakumar, T., et al. (2021). Future care planning: Concerns of elderly parents caring for a person with serious mental illness. Psychiatric Services, Open Forum, doi:10.1176/appi.ps.201900267; NASW Code of Ethics (2021), 1.02

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42. ASM-1627 (rev 3) · Best answer: A

A home health social worker meets with an adult child who cares full time for a 78-year-old parent with advanced Parkinson's disease. The caregiver reports back pain, poor sleep and missed work. When the social worker mentions an adult day program, the caregiver shakes their head and says, 'That's not how our family does things.' What should the social worker do **NEXT**?

A. Ask what kinds of help would feel acceptable to the caregiver

B. Arrange in-home aide visits in place of the day program

C. Urge the caregiver to try the day program for a week

Why each option is right or wrong

A **Correct.** The caregiver's remark signals a family or cultural norm about caregiving, but its meaning is still unclear: it may concern strangers, shame, duty or a past bad experience. Asking what help would feel acceptable explores that meaning and invites options the family can accept, such as relatives sharing shifts or help at home. Cultural values shape caregiving largely through how families use support (Knight & Sayegh, 2010; NASW Family Caregivers Standard 4). It beats B because in-home aides still bring outsiders into care, and the social worker does not yet know whether that would be acceptable.

B **Incorrect** (trap: Skips assessment). In-home help suits many caregivers and may fit this family. Arranging it now assumes the objection is only about leaving home; if the concern is outsiders providing care, aides will be refused as well.

C **Incorrect** (trap: Overrides the client's choice). Respite would ease the strain, but pressing a service the caregiver has just declined overrides the caregiver's stated values and may end the conversation. The program can be revisited later if the family's view changes.

Takeaway: When a caregiver declines help on family or cultural grounds, explore what support would fit their values before proposing alternatives.

Sources: Knight, B. G., & Sayegh, P. (2010). Cultural values and caregiving: The updated sociocultural stress and coping model. The Journals of Gerontology, Series B, 65B(1), 5-13, whole article; NASW Standards for Social Work Practice with Family Caregivers of Older Adults (2010), Standard 4: Cultural and Linguistic Competence
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43. ASM-1637 (rev 1) · Best answer: C

Parents of a 6-year-old in treatment for leukemia ask the hospital social worker for help with their 10-year-old, who has started skipping homework and slamming doors. Since the diagnosis, the parents have spent most nights at the hospital, and relatives have cared for the 10-year-old. The parents want a reward chart for school behavior. What should the social worker do **FIRST**?

A. Help the parents set up a reward chart for finishing homework
B. Refer the 10-year-old for an evaluation for ADHD
C. Hear from the 10-year-old how home life has changed

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A reward chart may help later, once the reasons for the behavior are understood. Starting with it treats the homework and door slamming as the problem and skips the family change that likely explains them.

B **Incorrect** (trap: Skips assessment). An ADHD evaluation would be considered if attention problems were longstanding and present across settings. These changes began after the sibling's diagnosis and the family's upheaval, so a new disorder is not the first explanation to pursue.

C **Correct.** A child's serious illness reorganizes the whole family. Parents' attention and routines shift to treatment, and well siblings often report reduced parental attention, distress and school problems in the first years after a diagnosis (Alderfer et al., 2010). The 10-year-old's behavior began after the diagnosis and the parents' absence, so the first step is to hear from the sibling about what has changed. It beats A because a reward chart targets the behavior without knowing what drives it and may miss the sibling's distress and need for connection.

Takeaway: When one child is seriously ill, assess how the family's reorganization affects well siblings before targeting their behavior.

Sources: Alderfer, M. A., Long, K. A., Lown, E. A., Marsland, A. L., Ostrowski, N. L., Hock, J. M., & Ewing, L. J. (2010). Psychosocial adjustment of siblings of children with cancer: A systematic review. Psycho-Oncology, 19(8), 789-805, whole article
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44. ASM-1105 (rev 2) · Best answer: B

At a community health clinic, a client who works at a nail salon seeks care for back pain. Privately, the client says the owner keeps the client's passport, deducts a growing 'travel debt' from each paycheck, and warns that complaining will bring immigration agents. The client wants to keep working for now but asks what help exists. What is the social worker's **BEST** response?

A. Report the salon owner's conduct to local police today
B. With consent, link the client to anti-trafficking services
C. Explain how the client can apply for a T visa

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Law enforcement may become involved later, and calling is right if there is imminent danger or the client chooses it. Reporting today overrides an adult's stated wishes and may deepen the client's fear of deportation.

B **Correct.** A held passport, a debt that keeps growing, and threats about immigration enforcement are markers of labor trafficking; federal law names debt bondage as a form of labor trafficking and counts threatened abuse of the legal process as coercion (22 U.S.C. 7102). The client is an adult who wants to keep working for now but asked about help, so a consent-based link to specialized anti-trafficking services respects self-determination (NASW Code 1.02) and opens legal, safety and immigration options. It beats A because a police report against the client's wishes can feed the very fear the owner exploits.

C **Incorrect** (trap: Outside the social work role). Immigration relief for trafficking survivors exists, but advising on eligibility and filing is legal work for an immigration attorney or accredited representative. The social worker's role is to connect the client to that expertise.

Takeaway: Adult trafficking indicators call for a consent-based link to specialized services, not overriding the client or giving legal advice.

Sources: Trafficking Victims Protection Act, 22 U.S.C. 7102(3), (7), (11); NASW Code of Ethics (2021), 1.02
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45. ASM-1106 (rev 1) · Best answer: A

In a video session, a 52-year-old client mentions plans to wire $6,000 to an online partner they have never met, who needs money for a 'customs fee' abroad. The client has already sent $9,000 in gift cards for earlier emergencies. The client otherwise manages money well and says, 'It's my choice, and I trust them.' What response is **MOST** helpful?

A. Describe common scam signs and offer to check the story together
B. Accept the client's choice and return to the session agenda
C. Urge the client to end contact with the online partner

Why each option is right or wrong

A **Correct.** Self-determination is strongest when the client decides with full information (NASW Code 1.02). A partner never met in person, repeated emergencies, and requests for gift cards and wire transfers are the romance scam signs the Federal Trade Commission describes. Naming these signs and offering to check the story together protects the client's money without taking over the decision. It beats B because accepting the plan without sharing what the social worker recognizes leaves the client open to further loss.

B **Incorrect** (trap: Good but not best). Respecting the decision matters, and the money is the client's. Staying silent about strong scam signs is not neutral, though; it leaves the client choosing without information the social worker has.

C **Incorrect** (trap: Too directive). Telling an adult to cut off the relationship is directive and often backfires, pushing the client to defend the partner. Sharing specific warning signs invites the client to weigh the evidence.

Takeaway: When a competent adult faces likely exploitation, share specific warning signs and offer help while leaving the decision with the client.

Sources: NASW Code of Ethics (2021), 1.02; Federal Trade Commission. What To Know About Romance Scams (consumer advice), Signs of a romance scam
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46. ASM-1107 (rev 2) · Best answer: C

A 15-year-old in a group home returns from a weekend away with new clothes, a second phone and cash. The youth says a 24-year-old 'partner' pays for hotel rooms and sometimes asks the youth to spend time with the partner's friends, who then give the youth money. The youth insists no one forces anything. What should the social worker do **FIRST**?

A. Ask the youth whether threats or force were ever used
B. Make a safety plan with the youth for the next outing
C. Make a child abuse report for suspected sex trafficking

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Asking about force fits an adult case, because force, fraud or coercion defines adult sex trafficking. For a 15-year-old, sex exchanged for money is enough, so this question delays a required report.

B **Incorrect** (trap: Right action, wrong time). Safety planning with the youth is part of the response and should follow quickly. Planning for the next outing before reporting treats ongoing exploitation of a minor as something to manage rather than report.

C **Correct.** Under federal law, anyone under 18 induced into a commercial sex act is a sex trafficking victim; force, fraud or coercion need not be shown (22 U.S.C. 7102(11)(A)), and CAPTA treats an identified child trafficking victim as a victim of child abuse (42 U.S.C. 5106g(b)(1)). Money from a much older partner's friends gives reasonable suspicion, the threshold for a mandated report. It beats A because the youth's denial of force does not change the legal picture, and seeking proof delays protection.

Takeaway: A minor in any commercial sex exchange is a trafficking victim; report on reasonable suspicion without proof of force.

Sources: Trafficking Victims Protection Act, 22 U.S.C. 7102(3), (7), (11); Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106g(b)(1)
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47. ASM-1184 (rev 2) · Best answer: C

A client at a community health center paid a smuggler to bring them across the border. On arrival, the smuggler said the client now owed twice the agreed fee, took the client's identity papers, and placed the client in a restaurant kitchen that pays nothing until the debt is cleared. The client says, 'I chose to come, so this is my problem.' Which term **MOST** likely describes the client's current situation?

A. Human smuggling
B. Wage theft
C. Labor trafficking

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Smuggling is the illegal movement of a person across a border, usually with that person's agreement, and the arrangement typically ends on arrival. It describes how the client entered, which is why it is tempting, but not the exploitation happening now.

B **Incorrect** (trap: Wrong model or concept). Wage theft is an employer's failure to pay wages owed, a labor-law violation. It misses the coercion: seized papers and an inflated debt that the client must work off.

C **Correct.** Trafficking is exploitation of a person through force, fraud or coercion to obtain labor or commercial sex; it does not require movement and can follow a smuggling arrangement (DHS Blue Campaign). Holding a person to work off an inflated debt is debt bondage, and taking identity papers adds coercion (TVPA, 22 U.S.C. 7102). It beats A because smuggling describes the client's agreed border crossing, while the forced unpaid work now is trafficking, whatever the client agreed to earlier.

Takeaway: Smuggling is a crime against a border; trafficking is exploitation of a person, and a smuggled person can become a trafficking victim.

Sources: U.S. Department of Homeland Security, Blue Campaign (2024). What Is Human Trafficking? (brochure), Human trafficking vs. human smuggling; Trafficking Victims Protection Act of 2000, 22 U.S.C. 7102(3), (7), (11)(B)
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48. ASM-1185 (rev 1) · Best answer: D

A 76-year-old client in a senior center counseling program pays part of an adult grandchild's rent each month. The client manages their own bills, which are current, and explains, 'I want to help while I'm alive to see it.' The client describes no pressure from the grandchild. The client's adult child calls the social worker, angry, insisting the payments are elder abuse that must be reported. What is the **BEST** response?

A. File an adult protective services report as the child asks
B. Ask the client to pause the payments until the family agrees
C. Share the client's finances with the adult child to ease worry
D. Support the client's choice and decline to file a report

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A report fits reasonable suspicion of exploitation, such as unpaid bills, pressure, deception or declining capacity. None is present here, so a report would act on a relative's disagreement and override a capable client's choice.

B **Incorrect** (trap: Overrides the client's choice). Asking a capable adult to stop gifts until relatives approve puts the family's wishes above the client's. It would be reasonable to revisit the payments if the client's needs began to go unmet.

C **Incorrect** (trap: Misreads law or ethics). Sharing the client's financial details with a relative needs the client's consent (NASW Code 1.07(b)). Reassuring the adult child this way would breach confidentiality, however well meant.

D **Correct.** Financial exploitation is improper or unauthorized use of an older person's resources for another's gain (42 U.S.C. 1397j(8)). A capable client who pays their own bills, gives freely and reports no pressure is making a gift, not being exploited, so self-determination governs (NASW Code 1.02). It beats A because a report needs reasonable suspicion, and nothing here suggests deception, pressure, incapacity or unmet needs. The social worker can listen to the adult child without sharing the client's information.

Takeaway: A capable older adult's voluntary gifts to family are not exploitation; report when there are signs of pressure, deception, incapacity or unmet needs.

Sources: Elder Justice Act, 42 U.S.C. 1397j(8); NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.07(b)
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49. ASM-0004 (rev 1) · Best answer: D

A parent tells the social worker that their 20-month-old, who used to explore the playground confidently, now clings to the parent and protests at separations, then pushes the parent away moments later. The child is otherwise healthy, and there have been no recent changes at home. What **MOST** likely explains the behavior?

A. Separation anxiety disorder
B. Insecure-avoidant attachment
C. Regression after a stressful event
D. Rapprochement subphase

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Distress at separation is developmentally expected in toddlers. The disorder requires fear beyond what is typical for the child's developmental level, with impairment, which the stem does not describe (DSM-5-TR).

B **Incorrect** (trap: Wrong model or concept). Avoidant attachment shows up as a child who seems indifferent to the caregiver and does not seek comfort. This toddler actively seeks the parent, which is the opposite pattern.

C **Incorrect** (trap: Skips assessment). Regression is a reasonable idea when a stressor is present, but the stem rules out recent changes at home. Choosing it adds facts that the stem does not give.

D **Correct.** In Mahler's separation-individuation theory, the rapprochement subphase (about 16 to 24 months) follows the confident practicing period. The toddler, newly aware of being separate, alternates between clinging and pushing away (Mahler, Pine & Bergman, 1975). The age, the earlier confident exploring and the cling-then-reject pattern all fit. It beats A because separation anxiety disorder requires fear that is excessive for the child's developmental level, and protest at separation is expected at 20 months.

Takeaway: Clinging that alternates with pushing away at 16 to 24 months is Mahler's rapprochement subphase, a normal phase.

Sources: Mahler, M. S., Pine, F., & Bergman, A. (1975). The Psychological Birth of the Human Infant. Basic Books, rapprochement subphase; DSM-5-TR (APA, 2022), Anxiety Disorders: Separation Anxiety Disorder
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50. ASM-0107 (rev 1) · Best answer: B

A 48-year-old client says work and home life feel empty. The client recently quit mentoring new employees and dropped out of a community youth program, saying, 'Nothing I do will matter to anyone after me.' The client spends most free time alone watching TV. According to Erikson, which psychosocial crisis is the client **MOST** likely facing?

A. Intimacy versus isolation
B. Generativity versus stagnation
C. Integrity versus despair
D. Identity versus role confusion

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Intimacy versus isolation is the young-adult task of forming close, committed relationships. The client spends time alone, but the core concern is a lack of lasting contribution to others, the hallmark of midlife.

B **Correct.** Erikson's midlife crisis, generativity versus stagnation, centers on contributing to the next generation through parenting, mentoring, work or community (Erikson, 1963). At 48, this client has withdrawn from mentoring and youth work and feels nothing will outlast them, which signals stagnation. It beats C because integrity versus despair is the late-life task of looking back over a whole life, usually after 65, while this client is in midlife and focused on contributing going forward. Work might aim at rebuilding meaningful contribution.

C **Incorrect** (trap: Wrong model or concept). Integrity versus despair is the late-life review of whether one's life had meaning, typically after 65. It is the nearest neighbor here, but this client is 48 and focused on contributing to future generations.

D **Incorrect** (trap: Wrong model or concept). Identity versus role confusion is the adolescent task of forming a stable sense of self. A 48-year-old questioning their impact on others is facing a later, midlife crisis.

Takeaway: Generativity versus stagnation is Erikson's midlife task of contributing to the next generation; integrity versus despair is the late-life review.

Sources: Erikson, E. H. (1963). Childhood and Society (2nd ed.). W. W. Norton, eight ages of man: generativity vs. stagnation

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51. ASM-1043 (rev 3) · Best answer: D

Which change in a 74-year-old fits normal cognitive aging rather than a neurocognitive disorder?

A. Getting lost while driving a familiar route
B. Needing help to pay monthly bills
C. Asking the same question minutes after an answer
D. Recalling a name slowly but remembering it later

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Becoming lost on a well-known route suggests decline in memory or visuospatial skills. It goes beyond normal slowing and warrants a cognitive evaluation.

B **Incorrect** (trap: Near-miss diagnosis). Needing help with bills, when it reflects cognitive change, is lost independence in a complex daily task. That kind of decline marks major neurocognitive disorder rather than normal aging.

C **Incorrect** (trap: Near-miss diagnosis). Repeating a question minutes after hearing the answer shows trouble holding new information. This is a typical early sign of a neurocognitive disorder, not ordinary slowing.

D **Correct.** Normal aging brings slower processing and occasional delays in retrieving names or words that come back later, with no loss of independence. Neurocognitive disorders involve decline from a previous level, and major neurocognitive disorder interferes with independence in daily activities such as managing bills (DSM-5-TR). It beats C because quickly forgetting a just-answered question reflects impaired learning of new information, a common early sign of Alzheimer's disease rather than ordinary slowing.

Takeaway: Slower recall that resolves is normal aging; losing new information, getting lost or needing help with daily tasks suggests a neurocognitive disorder.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major and Mild Neurocognitive Disorders

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52. ASM-1046 (rev 2) · Best answer: C

During a home visit, a parent worries that their 5-year-old talks and plays with an invisible friend. The child sets a place for the friend at meals but says, 'My friend is just pretend.' The child attends preschool, plays well with peers and sleeps normally. What is the **MOST** helpful response to the parent?

A. Recommend an evaluation for possible hallucinations
B. Suggest the parent gently discourage the pretend friend
C. Explain that imaginary companions are common at this age

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). An evaluation would fit if the child believed the friend was real, seemed distressed or showed other changes. A friend the child knows is pretend, with good functioning, is typical development.

B **Incorrect** (trap: Good but not best). Imaginary play is normal and can support social skills and creativity. Asking the parent to discourage it treats ordinary development as a problem and adds no benefit.

C **Correct.** Imaginary companions are common in early childhood, and children who have them usually know the friend is pretend and function well socially (Taylor, 1999). This child calls the friend pretend, attends preschool, plays with peers and sleeps normally, so nothing suggests mental illness. Explaining this answers the parent's worry directly. It beats B because discouraging normal pretend play is unnecessary and may signal to the child that something is wrong.

Takeaway: An imaginary companion that a young child knows is pretend, with good functioning, is typical development, not a sign of mental illness.

Sources: Taylor, M. (1999). Imaginary Companions and the Children Who Create Them. Oxford University Press, characteristics and prevalence of imaginary companions
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53. ASM-1500 (rev 1) · Best answer: B

A hospital social worker meets with a 5-year-old whose grandparent had a stroke this morning. The child says the stroke happened because the child refused to share toys at breakfast. Later, the child pats the hospital bed and says the bed feels sad. According to Piaget, this thinking is **MOST** likely typical of which stage?

A. Sensorimotor
B. Preoperational
C. Concrete operational

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The sensorimotor stage covers roughly birth to age 2, when infants learn through senses and movement and develop object permanence. A 5-year-old who builds a verbal story about cause and effect has moved past it into symbolic thought.

B **Correct.** In Piaget's preoperational stage (about ages 2 to 7), children think symbolically but egocentrically, often using magical thinking and animism (Piaget &

Inhelder, 1969). This 5-year-old believes a personal act caused the grandparent's stroke and gives feelings to a bed, both hallmarks of preoperational thought. It beats C because concrete operational children, from about age 7, reason logically about physical causes and usually understand that their behavior cannot cause another person's illness. Recognizing this helps the social worker correct the child's self-blame in simple, concrete words.

C **Incorrect** (trap: Wrong model or concept). Concrete operational thinking begins around age 7 and brings logical reasoning about real objects and events, including conservation. Children at that stage usually recognize that refusing to share cannot cause a stroke, so this child's self-blame and animism point to an earlier stage.

Takeaway: Self-blame through magical thinking and giving feelings to objects mark Piaget's preoperational stage, roughly ages 2 to 7.

Sources: Piaget, J., & Inhelder, B. (1969). The Psychology of the Child. Basic Books, preoperational thought: egocentrism and animism

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54. ASM-1501 (rev 1) · Best answer: A

During a home visit, the parent of a 20-month-old tells an early childhood social worker that the toddler used to say 'ball,' 'milk' and 'up' but stopped using any words about 3 months ago. The toddler does not point to show objects and seldom turns when called by name. Relatives say the child will grow out of it. What should the social worker do **FIRST**?

A. Refer the child for a prompt developmental evaluation

B. Plan to recheck the child's language at the next visit

C. Ask the parent how the family feels about the changes

D. Coach the parent in daily language-building play

Why each option is right or wrong

A **Correct.** Loss of words or social skills at any age is a developmental red flag that calls for prompt evaluation rather than watchful waiting (Centers for Disease Control and Prevention, Developmental Milestones). This toddler has lost all previously used words and also shows missing pointing and weak response to name, early signs linked to autism spectrum disorder (Hyman et al., 2020). It beats C because the family's feelings matter but do not change the need for a timely referral; they can be explored while the referral moves forward. Early evaluation opens the door to early intervention, which works best when started young.

B **Incorrect** (trap: Ignores risk). Waiting to recheck delays evaluation for a child who has already lost skills. Rescreening at a later visit can suit a borderline screening result with no regression; it is wrong once words have been lost.

C **Incorrect** (trap: Good but not best). Understanding the family's reactions supports engagement and can happen alongside the referral. Making it the first step, when the toddler has lost words and shows other red flags, puts a feelings conversation ahead of a time-sensitive evaluation.

D **Incorrect** (trap: Right action, wrong time). Language-building play is useful and may become part of an early intervention plan. Starting it in place of an evaluation treats the problem as simple delay before the cause of the lost words is known.

Takeaway: Loss of previously gained language or social skills at any age is a red flag that warrants prompt developmental evaluation, not waiting.

Sources: Centers for Disease Control and Prevention. Learn the Signs. Act Early.: CDC's Developmental Milestones, when to act early: lost skills; Hyman, S. L., Levy, S. E., Myers, S. M., & AAP Council on Children with Disabilities (2020). Identification, evaluation, and management of children with autism spectrum disorder. Pediatrics, 145(1), e20193447, early signs and referral for evaluation

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55. ASM-1502 (rev 1) · Best answer: D

A child welfare social worker observes a supervised visit between a 15-month-old in foster care and the birth parent. After a brief separation, the parent returns and holds out their arms. The toddler starts toward the parent, then stops and freezes with a blank stare. Next, the toddler backs toward the parent with face turned away. Which attachment pattern does this behavior **MOST** likely reflect?

A. Secure
B. Insecure-avoidant
C. Insecure-ambivalent
D. Disorganized

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Securely attached toddlers may protest separation but seek the caregiver on reunion and are comforted fairly quickly. Freezing and backing in with face turned away is not a secure pattern.

B **Incorrect** (trap: Wrong model or concept). Avoidant toddlers show little distress and calmly ignore or turn from the caregiver on reunion. The averted face fits partly, but avoidance is a consistent strategy, while this toddler freezes and approaches in a contradictory way.

C **Incorrect** (trap: Wrong model or concept). Ambivalent (resistant) toddlers are very distressed, cling yet push away angrily, and are hard to soothe (Ainsworth et al., 1978). This toddler shows a blank stare and freezing, not angry, clinging protest.

D **Correct.** Disorganized attachment shows contradictory or fearful behavior toward the caregiver on reunion, such as freezing, dazed expressions or approaching while looking away (Main & Solomon, 1990). This toddler starts toward the parent, freezes with a blank stare, then backs in with face averted, a mix that no organized strategy explains. It beats B because avoidant infants consistently ignore or turn from the caregiver and do not freeze or approach in contradictory ways. Disorganized attachment is most associated with maltreatment or frightening caregiving, which matters for visit planning.

Takeaway: Disorganized attachment appears as contradictory or fearful reunion behavior, such as freezing or approaching while looking away; avoidance is consistent ignoring.

Sources: Main, M., & Solomon, J. (1990). Procedures for identifying infants as disorganized/disoriented during the Ainsworth Strange Situation. In M. T. Greenberg, D. Cicchetti, & E. M. Cummings (Eds.), Attachment in the Preschool Years (pp. 121-160). University of Chicago Press, indices of disorganization; Ainsworth, M. D. S., Blehar, M. C., Waters, E., & Wall, S. (1978). Patterns of Attachment: A Psychological Study of the Strange Situation. Erlbaum, classification groups A, B, C

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56. ASM-1503 (rev 1) · Best answer: C

In a school-based substance use group, a 16-year-old says two classmates crashed after drinking because 'they're bad drivers.' The teen adds that driving after a few beers is fine for them because 'nothing like that is going to happen to me.' Which developmental concept **MOST** likely explains this belief?

A. Imaginary audience

B. Identity diffusion

C. Personal fable

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The imaginary audience is the adolescent belief that others are always watching and evaluating, which drives self-consciousness and concern about appearance. This teen's statement is about being exempt from danger, not about being observed.

B **Incorrect** (trap: Wrong model or concept). Identity diffusion is the identity status of a young person who has neither explored options nor made commitments. It describes a lack of direction in identity, not a belief that harmful consequences will skip oneself.

C **Correct.** Elkind's personal fable is the adolescent belief that one's experiences are unique and that risks which harm others will pass oneself by (Elkind, 1967). This teen accepts that drinking and driving hurt classmates but insists the same outcome will not happen to them, the core of the fable and a known driver of adolescent risk-taking. It beats A because the imaginary audience, also from Elkind, is the sense of being constantly watched and judged, which explains self-consciousness rather than feeling exempt from danger.

Takeaway: The personal fable is adolescents' sense of being unique and exempt from risks that harm others; the imaginary audience is feeling constantly watched.

Sources: Elkind, D. (1967). Egocentrism in adolescence. Child Development, 38(4), 1025-1034, personal fable and imaginary audience

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57. ASM-1504 (rev 1) · Best answer: A

A parent at a pediatric clinic tells the integrated behavioral health social worker that their 2-year-old has turned 'defiant.' The toddler insists on putting on shoes alone, says 'no' to many requests and has brief tantrums when stopped. The toddler sleeps well, speaks in short phrases and plays happily at child care. What is the **MOST** appropriate response?

A. Explain that these behaviors fit typical toddler development
B. Refer the toddler for a disruptive behavior disorder evaluation
C. Ask the parent to track every tantrum for 2 weeks

Why each option is right or wrong

A **Correct.** Erikson's stage of autonomy versus shame and doubt (about ages 1.5 to 3) centers on the toddler's push to act independently (Erikson, 1963). Insisting on doing things alone, frequent refusals and brief tantrums when stopped are expected at age 2, and the toddler's good sleep, language and play support typical development. It beats C because a tantrum log adds little when the stem already shows a normal pattern; it would help if tantrums were long, injurious or worsening. Guidance such as offering limited choices can follow.

B **Incorrect** (trap: Hands off too soon). A disruptive behavior evaluation suits persistent, severe or impairing behavior well beyond age norms. Referring a 2-year-old for normal independence-seeking pathologizes typical development and may alarm the parent without cause.

C **Incorrect** (trap: Good but not best). Tracking tantrums helps when their frequency, length or severity is unclear or concerning. Here the description already fits ordinary toddler behavior, so a 2-week log delays the reassurance and guidance the parent needs.

Takeaway: Toddler insistence on independence, frequent refusals and brief tantrums are typical autonomy-stage behavior; normalize and guide before considering referral.

Sources: Erikson, E. H. (1963). Childhood and Society (2nd ed.). W. W. Norton, eight ages of man: autonomy vs. shame and doubt

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58. ASM-1505 (rev 1) · Best answer: C

A 19-year-old at a university counseling center is failing pre-nursing courses. Both parents are nurses, and the client says the plan to become a nurse was set at age 12. Asked what other fields have interested them, the client looks puzzled and says they have never looked into anything else. Which identity status does this **MOST** likely reflect?

A. Identity moratorium
B. Identity achievement
C. Identity foreclosure
D. Identity diffusion

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Moratorium is active exploration without a firm commitment yet. This client is committed to nursing and has not explored alternatives, which is the reverse pattern.

B **Incorrect** (trap: Wrong model or concept). Identity achievement follows real exploration that ends in a chosen commitment. The client's firm commitment makes this tempting, but it was adopted from the parents at age 12 without examining other options.

C **Correct.** Marcia's foreclosure status describes a firm commitment made without exploring alternatives, often adopted from parents (Marcia, 1966). This client committed to nursing in childhood, following both parents, and has never considered other paths, even while failing the coursework. It beats B because identity achievement requires a period of active exploration before commitment; this client has the commitment but skipped the exploration. The academic setback may open a healthy period of exploration that the social worker can support.

D **Incorrect** (trap: Wrong model or concept). Diffusion means neither exploration nor commitment, often with drifting or apathy. This client is strongly committed to one path, so diffusion does not fit.

Takeaway: Foreclosure is commitment without exploration, often adopted from parents; achievement is commitment after exploration; moratorium is exploration without commitment.

Sources: Marcia, J. E. (1966). Development and validation of ego-identity status. Journal of Personality and Social Psychology, 3(5), 551-558, identity statuses

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59. ASM-1506 (rev 1) · Best answer: B

A school social worker meets with a 9-year-old who has stopped turning in math assignments. The child compares their test scores with classmates' scores, says 'Everyone is smarter than me,' and refuses to try new tasks in case they fail. Which Erikson stage is the child **MOST** likely working through?

A. Initiative versus guilt

B. Industry versus inferiority

C. Autonomy versus shame and doubt

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Initiative versus guilt is the preschool stage (about 3 to 6), when children start activities and plans and may feel guilt over overstepping. A 9-year-old's distress about academic competence belongs to the next stage.

B **Correct.** Erikson's school-age crisis, industry versus inferiority (about ages 6 to 12), centers on building competence through schoolwork and skills, judged against peers (Erikson, 1963). This 9-year-old measures themself against classmates, concludes they are less capable and stops trying, signs of a growing sense of inferiority. It beats A because initiative versus guilt is the preschool task of planning and starting activities, and its conflict is guilt over self-started play rather than competence at school.

C **Incorrect** (trap: Wrong model or concept). Autonomy versus shame and doubt is the toddler stage (about 1.5 to 3), centered on independence in basic self-care. Fear of

failing fits partly, but a 9-year-old judging their competence against classmates is facing the school-age crisis.

Takeaway: Industry versus inferiority (school age) concerns competence measured against peers; repeated failure and comparison can produce a sense of inferiority.

Sources: Erikson, E. H. (1963). Childhood and Society (2nd ed.). W. W. Norton, eight ages of man: industry vs. inferiority

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60. ASM-1507 (rev 1) · Best answer: B

In a court-ordered program, a client says they follow every rule, even ones they think are unfair, because 'if everyone picked which laws to obey, society would fall apart.' The client adds that changing a bad law is up to lawmakers, not individuals. In Kohlberg's theory, this reasoning **MOST** likely reflects which level?

A. Preconventional
B. Conventional
C. Postconventional

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Preconventional reasoning centers on avoiding punishment or gaining rewards. The client is not focused on personal consequences; the concern is keeping society orderly, which marks a later level.

B **Correct.** Kohlberg's conventional level judges right and wrong by social approval and by upholding laws and social order; stage 4 is the law-and-order orientation (Kohlberg, 1981). This client obeys even unfair rules because order would collapse otherwise and leaves change to lawmakers, the stage 4 view. It beats C because postconventional reasoning treats laws as social contracts that can be challenged when they violate rights or universal principles, a view this client rejects. Knowing the level can help the social worker frame program expectations.

C **Incorrect** (trap: Wrong model or concept). Postconventional reasoning sees laws as agreements that serve rights and justice and supports changing or resisting unjust laws on principle. The client's insistence on obeying unfair rules for the sake of order fits the conventional level instead.

Takeaway: Kohlberg's conventional level upholds rules and social order for their own sake; postconventional reasoning weighs laws against rights and principles.

Sources: Kohlberg, L. (1981). Essays on Moral Development, Vol. 1: The Philosophy of Moral Development. Harper & Row, stages of moral development: stage 4

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61. ASM-1508 (rev 1) · Best answer: C

A school social worker meets with the parent of a 14-year-old. Over the past 2 months, the teen has stopped seeing friends, quit the swim team, sleeps most afternoons and has dropped from B's to failing grades. The parent says, 'It's just a moody teenage phase.' What should the social worker do **FIRST**?

A. Agree that these changes are common in adolescence
B. Refer the teen to a psychiatrist for medication
C. Meet with the teen to screen for depression
D. Set up a daily homework check with teachers

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Ordinary adolescent moodiness is brief and does not erase friendships, activities and grades. Agreeing with the parent normalizes a cluster of changes that may signal depression and delays needed assessment.

B **Incorrect** (trap: Skips assessment). Medication decisions belong to a prescriber after a full evaluation. Sending the teen for medication before any screening skips the assessment that would show whether a psychiatric referral is needed.

C **Correct.** Some moodiness is typical in adolescence, but a marked, lasting change across several areas of functioning is atypical and calls for assessment (Zuckerbrot et al., 2018). Two months of withdrawal from friends, quitting a valued activity, excess sleep and a sharp grade drop suggest possible depression, so screening the teen directly, including asking about suicidal thoughts, comes first. It beats D because a homework plan targets the grades but not the likely cause, and could leave a depressed teen unassessed. Screening results then guide referral and school supports.

D **Incorrect** (trap: Right action, wrong time). School supports may help later, once the cause of the decline is known. A homework plan made first targets one symptom and leaves the possible mood disorder unassessed.

Takeaway: Distinguish typical adolescent moodiness from a lasting, multi-area change in functioning, which calls for direct screening before planning supports.

Sources: Zuckerbrot, R. A., Cheung, A., Jensen, P. S., Stein, R. E. K., Laraque, D., & GLAD-PC Steering Group (2018). Guidelines for Adolescent Depression in Primary Care (GLAD-PC): Part I. Pediatrics, 141(3), e20174081, identification and assessment

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62. ASM-1625 (rev 1) · Best answer: D

During a home visit, a parent describes their 13-month-old, who began walking last week. The toddler roams happily from room to room, shrugs off small tumbles and often seems to forget the parent is nearby. Every few minutes the child toddles back, pats the parent's knee and heads off again. Which of Mahler's subphases is this child **MOST** likely in?

A. Differentiation
B. Rapprochement
C. Object constancy
D. Practicing

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Differentiation, or 'hatching' (about 5 to 10 months), is when the infant first turns attention to the world beyond the parent, scanning faces and exploring the parent's body. It comes before independent walking, so it does not fit a toddler roaming the house.

B **Incorrect** (trap: Wrong model or concept). Rapprochement (about 16 to 24 months) also involves coming back to the parent, but the returns are driven by a new awareness of being separate. The toddler shadows and clings, then darts away, often with distress. This child shows elation and little concern, not that ambivalence.

C **Incorrect** (trap: Wrong model or concept). Object constancy develops from about 24 to 36 months, when the child holds a stable inner image of the parent and can tolerate longer separations. A 13-month-old who needs frequent physical check-ins has not reached it.

D **Correct.** In Mahler's theory, the practicing subphase (about 10 to 16 months) begins as the child crawls and then walks. The child explores with elation, seems almost unaware of the parent, and returns only briefly for emotional 'refueling' before setting off again. The 13-month-old's happy roaming, indifference to tumbles and quick touch-and-go returns fit this picture. It beats B because rapprochement, at about 16 to 24 months, brings a new awareness of separateness, with anxious clinging, shadowing and conflict rather than carefree exploring (Mahler, Pine & Bergman, 1975).

Takeaway: Mahler's practicing subphase features joyful exploration with brief check-ins; rapprochement adds awareness of separateness, clinging and ambivalence.

Sources: Mahler, M. S., Pine, F., & Bergman, A. (1975). The Psychological Birth of the Human Infant: Symbiosis and Individuation. Basic Books, separation-individuation subphases: practicing

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63. ASM-1629 (rev 1) · Best answer: A

A child welfare social worker visits a kinship home where a grandparent cares for a 15-month-old born 8 weeks preterm. The social worker has compared the child's skills with milestones for the corrected age of 13 months. The child is not yet walking alone and does not babble, wave or look up when called by name. The grandparent says a nurse told them preemies catch up. What should the social worker do **NEXT**?

A. Help the grandparent arrange an early intervention evaluation

B. Plan to recheck milestones at a corrected age of 18 months

C. Reassure the grandparent that preemies often walk later

D. Ask the grandparent to describe the child's daily routine

Why each option is right or wrong

A **Correct.** Corrected age is used to judge a premature child's milestones during the first 2 years, so not walking alone at a corrected 13 months is still within range. But babbling, waving and responding to one's name are expected by about 9 to 12 months even after correction. Missing them calls for a developmental evaluation now, and families can ask early intervention for one without waiting. It beats B because waiting 5 more months delays help for delays that prematurity does not explain (AAP, 2018; Centers for Disease Control and Prevention milestones).

B **Incorrect** (trap: Ignores risk). Rechecking later would be reasonable if the only concern were walking, which corrected age explains. The missing babbling, waving and response to name fall outside the range even for a corrected 13 months, so waiting postpones an evaluation the child needs now.

C **Incorrect** (trap: Good but not best). Late walking in a preterm child often does reflect prematurity, and correcting for age shows it is still typical. But this reassurance covers only the motor milestone and leaves the social and communication red flags unaddressed.

D **Incorrect** (trap: Right action, wrong time). Learning about routines belongs in a full assessment, but the social worker has already compared the child's skills with corrected-age milestones. Gathering more history now postpones the referral that the red flags already justify.

Takeaway: Correct for prematurity through age 2, but skills missing even for the corrected age still call for prompt developmental evaluation.

Sources: American Academy of Pediatrics, HealthyChildren.org (2018). Corrected Age for Preemies, use of corrected age during the first 2 years; Centers for Disease Control and Prevention. Learn the Signs. Act Early.: CDC's Developmental Milestones, 9-month and 12-month milestone checklists
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64. ASM-1635 (rev 1) · Best answer: A

A hospital social worker is helping an 8-year-old patient get ready for a scheduled surgery. The child asks many questions and wants to know exactly what will happen. Which approach is **BEST** suited to the child's stage of thinking?

A. Show the real mask and gown and walk through each step
B. Discuss possible risks and what happens if complications occur
C. Tell a story about a brave animal cured by magic medicine

Why each option is right or wrong

A **Correct.** Children of about 7 to 11 are typically in Piaget's concrete operational stage: they reason logically about real, observable things but struggle with abstract and hypothetical ideas. Handling the actual equipment and walking through the sequence gives the 8-year-old concrete, accurate information. Children's ideas about illness develop along the same stages (Bibace & Walsh, 1980). It beats B because talking through possible complications asks the child to reason about hypothetical events, a formal operational skill, and may raise fear without adding understanding (Piaget & Inhelder, 1969).

B **Incorrect** (trap: Wrong model or concept). Adolescents and adults who reason abstractly may want to discuss risks and contingencies. For an 8-year-old, hypothetical complications are hard to process and can raise anxiety without improving understanding.

C **Incorrect** (trap: Wrong model or concept). Stories with magical solutions fit the preoperational thinking of preschoolers. An 8-year-old who asks exact questions wants factual, concrete answers, and a magic story can leave the child confused or mistrustful.

Takeaway: Match explanations to cognitive stage: concrete, hands-on, step-by-step information for school-age children; abstract risk discussion suits formal operational thinkers.

Sources: Piaget, J., & Inhelder, B. (1969). The Psychology of the Child. Basic Books, concrete operations; Bibace, R., & Walsh, M. E. (1980). Development of children's concepts of illness. Pediatrics, 66(6), 912-917, whole article
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65. ASM-1640 (rev 1) · Best answer: C

Parents of a 23-year-old who lives at home ask for a family session. In 2 years, the young adult has changed jobs three times, switched college majors and moved in and out of a shared apartment. The young adult works part time, has close friends and says, 'I'm still figuring out what fits.' The parents worry something is wrong. Which response by the social worker is **BEST**?

A. Recommend vocational testing to settle on a career path
B. Suggest the parents set a firm date for the young adult to move out
C. Explain that such exploration is typical in the early twenties

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Vocational testing can help a young adult who wants direction and may be useful later if the young adult asks for it. Offered now, it treats normal exploration as a problem and bypasses the parents' question about whether something is wrong.

B **Incorrect** (trap: Too directive). A move-out date is the parents' choice to make, but recommending it frames typical exploration as failure. It raises family conflict before the parents understand what is typical at this age.

C **Correct.** Arnett describes the late teens through the twenties as emerging adulthood, a distinct period marked by identity exploration, instability in work, love and housing, and feeling in-between. The young adult's job and major changes, close friendships and steady part-time work fit this typical pattern rather than a disorder. It beats A because career testing assumes the exploration is a problem to fix, while the parents first need accurate information about what is typical at this age (Arnett, 2000).

Takeaway: Exploration and instability in work, study and housing are typical in emerging adulthood; distinguish them from impairment before treating them as problems.

Sources: Arnett, J. J. (2000). Emerging adulthood: A theory of development from the late teens through the twenties. American Psychologist, 55(5), 469-480, whole article
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66. ASM-1518 (rev 1) · Best answer: A

One view holds that barriers in buildings, policies and attitudes limit people more than their impairments do. Which model of disability is this?

A. Social model
B. Medical model
C. Moral model

Why each option is right or wrong

A **Correct.** The social model of disability distinguishes impairment, a feature of the body or mind, from disability, the exclusion created by barriers such as inaccessible buildings, rigid policies and negative attitudes (Oliver, 1990). Because the problem sits in society, the response targets those barriers. It beats B because the medical model locates disability in the individual's impairment and focuses on diagnosis, treatment or cure. Social workers draw on the social model when advocating for accommodations and accessible services.

B **Incorrect** (trap: Wrong model or concept). The medical model treats disability as a problem within the person, to be diagnosed, treated or cured by professionals. The question places the source of disability in the environment, which is the reverse emphasis.

C **Incorrect** (trap: Wrong model or concept). The moral model, the oldest view, treats disability as a sign of sin, a test of faith or a moral failing. It locates meaning in the person's character, not in environmental barriers.

Takeaway: The social model locates disability in environmental and attitudinal barriers; the medical model locates it in the person's impairment.

Sources: Oliver, M. (1990). The Politics of Disablement. Macmillan, social model of disability
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67. ASM-1519 (rev 1) · Best answer: A

A 47-year-old client with Down syndrome has lived in a supported apartment for 10 years. Over the past 6 months, staff report that the client gets lost on familiar bus routes, skips steps when cooking and has lost interest in a weekly bowling league. Staff call the client 'stubborn' and want a behavior plan. What should the social worker do **FIRST**?

A. Seek a medical and cognitive assessment of the decline
B. Create a reward plan for completing daily tasks
C. Ask the client how they feel about the recent changes
D. Reteach cooking steps using picture-based cues and prompts

Why each option is right or wrong

A **Correct.** New loss of daily skills in an adult with Down syndrome calls for medical and cognitive evaluation, because these adults face a greatly increased risk of Alzheimer-type dementia, and treatable causes such as thyroid problems, depression or vision loss must be ruled out (Alzheimer's Association). Calling the change stubbornness is diagnostic overshadowing: attributing new problems to the existing disability (Reiss et al., 1982). It beats D because retraining assumes a learning problem and could delay finding a treatable or progressive cause.

B **Incorrect** (trap: Skips assessment). A reward plan assumes the client is choosing not to perform tasks. Building it before the decline is evaluated risks treating a medical or neurocognitive change as a behavior problem and delaying care.

C **Incorrect** (trap: Good but not best). The client's view matters and should be included, but asking about feelings cannot explain a 6-month loss of skills. Making it the first step delays the evaluation that new decline requires.

D **Incorrect** (trap: Right action, wrong time). Visual supports and skill retraining may help later. Starting them first treats the decline as a learning issue before medical causes and dementia have been assessed.

Takeaway: New functional decline in an adult with an intellectual disability needs medical and cognitive evaluation; avoid attributing it to the disability or to behavior.

Sources: Alzheimer's Association. Differential Diagnosis of Down Syndrome (alz.org professional resources), risk, early signs and conditions to rule out; Reiss, S., Levitan, G. W., & Szyszko, J. (1982). Emotional disturbance and mental retardation: Diagnostic overshadowing. American Journal of Mental Deficiency, 86(6), 567-574, whole article

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68. ASM-1520 (rev 1) · Best answer: C

Staff at a senior center ask the social worker about a 76-year-old member who has stopped joining group activities. In the busy dining room, the member often answers questions that were not asked and asks others to repeat themselves. A relative wonders if dementia is starting. The social worker meets with the member in a quiet office, where they talk easily about recent events. What should the social worker do **NEXT**?

A. Move the member to smaller, quieter activity groups

B. Refer the member for a neurocognitive evaluation

C. Recommend that the member get a hearing evaluation

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Quieter groups may help the member participate and could become part of the plan. Doing this before checking hearing manages the symptom without identifying or treating the likely cause of the withdrawal.

B **Incorrect** (trap: Skips assessment). A neurocognitive evaluation is appropriate when memory or thinking problems appear across settings. Here the member does well in a quiet room, so referring for possible dementia first overlooks a likely sensory explanation.

C **Correct.** Hearing loss is common in older adults, and people who cannot hear well are often mistaken for confused, uncooperative or withdrawn (National Institute on Aging). This member struggles and answers off-target in a noisy room but converses easily and recalls recent events in a quiet office, a pattern that points to hearing rather than cognition. It beats B because jumping to a neurocognitive evaluation skips the simpler, likely explanation; if problems persist after hearing is addressed, cognitive evaluation becomes appropriate.

Takeaway: Rule out sensory loss, especially hearing, before attributing an older adult's withdrawal or off-target answers to cognitive decline.

Sources: National Institute on Aging (NIH). Hearing Loss: A Common Problem for Older Adults, signs and effects of hearing loss

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69. ASM-1631 (rev 1) · Best answer: C

At an aging services agency, a social worker meets with a 76-year-old client with macular degeneration who has had only routine eye exams. The client can no longer read pill labels or recipes and has stopped cooking and going to a weekly card game. The client's adult children want the client to move to assisted living. The client says, 'If I could just see well enough to manage, I'd stay home.' What should the social worker do **FIRST**?

A. Help the family compare nearby assisted living options
B. Arrange home-delivered meals to replace cooking
C. Refer the client for low vision rehabilitation
D. Hold a family meeting to weigh the proposed move

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Assisted living may become right if vision services cannot make home safe enough. Planning the move now overrides the client's stated wish to stay home before less restrictive options have been tried.

B **Incorrect** (trap: Good but not best). Meal delivery addresses one lost task and may help in the short term. It does nothing for medication safety or social withdrawal, and it accepts the loss of function instead of trying to restore it.

C **Correct.** Vision loss can erode daily function at any age, and rehabilitation can restore much of it through magnifiers, lighting, talking or large-print labels, and training in cooking and medication safety. The client wants to stay home and has never had these services, so restoring function comes before any decision about moving. It beats D because a family meeting about the move is more productive once everyone knows what the client can manage with vision services. Ophthalmology guidelines encourage linking people with vision loss to rehabilitation (AAO, 2022; NASW Code 1.02).

D **Incorrect** (trap: Right action, wrong time). A family meeting will likely be needed, since the children are worried. Holding it before the vision evaluation means debating the move without knowing what the client could manage with low vision services.

Takeaway: Before considering a more restrictive setting for an adult with sensory loss, refer for rehabilitation that may restore function.

Sources: American Academy of Ophthalmology (2022). Vision Rehabilitation Preferred Practice Pattern, providing rehabilitation information to patients with vision loss; NASW Code of Ethics (2021), 1.02
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70. ASM-1525 (rev 2) · Best answer: B

In the family life-cycle framework, what is the central emotional task for families in later life?

- **A.** Accepting many exits from and entries into the family
- **B.** Accepting the shift in generational roles
- **C.** Committing to a newly formed family system
- **D.** Loosening boundaries to allow children's independence

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). This is the key task of the launching stage, when grown children leave home and partners and grandchildren join. Later-life families may still see these changes, but their central task concerns how roles shift between generations as the oldest members age.

B **Correct.** In the family life-cycle framework, each stage has a key emotional process. For families in later life it is accepting the shift in generational roles: older adults keep up their own functioning and interests while the middle generation takes a more central role, and the family makes room for elders' experience without overfunctioning for them (Carter & McGoldrick, 2005). It beats A, the launching-stage task, because exits and entries describe children leaving and in-laws and grandchildren joining, not the reshuffling of roles as parents age.

C **Incorrect** (trap: Wrong model or concept). Commitment to a new system is the task of the new couple stage, when partners form their relationship and realign ties with their families of origin. It describes the start of a family, not its later years.

D **Incorrect** (trap: Wrong model or concept). Increasing the flexibility of boundaries belongs to families with adolescents, who need room to move in and out of the family. That stage also begins to account for grandparents' frailties, which is why it can tempt, but it is not the later-life task.

Takeaway: In later life, families accept shifting generational roles: elders keep their functioning while the middle generation takes on a more central role.

Sources: Carter, B., & McGoldrick, M. (Eds.) (2005). The Expanded Family Life Cycle: Individual, Family, and Social Perspectives (3rd ed., classic ed.). Allyn & Bacon, Ch. 1, Table 1.1 (p. 2): families in later life
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71. ASM-1526 (rev 1) · Best answer: A

The adult children of an 81-year-old who lives alone ask a senior center social worker for help. In 4 months, the parent has had two minor crashes, and new scrapes keep appearing on the car. The parent says both crashes were other drivers' fault and refuses to stop driving. One adult child wants to hide the car keys. Which recommendation is **BEST**?

A. Arrange a professional evaluation of the parent's driving ability
B. Suggest the parent limit driving to daylight hours near home
C. Support the adult children in removing access to the car
D. Ask the parent's physician to report the parent to the licensing agency

Why each option is right or wrong

A **Correct.** Driving often stands for independence in late life, so the response must protect both safety and self-determination (NASW Code 1.02). Repeated crashes and unexplained damage are red flags that call for an objective assessment, which can include clinical screening and a driver rehabilitation specialist's on-road evaluation (Pomidor, 2019). It beats B because self-imposed limits make sense once the parent's abilities are known; two crashes in 4 months make an unassessed compromise risky, and an evaluation gives the family a neutral basis for any next step.

B **Incorrect** (trap: Good but not best). Limiting driving to daylight and familiar roads is a common, useful adjustment for older drivers whose skills are known. Recommending it now guesses at the problem, may not address whatever is causing the crashes, and leaves the family's dispute unresolved.

C **Incorrect** (trap: Overrides the client's choice). Taking away access to the car overrides a competent adult's choice before anyone has assessed whether the parent can drive safely, and it can damage trust. Restricting driving may become necessary if an evaluation shows the parent is unsafe.

D **Incorrect** (trap: Skips assessment). A report to the licensing agency can be appropriate after an evaluation shows unsafe driving and the person keeps driving; reporting rules vary by state. Requesting one now skips the assessment and casts the physician as an enforcer before the facts are known.

Takeaway: When an older adult's driving safety is in question, seek an objective driving evaluation before restricting a competent person's independence.

Sources: NASW Code of Ethics (2021), 1.02; Pomidor, A. (Ed.) (2019). Clinician's Guide to Assessing and Counseling Older Drivers (4th ed.). American Geriatrics Society and National Highway Traffic Safety Administration, Ch. 2 red flags for unsafe driving; Ch. 5 driver rehabilitation
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72. ASM-1527 (rev 2) · Best answer: C

An employee assistance client tells the social worker that their widowed 78-year-old parent will move into the client's home next month. The parent drives, manages their own medications and volunteers weekly. The client's two teenagers will have to share a bedroom and are openly resentful, and the client's partner worries about privacy. What should the social worker recommend **FIRST**?

A. An in-home assessment of the parent's daily functioning
B. Individual counseling for the teenagers about their resentment
C. A meeting of the three generations to plan roles and space

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). An in-home functional assessment fits when an older adult's ability to manage daily tasks is in doubt. This parent is independent, so the assessment would not address the conflict over space and privacy that the household is already showing.

B **Incorrect** (trap: Hands off too soon). The teenagers' resentment is real, but it is one part of a household-wide change. Sending them to individual counseling treats a family reorganization as a personal problem and leaves the parent and partner out of the solution.

C **Correct.** A multigenerational move reorganizes the whole household. Family life-cycle theory notes that later-life transitions require families to shift generational roles and make room for elders without overfunctioning for them (Carter & McGoldrick, 2005). Bringing all three generations together before the move lets the teenagers, partner and parent negotiate space, privacy and household roles. It beats A because this parent drives, manages medications and volunteers, so the issue is family reorganization, not care needs; assessing functioning first would delay the conversation the family needs.

Takeaway: When an older parent joins a household, help all generations negotiate roles, space and routines before the move, not one member alone.

Sources: Carter, B., & McGoldrick, M. (Eds.) (2005). The Expanded Family Life Cycle: Individual, Family, and Social Perspectives (3rd ed., classic ed.). Allyn & Bacon, Ch. 1, Table 1.1 (p. 2): families in later life, second-order changes

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73. ASM-1633 (rev 1) · Best answer: B

A 74-year-old widowed client plans to remarry. The client's adult children request a family session. They say the new partner is replacing their late parent, and they worry about the family home and their inheritance. The client listens, then says the decision is made but that the client wants the children's blessing. Which response by the social worker is **BEST**?

A. Suggest the client delay the wedding until the children adjust
B. Help the children tell the client their worries directly
C. Request an evaluation of the client's decision-making capacity

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Postponing might ease the children's distress, but the client has made the decision and shows no wish to wait. Steering a competent older adult to delay a personal choice for others' comfort undercuts self-determination.

B **Correct.** Later-life transitions such as widowhood and repartnering require the whole family to reorganize, and they can stir loyalty to the late parent and worries about inheritance. The client has decided and wants the children's blessing, so helping the children put their fears into words with the client serves both the family bond and the client's right to choose. It beats A because asking a competent adult to postpone a personal decision to soothe others places the children's wishes above the client's self-determination (McGoldrick et al., 2016; NASW Code 1.02).

C **Incorrect** (trap: Skips assessment). A capacity evaluation fits when there are signs of impaired understanding or reasoning about a decision. Nothing in the client's statements suggests impairment, and family disagreement alone is not a reason to question an older adult's capacity.

Takeaway: Later-life repartnering can stir loyalty and inheritance fears; help the family voice them while respecting the older adult's choice.

Sources: McGoldrick, M., Garcia Preto, N., & Carter, B. (2016). The Expanding Family Life Cycle: Individual, Family, and Social Perspectives (5th ed.). Pearson, families in later life; NASW Code of Ethics (2021), 1.02
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74. ASM-1638 (rev 1) · Best answer: A

Which theory of aging holds that as people see their remaining time as limited, they narrow their social circle to the relationships that matter most to them?

A. Socioemotional selectivity theory
B. Disengagement theory
C. Activity theory

Why each option is right or wrong

A **Correct.** Carstensen's socioemotional selectivity theory proposes that when people perceive time as limited, as often happens in later life, they give priority to emotionally meaningful goals and let peripheral ties fade while keeping the closest ones. A smaller circle can therefore reflect choice rather than loss. It beats B because disengagement theory describes a mutual withdrawal of older adults and society from each other, not a chosen focus on valued relationships (Carstensen, Isaacowitz & Charles, 1999).

B **Incorrect** (trap: Wrong model or concept). Disengagement theory held that older adults and society naturally withdraw from each other. It describes a general retreat, not a deliberate narrowing toward the closest relationships based on how much time a person feels is left.

C **Incorrect** (trap: Wrong model or concept). Activity theory holds that older adults stay satisfied by keeping up activities and replacing lost roles with new ones. It predicts broad engagement, not a shrinking circle of close ties.

Takeaway: Socioemotional selectivity: perceived limited time leads people to focus on close relationships, so a smaller circle in later life can be adaptive.

Sources: Carstensen, L. L., Isaacowitz, D. M., & Charles, S. T. (1999). Taking time seriously: A theory of socioemotional selectivity. American Psychologist, 54(3), 165-181, whole article
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75. ASM-1515 (rev 2) · Best answer: C

A 20-year-old Mexican American college student meets with a campus social worker. Through high school, the student tried to fit in with white classmates and avoided speaking Spanish. After a Chicano history course, the student now spends time only with Latino peers and distrusts white faculty. The student feels angry about years of 'trying to be someone else.' In the racial/cultural identity development model, which stage does this **MOST** likely reflect?

A. Conformity
B. Dissonance
C. Resistance and immersion
D. Introspection

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Conformity describes preference for dominant-culture values and devaluing of one's own group. It matches the student's high school years, but the current pattern of rejecting the dominant culture marks a later stage.

B **Incorrect** (trap: Wrong model or concept). Dissonance is the period of confusion when experiences challenge earlier conformity attitudes. The history course may have triggered it, but the student now firmly embraces their own group and rejects the dominant one, which goes beyond questioning.

C **Correct.** In the racial/cultural identity development model, the resistance and immersion stage brings strong endorsement of one's own group, rejection of dominant-culture values and anger at past oppression or self-denial (Sue et al., 2022). This student has moved from trying to fit in with white peers to associating only with Latino peers, distrusting white faculty and feeling angry about earlier self-denial. It beats B because dissonance is the earlier stage of confusion and conflict, when prior beliefs are questioned but not yet replaced by full commitment to one's own group.

D **Incorrect** (trap: Wrong model or concept). Introspection follows resistance and immersion: the person starts to question rigid group loyalty and the energy spent opposing the dominant culture, often feeling torn. This student's exclusive same-group ties, blanket distrust and anger show the earlier stage is still in force, not yet being questioned (Sue et al., 2022).

Takeaway: In racial/cultural identity development, resistance and immersion means embracing one's own group and rejecting the dominant culture, often with anger.

Sources: Sue, D. W., Sue, D., Neville, H. A., & Smith, L. (2022). Counseling the Culturally Diverse: Theory and Practice (9th ed.). Wiley, Ch. 6: racial/cultural identity development model
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76. ASM-1516 (rev 1) · Best answer: D

In a telehealth session, a 22-year-old who moved away for graduate school says the faith absorbed from family and congregation no longer feels like their own. The client is reading widely, questioning teachings once accepted without thought and working out which beliefs to keep by personal choice. According to Fowler, which stage of faith does this **MOST** likely describe?

A. Synthetic-conventional
B. Conjunctive
C. Mythic-literal
D. Individuative-reflective

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Synthetic-conventional faith, common in adolescence, rests on beliefs absorbed from family and community and seldom examined. It describes where the client started, not the current critical questioning.

B **Incorrect** (trap: Wrong model or concept). Conjunctive faith, usually not before midlife, accepts paradox and draws meaning from several traditions after long self-examination. This client is in the earlier work of critically sorting inherited beliefs, not integrating paradox.

C **Incorrect** (trap: Wrong model or concept). Mythic-literal faith belongs to school-age children, who take religious stories and rules literally. A graduate student critically weighing inherited teachings is far beyond this stage.

D **Correct.** Fowler's individuative-reflective stage, often beginning in young adulthood when a person leaves home, involves critically examining inherited beliefs and taking personal responsibility for one's own commitments (Fowler, 1981). This client is questioning teachings once taken for granted and choosing which beliefs to keep. It beats A because synthetic-conventional faith, typical of adolescence, holds beliefs absorbed from the community without examining them, the stage the client is now leaving. Spiritual questioning at this age is a normal developmental transition, not a crisis by itself.

Takeaway: Fowler's individuative-reflective stage is the young-adult move from unexamined inherited faith to critically chosen, personally owned beliefs.

Sources: Fowler, J. W. (1981). Stages of Faith: The Psychology of Human Development and the Quest for Meaning. Harper & Row, Stage 4: individuative-reflective faith
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77. ASM-1517 (rev 3) · Best answer: B

A post-adoption social worker meets with parents whose 14-year-old was adopted from South Korea as an infant. The teen recently asked to take Korean language classes and join a Korean American youth group. The parents, who have spent most of the session voicing their fear that the teen is rejecting them, now ask the social worker to help talk the teen out of it. What is the **BEST** response?

A. Ask the parents to say more about their fear of rejection
B. Explain that exploring heritage is a normal part of identity development
C. Refer the family to therapy for adoption attachment issues
D. Suggest that the teen postpone these activities until the parents feel ready

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Inviting the parents to talk about their fear is useful early in the conversation, and they have already done so. Asking for more now delays the developmental information they need before they act to discourage the teen.

B **Correct.** Ethnic identity development in adolescence typically includes a period of actively exploring one's heritage before a secure identity forms (Phinney, 1990). For a transracially adopted teen, wanting language classes and a Korean American peer group is a healthy developmental step, not a rejection of the adoptive family. It beats A because the parents have already described their fear; exploring it further leaves their request to discourage healthy development unanswered. Reframing the behavior opens the way for the parents to support the teen's exploration.

C **Incorrect** (trap: Hands off too soon). Family therapy for attachment problems fits signs of a disturbed parent-child bond. Nothing here suggests an attachment problem; a teen exploring heritage is a typical developmental step, so referral pathologizes it.

D **Incorrect** (trap: Overrides the client's choice). Delaying the activities to ease the parents' discomfort puts their needs ahead of the teen's healthy identity development and signals that the teen's heritage threatens the family.

Takeaway: Adolescent exploration of ethnic or cultural heritage is typical identity development, including for transracial adoptees; help families support it rather than discourage it.

Sources: Phinney, J. S. (1990). Ethnic identity in adolescents and adults: Review of research. Psychological Bulletin, 108(3), 499-514, stages of ethnic identity development: exploration
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78. ASM-1630 (rev 1) · Best answer: D

Helms described a series of statuses in White racial identity. Which status brings guilt, shame and confusion as a person first faces racism and White advantage?

A. Contact
B. Reintegration
C. Pseudo-independence
D. Disintegration

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Contact is the starting status, marked by little awareness of racism and a belief that race does not matter. A person in contact has not yet felt the guilt and confusion that come with recognizing racism.

B **Incorrect** (trap: Wrong model or concept). Reintegration often follows disintegration as a retreat into idealizing one's own group and denying racism. Anger, not guilt, is its typical feeling, so it does not match the description.

C **Incorrect** (trap: Wrong model or concept). Pseudo-independence involves an intellectual grasp of institutional racism with limited emotional engagement. The person has moved past raw guilt and confusion to a more distant, analytical stance.

D **Correct.** In Helms's model, disintegration begins when a person can no longer ignore racism and White advantage. The new awareness clashes with beliefs about fairness and brings guilt, shame and confusion, and the person may stay silent when racism occurs. It beats B because reintegration is the retreat that can follow: the person falls back on idealizing their own group, denies racism, and replaces guilt with anger (Helms, 1990).

Takeaway: Helms: contact (unaware), disintegration (guilt, confusion), reintegration (retreat, anger), pseudo-independence (intellectual), then immersion/emersion and autonomy.

Sources: Helms, J. E. (Ed.). (1990). Black and White Racial Identity: Theory, Research, and Practice. Greenwood Press, White racial identity development model

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79. ASM-1643 (rev 2) · Best answer: B

During a home visit, a parent says their 4-year-old pointed at a person with darker skin in a store and asked loudly why the person's skin was 'dirty.' The embarrassed parent hushed the child. The parent says that race was never discussed at home growing up and now plans to avoid the topic so the child will not 'notice differences.' What is the **MOST** appropriate response by the social worker?

A. Agree to wait on race talk until the child is older

B. Say that preschoolers register race and gain from open talk

C. Explore further how the parent's family handled talk about race

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Many adults believe young children are too young to notice race, but this child's question shows the opposite. Waiting until the child is older does not prevent noticing; it leaves the child to form ideas without guidance.

B **Correct.** Children notice racial differences early, and by preschool many have begun forming racial attitudes. Research links open, age-appropriate conversation about race with lower bias in young children, while silence leaves them to draw their own conclusions. This child has already noticed, so the parent's plan rests on a misconception; correcting it gently and encouraging simple, honest answers supports

healthy development. It beats C because the parent has already described the family history behind the plan and now needs accurate information (Katz, 2003).

C **Incorrect** (trap: Good but not best). Exploring a parent's background helps when the source of discomfort is unclear. This parent has already described a family that never discussed race, so more exploration delays correcting the belief that silence keeps the child from noticing.

Takeaway: Young children notice race early; open, age-appropriate talk about differences supports healthy racial attitudes better than silence.

Sources: Katz, P. A. (2003). Racists or tolerant multiculturalists? How do they begin? American Psychologist, 58(11), 897-909, whole article

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80. ASM-0101 (rev 3) · Best answer: D

Parents in a high-conflict marriage each press their 14-year-old to agree that the other parent is at fault. Each parent also asks the teen to carry messages they refuse to say to each other. The teen agrees with whichever parent is talking and now has frequent stomachaches. Which family dynamic is **MOST** likely present?

A. Enmeshment
B. Cross-generational coalition
C. Emotional cutoff
D. Triangulation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Enmeshment describes diffuse boundaries across a family, with little privacy or autonomy for its members. The stem shows a specific pattern of routing parental conflict through one child, which triangulation captures more precisely.

B **Incorrect** (trap: Wrong model or concept). In a cross-generational coalition, one parent and a child form a stable alliance against the other parent. Here both parents compete for the teen, who agrees with each in turn, so no lasting alliance with one side has formed.

C **Incorrect** (trap: Wrong model or concept). Emotional cutoff is managing unresolved family tension by distancing or ending contact. These family members are in close, intense contact through the teen, which is the opposite of cutoff.

D **Correct.** Triangulation, a core concept in Bowen theory, occurs when two people in conflict pull a third person in to lower the tension between them (Bowen, 1978). Both parents route their conflict through the teen by demanding agreement and using the teen as a messenger, and the stomachaches reflect the strain. It beats B because a cross-generational coalition is a stable alliance of one parent and a child against the other parent, while this teen is pulled by both sides and agrees with each in turn (Minuchin, 1974).

Takeaway: Triangulation is drawing a third person into a two-person conflict; a coalition is a parent and child allied against the other parent.

Sources: Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, triangles; Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, coalitions and boundaries
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81. ASM-1531 (rev 1) · Best answer: C

When one family member's symptoms improve, another member sometimes develops problems. In family systems theory, what term describes the family's pull to keep its usual patterns?

A. Equifinality
B. Morphogenesis
C. Homeostasis
D. Family projection process

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Equifinality is the systems idea that different starting points can lead to the same outcome, so a family's history does not fix its future. It does not describe resistance to change after a member improves.

B **Incorrect** (trap: Wrong model or concept). Morphogenesis is a system's ability to change its structure and rules in response to new demands. It is the counterpart of the tendency described, which keeps old patterns in place.

C **Correct.** Homeostasis is a family system's tendency to keep its familiar balance of patterns and roles. Jackson (1957) observed that when one member improves, another may develop symptoms or the family may pull the member back into the old role, because the change upsets the system. It beats B because morphogenesis is the opposite process, a system's capacity to change its structure; the tendency described here works against change.

D **Incorrect** (trap: Wrong model or concept). The family projection process is Bowen's term for parents passing their anxiety onto a child, who becomes the focus of family concern. It explains how one child becomes symptomatic, not why symptoms shift to another member after improvement.

Takeaway: Families tend to preserve familiar patterns; when one member improves, another may become symptomatic unless the whole system adjusts.

Sources: Jackson, D. D. (1957). The question of family homeostasis. Psychiatric Quarterly Supplement, 31, 79-90, whole article
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82. ASM-1532 (rev 1) · Best answer: A

Research on families of adults with schizophrenia links which family pattern to higher relapse rates?

A. High expressed emotion
B. Double-bind communication
C. Disengaged boundaries
D. Emotional cutoff

Why each option is right or wrong

A **Correct.** Expressed emotion refers to relatives' critical comments, hostility and emotional overinvolvement toward a family member with mental illness. A meta-analysis found that living with relatives high in expressed emotion predicts relapse in schizophrenia (Butzlaff & Hooley, 1998), which is why family psychoeducation targets these patterns. It beats B because double-bind theory claimed that contradictory parental messages cause schizophrenia, an idea research did not support; expressed emotion affects the course of the illness, not its origin.

B **Incorrect** (trap: Wrong model or concept). Bateson and colleagues (1956) proposed that contradictory messages in families cause schizophrenia. The theory shaped early family therapy but was not supported by research, and blaming families for causing the illness is now rejected.

C **Incorrect** (trap: Wrong model or concept). Disengaged boundaries describe family members who are emotionally distant and underinvolved. Relapse research points instead to criticism and overinvolvement, which sit closer to the opposite of disengagement.

D **Incorrect** (trap: Wrong model or concept). Emotional cutoff is Bowen's term for managing unresolved family attachment through distance or silence. It is not the pattern that relapse studies in schizophrenia have measured.

Takeaway: Relatives' criticism, hostility and overinvolvement (high expressed emotion) predict schizophrenia relapse; families influence the course, not the cause.

Sources: Butzlaff, R. L., & Hooley, J. M. (1998). Expressed emotion and psychiatric relapse: A meta-analysis. Archives of General Psychiatry, 55(6), 547-552, whole article; Bateson, G., Jackson, D. D., Haley, J., & Weakland, J. (1956). Toward a theory of schizophrenia. Behavioral Science, 1(4), 251-264, double-bind hypothesis
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83. ASM-1533 (rev 2) · Best answer: D

A 38-year-old client moved across the country at age 19 and has had no contact with parents or siblings since. The client says, 'They have no hold on me anymore.' The client becomes furious and cannot sleep after the partner's parents offer any advice, and has ended two long friendships over minor disagreements. Which concept **MOST** likely describes this pattern?

A. Differentiation of self
B. Disengaged boundaries
C. Family projection process
D. Emotional cutoff

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Differentiation of self is the ability to stay emotionally connected to family while thinking and acting independently. The client's claim of independence can look like differentiation, but intense reactivity to in-laws and abrupt endings show low, not high, differentiation.

B **Incorrect** (trap: Wrong model or concept). Disengaged boundaries is a structural term for a household whose members are distant and underinvolved with one another. It describes current family organization, not an adult's long-term avoidance of the family of origin with reactivity carried into new relationships.

C **Incorrect** (trap: Wrong model or concept). The family projection process describes parents transmitting their anxiety to a child who becomes the family's focus. The stem says nothing about how the parents focused on this client, and the pattern described is the client's own distancing.

D **Correct.** Emotional cutoff is managing unresolved attachment to the family of origin through distance or silence. Bowen theory holds that cutoff reduces contact but not the underlying reactivity, which then shows up in other close relationships (Kerr & Bowen, 1988). The client claims freedom from family, yet reacts intensely to in-laws' advice and abruptly ends friendships. It beats A because a well-differentiated person stays connected while keeping a separate self and responds calmly; this client's distance coexists with high emotional reactivity.

Takeaway: Emotional cutoff reduces contact with family but not reactivity; unresolved attachment resurfaces as intense reactions in other close relationships.

Sources: Kerr, M. E., & Bowen, M. (1988). Family Evaluation: An Approach Based on Bowen Theory. W. W. Norton, emotional cutoff; differentiation of self

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84. ASM-1534 (rev 1) · Best answer: B

A pediatrician refers an 8-year-old with recurring stomachaches whose medical workup was normal. The child has missed 9 school days this term. During the first meeting, the parents mention that the stomachaches seem worst on days when they argue. What should the social worker do **FIRST**?

A. Recommend couples counseling to reduce the parents' arguing

B. Assess how each family member reacts when the pain begins

C. Teach the child relaxation skills to use when pain starts

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Couples work may become part of the plan if assessment confirms that parental conflict drives the symptoms. Recommending it now jumps from one comment to a conclusion and skips assessing how the whole family responds to the child's pain.

B **Correct.** Structural family theory notes that a child's symptom can detour parental conflict: the parents unite around the sick child instead of facing their own disagreement (Minuchin, Rosman & Baker, 1978). The parents' comment suggests such a link, but it needs assessment: what each member does when the pain begins and

what happens to the arguing. It beats A because recommending couples counseling acts on a hypothesis before the family pattern has been explored and may leave the child's role in it unaddressed.

C **Incorrect** (trap: Right action, wrong time). Relaxation training can help a child manage pain and may be added later. Starting with it treats the stomachaches as the child's individual problem before the family pattern around them is understood.

Takeaway: When a child's symptom seems tied to family conflict, assess the family's interaction around the symptom before choosing an intervention.

Sources: Minuchin, S., Rosman, B. L., & Baker, L. (1978). Psychosomatic Families: Anorexia Nervosa in Context. Harvard University Press, detouring of parental conflict through a child's symptom
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85. ASM-1535 (rev 1) · Best answer: C

Four months after a parent lost a factory job, a family of five meets with a social worker at a community agency to assess its strengths. Which observation **BEST** indicates a key family resilience process?

A. The parents keep the job loss from the children to protect them
B. Each member handles worries privately to avoid burdening others
C. Members describe the job loss as a challenge they will face together
D. The family keeps every routine exactly as it was before the loss

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Protecting children is a caring motive, but Walsh identifies clear, age-appropriate sharing of information as a resilience process. Secrecy often leaves children confused and anxious and keeps the family from facing the problem as a unit.

B **Incorrect** (trap: Good but not best). Shielding others from worry is well meant, yet open emotional sharing is one of the communication processes that support family resilience. Coping alone isolates members during a crisis.

C **Correct.** Walsh's family resilience framework identifies key processes in belief systems, organization and communication. Making meaning of adversity, seeing a crisis as a shared challenge the family can meet together, is a core belief-system process (Walsh, 2016). It beats D because stability can look like strength, but Walsh describes flexibility, the ability to reorganize roles and routines under new demands, as the resilient pattern; rigidly keeping everything the same after a major income loss can block needed adaptation.

D **Incorrect** (trap: Wrong model or concept). Some routines provide stability, but resilience depends on flexibility: changing roles, budgets and schedules to fit new circumstances. Keeping every routine exactly the same after a major income loss suggests rigidity rather than adaptation.

Takeaway: Family resilience shows in shared meaning-making, flexible organization and open communication, not in secrecy, private coping or rigid routines.

Sources: Walsh, F. (2016). Strengthening Family Resilience (3rd ed.). Guilford Press, key processes in family resilience

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86. ASM-1536 (rev 1) · Best answer: A

A 24-year-old client with obsessive-compulsive disorder lives with a parent. In a joint session, the parent says they rewash the client's clothes on request, answer the same reassurance questions for up to 2 hours each night, and no longer invite guests to the home. The client's rituals have increased over the past year. Which response is **MOST** appropriate?

A. Plan with both how the parent will reduce these accommodations

B. Begin exposure exercises with the client in individual sessions

C. Refer the parent to a caregiver support group for stress

Why each option is right or wrong

A **Correct.** Family accommodation, such as excessive reassurance or washing on a relative's behalf, is linked to more severe obsessive-compulsive symptoms and can predict poorer treatment outcomes, so reducing it is a key treatment goal (Lebowitz, Panza & Bloch, 2016). This parent's rewashing, nightly reassurance and avoidance of guests are feeding the client's rituals. It beats B because exposure work is a core treatment, but if the household keeps supplying reassurance and rewashing, gains are undercut; planning the reduction with both client and parent addresses the maintaining pattern directly.

B **Incorrect** (trap: Good but not best). Exposure with response prevention is a first-line treatment and will likely be part of care. Doing it only in individual sessions leaves the parent's nightly reassurance and rewashing in place, which keeps reinforcing the rituals between sessions.

C **Incorrect** (trap: Hands off too soon). The parent's strain deserves attention, and support may be added. A support group alone does not change the accommodating behaviors that are maintaining the client's symptoms.

Takeaway: Family accommodation can maintain OCD and anxiety symptoms; plan its reduction with the client and family rather than treating the client in isolation.

Sources: Lebowitz, E. R., Panza, K. E., & Bloch, M. H. (2016). Family accommodation in obsessive-compulsive and anxiety disorders: A five-year update. Expert Review of Neurotherapeutics, 16(1), 45-53, whole article

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87. ASM-1628 (rev 1) · Best answer: C

In a family session about a parent's alcohol use, an 11-year-old cracks jokes and makes faces whenever the adults' voices rise. The family laughs, and the tension drops for a moment. Teachers call the child the class comedian, but the child tells the social worker privately, 'I'm always scared something bad will happen.' In Wegscheider-Cruse's model, which family role is this child **MOST** likely filling?

A. Hero
B. Scapegoat
C. Mascot
D. Lost child

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The hero overachieves and takes on responsibility, giving the family pride and a sense that things are fine. This child manages tension with jokes rather than with achievement or caretaking.

B **Incorrect** (trap: Wrong model or concept). The scapegoat draws attention away from the drinking by acting out and getting into trouble, so the family blames the child. This child's behavior earns laughter and relief, not blame.

C **Correct.** Wegscheider-Cruse described roles that children take on in families affected by addiction. The mascot uses humor and antics to relieve family tension and pull attention away from the problem, often while hiding fear. The child's joking when voices rise, the family's relief and the private fear all fit this role. It beats B because the scapegoat diverts attention through defiance and trouble that earns blame, not through humor that earns laughter (Wegscheider, 1981).

D **Incorrect** (trap: Wrong model or concept). The lost child withdraws, stays quiet and avoids notice to keep from adding stress. This child does the opposite, stepping into the center of attention whenever conflict rises.

Takeaway: In addiction-affected families, the mascot eases tension with humor, the hero achieves, the scapegoat acts out, and the lost child withdraws.

Sources: Wegscheider, S. (1981). Another Chance: Hope and Health for the Alcoholic Family. Science and Behavior Books, family roles
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88. ASM-1634 (rev 1) · Best answer: C

At a family intake, parents say their 15-year-old's silence and closed bedroom door force them to keep checking in and asking questions. The teen says the constant questions are the reason for staying in the room. Each side asks the social worker to say who is right. What should the social worker do **FIRST** to understand how the family functions?

A. Determine which family member's behavior started the conflict
B. Coach the parents to give the teen more privacy
C. Map how each person's response sets off the other's

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Searching for who started it reflects linear cause-and-effect thinking. In an ongoing pattern each behavior maintains the other, so naming a first cause adds blame without explaining why the cycle continues.

B **Incorrect** (trap: Right action, wrong time). Giving the teen more privacy might later be part of a plan. Recommending it at intake sides with the teen and skips the assessment of how both sides keep the pattern going.

C **Correct.** Systems thinking treats family problems as circular: each person's behavior is both a response to and a trigger for the others'. Here the parents' questions and the teen's withdrawal feed each other, so the first assessment task is to trace that repeating sequence rather than assign blame. It beats B because coaching the parents is an intervention that takes the teen's side before the pattern has been mapped, and the teen's part in it would go unexamined (Nichols & Davis, 2020).

Takeaway: Assess family problems as circular sequences in which each member's behavior both triggers and responds to others', rather than seeking a first cause.

Sources: Nichols, M. P., & Davis, S. D. (2020). Family Therapy: Concepts and Methods (12th ed.). Pearson, systems concepts: circular causality

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89. ASM-1641 (rev 1) · Best answer: A

Parents who have decided to divorce after years of frequent shouting matches ask the social worker how to protect their children, ages 7 and 11. Which step would be **MOST** helpful to the children's long-term adjustment?

A. Help the parents keep the children out of their disputes

B. Help the parents keep the children in one home and school

C. Arrange individual therapy for each child right away

Why each option is right or wrong

A **Correct.** Children's adjustment after divorce depends heavily on how much conflict between parents they are exposed to. In one long-term study, children from high-conflict marriages that ended did nearly as well as peers from intact families, since the divorce ended their exposure. After years of shouting, the most protective step is a plan that keeps the children out of the parents' disputes. It beats B because a stable home and school help, but continued exposure to conflict would undercut those benefits (Amato, Loomis & Booth, 1995).

B **Incorrect** (trap: Good but not best). Keeping the same home and school offers continuity and is worth doing when possible. It protects less than reducing exposure to parental conflict, which keeps harming children whether or not their surroundings stay the same.

C **Incorrect** (trap: Hands off too soon). Individual therapy may help a child who shows distress after the divorce. Starting it for both children before any sign of need does less to protect them than changing the conflict they are exposed to.

Takeaway: After divorce, limiting children's exposure to conflict between parents protects their adjustment more than any single change in their surroundings.

Sources: Amato, P. R., Loomis, L. S., & Booth, A. (1995). Parental divorce, marital conflict, and offspring well-being during early adulthood. Social Forces, 73(3), 895-915, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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90. ASM-1552 (rev 1) · Best answer: B

A school social worker meets with a parent whose 10-year-old has missed 6 days this month. The family lost their apartment and is staying with relatives across town, about an hour away by bus. The parent wants the child to stay at the current school but assumes the move requires a transfer. The social worker has confirmed the family has no other housing. What should the social worker do **NEXT**?

A. Help the parent enroll the child at the school near the relatives
B. Connect the family with the district's homeless education liaison
C. Refer the family to a housing agency before addressing school
D. Start a truancy referral as the attendance rules require

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Enrolling near the relatives is a lawful option, but the parent prefers the current school and the law presumes that staying is in the child's best interest. A transfer fits if the family chooses it after learning the options.

B **Correct.** Under the McKinney-Vento Act, children who share others' housing because they lost their own are homeless (42 U.S.C. 11434a(2)(B)(i)). They may stay in their school of origin when it serves their best interest, with transportation provided, and each district has a liaison who arranges this (42 U.S.C. 11432(g)(1)(J), (g)(3)(A)). Connecting the family with the liaison addresses both the absences and the parent's wish. It beats A because no transfer is required, and changing schools would add another loss for a displaced child.

C **Incorrect** (trap: Right action, wrong time). Housing help matters and should follow, but it does not solve the immediate barrier: the child is missing school now because of the distance. School-stability rights apply right away, without waiting for new housing.

D **Incorrect** (trap: Misreads law or ethics). A truancy referral treats absences caused by homelessness as willful. The McKinney-Vento Act directs schools to remove barriers to attendance for homeless students; truancy steps fit only when absences have no such cause.

Takeaway: Children doubled up with others after losing housing count as homeless and may stay in their school of origin with transportation arranged.

Sources: McKinney-Vento Homeless Assistance Act, 42 U.S.C. 11434a(2)(B)(i); 42 U.S.C. 11432(g)(1)(J), (g)(3)(A)
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91. ASM-1553 (rev 1) · Best answer: C

At a family resource center, a parent asks for help applying for food assistance for their 4-year-old, a U.S. citizen. Partway through the form, the parent stops and says, 'I don't have papers. If I apply, could they use it against me or deport me?' The parent wants the child to eat well but is afraid to continue. What is the **BEST** action by the social worker?

A. Assure the parent that a child's application cannot affect them
B. Encourage finishing the application since the child is eligible
C. Connect the parent with a nonprofit immigration legal service
D. Explore the parent's past experiences with government agencies

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). The parent needs accurate information, but predicting immigration consequences is legal advice. Rules on public benefits and immigration are complex and have changed over time, so a blanket assurance could be wrong.

B **Incorrect** (trap: Overrides the client's choice). The child may well qualify, yet pushing ahead overrides the fear the parent just named. The parent decides once reliable legal information is available; the social worker can then help finish the application.

C **Correct.** Immigration status and displacement shape whether families use services, and fear of immigration consequences often keeps mixed-status families from benefits their citizen children can receive. The parent's question is a legal one whose answer depends on individual facts and changing rules. Social workers practice within their competence (NASW Code 1.04) and refer when specialized knowledge is needed (1.16). It beats A because a confident assurance the social worker cannot guarantee could mislead the parent about a real legal risk.

D **Incorrect** (trap: Good but not best). Exploring past experiences can build trust over time. The parent has already named a specific fear about deportation, and exploring history does not answer the question that is blocking the decision.

Takeaway: When immigration fears block access to services, connect families with qualified immigration legal help instead of predicting legal outcomes.

Sources: NASW Code of Ethics (2021), 1.04(a) Competence; NASW Code of Ethics (2021), 1.16(a) Referral for Services
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92. ASM-1554 (rev 1) · Best answer: A

At a refugee resettlement youth program, a 16-year-old who arrived with family 4 years ago now refuses to speak the family's language or attend community celebrations, calling them 'embarrassing.' Classmates mock the teen's accent, and the teen has stopped trying to join school activities. The teen says they do not fit in anywhere. Which acculturation strategy is the teen **MOST** likely showing?

A. Marginalization
B. Assimilation
C. Separation
D. Integration

Why each option is right or wrong

A **Correct.** Berry's (1997) model crosses two questions: is the heritage culture kept, and is contact with the larger society sought? Marginalization is low on both and often arises from enforced cultural loss combined with exclusion. This teen rejects the family's language and celebrations and, after being mocked, has stopped trying to join school activities. It beats B because assimilation means giving up the heritage culture while actively taking part in the host society; this teen has withdrawn from both.

B **Incorrect** (trap: Wrong model or concept). Assimilation fits the rejection of the family's language, but it also requires seeking daily participation in the new culture. The teen has stopped joining school activities after exclusion, so the host-culture side is missing.

C **Incorrect** (trap: Wrong model or concept). Separation means holding on to the heritage culture while avoiding the larger society. This teen is pulling away from the heritage culture too, so the first half of the pattern does not fit.

D **Incorrect** (trap: Wrong model or concept). Integration combines keeping the heritage culture with taking part in the larger society and is linked to the best adaptation. The teen is engaged in neither culture, the opposite of this pattern.

Takeaway: Berry's acculturation strategies rest on two dimensions, keeping heritage culture and joining the larger society; losing both is marginalization.

Sources: Berry, J. W. (1997). Immigration, acculturation, and adaptation. Applied Psychology: An International Review, 46(1), 5-34, acculturation strategies

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93. ASM-1655 (rev 2) · Best answer: A

A 56-year-old client has lived in emergency shelters for 8 years. Shelter staff tell the social worker that the client has fallen twice this month, struggles to climb to an upper bunk, and recently wet the bed. Staff add that the client is 'too young' for the agency's senior services. What is the **MOST** appropriate action?

A. Arrange a geriatric assessment of function, falls and continence

B. Move the client to a lower bunk to prevent further falls

C. Plan to apply for senior services once the client reaches age 60

Why each option is right or wrong

A **Correct.** People who have been homeless for years often develop geriatric conditions early. In a study of shelter residents averaging about 56 years old, about half had fallen in the past year or had incontinence, and functional impairment and incontinence exceeded rates in housed cohorts about 20 years older (Brown et al., 2012). This client's falls, mobility trouble and new incontinence call for a geriatric assessment of function now, whatever the calendar age. It beats B because a lower bunk reduces one hazard but leaves the causes of the falls and incontinence unexamined.

B **Incorrect** (trap: Good but not best). A lower bunk is a sensible safety step that can be done right away, but it treats one symptom. Repeated falls, mobility loss and new incontinence together need assessment to find causes and plan care.

C **Incorrect** (trap: Ignores risk). Waiting for an age cutoff accepts the staff's view that the client is too young for these problems. Long-term homelessness speeds up aging, so the client needs assessment now, not in several years.

Takeaway: Long-term homelessness brings geriatric conditions early; assess function, falls and continence based on need, not calendar age.

Sources: Brown, R. T., Kiely, D. K., Bharel, M., & Mitchell, S. L. (2012). Geriatric syndromes in older homeless adults. Journal of General Internal Medicine, 27(1), 16-22, whole article

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94. ASM-1555 (rev 2) · Best answer: C

A 15-year-old tells the school social worker that an online contact who seemed to be a peer received a private image from the teen and now demands money. The contact threatens to send the image to the teen's followers by tonight. Shaking, the teen says they want to delete everything and pretend it never happened. What should the social worker do **FIRST**?

A. Help the teen delete the chat and close the account

B. Ask the teen what led them to share the image

C. Help the teen save the messages and file a report

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Wanting to erase the problem is understandable, but deleting the messages and account destroys evidence. Blocking the contact is advised only after the profile and messages are saved for a report.

B **Incorrect** (trap: Right action, wrong time). Asking why the teen shared the image can sound blaming and delays action before tonight's deadline. Understanding what happened can come later, in a supportive conversation, after the report is made.

C **Correct.** Sextortion of a minor is a crime. FBI guidance tells young victims to report it to the FBI or NCMEC's CyberTipline, to block the offender without deleting the profile or messages, and not to pay; NCMEC can also help remove images (FBI, 2020; FBI Albuquerque press release). Saved messages help investigators identify the offender, and tonight's deadline makes prompt reporting the priority. It beats A because deleting everything, the teen's instinct, destroys the evidence needed to stop the threat.

Takeaway: In sextortion of a minor, preserve the evidence and report promptly; deleting messages or pausing to explore blame helps the offender.

Sources: FBI (2020). Sextortion, fbi.gov guidance for young victims: report; NCMEC CyberTipline; Take It Down; FBI Albuquerque Field Office (2022). FBI Media Alert: FBI in New Mexico Warns of Significant Increase in Sextortion, Press release, Dec. 19, 2022: block the predator but do not delete the profile or messages

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95. ASM-1556 (rev 1) · Best answer: B

What is the status of internet gaming disorder in the DSM-5-TR?

A. An official diagnosis among the addictive disorders
B. A condition for further study in Section III
C. A specifier used with gambling disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Only gambling disorder appears as a non-substance diagnosis in the substance-related and addictive disorders chapter. Internet gaming disorder was proposed but placed in Section III pending more research.

B **Correct.** DSM-5-TR places internet gaming disorder in Section III, Conditions for Further Study, meaning the evidence is not yet strong enough for official clinical use (DSM-5-TR). Gambling disorder remains the only behavioral addiction among the official diagnoses. Social workers can still assess how gaming affects sleep, school, work and relationships, and diagnose co-occurring conditions that meet criteria. It beats A because many people assume gaming joined gambling as an official diagnosis, and it has not.

C **Incorrect** (trap: Near-miss diagnosis). Gambling disorder has course and severity specifiers, none of which concern gaming. Gaming problems are described separately, as a proposed condition in Section III rather than a feature of gambling disorder.

Takeaway: Internet gaming disorder is a DSM-5-TR condition for further study, not an official diagnosis; assess gaming's effects and co-occurring disorders.

Sources: DSM-5-TR (APA, 2022), Section III, Conditions for Further Study: Internet Gaming Disorder; DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Gambling Disorder
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96. ASM-1557 (rev 1) · Best answer: D

At a health center visit, a client says their partner always knows where they have been and quotes their private texts back to them. The client found an unfamiliar app on their phone and wants the social worker's help deleting it right away. The client adds that the partner 'explodes' when not in control. What should the social worker do **FIRST**?

A. Help the client remove the app during the visit
B. Urge the client to report the tracking to police now
C. Explain that the partner's monitoring is a form of abuse
D. Plan for safety, using a device the partner cannot see

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Removing the app may be the right step later, and it is the client's choice. Done before a plan exists, it can tip off a controlling partner and erase evidence that the monitoring happened.

B **Incorrect** (trap: Overrides the client's choice). Police involvement is the client's decision and may become part of the plan (NASW Code 1.02). Urging an immediate

report overrides that choice and could raise danger if the partner learns of it before safety steps are ready.

C **Incorrect** (trap: Good but not best). Naming the behavior as abuse can validate the client's experience, but it does not address the immediate risk. The client is about to change the phone, so planning for safety is the pressing need.

D **Correct.** Technology can extend partner abuse: tracking apps let a partner follow location and messages. Survivor-safety guidance warns that some abusers escalate when monitoring stops, so safety planning comes before removal, and planning should happen on a device the partner cannot access (NNEDV Safety Net, 2022). This partner reacts explosively to losing control. It beats A because deleting the app during the visit, though it is what the client asked for, could alert the partner and destroy evidence before a plan is in place.

Takeaway: When a partner monitors a client's phone, plan for safety on a secure device before removing tracking tools, since removal can trigger escalation.

Sources: NNEDV Safety Net Project (2022). Stalkerware: Phone Surveillance & Safety for Survivors, removing stalkerware; safety planning first; NASW Code of Ethics (2021), 1.02 Self-Determination
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.21 Technology and social media effects on clients · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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97. ASM-1656 (rev 2) · Best answer: B

A 15-year-old who uses a wheelchair has few accessible activities nearby and spends most evenings in an online group for teens with the same condition. The teen calls these group members their closest friends. After a news story about online predators, the parent asks the social worker to support removing the teen's internet access. What is the **MOST** appropriate response?

A. Support the parent's plan until the teen is older
B. Review with the teen and parent the group's benefits and risks
C. Tell the parent the group is a strength that should continue
D. Suggest an in-person activity to replace the online group

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Removing access may feel protective, but it cuts the teen off from their main peer support and ignores the teen's view. Limits might fit later if assessment finds real danger, such as an adult seeking private contact.

B **Correct.** Technology brings both benefits and risks, so assessment should weigh both before anyone acts. The U.S. Surgeon General's 2023 advisory notes that social media can give youth positive community with others who share identities, abilities and interests, and it also names risks such as harassment and predatory contact. For a teen with few accessible local options, this group may be a key support. Exploring with the teen and parent what the group provides and what safety habits the teen uses lets them build a plan together. It beats C because endorsing the group before learning how it works dismisses the parent's real concern and skips the safety review.

C **Incorrect** (trap: Too directive). The group may well be protective, but declaring so before learning how the teen uses it skips assessment and brushes aside the parent's fear. A safety review comes before advising either way.

D **Incorrect** (trap: Good but not best). In-person activities are worth exploring, but few are accessible to this teen, and a replacement assumes the online friendships have no value. Adding options can come after the group's role is understood.

Takeaway: Assess both the supportive value and the risks of a client's online connections before endorsing or restricting them.

Sources: U.S. Surgeon General (2023). Social Media and Youth Mental Health: The U.S. Surgeon General's Advisory. U.S. Department of Health and Human Services, potential benefits and potential risks of social media
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98. ASM-1558 (rev 1) · Best answer: A

A social worker on a mobile outreach team serves a client with schizophrenia who takes an antipsychotic and lives alone in a top-floor apartment without air conditioning. A week of extreme heat is forecast. The client declines the city cooling center, saying crowds make the voices worse, but agrees the apartment 'gets like an oven.' What is the **BEST** plan?

A. Arrange a quieter cool place and daily check-ins during the heat
B. Ask the prescriber to hold the antipsychotic during the heat wave
C. Respect the refusal and review heat safety tips with the client

Why each option is right or wrong

A **Correct.** Extreme heat is an environmental hazard that falls hardest on people with serious mental illness. Antipsychotics can impair sweating and temperature regulation, and CDC advises clinicians to help patients reach a cool environment and name someone to check on them (CDC, Heat and Medications). This client lives alone in a hot apartment and turned down one option for a clinical reason, not all help. A quieter cooled space the client accepts, with daily contact, respects that choice while reducing risk. It beats C because tips alone leave a high-risk client in dangerous heat.

B **Incorrect** (trap: Ignores risk). Stopping an antipsychotic abruptly risks relapse, and CDC guidance cautions against stopping medications without a plan. Asking the prescriber about heat precautions is reasonable, but holding the medication is not the answer.

C **Incorrect** (trap: Good but not best). Respecting the refusal of the cooling center is right, but stopping there accepts a serious risk. The client has not refused all help, and impaired heat regulation makes advice alone insufficient.

Takeaway: During extreme heat, clients taking antipsychotics need an active plan for cooling and check-ins, built around what the client will accept.

Sources: CDC (Centers for Disease Control and Prevention) (2025). Heat and Medications - Guidance for Clinicians, Antipsychotics: interference with central thermoregulation; patient planning
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99. ASM-1559 (rev 1) · Best answer: C

During a home visit, a social worker sees a 2-year-old pick paint chips from a peeling windowsill and put them in their mouth. The family rents a house built in 1950. The parent says the child has been more irritable lately, sleeps poorly, and often has stomachaches. What should the social worker do **FIRST**?

A. Teach the parent calming routines for the irritability
B. Ask the landlord to repaint the windowsill
C. Help the parent arrange a blood lead test with the pediatrician

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Calming routines may help irritability from many causes, but they treat a symptom while possible lead exposure continues. Behavioral support fits after a medical cause has been checked.

B **Incorrect** (trap: Right action, wrong time). The hazard needs correcting, and renters can ask a landlord for a lead inspection. Testing the child comes first, though, because exposure may already have caused harm that needs medical follow-up.

C **Correct.** Peeling paint in homes built before 1978 is a major source of childhood lead exposure, and CDC states that no safe blood lead level in children has been identified. Even low levels are linked to learning and behavior problems. A blood lead test is the best way to learn whether this child has been exposed, so the social worker links the family to the pediatrician (CDC, Childhood Lead Poisoning Prevention). It beats B because fixing the windowsill matters but does not show whether the child already needs treatment.

Takeaway: When a young child may be eating old paint, arrange blood lead testing first; no safe blood lead level in children has been identified.

Sources: CDC (Centers for Disease Control and Prevention) (2025). About Childhood Lead Poisoning Prevention, sources (pre-1978 paint); no safe blood lead level; blood lead testing
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100. ASM-1560 (rev 2) · Best answer: B

A 20-year-old tells a social worker at the campus counseling center that worry about climate change has grown since a summer of wildfires near home. The student keeps up with classes, sleeps a little less after reading the news, and recently joined a campus sustainability group. The student asks, 'Is something wrong with me?' What is the **MOST** appropriate response?

A. Challenge the catastrophic thoughts with cognitive restructuring
B. Frame the concern as understandable and build meaning-focused coping
C. Refer the student for a medication evaluation for anxiety

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Cognitive restructuring fits when thoughts are distorted or exaggerated. Worry about climate risks the student has seen firsthand is realistic, so disputing it can feel invalidating and misses the coping need.

B **Correct.** Climate change is a real environmental threat, and worry about it can be a proportionate response rather than a disorder. This student's worry follows actual wildfire exposure, functioning is intact, and the student is already taking constructive action. Research suggests meaning-focused coping, such as finding hope and purpose through action, best supports both engagement and well-being (Clayton, 2020). It beats A because cognitive restructuring targets distorted thinking, and concern about documented climate risks is not a distortion.

C **Incorrect** (trap: Hands off too soon). A medication evaluation fits when distress meets criteria for an anxiety disorder and impairs functioning. This worry is focused on a real threat and the student functions well, so referral would pathologize a proportionate reaction (DSM-5-TR).

Takeaway: Proportionate worry about real climate threats is not pathology; support constructive, meaning-focused coping and watch for impairment.

Sources: Clayton, S. (2020). Climate anxiety: Psychological responses to climate change. Journal of Anxiety Disorders, 74, 102263, coping strategies; meaning-focused coping; DSM-5-TR (APA, 2022), Anxiety Disorders: Generalized Anxiety Disorder

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101. ASM-1657 (rev 1) · Best answer: D

Two days after an ice storm knocked out power, a social worker makes a scheduled home visit. A gasoline generator is running in the attached garage. The parent and two children report headaches and nausea, and the infant seems unusually sleepy. The parent says everyone feels better after time outside. What should the social worker do **FIRST**?

A. Ask each family member about fever, cough and other flu signs
B. Urge the parent to take the infant to the pediatrician today
C. Have the parent move the generator outdoors, away from windows
D. Get the family out of the house and call 911

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Asking about flu signs would fit if the illness had no clear pattern. Here several people share symptoms that ease outdoors while a generator runs in the garage. More questions delay leaving a possibly toxic home.

B **Incorrect** (trap: Ignores risk). Same-day care for the infant is too slow and leaves everyone else in the house. Possible carbon monoxide poisoning with a drowsy infant needs immediate removal from the source and emergency care.

C **Incorrect** (trap: Right action, wrong time). Moving the generator outside is necessary, but it is a later step. Gas already in the house keeps harming the family, and the drowsy infant needs emergency care now. Fire or emergency crews can then clear the home.

D **Correct.** Symptoms shared by several people in one home, easing away from the house, while a generator runs indoors point to carbon monoxide poisoning. The Centers for Disease Control and Prevention (CDC) warns never to run a generator inside a home or garage, even with doors open, and notes that infants are at higher

risk; poisoning can cause fainting and death, and treatment calls for oxygen. Leaving the house stops the exposure, and emergency services can treat the infant and clear the home. It beats C because moving the generator does not remove gas already inside or treat people who are already ill.

Takeaway: Shared headaches, nausea or drowsiness in a home with fuel-burning equipment suggest carbon monoxide; get everyone out and call emergency services.

Sources: Centers for Disease Control and Prevention (CDC). Carbon Monoxide Poisoning Basics (2024), symptoms; higher-risk groups; generator safety; Centers for Disease Control and Prevention (CDC). Clinical Guidance for Carbon Monoxide Poisoning Following Disasters and Severe Weather (2024), recognition and oxygen treatment

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102. ASM-0108 (rev 1) · Best answer: B

A hospital social worker meets with an 81-year-old patient who lives alone and was admitted after a fall. The team recommends assisted living, but the patient insists on going home. A screening last week showed some memory problems, and the patient cannot explain how they would manage their medications. The patient's adult child asks the social worker to arrange placement. What should the social worker do **FIRST**?

A. Arrange home services so the patient can return home

B. Assess the patient's ability to make this specific decision

C. Begin the assisted living placement the adult child requested

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). If the patient has capacity, supporting a return home with services respects self-determination even when the choice is risky. Acting on it before the doubts about understanding and medication management are assessed could leave the patient unsafe.

B **Correct.** Adults are presumed to have decision-making capacity, but when there is concrete reason for doubt, capacity for the specific decision is assessed before acting either way (Appelbaum, 2007). Here, memory problems and the patient's inability to explain medication management raise that doubt. Capacity covers the abilities to understand, appreciate, reason and express a choice. It beats A because arranging a discharge home honors the stated wish before confirming the patient grasps the risks; if capacity is present, A becomes the right next step (NASW Code 1.02, 1.14).

C **Incorrect** (trap: Overrides the client's choice). Placing the patient because a family member asks overrides a choice that may be the patient's right to make. A family's wishes do not replace the patient's decision unless the patient lacks capacity and a legally authorized person decides.

Takeaway: Presume capacity, but when there is reason for doubt, assess decision-specific capacity before honoring or overriding a risky choice.

Sources: Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, whole article; NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.14
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103. ASM-1115 (rev 2) · Best answer: B

After a stroke, a patient with left-side weakness wants to go home alone. The patient correctly repeats the team's explanation that the weakness makes walking unsafe without help. The patient then says, 'That's true for other stroke patients, but my leg is fine,' yet needs two staff to stand. Which decision-making ability is **MOST** likely impaired?

A. Understanding
B. Appreciation
C. Reasoning

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Understanding means grasping and restating the relevant information. The patient repeats the team's explanation correctly, which shows this ability is intact.

B **Correct.** Capacity rests on four abilities: communicating a choice, understanding the information, appreciating how it applies to oneself, and reasoning about options (Appelbaum, 2007). The patient restates the risk accurately, so understanding is intact, but denies that it applies to their own leg despite needing two staff to stand. That gap is appreciation. It beats A because understanding is shown by restating the facts, which the patient does. Capacity is decision-specific, so this finding concerns the discharge decision, not every choice.

C **Incorrect** (trap: Wrong model or concept). Reasoning means comparing options and their consequences logically. The core problem comes earlier: the patient does not accept that the risk applies to them, so the reasoning starts from a false premise.

Takeaway: A person who can restate risks but denies they apply to themselves lacks appreciation, one of the four capacity abilities.

Sources: Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, Legally relevant criteria for decision-making capacity
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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104. ASM-1116 (rev 1) · Best answer: C

On a medical unit, a 78-year-old patient treated for a urinary tract infection is confused each evening and refuses the planned move to a rehabilitation facility. Each morning the patient is alert, oriented and asking about home. The transfer is planned for 4 days from now. What should the social worker do **FIRST**?

A. Ask the adult child to make the transfer decision
B. Record the evening refusal as the patient's decision
C. Revisit the decision with the patient early in the day

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A family member may act as surrogate if the patient lacks capacity for the decision. That has not been shown, and the patient is clear each morning, so turning to the adult child now sidelines a patient who may decide for themselves.

B **Incorrect** (trap: Skips assessment). A refusal given while confused may not reflect the patient's real wishes. Treating it as final skips an assessment during the patient's clearer hours.

C **Correct.** Capacity can fluctuate, so it is best assessed when the person is at their clearest, especially when the decision can wait (Appelbaum, 2007). The confusion appears in the evening, a pattern typical of delirium, which DSM-5-TR describes as fluctuating over the day, and the transfer is 4 days away. Revisiting the move early in the day lets the patient make their own decision before anyone else steps in. It beats A because a surrogate is a later step, used only if the patient lacks capacity for this choice even when most alert.

Takeaway: When capacity fluctuates and the decision can wait, assess it during the person's clearest periods before involving a surrogate.

Sources: Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, Legally relevant criteria for decision-making capacity; DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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105. ASM-1117 (rev 1) · Best answer: B

A client with schizophrenia served by an assertive community treatment team asks to reduce home visits from daily to weekly. For 8 months, the client has kept a daily symptom log, twice called the team when sleep loss and suspicious thoughts began, and followed the agreed steps each time. A teammate wants to keep daily visits because of a hospitalization 2 years ago. What is the **BEST** response?

A. Keep daily visits until the client has a full year of stability
B. Plan a gradual step-down built on the client's warning-sign log
C. Transfer the client from the team to routine clinic care

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Caution after a past hospitalization is understandable. Setting an arbitrary waiting period, though, disregards 8 months of shown self-monitoring and overrides a reasonable request without a current reason.

B **Correct.** Self-determination includes the right to less intensive services when the client can manage safely (NASW Code 1.02). Monitoring one's own early warning signs is a recognized relapse-prevention skill (Birchwood et al., 1989), and this client has shown it for 8 months, twice catching early signs and acting on the plan. A gradual step-down that keeps the log and call plan in place honors the request while managing risk. It beats A because holding to daily visits rests on a 2-year-old event and ignores current evidence that the client monitors symptoms reliably.

C **Incorrect** (trap: Hands off too soon). Moving straight to routine clinic care removes intensive support all at once. A stepped reduction lets the team confirm that self-monitoring holds up with less contact before any transfer.

Takeaway: Shown ability to monitor one's own warning signs supports honoring a request for less intensive care through a planned step-down.

Sources: NASW Code of Ethics (2021), 1.02; Birchwood, M., Smith, J., Macmillan, F., Hogg, B., Prasad, R., Harvey, C., & Bering, S. (1989). Predicting relapse in schizophrenia: The development and implementation of an early signs monitoring system using patients and families as observers. Psychological Medicine, 19(3), 649-656, Early Signs Scale

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106. ASM-1118 (rev 2) · Best answer: A

A hospital patient with advanced major neurocognitive disorder needs a decision about a feeding tube. The team's assessment finds the patient cannot describe the choice or its risks after repeated simple explanations. The patient's health care agent, named in an advance directive, reports that the patient often said they never wanted 'to be kept alive by machines.' A nurse suggests seeking a court-appointed guardian. What should the social worker do **NEXT**?

A. Help the agent decide based on the patient's stated wishes

B. Support seeking a court guardian to replace the agent

C. Request an ethics consultation to settle the patient's care

Why each option is right or wrong

A **Correct.** When a patient lacks capacity for a decision, a valid health care agent decides using substituted judgment: what the patient would choose, based on known wishes and values (Beauchamp & Childress, 2019). The team found the patient cannot understand the choice, and the agent reports a clear, repeated wish to avoid life support by machines. Helping the agent apply that wish honors the patient's self-determination through the person the patient chose. It beats B because guardianship is meant for patients with no valid or suitable surrogate, and this patient chose an agent who knows their wishes. A best-interest standard would apply only if those wishes were unknown.

B **Incorrect** (trap: Misreads law or ethics). Guardianship is a court process for people with no valid surrogate or with a surrogate who is unsuitable. This patient named an agent in an advance directive, so a petition adds delay and cost and sidelines the patient's own choice of decision-maker.

C **Incorrect** (trap: Good but not best). Ethics consultation helps when surrogates and teams disagree or a patient's wishes are unclear. Here a valid agent reports clear, repeated wishes and no one disputes the agent's authority, so sending the decision to a committee adds delay and sidelines the decision-maker the patient chose.

Takeaway: A valid health care agent decides for a patient without capacity by applying the patient's known wishes; guardianship is then unnecessary.

Sources: Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Surrogate decision making: substituted judgment and best interests

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107. ASM-1190 (rev 1) · Best answer: B

A 16-year-old has lived with the same foster parents for 4 years, and they ask to adopt. The youth tells the child welfare social worker, 'I love them, but I won't give up my name or act like my birth family is gone.' The youth wants to stay in the home. Agency staff prefer adoption as the most permanent option. What is the **MOST** appropriate next step?

A. Ask the court to waive the youth's consent to adoption

B. Review adoption and guardianship choices with the youth

C. Change the goal to independent living without a legal parent

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Some courts can waive an older child's consent, but seeking a waiver overrides a capable 16-year-old's clear wishes before anyone has explored them. It also risks a placement the youth resents and may undermine.

B **Correct.** Nearly every state requires an older child's consent to adoption, with set ages of 10 to 14 (Child Welfare Information Gateway, 2021). Self-determination also means informed choice (NASW Code 1.02). The youth's concerns can be met in more than one way: adoption does not always mean a name change, and legal guardianship keeps the home permanent while preserving identity. Reviewing both with the youth respects the youth's voice and still pursues permanency. It beats A because a waiver overrides the youth before options are explored.

C **Incorrect** (trap: Good but not best). Independent living without a legal parent gives up lifelong family ties the youth clearly wants. It honors the objection to adoption but drops permanency, when other legal options would keep the youth in this home.

Takeaway: An adolescent's objection to adoption calls for informed review of permanency options, including guardianship, not override or loss of permanency.

Sources: Child Welfare Information Gateway (2021). Consent to Adoption (State Statutes Series), Consent of the adoptee; NASW Code of Ethics (2021), 1.02
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108. ASM-1191 (rev 1) · Best answer: A

An adult client with bipolar I disorder has kept a daily mood chart for a year. Reviewing it with the social worker, the client sees that the chart rated their mood as 'normal' during the 2 weeks before last spring's manic episode, while the client's partner noticed clear changes then. The client wants to use the chart alone to decide when to call the prescriber. What is the **BEST** response?

- **A.** Plan with the client to add the partner's observations
- **B.** Support the client's plan to rely on the mood chart
- **C.** Have the partner report mood changes to the prescriber directly
- **D.** Switch the client to a more detailed daily mood scale

Why each option is right or wrong

A **Correct.** Learning to spot early relapse signs and act quickly helps prevent mania (Perry et al., 1999). Insight often fades as mania begins, though, so family members can help recognize early warning signs (Miklowitz, 2019). The chart missed the last episode while the partner saw it, so planning with the client to add the partner's observations keeps the client in charge while covering the blind spot. It beats D because a more detailed scale still depends on the same self-ratings that failed.

B **Incorrect** (trap: Ignores risk). Self-monitoring is valuable and the client's wish matters, but the chart has already missed the run-up to a manic episode. Relying on it alone ignores evidence that it misses the signs that matter most.

C **Incorrect** (trap: Overrides the client's choice). Having the partner go straight to the prescriber cuts the client out of their own monitoring and may breach trust in the relationship. The partner's observations help most when the client agrees to them and helps decide how they are used.

D **Incorrect** (trap: Misuses the technique). A finer scale feels like a fix, but the problem is reduced awareness during early mania, not the scale. More detailed self-ratings would likely miss the same changes.

Takeaway: When self-monitoring misses early relapse signs, keep the client in charge but add a trusted observer's input rather than rely on self-report alone.

Sources: Perry, A., Tarrier, N., Morriss, R., McCarthy, E., & Limb, K. (1999). Randomised controlled trial of efficacy of teaching patients with bipolar disorder to identify early symptoms of relapse and obtain treatment. BMJ, 318, 149-153, whole article; Miklowitz, D. J. (2019). The Bipolar Disorder Survival Guide (3rd ed.). Guilford Press, recognizing early warning signs; family involvement
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109. ASM-1537 (rev 1) · Best answer: C

A parent remarried 4 months ago. The new spouse, who has no children, began enforcing strict curfews and chore rules with the parent's 13-year-old. The teen now slams doors and refuses to speak to the stepparent. The couple asks the social worker how to restore order at home. Which guidance is **BEST**?

A. Have the parent and stepparent agree on rules and enforce them equally
B. Have the stepparent stay out of the teen's daily life for now
C. Have the parent handle discipline while the stepparent builds a bond

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Agreeing on rules matters, and the couple should do so privately. Having the stepparent enforce them as an equal authority this early, before a relationship with the teen exists, tends to provoke the kind of rebellion the couple is seeing.

B **Incorrect** (trap: Misuses the technique). Stepping back from discipline is wise, but withdrawing from the teen's daily life altogether gives up the chance to build the relationship that later makes the stepparent's authority possible. Connection, not distance, is the stepparent's early task.

C **Correct.** Stepfamily guidance holds that a new stepparent has little authority at first and should build a relationship before taking on discipline, while the biological parent remains the main disciplinarian (Papernow, 2013). Four months in, a stepparent enforcing strict rules has set off the teen's resistance. It beats A because a united front on rules is important, but having the stepparent enforce them equally before a bond exists produces exactly this conflict; the adults can agree on rules privately while the parent delivers them.

Takeaway: In new stepfamilies, the biological parent leads discipline while the stepparent builds a relationship first; authority grows from connection.

Sources: Papernow, P. L. (2013). Surviving and Thriving in Stepfamily Relationships: What Works and What Doesn't. Routledge, parenting challenge: stepparent role and discipline
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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110. ASM-1538 (rev 2) · Best answer: B

In the eighth session of a weekly group for 15-year-olds at a community center, members jokingly treat the social worker as a parent and compete for the worker's attention like siblings. They now share personal struggles openly and compare the group to their own households. Early sessions were marked by testing rules and challenging the worker. Which stage of group development is the group **MOST** likely in?

A. Power and control
B. Intimacy
C. Differentiation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Power and control is the earlier stage of testing rules, status struggles and challenges to the leader. The stem places that testing in the first sessions; the group has since moved into more personal, family-like relating.

B **Correct.** Garland, Jones and Kolodny's model, as presented by Hepworth et al. (2022), describes an intimacy stage in which the group develops a familial frame of reference: members relate to one another like siblings and to the leader like a parent, and personal sharing deepens. The parent-like treatment of the worker, sibling-style rivalry and open sharing fit this stage. It beats C because differentiation comes later, when members accept one another as distinct individuals and the group develops its own identity rather than a family template.

C **Incorrect** (trap: Wrong model or concept). Differentiation follows intimacy. Members then accept differences, the group has its own identity and internal frame of reference, and members rely less on the leader, unlike the parent-centered rivalry described here.

Takeaway: In the Garland, Jones and Kolodny model, the intimacy stage brings a familial frame: sibling rivalry, the leader seen as parent, deeper sharing.

Sources: Hepworth et al. (2022). Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, stages of group development (Garland, Jones & Kolodny model)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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111. ASM-1539 (rev 1) · Best answer: A

At intake, a client seeks help for anxiety after losing a job. The client mentions living with two partners in a committed polyamorous relationship. Both partners have offered to help with bills and job leads. What should the social worker do **FIRST**?

A. Include both partners, as the client defines them, in assessing supports
B. Assess whether the arrangement with both partners is causing the anxiety
C. Refer the client to a provider who specializes in polyamory

Why each option is right or wrong

A **Correct.** Cultural humility means treating clients as experts on their own lives and cultures (NASW Code 1.05(c)), and the Code calls for understanding social diversity, including marital status (1.05(d)). This client presents with job-loss anxiety and names two partners who are actively helping, so the assessment should map them as supports in the way the client defines the relationship. It beats B because singling out the relationship structure as a likely cause has no basis in the stem and signals bias; relationship stress can be explored later if the client raises it.

B **Incorrect** (trap: Skips assessment). Relationships can add stress for any client, and the topic may come up. Assuming that a polyamorous structure is the cause, when nothing in the stem points there, pathologizes a relationship form and can damage trust at intake.

C **Incorrect** (trap: Hands off too soon). Referral fits when a worker lacks competence for the presenting concern. Job-loss anxiety is ordinary practice, and referring because of

the client's relationship structure hands off the client for who they are rather than for what they need.

Takeaway: Assess consensually nonmonogamous relationships as the client defines them, including partners as supports; do not presume the structure causes the problem.

Sources: NASW Code of Ethics (2021), 1.05(c)-(d)
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112. ASM-1540 (rev 2) · Best answer: C

A social worker leads a closed 12-week bereavement group for adults whose partners have died. In week 10, members are reviewing what they have learned and planning how to say goodbye. The intake coordinator asks the social worker to add a newly bereaved client who is struggling and on the waitlist. Which action is **BEST**?

A. Add the client now so they can gain support from the group
B. Ask the members to vote on whether to admit the client
C. Offer the client individual support until the next group starts

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Group support can help a grieving client, and open groups often add members. In a closed group two weeks from ending, a new member would join a group that is saying goodbye and would disrupt the members' work on separation.

B **Incorrect** (trap: Misuses the technique). Members' views matter, but a vote shifts a clinical and structural decision onto the group and can stir guilt or conflict during termination. The leader is responsible for decisions about a closed group's membership.

C **Correct.** Closed groups keep the same members from start to finish, which supports cohesion and lets members move through the group's stages together (Toseland & Rivas, 2017). In week 10, these members are doing ending work: reviewing gains and preparing to separate. A newly bereaved member would pull the group back to beginning tasks and would join a group about to end. It beats A because the new client's need is real, but individual support now, followed by a place in the next cycle, meets it without disrupting the current members' termination.

Takeaway: Closed groups keep stable membership; near termination, meet a new client's needs outside the group rather than disrupting the ending stage.

Sources: Toseland, R. W., & Rivas, R. F. (2017). An Introduction to Group Work Practice (8th ed.). Pearson, planning the group: open and closed membership
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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113. ASM-1541 (rev 2) · Best answer: B

Four months after their infant died, a couple meets with a hospital bereavement social worker for the first time. Within minutes, each partner says the other 'doesn't care' about the baby. One partner is tearful; the other sits quietly and mentions working long hours since the death. What should the social worker do **FIRST**?

A. Encourage the quieter partner to express feelings openly in sessions
B. Invite each partner to describe how they have been carrying the loss
C. Screen both partners for prolonged grief disorder
D. Help both partners see that each grieves in a different, valid way

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Pressing the quieter partner to show feelings assumes that open expression is the healthy way to grieve. Some people grieve mainly through thinking and action, and pressure can deepen that partner's sense of not being understood while siding with one style.

B **Correct.** Each partner reads the other's grief as indifference, but the social worker has not yet heard how either one is grieving. Inviting each to describe how they have been carrying the loss lets both feel heard and brings their styles into view, perhaps talk and tears for one and work or private routines for the other. Partners often grieve in different, equally valid ways, intuitive or instrumental (Doka & Martin, 2010), and seeing this eases blame, but the reframe lands best once each style has been described. It beats D because naming the pattern first can sound like a lecture that skips their pain.

C **Incorrect** (trap: Skips assessment). Prolonged grief disorder cannot be diagnosed in adults until at least 12 months after the death, and nothing here suggests impairment beyond expected grief (DSM-5-TR). Screening now treats a couple's conflict about grieving as an individual disorder.

D **Incorrect** (trap: Right action, wrong time). This reframe often helps couples who grieve differently, and it may well fit here. Offered before either partner has described their grief, it rests on an assumption and can feel dismissive of the hurt each one is carrying.

Takeaway: When grieving partners accuse each other of not caring, first hear how each one grieves; then help them see differing styles as valid.

Sources: Doka, K. J., & Martin, T. L. (2010). Grieving Beyond Gender: Understanding the Ways Men and Women Mourn (rev. ed.). Routledge, intuitive and instrumental grieving patterns; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder

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114. ASM-1639 (rev 3) · Best answer: B

A couple with a 4-month-old, their first child, comes for a first counseling session. In the opening minutes, they say they argue about chores and sleep and fear they are 'falling out of love.' Then both turn to the social worker, waiting for a verdict. What should the social worker do **FIRST**?

A. Help the couple make a plan for sharing chores and night feedings
B. Ask each partner how life together has changed since the birth
C. Explain that satisfaction often dips with a new baby
D. Refer the couple to a specialized couples therapy program

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A fair plan for chores and night feedings is a sensible later step. Building it now assumes the arguments are only about tasks before the social worker knows how each partner sees the problem.

B **Correct.** At a first session, the first task is to hear how each partner experiences the change: sleep, roles, closeness and support since the birth. Exploring both views engages each partner and gives the social worker the facts needed to judge whether this is the common strain of new parenthood or something more. Relationship quality often declines after a first child (Doss et al., 2009), but saying so before hearing them can sound like a verdict that sidesteps their fear. It beats C because that reassurance fits only once the social worker knows the strain matches the common pattern.

C **Incorrect** (trap: Right action, wrong time). This is often true and can bring relief, and it may well fit this couple. Offered in the opening minutes, before either partner has described what is happening, it can sound like a verdict that brushes past their fear.

D **Incorrect** (trap: Hands off too soon). Specialized couples therapy may help if problems prove severe, involve violence or exceed the social worker's skills. Referring at a first session hands off concerns that have not yet been assessed and may reflect a common transition.

Takeaway: Hear each partner's view of a family transition before teaching what is typical or planning changes.

Sources: Doss, B. D., Rhoades, G. K., Stanley, S. M., & Markman, H. J. (2009). The effect of the transition to parenthood on relationship quality: An 8-year prospective study. Journal of Personality and Social Psychology, 96(3), 601-619, whole article

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115. ASM-1650 (rev 1) · Best answer: C

In the fourth session of a community agency's early-recovery group, a member mentions taking buprenorphine. Two longtime members reply that people on 'replacement drugs' are not truly sober, and several others nod. The member goes quiet. Over the past two sessions, members have made similar remarks about anyone who uses medication. What is the social worker's **BEST** response?

A. Talk with the member one-on-one once the group has ended
B. Let the debate continue so members reach their own view
C. Name the pattern and open discussion of different recovery paths
D. Suggest the member move to a recovery group for people on medication

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A private check-in is a reasonable follow-up, but the problem is a norm the whole group is building. Handling it only outside the session signals that the remarks are acceptable in group and leaves the member isolated there.

B **Incorrect** (trap: Misuses the technique). Leaving members to settle it lets the loudest voices set the norm. When a pattern shames a member and spreads misinformation about treatment, the leader steps in rather than waiting for the group to correct itself.

C **Correct.** Group norms are the shared expectations that shape what members may say and do, and leaders have the most influence on them while a group is young (Toseland & Rivas, 2017). Similar remarks across three sessions show a norm forming that shames medication use. Naming the pattern in the room and inviting talk about varied pathways corrects it for everyone; SAMHSA's recovery principles state that recovery occurs through many pathways. It beats A because a private check-in supports one member but leaves the group-level norm in place for the next member who takes medication.

D **Incorrect** (trap: Hands off too soon). Moving the member out treats the member as the problem and teaches the group that other paths are unwelcome. A specialized group could fit later if the member wants one, but it does nothing about the norm forming here.

Takeaway: When a harmful norm emerges in a group, address it with the whole group in session, not only privately with the affected member.

Sources: Toseland, R. W., & Rivas, R. F. (2017). An Introduction to Group Work Practice (8th ed.). Pearson, group dynamics: norms and social control; SAMHSA (2012) Working Definition of Recovery, Guiding principles: recovery occurs via many pathways
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116. ASM-0001 (rev 1) · Best answer: B

Which bruise location is **MOST** concerning for physical abuse in an older adult?

A. Front of the lower leg
B. Upper arm
C. Back of the hand
D. Forearm below the elbow

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Shins strike furniture, stairs and walkers often. Lower-leg bruises are among the most common accidental bruises in older adults and are weak evidence of abuse on their own.

B **Correct.** Research on older adults found that bruises on the upper arm, head, neck, torso, buttocks and soles of the feet are more often linked to abuse. Bruises on the shins, the forearm below the elbow and the back of the hand are common after accidental bumps and falls (Mosqueda, 2015). Upper-arm bruises can come from being grabbed or shaken. It beats D because the forearm below the elbow is a typical site for bruises from catching oneself in a fall or bumping into objects. A worrying location calls for careful, private assessment, not a conclusion.

C **Incorrect** (trap: Near-miss diagnosis). The backs of the hands bruise easily with fragile aging skin and everyday bumps. This location is more often accidental than abusive (Mosqueda, 2015).

D **Incorrect** (trap: Near-miss diagnosis). The forearm below the elbow is a typical site for accidental bruising, for example when someone catches themself during a fall. Location alone points away from abuse here, though the full history still matters.

Takeaway: In older adults, bruises on the upper arm, head, neck or torso raise abuse concern; shins, forearms and hands are usually accidental.

Sources: Mosqueda, L. (2015). Forensic Markers of Elder Abuse. NAPSA/NCPEA Research to Practice Brief, bruise location findings

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117. ASM-0102 (rev 1) · Best answer: A

In an emergency department, a social worker is asked to see a patient with a broken wrist. The patient says the injury came from a fall, but the physician notes bruises of different ages on both upper arms. The patient's partner stays in the room and answers most questions. What should the social worker do **FIRST**?

A. Arrange to speak with the patient alone

B. Ask about safety at home with the partner present

C. Give the patient domestic violence hotline information

Why each option is right or wrong

A **Correct.** Possible intimate partner violence should be explored in private, because asking in front of a suspected abuser can put the patient at risk and makes honest disclosure unlikely (WHO, 2013). The inconsistent history, bruises of different ages and a partner who answers for the patient are warning signs. It beats C because handing over hotline materials before a private conversation skips assessment, and the partner may see them, which can raise danger. Once alone, the social worker can ask directly and assess safety.

B **Incorrect** (trap: Ignores risk). Asking about violence while the partner listens can expose the patient to retaliation and almost guarantees a guarded answer. Direct questions about safety are right, but only once the patient can talk privately.

C **Incorrect** (trap: Right action, wrong time). Resource information belongs after a private conversation and safety assessment. Given now, it skips the assessment and could be noticed by the partner, which may increase risk (WHO, 2013).

Takeaway: When abuse indicators appear and a possible abuser is present, first create a private space before asking about violence or offering resources.

Sources: World Health Organization (2013). Responding to Intimate Partner Violence and Sexual Violence Against Women: WHO Clinical and Policy Guidelines, Recommendations 1 and 3: first-line support and clinical enquiry
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118. ASM-1093 (rev 2) · Best answer: C

When a caregiver says a young child's injury came from a fall at home, which bruise **MOST** warrants an evaluation for physical abuse?

A. Bruised shins on an active 4-year-old
B. A forehead bruise on a toddler who just began walking
C. Bruising on the ear of a 2-year-old

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Bruises over the shins are common in active preschoolers, because the shins hit furniture and the ground during ordinary play. They fit a fall explanation.

B **Incorrect** (trap: Near-miss diagnosis). Newly walking toddlers often fall forward and bruise the forehead, a bony area. This pattern is expected and fits the explanation given.

C **Correct.** Validated bruising rules such as TEN-4-FACESp flag bruises on the torso, ears or neck in children under 4, and any bruise in a young infant, as strongly associated with abuse (Pierce et al., 2021). The ear is a protected site rarely injured in ordinary falls, so a fall explanation does not fit well. It beats B because forehead bruises are expected in toddlers learning to walk; location, not just age, separates typical from concerning bruises.

Takeaway: Bruises on the ears, neck or torso of a child under 4 warrant abuse evaluation; bruises on bony, exposed areas usually reflect accidents.

Sources: Pierce, M. C., et al. (2021). Validation of a clinical decision rule to predict abuse in young children based on bruising characteristics. JAMA Network Open, 4(4), e215832, TEN-4-FACESp bruising regions
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119. ASM-1094 (rev 1) · Best answer: B

At a health clinic, a client says their partner 'grabbed my throat until I almost blacked out' last month during an argument about the client moving out. The client laughs it off: 'It only happened once, and I'm fine now.' The client still plans to leave in a few weeks. What is the social worker's **BEST** response?

A. Accept the client's view that it was a one-time event
B. Explain that strangling sharply raises the risk of homicide
C. Explore why the client sees the incident as minor

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). The client's view deserves respect, and the client decides what to do. Accepting it without sharing what is known about strangulation leaves the client choosing without a key fact about lethal danger.

B **Correct.** Prior nonfatal strangulation is one of the strongest markers that an abusive partner may later kill (Glass et al., 2008), and leaving the relationship is a period of heightened danger (Campbell et al., 2003). The client minimizes the event and plans to leave soon, so sharing this risk, then offering safety planning, supports an informed choice without overriding it. It beats C because exploring the client's view matters, but the client is making a high-stakes decision without knowing that this act signals lethal risk.

C **Incorrect** (trap: Good but not best). Understanding the client's minimization can guide later work. With a planned move-out weeks away, exploring first delays information the client needs to plan the departure safely.

Takeaway: Nonfatal strangulation and planned separation are major lethality markers in partner violence; share this risk so the client can plan safely.

Sources: Glass, N., et al. (2008). Non-fatal strangulation is an important risk factor for homicide of women. Journal of Emergency Medicine, 35(3), 329-335, whole article; Campbell, J. C., et al. (2003). Risk factors for femicide in abusive relationships: Results from a multisite case control study. American Journal of Public Health, 93(7), 1089-1097, Separation and lethality

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120. ASM-1095 (rev 1) · Best answer: A

A 10-year-old client in a child advocacy center program disclosed last month that a parent's partner sexually abused them. The partner then moved out, and the family lost much of its income. Today the client says, 'I made it up,' while looking at the floor, and adds that the parent cries every night. What is the **MOST** appropriate action for the social worker?

A. Tell the investigating caseworker about the recantation
B. Accept the recantation and note the report was likely false
C. Question the client in detail to learn which account is true

Why each option is right or wrong

A **Correct.** Recantation occurs in a meaningful share of substantiated child sexual abuse cases, especially when the child is young, the alleged abuser is a parent figure and the nonoffending caregiver is unsupportive or distressed. It does not by itself mean the first report was false (Malloy, Lyon & Quas, 2007). The partner's removal, lost income and the parent's distress are typical pressures. The caseworker needs this to protect the child. It beats C because probing for the 'true' account belongs to trained forensic interviewers, and repeated questioning can distress the child and taint later statements.

B **Incorrect** (trap: Ignores risk). Taking a child's words seriously is important, but research on recantation shows it often follows family pressure rather than a false report. Treating it as proof of fabrication could leave the child unprotected.

C **Incorrect** (trap: Outside the social work role). Wanting clarity is understandable. Detailed questioning by the social worker risks leading the child, adds pressure and can compromise the investigation; forensic interviews belong to trained staff.

Takeaway: A child's recantation under family pressure does not mean abuse did not occur; share it with investigators rather than settling the question yourself.

Sources: Malloy, L. C., Lyon, T. D., & Quas, J. A. (2007). Filial dependency and recantation of child sexual abuse allegations. Journal of the American Academy of Child and Adolescent Psychiatry, 46(2), 162-170, whole article
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121. ASM-1096 (rev 1) · Best answer: B

A home-delivered meals worker refers a 78-year-old client with mild memory problems. A new acquaintance moved into the client's home 2 months ago and now collects the mail. The electricity is about to be shut off despite a steady pension, and the bank has called about large ATM withdrawals. The client says the acquaintance 'handles everything.' What form of mistreatment is **MOST** likely occurring?

A. Self-neglect
B. Financial exploitation
C. Caregiver neglect
D. Psychological abuse

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Self-neglect could explain unpaid bills in a person with memory problems. It does not explain large withdrawals and a new housemate taking over the client's mail and affairs.

B **Correct.** Financial exploitation is the improper use of an older adult's money or property. Red flags include sudden unexplained bank activity such as large withdrawals, bills left unpaid despite adequate resources, and someone new taking control of finances (U.S. Department of Justice, Elder Justice Initiative). A new housemate who collects the mail while withdrawals rise and utilities lapse fits this pattern. It beats A because self-neglect could explain unpaid bills but not the withdrawals and the new person managing the client's affairs.

C **Incorrect** (trap: Near-miss diagnosis). Caregiver neglect is failure to meet basic needs such as food, hygiene or medical care. The facts here center on money moving out of the account, not on unmet physical care.

D **Incorrect** (trap: Near-miss diagnosis). Psychological abuse involves threats, humiliation or isolation. Control by the housemate may involve some of this, but the clearest pattern is the misuse of the client's money.

Takeaway: Unexplained withdrawals, unpaid bills despite adequate income and a new person controlling finances signal elder financial exploitation.

Sources: U.S. Department of Justice, Elder Justice Initiative (2023). Red Flags of Elder Abuse, Financial Exploitation

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122. ASM-1100 (rev 2) · Best answer: B

At a family clinic, the parent of a 4-year-old says the child touches their own genitals at nap time and recently asked why a baby sibling 'looks different.' The touching stops when the parent offers a toy. The parent's voice shakes: 'I just have a bad feeling something happened.' What should the social worker do **FIRST**?

A. Make a report of possible sexual abuse

B. Ask the parent what has made them worried

C. Ask the child directly whether anyone has touched them

D. Explain to the parent that this is typical curiosity for the age

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A report rests on reasonable suspicion, such as a disclosure, sexual knowledge beyond the child's age or coercive acts with other children. Self-touch that stops with simple redirection does not reach that threshold, though what the parent says next might. Reporting now would start an investigation on facts that do not yet point to abuse.

B **Correct.** Touching one's own genitals and curiosity about others' bodies are common in preschoolers; concern rises when behavior goes beyond the child's stage, persists despite redirection or is coercive (NCTSN, 2009). The behaviors described are typical, but the parent's sense that 'something happened' may rest on facts not yet shared, such as a new caregiver, a remark by the child or a change in mood. Exploring the worry first respects the parent and gathers the facts that decide between reassurance and a report. It beats D because reassuring before hearing what drives the worry could miss a real warning sign.

C **Incorrect** (trap: Outside the social work role). Questions about abuse belong with trained interviewers once indicators are present, and they should be open-ended. Asking a 4-year-old a pointed question now risks leading the child, adding fear and weakening any later forensic interview.

D **Incorrect** (trap: Right action, wrong time). The behaviors described are typical for a 4-year-old, so this explanation will likely be part of the answer. Offered before the

social worker learns what lies behind the parent's 'bad feeling,' it may dismiss information that matters.

Takeaway: When a parent fears abuse on the basis of age-typical behavior, first learn what drives the worry; then reassure or report based on the facts.

Sources: National Child Traumatic Stress Network (2009). Sexual Development and Behavior in Children: Information for Parents and Caregivers, Common sexual behaviors; when to be concerned
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123. ASM-1101 (rev 1) · Best answer: A

A 12-year-old with type 1 diabetes is admitted with diabetic ketoacidosis for the second time in 3 months. The parent missed 5 of 6 clinic visits despite transportation vouchers and home nurse visits, and insulin is supplied free. The parent can describe the insulin plan but says the child 'is old enough to handle it.' After meeting with the parent, what should the social worker do **NEXT**?

A. Report suspected medical neglect to child welfare authorities
B. Arrange another round of diabetes teaching for the parent and child
C. Coach the child to take over the daily insulin doses

Why each option is right or wrong

A **Correct.** Medical neglect is considered when a child is harmed by missed care, effective care is accessible, and the caregiver understands the advice but does not use it. Less intrusive steps come first, and a protective services report follows when they fail (Jenny & AAP, 2007). Vouchers and nurse visits have already been tried, the parent understands the plan, and the child has had two dangerous admissions. It beats B because more teaching repeats a support that has not worked while the danger grows. The report rests on reasonable suspicion, not proof.

B **Incorrect** (trap: Good but not best). Teaching is the right step when a family does not understand the care plan. This parent can describe the plan, and the family has already had home nurse visits, so another round of teaching delays protection after a second dangerous admission.

C **Incorrect** (trap: Ignores risk). Children build self-care skills gradually, but a 12-year-old still needs an adult to supervise insulin. Shifting full control to the child after two crises moves the parent's duty onto the child and raises the risk of another emergency.

Takeaway: When a child is harmed by missed care that the caregiver understands and can access, and supports have failed, report suspected medical neglect.

Sources: Jenny, C., & AAP Committee on Child Abuse and Neglect (2007). Recognizing and responding to medical neglect. Pediatrics, 120(6), 1385-1389, Diagnosis of medical neglect; responding to medical neglect
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124. ASM-1180 (rev 1) · Best answer: A

A 15-year-old in foster care was sexually abused for years by a stepparent while the client's other parent knew and did nothing. In sessions, the client says adults 'only pretend to care,' breaks rules to test each new foster parent, and refuses to lean on any staff member. In Finkelhor and Browne's model, which traumagenic dynamic do these reactions **MOST** likely reflect?

A. Betrayal
B. Powerlessness
C. Stigmatization
D. Traumatic sexualization

Why each option is right or wrong

A **Correct.** Finkelhor and Browne (1985) describe four traumagenic dynamics of child sexual abuse. Betrayal arises when someone the child depended on causes harm or fails to protect, and it shows up as distrust, testing and refusing to depend on caregivers. A parent who knew and did nothing, followed by testing foster parents and rejecting staff, fits betrayal. It beats B because powerlessness centers on fear, helplessness or a need for control rather than on distrust of caregivers.

B **Incorrect** (trap: Wrong model or concept). Powerlessness comes from repeated invasion of the body against the child's will. It tends to produce anxiety, fear, a sense of helplessness or efforts to control. Rule testing can look like control, but the client's words point to distrust of caregivers.

C **Incorrect** (trap: Wrong model or concept). Stigmatization refers to shame, guilt and a sense of being damaged or different, often leading to isolation or self-harm. The client's statements are about whether adults can be relied on, not about personal shame.

D **Incorrect** (trap: Wrong model or concept). Traumatic sexualization involves developmentally inappropriate sexual knowledge, behavior or confusion about sex. Nothing in the stem describes sexual behavior or confusion.

Takeaway: Abuse by or with the knowledge of a trusted caregiver can produce betrayal reactions: distrust, testing and refusal to depend on adults.

Sources: Finkelhor, D., & Browne, A. (1985). The traumatic impact of child sexual abuse: A conceptualization. American Journal of Orthopsychiatry, 55(4), 530-541, traumagenic dynamics
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125. ASM-1181 (rev 2) · Best answer: C

A hospital social worker is asked to meet with the parent of a 6-year-old whose lab results show gonorrhea. The physician confirms the infection could not have been passed at birth. The parent says the child must have caught it from shared towels at a relative's home. What should the social worker do **FIRST**?

A. Record the towel explanation and arrange the child's treatment
B. Arrange a forensic interview at a children's advocacy center
C. Notify the child abuse hotline of the possible sexual abuse

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Treatment follow-up is needed, and the parent's account belongs in the record. But shared towels do not credibly explain gonorrhea in a child; past the newborn period it strongly indicates sexual abuse (CDC, 2021). Accepting the explanation leaves the child unprotected.

B **Incorrect** (trap: Right action, wrong time). A forensic interview is the right way to hear from the child, but it is arranged through the investigation the report sets in motion. Arranging it first delays the mandated report.

C **Correct.** An STI such as gonorrhea in a child past the newborn period strongly indicates sexual abuse (CDC, 2021). That meets the reasonable-suspicion standard behind mandated reporting laws (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)). The towel explanation does not remove the duty, because reporters do not need proof and do not judge whether abuse occurred. It beats B because the forensic interview is arranged through the investigation that the report starts.

Takeaway: Gonorrhea or another STI acquired after birth in a young child signals likely sexual abuse and warrants a report, whatever caregivers say.

Sources: CDC (2021). Sexually Transmitted Infections Treatment Guidelines, Sexual Assault or Abuse of Children; Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i)

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126. ASM-1182 (rev 1) · Best answer: B

At a community health center, a 24-year-old client tells the social worker that their partner has flushed the client's birth control pills twice and says a baby would 'keep us together.' In answer to safety questions, the client describes no physical violence. The client does not want to become pregnant now and does not want to end the relationship. What is the **MOST** appropriate response?

A. Encourage the client to leave the relationship for their safety

B. Connect the client with the provider about less detectable methods

C. Suggest couples counseling so both partners can discuss family plans

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Leaving is the client's decision, and the client has said they want to stay. Pressing a person to leave a controlling partner can damage trust, and the period around separation can bring added risk. Harm reduction respects the client's choice.

B **Correct.** Sabotaging a partner's contraception to cause pregnancy is reproductive coercion, a form of intimate partner violence. ACOG (2013, No. 554) recommends harm-reduction counseling and offering long-acting methods that are less detectable to partners. The client's goals are to avoid pregnancy and keep the relationship, so linking the client with the provider for a discreet method protects those goals. It beats A because pressing a client to leave overrides the stated choice and does not prevent pregnancy now.

C **Incorrect** (trap: Ignores risk). Couples counseling treats coercion as a communication problem and can expose the client to retaliation for disclosing. Couples work is generally avoided when one partner is controlling the other's reproductive choices.

Takeaway: Reproductive coercion is partner abuse; respond with harm reduction, such as discreet contraception, while respecting the client's relationship choices.

Sources: American College of Obstetricians and Gynecologists (2013). Reproductive and Sexual Coercion. Committee Opinion No. 554, Recommendations
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127. ASM-0002 (rev 1) · Best answer: C

A social worker in a rehabilitation facility meets with an 82-year-old patient recovering from hip surgery. Staff report that the patient, who was alert and oriented last week, has become confused over the past 2 days. The patient's attention drifts, and the patient says insects are crawling on the wall. What should the social worker do **FIRST**?

A. Administer a cognitive screening to document the decline
B. Meet with family to discuss memory care placement
C. Notify the medical team of the sudden change in mental status
D. Request a psychiatric consultation for new-onset psychosis

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Screening can document the deficits, but it delays the urgent step. Acute confusion with fluctuating attention needs medical evaluation first, and formal cognitive testing during delirium gives misleading results.

B **Incorrect** (trap: Skips assessment). Planning memory care assumes a lasting neurocognitive disorder. The rapid onset points to delirium, which often clears when the cause is treated, so placement talk is premature and may be unnecessary.

C **Correct.** Confusion that develops over hours to days, with drifting attention and visual hallucinations, in an older patient after surgery strongly suggests delirium. Delirium is a medical emergency with treatable causes such as infection, medication effects or dehydration (DSM-5-TR; Inouye et al., 2014). The first step is making sure the medical team evaluates the change right away. It beats A because a cognitive screen adds information, but scoring the confusion does not get a possible medical emergency treated.

D **Incorrect** (trap: Near-miss diagnosis). Visual hallucinations with acute confusion in an older medical patient point to a medical cause rather than a primary psychotic disorder. Psychiatry may help later, but the medical team must look for underlying causes first.

Takeaway: Sudden confusion with fluctuating attention in an older adult suggests delirium, a medical emergency; get medical evaluation first.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium; Inouye, S. K., Westendorp, R. G. J., & Saczynski, J. S. (2014). Delirium in elderly people. The Lancet, 383(9920), 911-922, whole article
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128. ASM-0103 (rev 2) · Best answer: C

A primary care provider refers a 74-year-old client for memory concerns that began about 2 months ago. The client says, 'My mind is gone,' and answers 'I don't know' to many screening questions. The client also reports poor sleep, weight loss, lost interest in gardening and feeling like a burden. The client still manages bills and medications, alertness is steady and medical tests are normal. What is the **MOST** likely explanation?

A. Major neurocognitive disorder
B. Delirium
C. Major depressive disorder
D. Mild neurocognitive disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Major neurocognitive disorder requires decline that interferes with independence, such as needing help with bills or medications. This client handles both. People with Alzheimer's disease also tend to minimize deficits rather than stress them.

B **Incorrect** (trap: Near-miss diagnosis). Delirium develops over hours to days and involves fluctuating attention and awareness, usually with a medical cause. Steady alertness, a 2-month course and normal medical tests make it unlikely. A sudden change would call for urgent medical evaluation.

C **Correct.** Depression in older adults often presents as memory complaints. Clues include a fairly recent, datable onset, emphasis on deficits, poor effort on testing ('I don't know') and other depressive symptoms (DSM-5-TR). This client also has loss of interest, insomnia, weight loss and feelings of worthlessness. It beats D because mild neurocognitive disorder involves modest measured decline not better explained by another disorder, and DSM-5-TR lists depression as a key differential. Treating the depression and then rechecking cognition is the standard path.

D **Incorrect** (trap: Near-miss diagnosis). Mild neurocognitive disorder fits modest decline with preserved independence, so it is tempting. But a full depressive syndrome with a datable onset better explains the complaints. Cognition should be rescreened once the depression is treated.

Takeaway: In older adults, recent memory complaints with depressive symptoms and poor effort on testing suggest depression; treat it and reassess cognition.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder; DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major and Mild Neurocognitive Disorders (differential diagnosis)
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129. ASM-1039 (rev 2) · Best answer: C

Which mood can count toward a depressive episode in children and adolescents in place of sad or depressed mood?

A. Anxious mood
B. Labile mood
C. Irritable mood
D. Detached mood

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Anxiety often accompanies depression in young people and can be noted with the anxious distress specifier. It does not replace depressed mood as a core symptom of a depressive episode.

B **Incorrect** (trap: Wrong model or concept). Labile mood, quick shifts between emotions, can appear in bipolar disorder, personality disorders and brain injury. DSM-5-TR does not name it as a substitute for depressed mood in young people; irritability is the recognized variant.

C **Correct.** DSM-5-TR allows irritable mood to stand in for depressed mood in children and adolescents, both for a major depressive episode and for persistent depressive disorder. Young people often show depression as crankiness, anger or low frustration tolerance rather than reported sadness, so an irritable child should be screened for depression, not only for behavior problems. It beats A because anxiety commonly co-occurs with youth depression but does not substitute for the core mood criterion.

D **Incorrect** (trap: Wrong model or concept). Detachment can accompany depression, trauma or dissociation, but DSM-5-TR does not name it as a substitute for depressed mood in youth. Irritability is the recognized developmental variant.

Takeaway: In youth, irritable mood can replace sad mood in depression criteria; irritable children need depression screening, not just behavior plans.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
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130. ASM-1040 (rev 1) · Best answer: A

During a home visit 7 weeks after delivery, a parent tearfully says they keep getting sudden images of the baby falling down the stairs. The images horrify them, so they now avoid carrying the baby near the staircase. The parent also reports feeling worthless and sleeping poorly even when the baby sleeps. What should the social worker do **FIRST**?

A. Assess the thoughts, mood, safety and any intent to act
B. Refer the parent to a perinatal mental health specialist
C. Explain that such unwanted thoughts are common after birth

Why each option is right or wrong

A **Correct.** Unwanted, horrifying images of accidental harm to an infant are common after birth; when the parent is distressed by them and avoids acting on them, they usually signal anxiety or depression rather than danger (Fairbrother & Woody, 2008). Worthlessness and poor sleep suggest a depressive episode, which also calls for checking suicidal thinking (DSM-5-TR). Assessing the thoughts, mood, intent and safety comes first. It beats C because reassurance fits only after the social worker confirms the thoughts are unwanted and finds no intent, psychosis or suicidal thinking.

B **Incorrect** (trap: Right action, wrong time). A perinatal specialist referral is likely the right step soon. Making it before assessing intent, psychotic symptoms and suicidal thinking skips the information that decides how urgent the referral must be.

C **Incorrect** (trap: Skips assessment). Normalizing unwanted thoughts helps many new parents, but it should follow an assessment. Reassuring first assumes the thoughts are harmless before ruling out intent, psychosis or suicidal thinking.

Takeaway: When a new parent reports intrusive harm images, assess the thoughts, mood and intent before reassuring, referring or reporting.

Sources: Fairbrother, N., & Woody, S. R. (2008). New mothers' thoughts of harm related to the newborn. Archives of Women's Mental Health, 11(3), 221-229, whole article; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder

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131. ASM-1041 (rev 1) · Best answer: B

A high school social worker meets a 15-year-old cross-country runner referred by a coach. The student has lost a lot of weight over 3 months, skips lunch and runs extra miles. Today the student fainted at practice, and the athletic trainer recorded a very slow resting heart rate. The student says they feel fine. What should the social worker do **FIRST**?

A. Explore the student's thoughts about weight and body shape

B. Have the parents take the student to a doctor right away

C. Refer the student to an eating disorder therapy program

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Body image questions are central to assessing a possible eating disorder and will come soon. They follow medical safety when fainting and a slow heart rate point to possible instability.

B **Correct.** Fainting and a very slow resting heart rate in a teen with rapid weight loss and food restriction are signs of medical instability, which can require hospital care in restrictive eating disorders (Hornberger & Lane, 2021). Safety comes first, so the student needs a same-day medical evaluation with parents involved. It beats A because exploring body image is a core part of the eating disorder assessment, but it can wait until the student is medically safe.

C **Incorrect** (trap: Ignores risk). Eating disorder treatment is likely needed, but an outpatient therapy referral leaves the immediate medical risk unaddressed. Medical stability is checked first, and it also guides the level of care.

Takeaway: Fainting or a very slow heart rate in a restricting teen signals medical instability; get same-day medical care before psychological assessment or therapy.

Sources: Hornberger, L. L., Lane, M. A., & AAP Committee on Adolescence (2021). Identification and management of eating disorders in children and adolescents. Pediatrics, 147(1), e2020040279, medical evaluation and indications for hospitalization

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132. ASM-1042 (rev 1) · Best answer: C

Parents bring their 16-year-old to an outpatient family clinic after 4 months of falling grades and pulling away from friends. The teen says people at school seem to send hidden signals and sometimes a faint voice calls their name, but adds that it might be their mind playing tricks. The parents believe it is a phase. What is the social worker's **BEST** recommendation?

A. Monitor the symptoms and reassess at monthly visits
B. Begin family sessions to support the teen at school
C. Refer the teen for a specialized early psychosis evaluation

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Monitoring is reasonable for vague, isolated changes. Here, perceptual changes and ideas of hidden signals with declining function warrant specialist evaluation now; waiting risks a longer period of untreated psychosis if symptoms progress.

B **Incorrect** (trap: Good but not best). Family support helps, and families are part of early psychosis care. Family sessions alone would not evaluate the emerging psychotic symptoms that drive the concern.

C **Correct.** Mild psychotic-like experiences, such as ideas that others send hidden signals and faint voices while doubt is still intact, combined with falling function in adolescence, are warning signs of possible emerging psychosis (DSM-5-TR, attenuated psychosis syndrome). Coordinated specialty care for early psychosis improves outcomes, especially when untreated psychosis is brief (Kane et al., 2016). It beats A because waiting month to month delays evaluation during the period when early intervention helps most.

Takeaway: Teens with mild psychotic-like experiences and falling function need prompt specialized early psychosis evaluation, not watchful waiting.

Sources: DSM-5-TR (APA, 2022), Conditions for Further Study: Attenuated Psychosis Syndrome; Kane, J. M., et al. (2016). Comprehensive versus usual community care for first-episode psychosis: 2-year outcomes from the NIMH RAISE Early Treatment Program. American Journal of Psychiatry, 173(4), 362-372, whole article
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133. ASM-1044 (rev 1) · Best answer: A

A client with bipolar I disorder, stable for 2 years, meets the social worker by video. The client reports sleeping 4 hours a night for 3 nights, feeling 'better than ever' and planning a new business. The relapse plan the client wrote while well lists short sleep as a warning sign that means calling the prescriber. The client now wants to wait for next month's appointment. What should the social worker do **FIRST**?

A. Help the client contact their medication provider today
B. Explore the client's concerns about medication changes
C. Agree to track sleep together until the next appointment

Why each option is right or wrong

A **Correct.** Less sleep with rising mood is a common early warning sign of manic relapse, and teaching clients to act on early signs through a prearranged plan reduces manic relapses (Perry et al., 1999). The client wrote this plan while well, so helping the client follow it supports, rather than overrides, self-determination (NASW Code 1.02). It beats B because worries about medication deserve attention, but exploring them first delays a time-sensitive step; they can be discussed while the call is made.

B **Incorrect** (trap: Right action, wrong time). Ambivalence about medication is common and worth exploring. With 3 nights of short sleep and rising mood, exploration comes after the client reaches the provider, since early warning signs can escalate within days.

C **Incorrect** (trap: Ignores risk). Tracking sleep is useful alongside treatment, but waiting a month ignores the warning sign the client's own plan names. Early manic symptoms can progress quickly.

Takeaway: Early warning signs named in a client's relapse plan call for prompt contact with the treating provider, honoring choices the client made while well.

Sources: Perry, A., Tarrier, N., Morriss, R., McCarthy, E., & Limb, K. (1999). Randomised controlled trial of efficacy of teaching patients with bipolar disorder to identify early symptoms of relapse and obtain treatment. BMJ, 318(7177), 149-153, whole article; NASW Code of Ethics (2021), 1.02

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134. ASM-1045 (rev 2) · Best answer: B

At a community mental health center, the parents of a 23-year-old with schizophrenia say their adult child now showers rarely, speaks little and sits for hours without starting anything. The voices have faded with treatment. The parents call it laziness and have started criticizing the client every day. What is the social worker's **BEST** response?

A. Help the parents set firm rewards and consequences for chores

B. Explain that these behaviors are common symptoms of the illness

C. Link the client to a vocational rehabilitation program

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Structure and positive reinforcement can help with negative symptoms later. Starting with consequences while the parents see the behavior as laziness is likely to increase the criticism linked to relapse.

B **Correct.** Reduced motivation, sparse speech and poor self-care are negative symptoms of schizophrenia (avolition and alogia), signs of the illness rather than choices (DSM-5-TR). Daily criticism is a form of high expressed emotion, which predicts relapse (Butzlaff & Hooley, 1998), so helping the parents see these behaviors as symptoms comes first. It beats A because structured routines can help later, but a consequence plan framed around laziness would likely add to the criticism that raises relapse risk.

C **Incorrect** (trap: Right action, wrong time). Vocational services help many people with schizophrenia and may fit later. It does not address the family's misreading of symptoms or the daily criticism at home.

Takeaway: Help families recognize negative symptoms as illness, not laziness; reducing criticism lowers relapse risk.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; Butzlaff, R. L., & Hooley, J. M. (1998). Expressed emotion and psychiatric relapse: A meta-analysis. Archives of General Psychiatry, 55(6), 547-552, whole article
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135. ASM-1143 (rev 1) · Best answer: C

A primary care physician refers a 76-year-old client after a full workup found no cause for months of fatigue, aches and poor appetite. The client has stopped gardening and no longer meets friends for cards but says, 'I'm not sad, just worn out.' What should the social worker do **NEXT**?

A. Encourage the client to rejoin a gardening group
B. Accept the denial of sadness and focus on coping with pain
C. Assess for depression, including lost interest and hopelessness

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Reengaging in valued activities is a sound part of treating late-life depression. Suggesting it before assessing treats the withdrawal as a lifestyle issue and may miss depression and any suicide risk.

B **Incorrect** (trap: Good but not best). Denial of sadness does not rule out depression in later life. Focusing on pain coping accepts the physical framing and leaves the loss of interest unexamined.

C **Correct.** Older adults with depression often deny sadness and present instead with physical complaints, fatigue and loss of interest (Gallo & Rabins, 1999). DSM-5-TR allows loss of interest or pleasure, without depressed mood, to anchor a major depressive episode. With medical causes ruled out and clear withdrawal from valued activities, the social worker should assess for depression, including hopelessness and suicidal thoughts. It beats A because activity can help once an assessment guides the plan, but prompting first may miss a treatable and risky illness.

Takeaway: Older adults may show depression through physical complaints and lost interest without sadness; assess for depression once medical causes are excluded.

Sources: Gallo, J. J., & Rabins, P. V. (1999). Depression without sadness: Alternative presentations of depression in late life. American Family Physician, 60(3), 820-826, whole article; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
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136. ASM-1150 (rev 1) · Best answer: B

A 34-year-old group home resident with moderate intellectual developmental disorder communicates with a few words and gestures. Over the past 3 weeks, staff have seen the resident strike the side of their own jaw, wake often at night, and push meals away. The house manager asks the social worker to write a behavior plan. What should the social worker do **FIRST**?

A. Complete a functional assessment of the jaw striking
B. Arrange a medical and dental exam for possible pain
C. Refer the resident to psychiatry to evaluate depression
D. Draft a plan that rewards calm mealtimes and sleep

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A functional assessment is the usual basis for a behavior plan and would come next if no medical cause is found. Done first, it may label pain-driven behavior as attention or escape seeking and delay treatment.

B **Correct.** When a person with limited speech shows a sudden change in behavior, sleep or eating, physical causes such as pain are checked first. NICE guidance on behavior that challenges says initial assessment should take physical health problems into account (NICE NG11, 1.5.5). Striking the jaw, broken sleep and pushing food away over 3 weeks fit untreated dental or other pain. It beats A because a functional assessment is the right next step if the exam finds no medical cause; done first, it risks building a plan around pain that only treatment can relieve.

C **Incorrect** (trap: Right action, wrong time). Sleep and appetite changes can signal depression, which is often missed in people with intellectual disabilities. A psychiatric evaluation is reasonable once physical causes such as pain are ruled out, because pain can produce the same picture.

D **Incorrect** (trap: Skips assessment). A reward plan treats the behavior before anyone knows why it started. If pain is the cause, rewards will not help, and the resident's distress will continue unrecognized.

Takeaway: A sudden behavior change in a person with limited speech calls for a medical check for pain or illness before behavioral or psychiatric plans.

Sources: NICE (National Institute for Health and Care Excellence) (2015). Challenging behaviour and learning disabilities: prevention and interventions for people with learning disabilities whose behaviour challenges (NICE NG11), Recommendation 1.5.5

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137. ASM-1151 (rev 1) · Best answer: D

A client with schizophrenia has worked part-time stocking shelves for 4 months through a supported employment program. The supervisor reports that the client arrives early, chats easily with coworkers, and calls the job the best part of the week. However, the client cannot hold multistep instructions in mind and gets lost when a task changes midway. Which problem is **MOST** likely affecting job performance?

A. Negative symptoms
B. Disorganized thinking
C. Depressive symptoms
D. Cognitive impairment

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Negative symptoms include reduced motivation, pleasure and expression, and they often hurt job performance in schizophrenia. This client is eager, socially engaged and finds the work rewarding, which points away from them.

B **Incorrect** (trap: Near-miss diagnosis). Disorganized thinking shows up in speech that derails or loses coherence. The client chats easily with coworkers; the trouble lies in holding and updating instructions, not in how thoughts connect in speech.

C **Incorrect** (trap: Near-miss diagnosis). Depression can slow concentration, but it comes with low mood or loss of interest. The client calls the job the best part of the week, which does not fit a depressive picture.

D **Correct.** Cognitive deficits, such as problems with working memory, attention and executive function, are common in schizophrenia and are closely tied to work and daily functioning, even when other symptoms are controlled (DSM-5-TR). Following multistep instructions and adjusting when a task shifts depend on working memory and mental flexibility. It beats A because negative symptoms appear as low drive, little pleasure or reduced speech, while this client arrives early, enjoys the job and talks easily with coworkers.

Takeaway: Working-memory and executive deficits in schizophrenia can impair work even when the client is motivated and other symptoms are controlled.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia (Associated Features)
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138. ASM-1152 (rev 1) · Best answer: A

A school nurse refers a 9-year-old who comes to the health office with stomachaches on most test days and many Monday mornings. A pediatrician found no medical cause. The parent asks the school social worker to support a plan that sends the child home whenever the pain starts. What should the social worker do **FIRST**?

A. Assess the child's worries and avoidance with child and parent
B. Support the parent's go-home plan to ease distress
C. Teach the child slow breathing to use before each test

Why each option is right or wrong

A **Correct.** Assessment comes before intervention. Stomachaches and headaches are common in anxious children, and this pattern, pain tied to tests and the start of the school week with a normal medical workup, points toward anxiety. The AACAP practice parameter calls for gathering information from both the child and adults, because each sees different parts of the problem (Connolly & Bernstein, 2007). It beats C because breathing practice is a reasonable coping tool, but choosing it before learning what the child fears and avoids skips the assessment that should guide the plan.

B **Incorrect** (trap: Skips assessment). Sending the child home relieves distress in the moment but can reward avoidance and let school-related anxiety grow. Any plan for handling the pain at school should follow an assessment of what drives it.

C **Incorrect** (trap: Right action, wrong time). Relaxation skills are often part of treatment for childhood anxiety. Taught now, though, they rest on a guess about the problem; the social worker first needs to learn what the child worries about and avoids.

Takeaway: When a medical workup is negative, assess for anxiety behind a child's school-linked physical complaints before choosing accommodations or skills.

Sources: Connolly, S. D., & Bernstein, G. A. (2007). Practice parameter for the assessment and treatment of children and adolescents with anxiety disorders. Journal of the American Academy of Child and Adolescent Psychiatry, 46(2), 267-283, Clinical presentation; Recommendation 1

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139. ASM-0104 (rev 1) · Best answer: C

At a community mental health agency, a client has had major depressive disorder since adolescence and began drinking heavily 2 years ago. The client now meets criteria for moderate alcohol use disorder and denies suicidal thoughts. An intake staff member suggests the client get sober before starting mental health care. What is the **BEST** treatment approach?

A. Address the drinking first, then reassess the depression

B. Refer to separate agencies for each disorder at once

C. Treat both disorders together in one coordinated plan

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Requiring sobriety before mental health care is the sequential model. A period of abstinence can help clarify a diagnosis when depression may be alcohol-induced, but this client's depression predates the drinking by years. Waiting leaves an independent disorder untreated.

B **Incorrect** (trap: Good but not best). Parallel treatment addresses both disorders, but in separate systems that may not coordinate. Clients can receive conflicting messages and drop out between programs. It is better than no care, but it falls short of integrated treatment.

C **Correct.** Integrated treatment, which addresses both disorders at the same time through one team or coordinated plan, is the preferred model for co-occurring disorders (SAMHSA TIP 42, Ch. 2). Here the depression began years before the drinking, so it is an independent disorder rather than a result of alcohol use, and

delaying its care risks worsening both. It beats B because parallel care at separate agencies splits the plan, sends mixed messages and often loses clients between systems.

Takeaway: For co-occurring mental health and substance use disorders, integrated treatment of both at once is preferred over sequential or parallel care.

Sources: SAMHSA TIP 42 (2020), Ch. 2: Guiding Principles for Working With People Who Have Co-Occurring Disorders (Achieving Integrated Treatment); DSM-5-TR (APA, 2022), Depressive Disorders: Substance/Medication-Induced Depressive Disorder
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140. ASM-1047 (rev 2) · Best answer: B

A client entered residential treatment for severe alcohol use disorder 6 days ago after years of heavy daily drinking. The client reports low mood, poor sleep and guilt, and says the low mood began after the drinking became heavy 3 years ago. The client answers no to direct questions about suicide. The staff psychiatrist asks for the social worker's diagnostic input. What is the social worker's **BEST** recommendation?

A. Recommend diagnosing major depressive disorder now
B. Reassess the mood after about a month of abstinence
C. Treat the low mood as alcohol-related and stop assessing it
D. Suggest starting an antidepressant during detox

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Major depressive disorder may prove correct, but the low mood began with heavy drinking and is being judged in the first week of abstinence. DSM-5-TR asks clinicians to consider a substance-induced cause first.

B **Correct.** Depressive symptoms are common during heavy drinking and early abstinence and often lift within weeks. DSM-5-TR points to an independent depressive disorder when depression began before the substance use or persists for a substantial period, about a month, after acute withdrawal or intoxication ends. This client's low mood began with heavy drinking and is being assessed on day 6, so reassessing after about a month of abstinence, while monitoring mood and safety, gives the clearest answer. It beats A because diagnosing major depression now cannot separate alcohol's effects from an independent disorder.

C **Incorrect** (trap: Ignores risk). Alcohol may well explain the mood, but closing the question risks missing an independent depression that persists after abstinence. Co-occurring disorders are common and need continued assessment.

D **Incorrect** (trap: Skips assessment). Medication decisions belong to the prescriber, and starting an antidepressant during early withdrawal assumes an independent depression that the timeline does not yet support. It may fit if low mood persists.

Takeaway: Depression that starts with heavy substance use is reassessed after about a month of abstinence before diagnosing an independent depressive disorder.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Substance/Medication-Induced Depressive Disorder
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141. ASM-1050 (rev 1) · Best answer: B

An adult with schizophrenia in supportive housing has said for 2 days that 'the devil is twisting my stomach.' The client has voiced similar beliefs before. Today the client is hunched over, has vomited twice, refuses meals and feels warm to the touch. Staff ask the social worker to treat it as a familiar delusion. What should the social worker do **FIRST**?

A. Ask the psychiatrist to review the antipsychotic dose
B. Arrange a same-day medical evaluation of the stomach symptoms
C. Explore what the stomach image means to the client

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A medication review may help if psychotic symptoms are worsening, but it treats the complaint as purely psychiatric. Vomiting, food refusal and warmth are physical signs that need medical evaluation first; the psychiatrist can be updated afterward.

B **Correct.** People with serious mental illness have higher rates of physical illness, and their physical complaints are often wrongly credited to the psychiatric condition, a pattern called diagnostic overshadowing. Vomiting, refusing food and feeling warm are new physical signs that need same-day medical evaluation, whatever the delusional framing. This beats A because a medication review addresses the psychosis but leaves a possible abdominal emergency, such as appendicitis, unexamined (Jones, Howard & Thornicroft, 2008; De Hert et al., 2011).

C **Incorrect** (trap: Ignores risk). Exploring the meaning of a delusional image can support engagement once the client is medically safe. Doing it first leaves possible acute illness unassessed while the client vomits and refuses food.

Takeaway: New physical signs in a client with serious mental illness need medical evaluation, even when the complaint is voiced through a delusion.

Sources: Jones, S., Howard, L., & Thornicroft, G. (2008). 'Diagnostic overshadowing': Worse physical health care for people with mental illness. Acta Psychiatrica Scandinavica, 118(3), 169-171, whole article; De Hert, M., et al. (2011). Physical illness in patients with severe mental disorders. I. Prevalence, impact of medications and disparities in health care. World Psychiatry, 10(1), 52-77, pp. 52-77
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142. ASM-1051 (rev 2) · Best answer: C

An adult in an intensive outpatient program for moderate alcohol use disorder has cut drinking from daily to 2 evenings a week over 4 weeks. The client also has PTSD from an assault 2 years ago, with nightmares and avoidance that the client links to drinking. The client has stable housing, no withdrawal signs and no suicidal thoughts, and asks to start trauma treatment now. What is the social worker's **BEST** recommendation?

A. Defer trauma therapy until 6 months of sobriety
B. Refer to a separate trauma clinic after the program ends
C. Begin trauma-focused therapy alongside current care

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Requiring a long stretch of sobriety before trauma treatment was once common practice, but trauma-focused therapy is safe and effective alongside substance use treatment. Delay fits only when acute instability, such as withdrawal or crisis, prevents engagement.

B **Incorrect** (trap: Hands off too soon). Sending PTSD care elsewhere after the program ends splits and delays treatment. Separate, sequential care is weaker than integrated care and leaves untreated the nightmares and avoidance that the client ties to drinking.

C **Correct.** Current guidance holds that a co-occurring substance use disorder does not rule out trauma-focused psychotherapy for PTSD, and integrated care that treats both conditions together is preferred. This client is engaged, has cut drinking, has stable housing, shows no withdrawal or suicide risk, and links drinking to trauma symptoms, so adding trauma-focused therapy now fits. It beats A because waiting months for sobriety leaves untreated the symptoms that drive drinking (VA/DoD, 2023; SAMHSA, 2021).

Takeaway: Co-occurring substance use does not rule out trauma-focused PTSD therapy; treat both together when the client is stable enough to engage.

Sources: VA/DoD (2023). Clinical Practice Guideline for the Management of Posttraumatic Stress Disorder and Acute Stress Disorder. U.S. Department of Veterans Affairs & Department of Defense, Treatment recommendations: trauma-focused psychotherapy with co-occurring conditions; SAMHSA (2021). Advisory: Substance Use Disorder Treatment for People With Co-Occurring Disorders (PEP20-06-04-006), Key Messages; Practice Principles of Integrated Treatment

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143. ASM-1052 (rev 1) · Best answer: D

A client in private practice has major depressive disorder. After 12 weeks of treatment, the client's mood, energy and interest have largely returned. However, the client still takes over an hour to fall asleep most nights, as for the past 5 months, and feels exhausted at work. The prescriber expects sleep to recover as the depression lifts. What is the **MOST** appropriate recommendation?

A. Wait for sleep to improve as the depression resolves
B. Ask the prescriber about adding a sleep medication
C. Give the client a handout on sleep hygiene
D. Add CBT for insomnia to the client's care plan

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Waiting is reasonable early in depression treatment, when poor sleep is part of the episode. Here mood and interest have largely recovered while sleep has not, so the insomnia now needs direct attention.

B **Incorrect** (trap: Right action, wrong time). Medication is an option the prescriber may weigh, but guidelines place CBT-I first for chronic insomnia because its gains last after treatment ends. Medication fits if CBT-I is unavailable or does not help.

C **Incorrect** (trap: Good but not best). Sleep hygiene advice is useful, but used alone it is not an effective treatment for chronic insomnia (Edinger et al., 2021). It is one piece of CBT-I, which adds stimulus control, sleep restriction and cognitive work.

D **Correct.** Insomnia often persists after depression improves, and it can be diagnosed as a co-occurring insomnia disorder when the other condition no longer adequately explains it (DSM-5-TR). Here mood and interest have recovered, yet trouble falling asleep has lasted 5 months and impairs work, so it warrants its own treatment. Cognitive behavioral therapy for insomnia (CBT-I) is the recommended first-line treatment (Qaseem et al., 2016). It beats A because waiting assumes the depression explains a problem that has outlasted it.

Takeaway: Insomnia that outlasts improvement in depression is a co-occurring disorder that needs its own treatment, with CBT-I first.

Sources: DSM-5-TR (APA, 2022), Sleep-Wake Disorders: Insomnia Disorder; Qaseem, A., et al. (2016). Management of chronic insomnia disorder in adults: A clinical practice guideline from the American College of Physicians. Annals of Internal Medicine, 165(2), 125-133, Recommendation 1; Edinger, J. D., et al. (2021). Behavioral and psychological treatments for chronic insomnia disorder in adults: An American Academy of Sleep Medicine clinical practice guideline. Journal of Clinical Sleep Medicine, 17(2), 255-262, Recommendations: sleep hygiene

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144. ASM-1053 (rev 2) · Best answer: A

An adult with bipolar I disorder comes to a community mental health center for help with worsening depression. The client drinks 2 or 3 beers most evenings and has never had withdrawal symptoms. The intake coordinator says center policy sends anyone who drinks to an addiction program before mental health care. What is the social worker's **BEST** response?

A. Assess both the mood episode and alcohol use today
B. Refer the client to the addiction program first
C. Help the client find an integrated program elsewhere

Why each option is right or wrong

A **Correct.** Co-occurring disorders are expected, and there is no wrong door: whichever program a person enters should assess and begin to address both conditions (SAMHSA, 2021). This client has worsening bipolar depression and regular evening drinking without withdrawal history, so an integrated intake now meets the immediate need. It beats C because sending the client to another program, even a good one, delays care for a worsening mood episode and risks losing the client between agencies. The policy itself can be raised afterward through channels.

B **Incorrect** (trap: Misreads law or ethics). Sequential care, addiction first, follows the policy but leaves a worsening bipolar depression untreated and risks losing the client in transfer. Policy does not outweigh the duty to serve a client who needs care now.

C **Incorrect** (trap: Hands off too soon). Linking to an integrated program sounds client-centered but still turns the client away from a door they already came through. A transfer fits only if assessment shows needs this center cannot meet.

Takeaway: There is no wrong door for co-occurring disorders: the program a client enters assesses and begins treating both conditions.

Sources: SAMHSA (2021). Advisory: Substance Use Disorder Treatment for People With Co-Occurring Disorders (PEP20-06-04-006), Key Messages
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145. ASM-1054 (rev 1) · Best answer: B

An ACT team social worker visits a 23-year-old client 6 months after a first hospitalization for schizophrenia, during which the voices stopped. The client now speaks of having 'ruined everything,' feels guilty about burdening family, sleeps poorly and no longer enjoys music. The client is tearful and expressive while talking. A team member calls this negative symptoms. What is the **MOST** likely explanation?

A. Negative symptoms of schizophrenia
B. Co-occurring depression
C. Antipsychotic-induced parkinsonism
D. Schizoaffective disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Negative symptoms such as diminished expression and low motivation can resemble depression, but they lack prominent guilt and hopelessness. This client is tearful and expressive, which argues against blunted emotional expression.

B **Correct.** Depression commonly co-occurs with schizophrenia, often after a first episode as insight returns, and it raises suicide risk. Guilt, hopelessness ('ruined everything'), poor sleep and lost enjoyment, with visible tearfulness, point to depression: the client feels sadness and shows it. It beats A because negative symptoms involve reduced expression and drive without this felt sadness and guilt. Recognizing co-occurring depression should prompt a suicide risk assessment and treatment planning (DSM-5-TR; Kaplan & Sadock).

C **Incorrect** (trap: Near-miss diagnosis). Antipsychotic-induced parkinsonism causes a masklike face, slowed movement and stiffness that can mimic depression. It does not explain guilt or hopeless statements, and this client shows expressive emotion.

D **Incorrect** (trap: Near-miss diagnosis). Schizoaffective disorder requires a major mood episode during the active psychotic phase, with mood symptoms present for most of the illness. Here low mood began after the psychosis remitted, which fits depression co-occurring with schizophrenia.

Takeaway: In schizophrenia, guilt, hopelessness and felt sadness point to co-occurring depression rather than negative symptoms and call for suicide risk assessment.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Schizophrenia: depression and suicide; differential of negative symptoms
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146. ASM-1153 (rev 1) · Best answer: C

A client with schizophrenia who lives in supportive housing smokes two packs of cigarettes a day. The client tells the social worker they want to quit because cigarettes use up most of their spending money. A staff member says quitting will worsen the client's symptoms and that smoking is the client's only pleasure. The client's symptoms have been stable for a year. What is the social worker's **BEST** response?

A. Suggest waiting until the client's stability is more established
B. Encourage the client to cut down gradually on their own
C. Help the client get cessation treatment through the prescriber

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Waiting reflects a common belief that quitting destabilizes people with schizophrenia. Evidence does not support routine delay, and this client has been stable for a year and wants to act now. Delay keeps money and health going to cigarettes.

B **Incorrect** (trap: Good but not best). Supporting a gradual cutback respects the client's goal, but unaided attempts succeed far less often than treatment. Medication plus counseling works better than either alone, and the prescriber needs to be involved because smoking affects levels of some antipsychotics.

C **Correct.** Smoking is far more common among people with serious mental illness, and it is a major cause of their early deaths. In a large trial, cessation medicines helped smokers with psychiatric disorders quit without a significant rise in serious psychiatric side effects (Anthenelli et al., 2016), and the Public Health Service guideline recommends combining medication with counseling (Fiore et al., 2008). The prescriber also needs to know, since smoking speeds the breakdown of some antipsychotics. It beats B because unaided cutting down works far less often than treatment.

Takeaway: Offer evidence-based tobacco treatment to clients with stable serious mental illness who want to quit, and involve the prescriber.

Sources: Anthenelli, R. M., Benowitz, N. L., West, R., et al. (2016). Neuropsychiatric safety and efficacy of varenicline, bupropion, and nicotine patch in smokers with and without psychiatric disorders (EAGLES): A double-blind, randomised, placebo-controlled clinical trial. Lancet, 387(10037), 2507-2520, whole article; Fiore, M. C., et al. (2008). Treating Tobacco Use and Dependence: 2008 Update. Clinical Practice Guideline. U.S. Department of Health and Human Services, Public Health Service, Recommendations: combining counseling and medication

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147. ASM-1154 (rev 1) · Best answer: B

A client with schizoaffective disorder served by an assertive community treatment team has gained weight over the past year. At the client's apartment, the client mentions being thirsty all the time, getting up several times a night to urinate, and having blurry vision. The client distrusts doctors and plans to bring it up at a psychiatric appointment in 5 weeks. What is the **MOST** appropriate action?

A. Agree to the client's plan and check on the symptoms next month
B. Offer to go with the client to a primary care visit this week
C. Encourage the client to switch to water and sugar-free drinks

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). The plan respects the client's wishes, but it leaves possible high blood sugar unchecked for more than a month. Untreated diabetes can worsen quickly, so a sooner medical visit is needed.

B **Correct.** People with serious mental illness have higher rates of diabetes than the general population, and their physical illnesses are often missed or treated late (De Hert et al., 2011). Constant thirst, frequent urination and blurry vision are classic signs of high blood sugar (CDC). Offering to go along respects the client's distrust while getting a timely evaluation. It beats A because honoring the client's timeline suits minor concerns, but 5 weeks is a long delay for possible untreated diabetes, and a psychiatric visit is not set up to test for it.

C **Incorrect** (trap: Good but not best). Cutting sugary drinks is healthy advice, but it treats a possible medical condition without a diagnosis. The symptoms need testing by a medical provider before lifestyle steps are planned.

Takeaway: Clients with serious mental illness face high, often missed, rates of diabetes; link new classic symptoms to timely medical care.

Sources: De Hert, M., Correll, C. U., Bobes, J., et al. (2011). Physical illness in patients with severe mental disorders. I. Prevalence, impact of medications and disparities in health care. World Psychiatry, 10(1), 52-77, whole article; CDC (Centers for Disease Control and Prevention). Symptoms of Diabetes, Symptoms shared by type 1 and type 2 diabetes

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148. ASM-1155 (rev 1) · Best answer: A

Six weeks after a heart attack, a patient in a hospital cardiac rehabilitation program has missed several exercise sessions. The patient tells the social worker they feel down most days, have lost interest in hobbies, and sleep poorly. The patient adds, 'I don't need a shrink. I just need my strength back.' A nurse calls this a normal adjustment. What is the **MOST** appropriate action?

A. Screen for depression as a routine part of rehab care
B. Focus on exercise goals as the patient prefers
C. Arrange a psychiatric consult to consider antidepressants
D. Reassure the patient that low mood after a heart attack passes

Why each option is right or wrong

A **Correct.** Depression after a heart attack is common and is linked to higher rates of later cardiac events and death (Lichtman et al., 2014). Weeks of low mood, lost interest and poor sleep call for a standardized screen such as the PHQ-9 (Kroenke et al., 2001). Presenting it as routine rehab care eases the stigma the patient voices. It beats B because following the patient's focus is usually sound, but here untreated depression may be what is undermining the exercise goals.

B **Incorrect** (trap: Good but not best). Starting where the client is matters, and exercise goals belong in the plan. Setting the mood concern aside, though, ignores a condition that predicts poorer heart outcomes and may explain the missed sessions.

C **Incorrect** (trap: Skips assessment). Medication may eventually help, but referral comes after screening shows the size of the problem. A psychiatric referral now also runs into the stigma the patient has just expressed.

D **Incorrect** (trap: Good but not best). Some low mood after a heart attack does fade. Weeks of lost interest and poor sleep go beyond that, and reassurance without screening risks missing depression that affects survival.

Takeaway: Depression after a heart attack predicts worse cardiac outcomes; screen persistent symptoms rather than accept them as normal adjustment.

Sources: Lichtman, J. H., Froelicher, E. S., Blumenthal, J. A., et al. (2014). Depression as a risk factor for poor prognosis among patients with acute coronary syndrome: Systematic review and recommendations. A scientific statement from the American Heart Association. Circulation, 129(12), 1350-1369, whole article; Kroenke, K., Spitzer, R. L., & Williams, J. B. W. (2001). The PHQ-9: Validity of a brief depression severity measure. Journal of General Internal Medicine, 16(9), 606-613, whole article

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149. ASM-1561 (rev 1) · Best answer: A

Engel's biopsychosocial model was proposed as a challenge to which approach to illness?

A. The biomedical model

B. The psychoanalytic model

C. The ecological model

Why each option is right or wrong

A **Correct.** Engel (1977) argued that the dominant biomedical model, which explains illness only through biological processes, could not account for how psychological and social factors shape illness, help-seeking and recovery. His biopsychosocial model became the frame for social work assessment across biological, psychological and social domains. It beats B because Engel's critique targeted medicine's reliance on biology alone, not psychoanalytic theory.

B **Incorrect** (trap: Wrong model or concept). Psychoanalytic theory explains distress through unconscious processes and was not Engel's target. His critique addressed medicine's practice of explaining illness through biological causes alone.

C **Incorrect** (trap: Wrong model or concept). The ecological perspective, which stresses the fit between person and environment, is compatible with Engel's model. It was not the approach his paper set out to challenge.

Takeaway: The biopsychosocial model holds that biological, psychological and social factors together shape illness, replacing biology-only explanations.

Sources: Engel, G. L. (1977). The need for a new medical model: A challenge for biomedicine. Science, 196(4286), 129-136, whole article

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150. ASM-1562 (rev 3) · Best answer: C

During a biopsychosocial assessment in a hospital dialysis unit, a patient mentions that prayer has helped them cope, but adds that they have not attended services since starting treatment 3 months ago. The intake form already lists the patient's religious affiliation. What should the social worker do **NEXT**?

A. Refer the patient to the hospital chaplain for support
B. Move on to the social history section of the assessment
C. Ask what faith means to the patient and how it affects care

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Chaplain involvement may well help and fits the care-planning part of a spiritual history. Referring before asking what the patient wants hands off a domain the social worker should first assess.

B **Incorrect** (trap: Good but not best). Recording affiliation alone misses the spiritual domain. The patient has signaled both a coping strength and a lost community connection, and both affect support during dialysis.

C **Correct.** A biopsychosocial-spiritual assessment covers beliefs, their importance, community and how they should be addressed in care, the areas of the FICA spiritual history (Puchalski & Romer, 2000). This patient named prayer as a coping resource and has lost contact with a faith community since treatment began, so learning what faith means to the patient and how it shapes care is the next step. It beats A because a chaplain referral may be useful, but it should follow the patient's own account of needs and wishes.

Takeaway: A biopsychosocial assessment includes a spiritual history: beliefs, their importance, community and how the client wants them addressed in care.

Sources: Puchalski, C., & Romer, A. L. (2000). Taking a spiritual history allows clinicians to understand patients more fully. Journal of Palliative Medicine, 3(1), 129-137, FICA spiritual history
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151. ASM-1563 (rev 1) · Best answer: B

A court refers a client for assessment and describes the problem as 'anger and aggression.' At intake, the client says the real problem is losing visits with their children since a breakup and not knowing how to get them back. The social worker is writing the presenting problem section of the biopsychosocial assessment. What is the **BEST** way to record it?

A. Record the court's description as the official presenting problem
B. Record the client's own words along with the referral reason
C. Record the social worker's view of the underlying problem

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The court's concern belongs in the record as the referral reason, but treating it as the whole presenting problem leaves out how the client defines the difficulty, which is the starting point for engagement.

B **Correct.** The presenting problem is recorded in the client's own words, with the referral source's reason noted separately, because what a referral says and what a client identifies often differ (Hepworth et al., assessment of problems and strengths). Recording both keeps the court's concern visible while starting where the client is: losing visits with their children. It beats A because recording only the court's label replaces the client's view of the problem and can weaken engagement from the first session.

C **Incorrect** (trap: Misuses the technique). Clinical interpretation belongs later, in the assessment summary or formulation. Placing it in the presenting problem section mixes the worker's hypotheses with the client's own account.

Takeaway: Record the presenting problem in the client's own words, and note the referral source's stated reason separately.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: problems and strengths (presenting problem)
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152. ASM-1564 (rev 1) · Best answer: B

A social worker is writing a biopsychosocial formulation for a 71-year-old with depression. The client's parent had depression, and symptoms began after a hip fracture last spring. Since then the client rarely leaves the house, and mood dips further with each week spent alone indoors. A neighbor visits daily. In a 4P formulation, how is the client's staying home classified?

A. Precipitating factor
B. Perpetuating factor
C. Predisposing factor
D. Protective factor

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Precipitating factors are events just before onset that trigger the problem, here the hip fracture. Staying home came afterward and keeps the depression going, so it belongs elsewhere.

B **Correct.** In a 4P formulation, perpetuating factors are those that keep current difficulties going, such as ongoing behaviors and thinking patterns (Macneil et al., 2012). The client's withdrawal deepens low mood week by week, so it sustains the depression rather than starting it. Sorting biological, psychological and social data this way links the assessment to treatment targets. It beats A because the precipitating factor is the event that set off the episode, the hip fracture, not the pattern that followed.

C **Incorrect** (trap: Wrong model or concept). Predisposing factors are background vulnerabilities, such as the parent's history of depression in this case. They raise risk before any trigger occurs.

D **Incorrect** (trap: Wrong model or concept). Protective factors are strengths that buffer the problem, such as the neighbor's daily visits. Withdrawal works against recovery, so it is not protective.

Takeaway: A 4P formulation sorts biopsychosocial data into predisposing, precipitating, perpetuating and protective factors that point to treatment targets.

Sources: Macneil, C. A., Hasty, M. K., Conus, P., & Berk, M. (2012). Is diagnosis enough to guide interventions in mental health? Using case formulation in clinical practice. BMC Medicine, 10, 111, case formulation: predisposing, precipitating, perpetuating, protective factors

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153. ASM-1565 (rev 1) · Best answer: A

In a primary care clinic, a patient with 2 years of low back pain is referred to the social worker after normal imaging. The patient lost a warehouse job, sleeps poorly, and avoids walking for fear of reinjury. The patient asks, 'So the doctor thinks the pain is all in my head?' Which response is **MOST** helpful?

A. Explain that real pain is affected by mood, stress and activity

B. Reassure the patient that the pain has a purely physical cause

C. Explain that emotional stress is the likely cause of the pain

Why each option is right or wrong

A **Correct.** The biopsychosocial model of chronic pain holds that pain is a real experience shaped by interacting biological, psychological and social factors, including mood, sleep, fear of movement and job loss (Gatchel et al., 2007). Normal imaging does not mean the pain is imagined. Explaining this validates the patient and opens the door to work on sleep, avoidance and stress. It beats B because insisting on a purely physical cause would discourage the psychosocial work that can reduce pain and disability.

B **Incorrect** (trap: Good but not best). Reassurance may ease the patient's worry, but claiming a purely physical cause contradicts the biopsychosocial view and makes it harder to accept help with sleep, fear of movement and job loss.

C **Incorrect** (trap: Wrong model or concept). Saying stress causes the pain repeats the mind-body split the patient fears and can feel dismissive. Psychological factors influence pain alongside biology rather than replacing it as the explanation.

Takeaway: Chronic pain is real and shaped by interacting biological, psychological and social factors; explain it that way, not as either physical or mental.

Sources: Gatchel, R. J., Peng, Y. B., Peters, M. L., Fuchs, P. N., & Turk, D. C. (2007). The biopsychosocial approach to chronic pain: Scientific advances and future directions. Psychological Bulletin, 133(4), 581-624, biopsychosocial model of chronic pain

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154. ASM-1566 (rev 1) · Best answer: C

A social worker in private practice has seen a client for 8 sessions for work stress, based on a biopsychosocial assessment completed at intake. Today the client mentions, for the first time, an eating disorder in their teens and says they have started skipping meals again during a busy project. What should the social worker do **FIRST**?

A. Refer the client to an eating disorder treatment program

B. Continue the stress plan and monitor meals in later sessions

C. Ask about current eating patterns and physical health

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Specialized treatment may be needed, and referral becomes appropriate if assessment shows significant restriction or medical risk. Referring before learning the current pattern hands the client off without knowing what level of care fits.

B **Incorrect** (trap: Ignores risk). Continuing unchanged treats a possible relapse as background detail. A past eating disorder plus renewed restriction calls for prompt assessment, not watchful waiting.

C **Correct.** Biopsychosocial assessment is ongoing rather than a one-time intake task, and new information calls for updating it (Hepworth et al.). A past eating disorder with renewed meal skipping changes both the biological and psychological picture. Asking now about frequency, weight change, purging, physical symptoms and medical care updates that picture before the plan is revised. It beats A because the right level of care cannot be chosen until current severity is known; a specialized referral may well follow.

Takeaway: Biopsychosocial assessment is ongoing; when new history or symptoms emerge, update the assessment before revising the plan or referring.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment as an ongoing process
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155. ASM-1658 (rev 1) · Best answer: A

During a biopsychosocial intake at a primary care clinic, a 47-year-old client reports 2 weeks of low mood, irritability, tearfulness and sleeping about 3 hours a night. The medical history shows the client began high-dose prednisone for a lupus flare 3 weeks ago. The client has no past mood episodes and wants help 'getting back to normal.' What should the social worker do **FIRST**?

A. Ask consent to tell the prescriber about the mood changes
B. Begin weekly CBT sessions targeting the low mood and sleep
C. Use a PHQ-9 to confirm and record a depressive disorder

Why each option is right or wrong

A **Correct.** The biological part of a biopsychosocial assessment includes medications, because some drugs cause psychiatric symptoms. Corticosteroids such as prednisone commonly produce mood changes, irritability and insomnia, especially at high doses (Kaplan & Sadock). Symptoms that began a week after high-dose prednisone started, with no prior mood history, point to a possible medication effect that only the prescriber can weigh. Sharing the timing, with consent, lets the prescriber decide on any change. It beats B because starting therapy first treats a possible drug effect as a primary mood disorder; DSM-5-TR separates medication-induced depressive symptoms from independent episodes.

B **Incorrect** (trap: Right action, wrong time). Support for low mood and sleep may help, and therapy can run alongside medical care. Starting it first assumes a primary mood disorder before the prescriber has weighed a likely medication effect.

C **Incorrect** (trap: Skips assessment). A PHQ-9 can track symptoms, but a screener does not diagnose, and the timing suggests the symptoms may be medication-induced. Recording a depressive disorder now could mislabel a drug effect.

Takeaway: When symptoms begin soon after a new medication starts, share the timing with the prescriber before treating them as a primary psychiatric disorder.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), medication-induced psychiatric symptoms: corticosteroids; DSM-5-TR (APA, 2022), Depressive Disorders: Substance/Medication-Induced Depressive Disorder

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156. ASM-1659 (rev 2) · Best answer: C

An emergency department social worker is asked to see a 68-year-old patient who was treated for a minor fall and is medically ready to leave. Transport home is arranged for 1 hour from now. The nurse notes that the patient seemed unsure how they would manage at home. What is the **BEST** way to approach the assessment?

A. Complete a full biopsychosocial history before the ride home arrives

B. Leave the assessment to the patient's primary care provider

C. Focus on home safety, supports and daily functioning now

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). A full history is the ideal in ongoing care, but it is not realistic in an hour and is not needed for this decision. Spreading attention across every domain risks missing the home-safety facts that matter now.

B **Incorrect** (trap: Hands off too soon). Primary care can follow up, but the nurse's concern bears on today's discharge. Deferring the whole assessment sends an uncertain patient home without anyone checking supports or safety.

C **Correct.** A biopsychosocial assessment is tailored to the setting, the purpose and the time available, so its depth follows the decision at hand (Hepworth et al.). Here the decision is whether this patient can safely go home within the hour. The priority areas are home safety, available help, daily functioning and any risk. Less urgent areas, such as developmental or detailed family history, can be gathered later by community providers. It beats A because trying to cover every domain in an hour would thin out the very questions that decide a safe discharge.

Takeaway: Match the depth and focus of a biopsychosocial assessment to the setting, purpose and time; start with the domains that drive the immediate decision.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Multidimensional assessment: adapting scope to setting and purpose

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157. ASM-1660 (rev 2) · Best answer: B

A hospital social worker begins a biopsychosocial assessment with a 74-year-old patient admitted overnight after a fall. While describing their living situation, the patient becomes anxious and says their spouse, who has major neurocognitive disorder and needs daily help, has been home alone since the ambulance came yesterday afternoon. The patient cannot name anyone who has checked on the spouse. What should the social worker do **FIRST**?

A. Finish the assessment, then plan for the spouse's care
B. Arrange for someone to see the spouse at home right away
C. Report the spouse's situation to adult protective services

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Completing the assessment is the usual order, and the remaining domains still matter. But an adult who cannot care for themselves has been alone for many hours, and waiting until the interview ends risks harm that can be prevented now.

B **Correct.** When an assessment uncovers an immediate safety need, that need takes priority over finishing the interview; safety and basic needs come first (Maslow, 1943). A spouse with major neurocognitive disorder who needs daily help has been alone since yesterday, and no one has checked. Arranging an urgent in-person check, through a relative, a neighbor or, if needed, a police welfare check, protects someone who cannot protect themselves. It beats C because an adult protective services referral takes time to reach the home; it fits if no safe care can be arranged once the spouse has been seen.

C **Incorrect** (trap: Right action, wrong time). Adult protective services may be needed if no safe care can be arranged, but a report does not get anyone to the home quickly. The urgent check comes first, and a report can follow if needed.

Takeaway: When assessment reveals an immediate safety need, such as a dependent adult left alone, act on it before completing the assessment.

Sources: Maslow, A. H. (1943). A theory of human motivation. Psychological Review, 50(4), 370-396, hierarchy of needs: safety and physiological needs
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158. ASM-0105 (rev 1) · Best answer: A

A client in an outpatient program arrives restless and irritable after missing 2 days of treatment. The social worker notices yawning, a runny nose, watery eyes, dilated pupils and goosebumps. The client reports muscle aches and diarrhea. Which condition is the client **MOST** likely experiencing?

A. Opioid withdrawal
B. Opioid intoxication
C. Stimulant intoxication
D. Alcohol withdrawal

Why each option is right or wrong

A **Correct.** Opioid withdrawal produces flu-like signs: yawning, runny nose, tearing, dilated pupils, goosebumps, muscle aches and diarrhea, along with restlessness and low mood (DSM-5-TR). This client shows nearly the full cluster. It beats D because alcohol withdrawal centers on tremor, sweating, rapid pulse and possible seizures or hallucinations, without the tearing, runny nose, yawning and goosebumps typical of opioids. Opioid withdrawal is rarely life-threatening in healthy adults, but it raises overdose risk if the client returns to use with lower tolerance.

B **Incorrect** (trap: Near-miss diagnosis). Opioid intoxication looks nearly opposite: constricted (pinpoint) pupils, drowsiness, slurred speech and slowed breathing. Dilated pupils and restlessness point away from intoxication.

C **Incorrect** (trap: Near-miss diagnosis). Stimulant intoxication can cause dilated pupils and restlessness, which makes it tempting. It does not explain yawning, tearing, runny nose, goosebumps and diarrhea, which together point to opioid withdrawal.

D **Incorrect** (trap: Near-miss diagnosis). Alcohol withdrawal brings tremor, sweating, rapid heartbeat, insomnia and, in severe cases, seizures or hallucinations. It lacks the runny nose, tearing, yawning and goosebumps seen here.

Takeaway: Opioid withdrawal: yawning, tearing, runny nose, dilated pupils, goosebumps, aches and diarrhea. Opioid intoxication: pinpoint pupils and sedation.

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Opioid Withdrawal; DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Opioid Intoxication
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159. ASM-1055 (rev 1) · Best answer: C

A school social worker meets a quiet 10-year-old whose parent drinks heavily. Teachers describe the student as 'no trouble at all.' The student spends recess alone drawing, rarely asks for help and is often forgotten when groups are formed. At home, the student stays in their room for hours. Which family role in alcohol-affected families does this pattern **MOST** likely reflect?

A. Family hero
B. Scapegoat
C. Lost child
D. Mascot

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The family hero is the high achiever who takes on responsibility and earns praise to make the family look good. Being undemanding resembles the hero's good behavior, but this student seeks no achievement or recognition.

B **Incorrect** (trap: Wrong model or concept). The scapegoat acts out and draws blame and negative attention away from the parent's drinking. This student avoids attention entirely, the opposite of the scapegoat's defiance.

C **Correct.** Wegscheider-Cruse described roles children may adopt in families affected by addiction. The lost child copes by withdrawing: quiet, solitary, undemanding and easily

overlooked, which matches this student's isolation and invisibility in groups. It beats A, the family hero, because the hero seeks achievement and approval that bring the family credit, whereas this student draws no attention at all. The roles are descriptive, not diagnoses, but they help workers notice children whose distress is easy to miss (Wegscheider-Cruse, 1989).

D **Incorrect** (trap: Wrong model or concept). The mascot uses humor and clowning to ease family tension and draw attention. This student is quiet and solitary rather than playful or attention-seeking.

Takeaway: In addiction-affected families, a withdrawn, undemanding, overlooked child fits the lost child role, whose distress is easily missed.

Sources: Wegscheider-Cruse, S. (1989). Another Chance: Hope and Health for the Alcoholic Family (2nd ed.). Science and Behavior Books, family roles

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160. ASM-1056 (rev 2) · Best answer: A

Staff at a youth drop-in center ask a social worker to see a 13-year-old who arrived dizzy and unsteady. The youth's speech is slurred, the eyes make jerky side-to-side movements, and there is silver paint around the nose and mouth. The youth's clothes smell strongly of chemicals. Which condition is **MOST** likely?

A. Inhalant intoxication

B. Alcohol intoxication

C. Sedative-hypnotic intoxication

Why each option is right or wrong

A **Correct.** Inhalant intoxication brings dizziness, slurred speech, unsteady gait and nystagmus (jerky eye movements), and it is most common in young adolescents because household products are easy to get. Paint residue around the nose and mouth and a chemical odor on clothing are telltale signs of breathing in vapors from paints or solvents. It beats B, alcohol intoxication, which shares the slurring, unsteadiness and nystagmus but would not explain paint on the face or a solvent smell (DSM-5-TR). Inhalants can cause sudden death, so medical assessment is warranted.

B **Incorrect** (trap: Near-miss diagnosis). Alcohol intoxication produces slurred speech, poor coordination and nystagmus, so it fits much of the picture. It does not account for the paint residue around the nose and mouth or the chemical odor.

C **Incorrect** (trap: Near-miss diagnosis). Sedative-hypnotic intoxication, as from benzodiazepines, also brings slurred speech, unsteadiness and nystagmus, so it fits the physical signs. It does not explain the paint residue around the nose and mouth or the chemical odor on the clothing.

Takeaway: Dizziness, slurring and nystagmus with paint or solvent residue and chemical odor indicate inhalant intoxication, most common in young adolescents.

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Inhalant Intoxication

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161. ASM-1057 (rev 2) · Best answer: C

An adult walks into an outpatient clinic asking for help staying sober. The client drank about a fifth of liquor daily for years and stopped on their own 30 hours ago. The client's hands shake, they are sweating and restless, and they briefly saw insects on the wall this morning. What should the social worker do **FIRST**?

A. Help the client enroll in outpatient withdrawal care tomorrow
B. Book an intake for the intensive outpatient program
C. Arrange same-day medical care for alcohol withdrawal

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Ambulatory withdrawal management suits many people with mild to moderate withdrawal, so this is tempting. Hallucinations 30 hours after heavy daily drinking signal more serious withdrawal during the peak seizure window, so medical evaluation cannot wait until tomorrow (ASAM, 2020).

B **Incorrect** (trap: Right action, wrong time). Intensive outpatient care may suit the client after withdrawal is managed. Booking it now leaves active withdrawal with hallucinations untreated during the period of highest seizure risk.

C **Correct.** Withdrawal after heavy daily drinking can progress to seizures and delirium tremens, which can be fatal, so it needs medical management. Tremor, sweating and restlessness 30 hours after the last drink, plus brief hallucinations, show withdrawal is under way in the window when seizures most often occur. Same-day medical care comes before further screening or program planning. It beats A because starting care tomorrow leaves the client untreated through the hours when seizures are most likely, and hallucinations call for a medical judgment about the safe level of care now (ASAM, 2020; DSM-5-TR).

Takeaway: Visible alcohol withdrawal after heavy daily drinking needs same-day medical management before screening or program placement.

Sources: ASAM (2020). The ASAM Clinical Practice Guideline on Alcohol Withdrawal Management. Journal of Addiction Medicine, 14(3S), 1-72, Assessment and level of care; DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Alcohol Withdrawal

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162. ASM-1058 (rev 2) · Best answer: B

A child protective services social worker receives a report that the parent of a 3-year-old is 'on methadone.' With the parent's consent, the opioid treatment program confirms 8 months of steady attendance, negative drug screens and newly approved take-home doses. The child appears healthy at the first home visit. What is the social worker's **BEST** approach?

A. Close the report, since the parent's methadone is lawful care
B. Assess parenting and safe storage of the take-home doses
C. Require tapering the doses before the case can close

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Methadone is lawful, effective treatment, and the report should not be treated as proof of neglect. Closing without any assessment, though, skips the storage of take-home doses around a 3-year-old, the risk that matters here.

B **Correct.** Medication for opioid use disorder (MOUD) is evidence-based treatment (SAMHSA TIP 63), so receiving it is not by itself evidence of maltreatment. The real child-safety questions are the parent's daily functioning and whether the new take-home methadone is stored out of a young child's reach, since accidental ingestion can be fatal. Federal rules require programs to use child-resistant packaging and teach safe home storage (42 CFR 8.12). It beats A because closing without checking storage skips the one concrete risk the facts present.

C **Incorrect** (trap: Outside the social work role). Medication decisions belong to the parent and the prescriber. Requiring a taper reflects stigma, raises relapse and overdose risk, and runs against evidence that MOUD supports stable parenting.

Takeaway: A parent's MOUD is treatment, not maltreatment; assess actual parenting and safe storage of take-home doses.

Sources: SAMHSA TIP 63 (2021), Part 1: Introduction to Medications for Opioid Use Disorder Treatment; 42 CFR Part 8, 42 CFR 8.12(i)(4)
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163. ASM-1059 (rev 1) · Best answer: A

On a home visit, a 77-year-old client who was widowed last year has had 2 falls in 3 months and forgets recent conversations. An adult child asks the social worker to arrange memory testing. The social worker notices many empty wine bottles in the recycling bin, and the client says a few glasses help with sleep. What should the social worker do **FIRST**?

A. Ask about the amount and timing of the client's drinking
B. Refer the client for the memory testing the family requested
C. Arrange a home safety evaluation to prevent further falls

Why each option is right or wrong

A **Correct.** In older adults, alcohol misuse often shows up as falls, memory problems, poor sleep or low mood and is easily mistaken for normal aging or dementia (SAMHSA TIP 26). The empty bottles and the client's use of wine for sleep after a major loss make drinking a likely contributor to both the falls and the forgetfulness. A respectful, specific question about quantity and pattern comes first because it shapes every next step. It beats B because memory testing done during heavy drinking may credit alcohol effects to a neurocognitive disorder.

B **Incorrect** (trap: Right action, wrong time). Memory evaluation may well be needed, but ordering it first ignores a visible, treatable contributor. Testing is more accurate once drinking has been assessed and, if needed, addressed.

C **Incorrect** (trap: Good but not best). Fall-proofing the home is worthwhile and may follow. It treats the falls as an environmental problem, though, and leaves a possible alcohol cause of falls and forgetfulness unassessed.

Takeaway: In older adults, falls and forgetfulness can signal alcohol misuse; assess drinking before attributing problems to aging or dementia.

Sources: SAMHSA TIP 26 (2020), Ch. 4: Treating Alcohol Misuse in Older Adults
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164. ASM-1060 (rev 1) · Best answer: D

A 7-year-old has lived with a relative caregiver since infancy. Child welfare records show the birth parent drank heavily throughout the pregnancy. The 7-year-old repeats the same mistakes despite consequences, forgets rules learned the day before and has a small head size noted by the pediatrician. The school asks the social worker to arrange an ADHD evaluation. What is the social worker's **MOST** appropriate recommendation?

A. Arrange the ADHD evaluation that the school requested
B. Start a home reward chart to teach rule following
C. Refer for trauma-focused therapy about early separation
D. Request a fetal alcohol spectrum disorder evaluation

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). ADHD is common in children with prenatal alcohol exposure, so attention should be assessed. An ADHD-only evaluation, though, can miss the wider memory and learning deficits that the exposure history points to.

B **Incorrect** (trap: Skips assessment). Reward charts rely on learning from consequences, the very skill this child struggles with. Home strategies should follow an evaluation that clarifies the cause of the difficulties.

C **Incorrect** (trap: Near-miss diagnosis). Early separation can matter, but the child has had one stable caregiver since infancy, and the memory and learning pattern with small head size points to a neurodevelopmental cause.

D **Correct.** Prenatal alcohol exposure can cause lifelong problems with memory, learning from consequences and attention, sometimes with physical signs such as small head size. With documented heavy drinking in pregnancy plus these features, a specialized fetal alcohol spectrum disorder (FASD) evaluation is the most complete assessment, and it also examines attention. It beats A because an ADHD-only evaluation may label the attention problems while missing the broader cause that shapes school supports and caregiver strategies (Hoyme et al., 2016; DSM-5-TR).

Takeaway: Documented prenatal alcohol exposure with learning, memory and physical signs warrants a specialized FASD evaluation, not an ADHD-only workup.

Sources: Hoyme, H. E., et al. (2016). Updated clinical guidelines for diagnosing fetal alcohol spectrum disorders. Pediatrics, 138(2), e20154256, whole article; DSM-5-TR (APA, 2022), Conditions for Further Study: Neurobehavioral Disorder Associated With Prenatal Alcohol Exposure
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165. ASM-1156 (rev 2) · Best answer: C

Teachers ask a high school social worker about a 16-year-old with strong past grades who, over the last 3 weeks, has become restless, irritable, anxious, and unable to focus in morning classes. The changes began when the school installed vape sensors in the restrooms. A classmate says the student used to vape between every class. The student now asks to leave campus at lunch and comes back calmer. Which explanation is **MOST** likely?

A. Attention-deficit/hyperactivity disorder
B. Generalized anxiety disorder
C. Tobacco withdrawal

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). ADHD is a common referral question when focus drops, but its symptoms must begin in childhood and show up across settings. A sudden change at 16 in a student with a strong record, tied to a specific school event, does not fit.

B **Incorrect** (trap: Near-miss diagnosis). Generalized anxiety brings restlessness and poor concentration, but it centers on months of hard-to-control worry about many things. These symptoms began abruptly with the sensors and ease after time off campus, a pattern that points to a substance.

C **Correct.** Tobacco withdrawal follows stopping or sharply cutting back daily nicotine use and brings irritability, restlessness, anxiety and trouble concentrating, often within a day (DSM-5-TR). A student who vaped between every class likely had daily use, and the restroom sensors cut it off during the school day, and the symptoms ease after leaving campus at lunch. It beats A because ADHD symptoms must start in childhood and appear across settings, while these began 3 weeks ago in a student with a strong record.

Takeaway: New irritability, restlessness and poor focus that start when daily nicotine use is interrupted suggest tobacco withdrawal, not a new disorder.

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Tobacco Withdrawal; DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder
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166. ASM-1157 (rev 1) · Best answer: A

In the lobby of a community mental health center, the social worker finds a client who has opioid use disorder slumped in a chair. The client does not respond to a loud voice or a firm knuckle rub on the breastbone. The client's lips look bluish, and they are making slow snoring sounds. Naloxone is kept at the front desk. What should the social worker do **FIRST**?

A. Call 911 and give the naloxone
B. Place the client on their side and hold naloxone in reserve
C. Ask the front desk whether the client took anything

Why each option is right or wrong

A **Correct.** Unresponsiveness, bluish lips and slow, snoring breaths are signs of opioid overdose; the snoring signals blocked or failing breathing, not sleep. SAMHSA's toolkit directs responders to call 911 and give naloxone, then support breathing and stay with the person (SAMHSA, 2018). It beats C because learning what the client took can help responders, but every minute without naloxone deepens oxygen loss, and the signs already present are enough to act on.

B **Incorrect** (trap: Ignores risk). Placing the person on their side protects the airway and is part of overdose care. Holding naloxone back while the client is unresponsive and barely breathing delays the drug that reverses the overdose; it should be given right away.

C **Incorrect** (trap: Right action, wrong time). Information about what was taken is useful to emergency staff and can be gathered once help is on the way. Seeking it first delays naloxone when the overdose signs are already clear.

Takeaway: Unresponsiveness with slow or snoring breathing and bluish lips means possible opioid overdose: call 911 and give naloxone without delay.

Sources: SAMHSA (2018) Opioid Overdose Prevention Toolkit, Five Essential Steps for First Responders
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167. ASM-1158 (rev 2) · Best answer: B

An emergency department asks the social worker to see a young adult patient on a fourth visit this year for days of repeated vomiting. Staff wonder about an eating disorder. Labs and imaging have been normal. The patient describes smoking cannabis several times a day for years, taking long hot showers that ease the nausea, and smoking more to settle the stomach. What should the social worker do **NEXT**?

A. Tell the patient that cannabis is causing the vomiting
B. Share the use and bathing history with the physician
C. Refer the patient for an eating disorder evaluation

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Cannabis education will matter once the physician confirms the cause, so this step belongs later. Stating the cause to the patient now, before the medical team has weighed the history, amounts to a diagnosis, which is outside the social work role.

B **Correct.** Social workers gather history but do not diagnose medical conditions. Years of daily cannabis use, repeated vomiting episodes and relief from hot showers form the typical pattern of cannabinoid hyperemesis syndrome, which stopping cannabis usually resolves (Sorensen et al., 2017). Because patients often use more cannabis to ease the nausea, the pattern is easy to miss. It beats A because the information is likely accurate, but the diagnosis belongs to the physician, who can confirm it and then explain it to the patient.

C **Incorrect** (trap: Skips assessment). Eating disorders can involve vomiting, but the history the patient gave points to a substance-related cause. Referring now acts on staff's first guess and sets aside information the medical team should weigh first.

Takeaway: Pass substance-use history that may explain physical symptoms to the medical team; diagnosis belongs to the physician.

Sources: Sorensen, C. J., DeSanto, K., Borgelt, L., Phillips, K. T., & Monte, A. A. (2017). Cannabinoid hyperemesis syndrome: Diagnosis, pathophysiology, and treatment, a systematic review. Journal of Medical Toxicology, 13(1), 71-87, Clinical features; treatment

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168. ASM-1545 (rev 2) · Best answer: A

A school social worker learns that a 7-year-old often arrives hungry and wears the same clothes several days in a row. The parent, who works two part-time jobs, explains that the family's food benefits stopped 2 months ago after a missed renewal appointment. The child is clean and up to date on checkups, and the parent attends every school conference. What should the social worker do **FIRST**?

A. Assist with restoring the lapsed aid and arranging free meals

B. Notify child protective services of possible neglect

C. Interview the child privately about conditions at home

Why each option is right or wrong

A **Correct.** Poverty is not neglect. Child welfare guidance stresses separating the two and meeting families' concrete needs, because neglect involves a caregiver's failure to provide care, not a temporary lack of resources (Child Welfare Information Gateway, 2023). This parent is working, engaged and seeking help, and the hunger traces to a lapsed benefit. Restoring the lapsed food benefits and arranging free school meals addresses the cause directly. It beats B because a report would bring an investigation to a family whose problem is a benefits gap; a report becomes appropriate if needs go unmet after help is offered or other signs of harm appear.

B **Incorrect** (trap: Misreads law or ethics). Mandated reporters report reasonable suspicion of maltreatment, but these facts point to a temporary loss of income support, not a caregiver's failure to provide. Reporting first treats poverty as neglect and can harm a family that is already seeking help.

C **Incorrect** (trap: Good but not best). Hearing from the child matters when the cause of hunger is unclear. Here the parent has explained the lapse in benefits and other care is consistent, so a private interview delays the practical help the child needs now.

Takeaway: Distinguish poverty from neglect: when a family's unmet needs stem from lack of resources, connect them to concrete supports first.

Sources: Child Welfare Information Gateway (2023). Separating Poverty From Neglect in Child Welfare, bulletin for professionals

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169. ASM-1546 (rev 2) · Best answer: B

A 52-year-old patient is readmitted to the hospital with high blood sugar for the third time in 4 months. A team note calls the patient 'noncompliant with insulin.' The patient works two part-time jobs without health benefits and takes two buses to reach the nearest pharmacy. What should the social worker do **FIRST**?

A. Teach the patient about complications of skipping insulin
B. Ask the patient what makes getting and using insulin difficult
C. Refer the patient to a diabetes self-management education class

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Explaining complications assumes the patient does not understand the risks and can feel like a lecture. A patient readmitted three times likely knows the danger; without learning the barriers, more information is unlikely to change anything.

B **Correct.** Social determinants of health, including economic stability and access to health care, shape outcomes (ODPHP, Healthy People 2030). Labeling a patient noncompliant can hide barriers such as cost, lack of coverage, transportation and work schedules, all hinted at here. Asking the patient directly what gets in the way identifies the actual barrier before choosing a response. It beats C because diabetes education assumes a knowledge gap; if the problem is paying for insulin or reaching the pharmacy, classes will not prevent the next readmission.

C **Incorrect** (trap: Right action, wrong time). Self-management education is valuable when knowledge or skills are missing. Referring before asking about barriers may send a patient who cannot afford insulin or reach the pharmacy to a class that addresses neither problem.

Takeaway: Before labeling a patient nonadherent, assess social determinants such as cost, coverage, transportation and work demands that block treatment.

Sources: Office of Disease Prevention and Health Promotion (ODPHP) (2020). Healthy People 2030: Social Determinants of Health, SDOH domains: economic stability; health care access and quality
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170. ASM-1550 (rev 2) · Best answer: D

A client with bipolar disorder attends a supported employment program and receives Social Security disability benefits and Medicaid. The employer offers the client more weekly hours. The client says, 'I want the hours, but if I earn more I could lose my check and my medication coverage.' Unsure how earnings affect either benefit, the client asks the social worker what to do. What is the **BEST** response?

A. Encourage the client to accept the hours as a recovery step
B. Explain that the added earnings will end the client's benefits
C. Explore what the extra hours would mean to the client
D. Refer the client to a work incentives benefits planner

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Work can support recovery, but urging the client to accept brushes past a real financial risk the client raised. Encouragement fits once the client has accurate information about how earnings affect benefits and chooses to go ahead.

B **Incorrect** (trap: Outside the social work role). Work incentive rules often let beneficiaries earn some income while keeping benefits or health coverage, so this statement may be wrong. Predicting benefit outcomes requires specialized knowledge the social worker does not have.

C **Incorrect** (trap: Good but not best). Exploring meaning helps when a client is unsure what they want. This client already wants the hours and has named a specific, factual barrier, so exploration alone leaves the real obstacle in place.

D **Correct.** Fear of losing cash benefits and health coverage is a common poverty trap that shapes work decisions. How earnings affect Social Security benefits and Medicaid is complex and individual, and free work incentives counseling is available to Social Security disability beneficiaries ages 14 to full retirement age (SSA, Ticket to Work). Referring when another professional's specialized knowledge is needed follows NASW Code 1.16. It beats C because the client has already stated both the wish and the fear; what is missing is accurate benefits information, not more exploration.

Takeaway: When benefit rules tied to poverty shape a client's work decisions, connect the client to specialized benefits counseling before the decision is made.

Sources: Social Security Administration (SSA), Ticket to Work (2025). Benefits Counseling and the Path to Employment, Work Incentives Planning and Assistance (WIPA) fact sheet; NASW Code of Ethics (2021), 1.16(a) Referral for Services

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171. ASM-1551 (rev 2) · Best answer: A

A community mental health client with depression has missed 3 of 5 appointments, and clinic policy allows closure after 3 missed visits. When reached, the client explains that their hourly job posts shifts only 2 days ahead and their phone is often shut off for unpaid bills, so reminder calls fail. The client says they want to keep coming. What is the **MOST** appropriate response?

A. Offer short-notice scheduling and a workable contact method

B. Explore the client's ambivalence about continuing treatment

C. Close the case and invite the client to reapply later

Why each option is right or wrong

A **Correct.** Poverty shapes service use: unpredictable hourly shifts and unstable phone service are economic barriers, not a lack of motivation (ODPHP, Healthy People 2030). Because the client wants care, the agency adapts how it delivers services. Short-notice slots fit a schedule posted 2 days ahead, and an agreed contact method replaces reminder calls that never arrive. It beats B because nothing in the client's account suggests mixed feelings about therapy; exploring ambivalence would treat a structural barrier as a motivational one.

B **Incorrect** (trap: Good but not best). Missed visits can signal ambivalence, and exploring it fits when a client seems unsure about therapy. This client named concrete scheduling and phone barriers and wants to continue, so the problem is access rather than motivation.

C **Incorrect** (trap: Hands off too soon). Closing the case penalizes the client for the effects of low-wage, unstable work. Closure could follow if the client chose to stop or barriers remained after real accommodations, but no adjustment has been tried yet.

Takeaway: When missed appointments stem from poverty-related barriers, adapt how services are delivered before treating attendance as a motivation problem.

Sources: Office of Disease Prevention and Health Promotion (ODPHP) (2020). Healthy People 2030: Social Determinants of Health, SDOH domain: economic stability

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172. ASM-1653 (rev 2) · Best answer: C

A teacher refers a 9-year-old to the school social worker for an ADHD evaluation, noting that the child cannot focus after mid-morning and puts their head down by lunch. In a phone call, the parent explains that their work hours were cut and food often runs out before payday. The child usually skips breakfast. What should the social worker do **FIRST**?

A. Collect parent and teacher ratings of the child's attention

B. Suggest the parent ask the pediatrician about ADHD medication

C. Connect the family to free meal programs and food aid

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Multi-informant rating scales are a sound part of an ADHD evaluation, but completing them while the child is hungry would blur the picture. They belong after the food problem is addressed, if attention difficulties continue.

B **Incorrect** (trap: Skips assessment). Raising medication jumps past both assessment and the plain explanation in the stem. A social worker also does not steer families toward a specific medication; a pediatric evaluation fits if problems persist after basic needs are met.

C **Correct.** Basic needs come before deeper assessment, and hunger alone can impair a child's concentration. The American Academy of Pediatrics links food insecurity to trouble concentrating in school and to behavior problems (AAP, 2015). Here focus fades as the morning goes on, and the child arrives without breakfast from a household that runs out of food. Linking the family to free school meals and other food aid addresses the likely cause quickly. It beats A because rating scales collected now would be clouded by hunger; if focus problems persist once the child eats regularly, an ADHD evaluation is the right next step.

Takeaway: Address unmet basic needs such as hunger before attributing a child's inattention to ADHD; reassess attention once needs are met.

Sources: American Academy of Pediatrics (AAP), Council on Community Pediatrics & Committee on Nutrition (2015). Promoting food security for all children. Pediatrics, 136(5), e1431-e1438. doi:10.1542/peds.2015-3301, effects of food insecurity on children

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173. ASM-1654 (rev 1) · Best answer: D

A 34-year-old client with schizophrenia grew up in a middle-income family and was in college when symptoms began at age 20. Since then, the client has lost a series of jobs and now lives on disability benefits in low-cost housing. A new staff member remarks that the client's poverty probably caused the illness. Which account of the link between low income and mental illness does this history **MOST** likely support?

A. Social causation
B. Culture of poverty
C. Intergenerational transmission
D. Social selection

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Social causation holds that the stresses of poverty raise the risk of disorder. It fits when hardship comes first, as research suggests for some depression. Here the client's income fell after the illness began, so the history runs the other way.

B **Incorrect** (trap: Wrong model or concept). The culture of poverty is a contested idea describing values said to keep families poor across generations. It does not explain a person moving from middle income into poverty after a psychiatric illness.

C **Incorrect** (trap: Wrong model or concept). Intergenerational transmission describes poverty passed from parents to children through limited resources and opportunity. This client did not grow up poor, so the concept does not fit the history given.

D **Correct.** The social selection, or drift, explanation holds that a disorder leads to lower socioeconomic status as symptoms disrupt school and work. This client started out middle-income and became poor only after onset, the order selection predicts. Dohrenwend and colleagues found that selection appears more important for schizophrenia, while causation appears more important for depression in women and for antisocial personality and substance use disorders in men (Dohrenwend et al., 1992). It beats A because social causation requires poverty and its stresses to come before the illness, which this history does not show.

Takeaway: Social selection (drift): illness lowers status. Social causation: poverty's stress raises illness risk. Evidence for schizophrenia leans toward selection.

Sources: Dohrenwend, B. P., Levav, I., Shrout, P. E., Schwartz, S., Naveh, G., Link, B. G., Skodol, A. E., & Stueve, A. (1992). Socioeconomic status and psychiatric disorders: The causation-selection issue. Science, 255(5047), 946-952, whole article

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II.B Assessment Methods and Techniques

174. ASM-0005 (rev 1) · Best answer: B

During a routine session, a 70-year-old client says, 'Lately I've been thinking about ending my life.' The social worker thanks the client for sharing and stays calm. What should the social worker do **NEXT**?

A. Begin a written safety plan with coping strategies
B. Ask about a plan, intent and access to lethal means
C. Arrange an emergency psychiatric evaluation later today

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A safety plan is a core intervention, but writing it before knowing whether there is a plan, intent or access to means skips the assessment it should rest on.

B **Correct.** Once a client discloses suicidal thoughts, the social worker completes the risk assessment by asking directly about a plan, intent, preparation and access to lethal means. The answers set the level of risk and the response (Posner et al., 2011). Asking directly does not increase suicidal thinking (Dazzi et al., 2014). It beats A because safety planning is an important intervention, but it should be built on what the assessment shows. For example, an older adult's access to firearms or stored medications would shape the plan.

C **Incorrect** (trap: Skips assessment). Emergency evaluation is right if the assessment shows intent, a plan or imminent danger. Arranging it now escalates before the social worker knows the level of risk.

Takeaway: After a suicidal disclosure, finish the risk assessment, including plan, intent and means, before choosing safety planning or emergency action.

Sources: Posner, K., et al. (2011). The Columbia-Suicide Severity Rating Scale: Initial validity and internal consistency findings from three multisite studies with adolescents and adults. American Journal of Psychiatry, 168(12), 1266-1277, whole article; Dazzi, T., Gribble, R., Wessely, S., & Fear, N. T. (2014). Does asking about suicide and related behaviours induce suicidal ideation? What is the evidence? Psychological Medicine, 44(16), 3361-3363, whole article

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175. ASM-0109 (rev 1) · Best answer: A

In an integrated primary care clinic, a client who lost their job 3 months ago meets with the social worker about stress. The client says, 'My family would be better off without me,' and mentions giving a prized guitar to a friend last week. The client then changes the subject to money worries. What should the social worker do **FIRST**?

A. Ask the client directly about thoughts of suicide
B. Arrange an emergency psychiatric evaluation today
C. Reflect the client's feeling of being a burden

Why each option is right or wrong

A **Correct.** Indirect statements about being a burden, combined with giving away a valued possession, are warning signs that call for a direct question about suicidal thoughts. Asking does not plant the idea or raise risk (Dazzi et al., 2014), and a structured inquiry into ideation, plan, intent and means guides the response (Posner et al., 2011). It beats C because reflecting the feeling is supportive but lets the change of subject stand and leaves the risk unassessed. Reflection can lead into the direct question.

B **Incorrect** (trap: Skips assessment). An emergency evaluation is right when assessment shows a plan, intent or imminent danger. Nothing about plan or intent is known yet, so arranging it now escalates before the facts are gathered and may damage trust.

C **Incorrect** (trap: Good but not best). Reflecting the feeling of being a burden shows empathy and can open the door, but on its own it does not find out whether the client is thinking of suicide. With these warning signs, the risk question needs to be asked plainly.

Takeaway: Indirect suicidal statements or warning signs call for a direct question about suicidal thoughts before choosing any intervention.

Sources: Posner, K., et al. (2011). The Columbia-Suicide Severity Rating Scale: Initial validity and internal consistency findings from three multisite studies with adolescents and adults. American Journal of Psychiatry, 168(12), 1266-1277, whole article; Dazzi, T., Gribble, R., Wessely, S., & Fear, N. T. (2014). Does asking about suicide and related behaviours induce suicidal ideation? What is the evidence? Psychological Medicine, 44(16), 3361-3363, whole article

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176. ASM-1080 (rev 2) · Best answer: C

A client with borderline personality disorder has had near-daily passive thoughts of death for years and attends weekly DBT. After a breakup this week, the client says the thoughts feel different. In the risk assessment that follows, the client says they bought two large bottles of acetaminophen yesterday and will 'probably' take them this weekend. The client cannot name anything that would keep them safe. What should the social worker do **NEXT**?

A. Continue DBT skills, noting the thoughts are long-standing
B. Complete a safety plan and add a midweek session
C. Have the client evaluated for hospitalization today
D. Explore what the breakup means to the client

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Chronic passive thoughts can be managed in ongoing DBT. Treating this as baseline ignores the new facts, bought pills, a time frame and no way to stay safe, which signal a sharp rise in acute risk.

B **Incorrect** (trap: Good but not best). A safety plan with more frequent contact suits intermediate acute risk, when a client can keep themselves safe. This client has a method in hand, a likely timeline and cannot name anything that would keep them safe, so outpatient steps are not enough today.

C **Correct.** Suicide risk is best judged on two dimensions: chronic baseline risk and current acute risk (Wortzel et al., 2014). Years of passive thoughts set a high baseline, but the new facts, bought pills, a likely time frame and inability to stay safe, mark high acute risk. VA's risk stratification guidance says high acute risk typically calls for hospitalization with observation until then (Rocky Mountain MIRECC, 2024). It beats B because a safety plan and extra session fit someone who can maintain safety alone; this client cannot.

D **Incorrect** (trap: Right action, wrong time). The breakup's meaning matters and will be part of treatment. With a stated plan, acquired means and a timeline already assessed, exploration now delays the protective step the risk requires.

Takeaway: In chronically suicidal clients, rate acute risk separately; a new plan, acquired means and inability to stay safe call for hospital-level evaluation.

Sources: Wortzel, H. S., Homaifar, B., Matarazzo, B., & Brenner, L. A. (2014). Therapeutic risk management of the suicidal patient: Stratifying risk in terms of severity and temporality. Journal of Psychiatric Practice, 20(1), 63-67, whole article; Rocky Mountain MIRECC for Suicide Prevention (2024). Therapeutic Risk Management: Risk Stratification Table. U.S. Department of Veterans Affairs, Acute risk: high

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177. ASM-1081 (rev 1) · Best answer: A

At a home visit 2 weeks after delivery, a parent has barely slept for 4 days and speaks rapidly about 'signs' that the baby is evil. When asked directly about harm, the parent calmly says the baby 'needs to be sent back to God' and mentions the bathtub. The parent's partner is in the next room. What should the social worker do **NEXT**?

A. Get the parent to a psychiatric emergency service now
B. Book a perinatal psychiatry appointment for this week
C. Explore the parent's spiritual beliefs about the baby
D. Ask the partner to stay with the baby until follow-up

Why each option is right or wrong

A **Correct.** Postpartum psychosis is a psychiatric emergency linked to suicide and infanticide, and hospitalization is generally indicated (ACOG, 2023). Severe sleep loss, rapid speech, delusions about the infant and a calmly stated method show the direct assessment is done and the danger is high. The parent needs emergency evaluation today, with the partner staying with the baby in the meantime. It beats D because supervision alone leaves an acute, treatable psychosis untreated and the infant still exposed to a parent who may act on the delusion.

B **Incorrect** (trap: Ignores risk). Specialized perinatal care fits postpartum depression or anxiety without immediate danger. Psychosis with a stated way of harming the infant cannot wait days for an appointment.

C **Incorrect** (trap: Right action, wrong time). Cultural and religious meaning deserves respect. These beliefs, though, arose with days of sleeplessness and rapid speech after delivery and include a method of harm, which marks psychosis needing emergency care rather than further exploration.

D **Incorrect** (trap: Ignores risk). Keeping the baby with the partner is a useful short-term protective step. As the only response, it leaves the parent's psychosis untreated and relies on round-the-clock supervision that cannot be guaranteed.

Takeaway: Postpartum psychosis with thoughts of harming the infant is a psychiatric emergency: arrange same-day emergency evaluation.

Sources: American College of Obstetricians and Gynecologists (2023). Clinical Practice Guideline No. 5: Treatment and Management of Mental Health Conditions During Pregnancy and Postpartum, Postpartum psychosis
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178. ASM-1082 (rev 1) · Best answer: C

In a youth residential program, staff find fresh, shallow cuts on a 15-year-old resident's thigh. The resident tells the social worker, 'It's not like that. It just takes the pressure off when I get overwhelmed.' The unit manager wants to start one-to-one suicide watch and remove the resident's privileges for a week. What should the social worker do **FIRST**?

A. Start one-to-one observation until the psychiatrist visits
B. Teach replacement coping skills for overwhelming feelings
C. Assess the self-injury's function and any wish to die

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Close observation is right if assessment shows suicidal intent. Started before any assessment, and paired with lost privileges, it can feel punitive, push self-injury into hiding and still leave intent unknown.

B **Incorrect** (trap: Right action, wrong time). Skills such as distress tolerance are a core treatment for self-injury. They come after the social worker knows whether the cutting relieves emotion or carries intent to die, which changes the immediate plan.

C **Correct.** Nonsuicidal self-injury is self-harm without intent to die, often used to relieve intense emotion, while any intent to die makes an act suicidal (Nock, 2010; DSM-5-TR). The resident's words suggest emotional relief, but only a direct assessment of function, intent and history can tell the two apart and set the response. It beats A because one-to-one watch fits if the assessment finds suicidal intent; imposing it with punishment first can shame the youth and does not replace the assessment.

Takeaway: Self-injury needs assessment of its function and of suicidal intent before deciding on restrictions or treatment.

Sources: Nock, M. K. (2010). Self-injury. Annual Review of Clinical Psychology, 6, 339-363, Definitions; functions of self-injury; DSM-5-TR (APA, 2022), Conditions for Further Study: Nonsuicidal Self-Injury Disorder
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179. ASM-1083 (rev 2) · Best answer: B

A rural client in their 60s, in treatment for depression after losing the family farm, reports passive thoughts of death most days. The client keeps several hunting rifles at home and refuses to sell them, saying hunting is part of who they are. The client's sibling lives nearby and owns a gun safe. What is the social worker's **BEST** response?

A. Accept the refusal and note the rifles in the chart
B. Discuss temporary storage at a relative's home
C. Arrange a short voluntary hospital stay for safety

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Respecting the client's attachment to hunting matters, and documenting firearm access is necessary. Recording the rifles without acting leaves the most lethal method at hand for someone with near-daily thoughts of death.

B **Correct.** Lethal-means counseling asks at-risk people and their families to keep firearms away from the person until the crisis passes, often through temporary off-site storage with a relative or friend (Barber & Miller, 2014). Suicidal crises can be brief, so distance from a gun buys time. Temporary storage with the sibling honors the refusal to sell and the client's identity as a hunter while lowering risk. It beats A because noting the rifles without action leaves the means in place.

C **Incorrect** (trap: Skips assessment). Hospitalization fits high acute risk with intent or a plan. Passive thoughts of death, without stated intent or a plan, call for less restrictive steps, and a hospital stay does nothing about the rifles waiting at home.

Takeaway: For an at-risk client who keeps firearms, counsel temporary off-site storage with a trusted person until the risk passes.

Sources: Barber, C. W., & Miller, M. J. (2014). Reducing a suicidal person's access to lethal means of suicide: A research agenda. American Journal of Preventive Medicine, 47(3S2), S264-S272, Firearms: temporary off-site storage

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180. ASM-1084 (rev 2) · Best answer: A

Police bring an adult to the emergency department after a neighbor heard the person shouting that they would 'end it tonight.' The patient smells of alcohol, slurs words and walks unsteadily. Now the patient laughs it off, says, 'I was just drunk,' and asks to leave. What should the social worker do **FIRST**?

A. Observe until sober, then do a full assessment
B. Complete the full suicide risk assessment right now
C. Accept the retraction and discharge with a crisis line

Why each option is right or wrong

A **Correct.** Intoxication impairs judgment and makes both suicidal statements and retractions unreliable. The conservative approach is to keep the patient safe and observed until sober enough to take part in a full risk assessment, and not to let the patient leave before it is done (Betz & Boudreaux, 2016). A public threat to 'end it tonight' is not cancelled by a drunken retraction. It beats B because answers given while the patient slurs and staggers cannot be trusted and may falsely reassure; the full assessment belongs after sobering.

B **Incorrect** (trap: Right action, wrong time). A thorough assessment is the goal, and basic safety questions can be asked now. A full assessment while the patient is visibly intoxicated, though, yields unreliable answers. It should be completed once the patient can take part meaningfully.

C **Incorrect** (trap: Ignores risk). A retraction may be sincere, but one made while intoxicated, hours after a threat heard by a neighbor, cannot be relied on. Discharge before a sober assessment risks releasing someone at real risk.

Takeaway: Suicidal statements or retractions made while intoxicated are unreliable: keep the person safe and complete the assessment once sober.

Sources: Betz, M. E., & Boudreaux, E. D. (2016). Managing suicidal patients in the emergency department. Annals of Emergency Medicine, 67(2), 276-282, Intoxicated patients

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181. ASM-1085 (rev 1) · Best answer: C

In a nursing facility, a 79-year-old resident with heart failure has refused most meals and several medications since their spouse died 6 weeks ago. The resident says, 'I'm just done. Let me be.' The resident has no advance directive about refusing care. Staff want to honor the refusal as the resident's right. What is the **MOST** appropriate action for the social worker?

A. Support the refusal and document it as an informed choice

B. Request a hospice referral to focus on comfort care

C. Evaluate the refusal for depression and a wish to die

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). A capable resident may refuse care, and staff are right to value that. A refusal that began with a recent loss and sounds hopeless may come from treatable depression, so it is not yet a settled, informed choice.

B **Incorrect** (trap: Skips assessment). Hospice can be right for advanced heart failure when comfort is the resident's considered goal. Referring now treats grief-driven refusal as an end-of-life decision before anyone has assessed mood or intent.

C **Correct.** SAMHSA's toolkit for senior living communities lists refusing food, medicine or other treatment among signs that a person needs help right away (SAMHSA, 2011). Six weeks after a spouse's death, 'I'm just done' with refusal of meals and medicine may reflect depression and a passive wish to die. Assessing mood, hopelessness and intent comes before deciding whether the refusal is an informed choice. It beats A

because honoring the refusal now skips the assessment that tells treatable despair from a considered decision.

Takeaway: In older adults, refusing food or medicine after a loss can signal suicidal intent; assess depression before treating it as an informed choice.

Sources: SAMHSA (2011). Promoting Emotional Health and Preventing Suicide: A Toolkit for Senior Living Communities (SMA10-4515), Fact Sheets for Residents: signs that a person needs help now
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182. ASM-1086 (rev 1) · Best answer: D

A psychiatric inpatient team is planning discharge for a patient with schizophrenia. The record shows the patient attacked neighbors twice in the past 3 years, a current belief that a neighbor is poisoning the water supply, and heavy drinking on weekends. Which factor **MOST** strongly predicts future violence?

A. The persecutory belief about the neighbor
B. Heavy weekend alcohol use
C. The diagnosis of schizophrenia
D. Prior violent behavior

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Persecutory beliefs can raise concern, especially when a specific person is the focus. Their link with violence is weaker and less consistent than a documented history of actual assaults.

B **Incorrect** (trap: Near-miss diagnosis). Substance use is an important, changeable risk factor and should be addressed in discharge planning. It adds to risk but predicts violence less strongly than a record of past violent acts.

C **Incorrect** (trap: Near-miss diagnosis). Most people with schizophrenia are never violent, and the diagnosis alone is a weak predictor. Treating the label as the main risk factor overlooks the behavioral history that matters most.

D **Correct.** Past violence is the most consistent predictor of future violence in people with and without mental disorders; in the MacArthur study, prior arrests and violent acts were among the strongest risk factors (Monahan et al., 2001). Two attacks on neighbors in 3 years outweigh the other factors listed. It beats A because persecutory beliefs may add risk, but their association with violence is weaker and less consistent than a record of actual assaults (Kaplan & Sadock).

Takeaway: Past violent behavior is the strongest single predictor of future violence; diagnosis alone predicts poorly.

Sources: Monahan, J., Steadman, H. J., Silver, E., Appelbaum, P. S., Robbins, P. C., Mulvey, E. P., Roth, L. H., Grisso, T., & Banks, S. (2001). Rethinking Risk Assessment: The MacArthur Study of Mental Disorder and Violence. Oxford University Press, Prior violence and arrest history as risk factors; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Psychiatric emergencies: violence and assaultive behavior
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183. ASM-1087 (rev 1) · Best answer: A

Which finding is a warning sign of near-term suicide risk rather than a long-term risk factor?

A. Searching online for ways to die
B. A family history of death by suicide
C. Childhood physical abuse
D. A chronic pain condition

Why each option is right or wrong

A **Correct.** Warning signs are current behaviors or states that signal suicide risk in the coming minutes to days, such as looking for a way to kill oneself or talking about wanting to die (Rudd et al., 2006). Risk factors are more stable traits or history that raise risk over a lifetime. Searching for methods is a preparatory behavior that calls for immediate assessment. It beats B because a family history of suicide raises long-term risk but says nothing about whether danger is rising now.

B **Incorrect** (trap: Wrong model or concept). Family history of suicide is a well-established risk factor. It is a fixed part of a person's history and does not show that risk is rising at this moment.

C **Incorrect** (trap: Wrong model or concept). Childhood abuse raises lifetime suicide risk. As a historical factor, it does not mark near-term danger the way current preparatory behavior does.

D **Incorrect** (trap: Wrong model or concept). Chronic pain is a recognized risk factor, especially in later life. It is an ongoing condition rather than a sign that a crisis is building now.

Takeaway: Warning signs, such as seeking methods, signal near-term danger; risk factors, such as family history, raise long-term risk.

Sources: Rudd, M. D., et al. (2006). Warning signs for suicide: Theory, research, and clinical applications. Suicide and Life-Threatening Behavior, 36(3), 255-262, whole article
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184. ASM-1088 (rev 1) · Best answer: B

A client with schizoaffective disorder tells a social worker at a supportive housing program that for 3 days the voices have been telling them to step in front of a bus. The client looks tense but is talking openly and agrees to keep meeting today. What should the social worker do **FIRST**?

A. Call for an emergency evaluation for hospitalization
B. Ask whether the client intends to obey the voices
C. Notify the prescriber that the voices have increased

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Emergency evaluation is right if the client intends to obey, has acted on the voices or cannot resist them. With an engaged client who is talking openly, a few minutes of direct assessment come first and guide the level of response.

B **Correct.** Not everyone who hears harmful commands acts on them. Compliance depends on the person's beliefs about the commands and on related delusions (Shawyer et al., 2008), so risk turns on whether the client believes the voices, intends to obey and has a plan. Asking directly sets the response. It beats A because emergency evaluation follows if the client intends to obey or cannot resist; the client is engaged and talking openly, so assessment can and should come first.

C **Incorrect** (trap: Right action, wrong time). The prescriber should learn that the voices have increased. That call belongs after the immediate risk is assessed; it does not answer whether the client may act on the command today.

Takeaway: When a client reports command hallucinations to self-harm, assess intent to obey and any plan before choosing the level of response.

Sources: Shawyer, F., Mackinnon, A., Farhall, J., Sims, E., Blaney, S., Yardley, P., Daly, M., Mullen, P., & Copolov, D. (2008). Acting on harmful command hallucinations in psychotic disorders: An integrative approach. Journal of Nervous and Mental Disease, 196(5), 390-398, whole article

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185. ASM-1089 (rev 1) · Best answer: A

A 66-year-old hospital patient with advanced kidney disease tells the social worker, 'I've turned into nothing but a cost to my kids. They'd manage fine without me.' Which construct from the interpersonal theory of suicide does this statement **MOST** likely reflect?

A. Perceived burdensomeness

B. Thwarted belongingness

C. Acquired capability

Why each option is right or wrong

A **Correct.** Joiner's interpersonal theory holds that the desire for suicide grows from perceived burdensomeness, the belief that one's existence costs others more than it gives, together with thwarted belongingness; acquired capability explains who can act on that desire (Van Orden et al., 2010). Calling oneself a cost the family would be better without is the core burdensomeness belief. It beats B because thwarted belongingness centers on feeling alone or disconnected, not on being a drain on others.

B **Incorrect** (trap: Wrong model or concept). Thwarted belongingness is the painful sense of not belonging or of being cut off from others. The patient's statement is about costing the family, not about feeling alone.

C **Incorrect** (trap: Wrong model or concept). Acquired capability is a reduced fear of death and higher pain tolerance built through painful experiences. It concerns the ability to act on suicidal desire, not the belief expressed here.

Takeaway: Statements that others would be better off without oneself reflect perceived burdensomeness, a core driver of suicidal desire.

Sources: Van Orden, K. A., Witte, T. K., Cukrowicz, K. C., Braithwaite, S. R., Selby, E. A., & Joiner, T. E. (2010). The interpersonal theory of suicide. Psychological Review, 117(2), 575-600, Perceived burdensomeness; thwarted belongingness; acquired capability

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186. ASM-1090 (rev 2) · Best answer: C

On a night of subzero temperatures, an outreach social worker finds a client with schizophrenia sleeping outside in a light jacket. The client's fingers are white and numb. The client insists that shelters 'pump poison through the vents,' cannot describe any other way to stay warm, and refuses a blanket, a ride and the warming center. What should the social worker do **NEXT**?

A. Leave blankets nearby and return early tomorrow
B. Explore what would make a shelter feel safe
C. Call emergency services for medical and psychiatric care

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Respecting refusal and returning is right on most nights. Tonight, with numb, white fingers and no way to stay warm, waiting until morning risks severe frostbite or death.

B **Incorrect** (trap: Right action, wrong time). Engagement around what feels safe is usually the best path with a client who distrusts shelters. Here several options have already been offered and refused, a delusion blocks every route to warmth, and frostbite has begun.

C **Correct.** Nearly all states treat an inability to provide for basic needs because of mental illness as a form of danger to self, which can justify emergency evaluation even over objection (Kaplan & Sadock). Here a delusion blocks every route to warmth on a dangerously cold night, and white, numb fingers suggest frostbite, a medical emergency. Less restrictive options were offered and refused. It beats B because continued engagement fits most nights; tonight delay risks serious injury or death.

Takeaway: When mental illness leaves a person unable to meet basic survival needs in immediate danger, emergency evaluation can override refusal.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Forensic psychiatry: involuntary hospitalization criteria
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187. ASM-1091 (rev 1) · Best answer: B

An adult child calls a geriatric care program about a parent with moderate Alzheimer's disease who keeps a loaded handgun by the bed. Last week the parent woke at night, did not recognize the adult child, and pointed the gun at them. The parent insists the gun is needed for protection. What is the social worker's **BEST** response?

A. Suggest unloading the gun and hiding the ammunition
B. Help the family remove the gun from the home now
C. Wait for a capacity evaluation before changing anything

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Unloading and separate storage can reduce risk in milder impairment with supervision. With moderate dementia and an episode of aiming the gun at a family member, the parent may find or reload it, so partial steps fall short.

B **Correct.** Dementia impairs memory, judgment and recognition, and guidance on firearms holds that people with moderate to severe dementia should have no access; families should remove firearms (Betz et al., 2018). An episode of pointing the gun at an unrecognized family member shows immediate danger to others, which outweighs the parent's wish to keep it. It beats A because unloading and hiding ammunition can suit early stages, but a confused person who has already aimed the gun may find or reload it.

C **Incorrect** (trap: Ignores risk). A capacity evaluation can help with longer-term decisions. Waiting for one leaves a loaded gun with a person who has already pointed it at a family member during nighttime confusion.

Takeaway: In moderate dementia, firearm access endangers others and the person; families should remove firearms rather than rely on partial safeguards.

Sources: Betz, M. E., McCourt, A. D., Vernick, J. S., Ranney, M. L., Maust, D. T., & Wintemute, G. J. (2018). Firearms and dementia: Clinical considerations. Annals of Internal Medicine, 169(1), 47-49, whole article
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188. ASM-1092 (rev 2) · Best answer: A

A middle school social worker learns from a teacher that a 13-year-old student wrote a list titled 'people who will pay' with five classmates' names. Classmates say the student recently talked about a relative's guns. In a brief meeting, the student shrugs and calls the list a joke. What should the social worker do **FIRST**?

A. Refer the concern to the threat assessment team today
B. Call the parents to ask whether the relative's guns are locked
C. Call the police to search the student's home for weapons

Why each option is right or wrong

A **Correct.** School threat assessment relies on a trained multidisciplinary team that gathers information from many sources, including access to weapons, motives and stressors, and then plans risk management (U.S. Secret Service NTAC, 2018). A written list of named classmates plus talk of available guns is concerning behavior that needs this structured assessment, whatever the student's explanation. It beats C because immediate police action is reserved for imminent threats; otherwise the team, which can include law enforcement, decides how police are involved.

B **Incorrect** (trap: Right action, wrong time). Checking access to firearms is essential, and the family will likely be contacted. Done alone and first, though, it replaces the structured team process that gathers information from many sources and decides who contacts the family, when and how.

C **Incorrect** (trap: Skips assessment). Police must be called at once when a threat is imminent. Here nothing shows an attack is under way, so the threat assessment team should gather information and decide, with law enforcement as a partner, what involvement is needed.

Takeaway: A student's concerning statements about harming others go to a multidisciplinary threat assessment team for structured evaluation.

Sources: U.S. Secret Service National Threat Assessment Center (2018). Enhancing School Safety Using a Threat Assessment Model: An Operational Guide for Preventing Targeted School Violence, Steps 1, 4 and 5
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189. ASM-1167 (rev 1) · Best answer: A

A 79-year-old client has cared for a spouse with advanced Parkinson's disease for 6 years. In a caregiver support session, the client says, 'Some nights I think it would be kinder if neither of us woke up.' The client mentions keeping a handgun in the bedroom. What should the social worker do **FIRST**?

A. Ask whether the thoughts include ending both their lives
B. Arrange respite care to ease the caregiving load
C. Counsel the client to store the handgun elsewhere
D. Explore the grief of watching the spouse decline

Why each option is right or wrong

A **Correct.** Assessment comes before intervention. 'Neither of us' signals possible thoughts of harming the spouse as well as oneself, and studies of spousal homicide-suicide in later life find that perpetrators are often caregivers with depressed mood (Malphurs & Cohen, 2005). Asking directly about intent toward both people shows how urgent and what kind of response is needed. It beats C because lethal-means counseling is essential and should follow quickly, but the plan depends on whether the risk includes the spouse.

B **Incorrect** (trap: Right action, wrong time). Respite may relieve the strain behind the statement and belongs in the plan. Arranging it before asking about intent leaves possible danger to the client and spouse unassessed.

C **Incorrect** (trap: Right action, wrong time). Removing access to the handgun is a vital step once risk is clarified, and it may come within minutes. First the social worker needs to know whether the thoughts involve the spouse, which shapes who must be protected.

D **Incorrect** (trap: Good but not best). Anticipatory grief is real and worth exploring later. Focusing on it now treats the statement as sadness alone and skips assessing a possible plan to harm two people.

Takeaway: When a caregiver's death wish includes the care recipient, assess thoughts of harming both before planning support or means safety.

Sources: Malphurs, J. E., & Cohen, D. (2005). A statewide case-control study of spousal homicide-suicide in older persons. American Journal of Geriatric Psychiatry, 13(3), 211-217, whole article
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190. ASM-1168 (rev 1) · Best answer: C

In a pediatric emergency department, a 15-year-old being treated for an asthma attack completes the hospital's suicide risk screen with a nurse. The teen answers yes when asked about having thoughts of killing themselves right now. The parent is in the waiting room. What should the social worker do **NEXT**?

A. Schedule an outpatient mental health visit for this week
B. Ask the parent whether the teen has talked about suicide before
C. Ensure the teen is watched until an urgent safety evaluation

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Outpatient follow-up suits a non-acute positive screen. A yes to thoughts of suicide right now signals possible imminent risk, which calls for same-visit evaluation and observation, not a later appointment.

B **Incorrect** (trap: Right action, wrong time). Collateral history helps judge risk and will be part of the full evaluation. Seeking it first, before the teen is under observation and an urgent evaluation is requested, puts information ahead of safety.

C **Correct.** A yes to current suicidal thoughts on a screen such as the NIMH Ask Suicide-Screening Questions (ASQ) marks an acute positive result. The toolkit calls for keeping the patient in sight, removing dangerous objects and getting an urgent full safety evaluation before the patient leaves (NIMH ASQ). It beats B because history from the parent is valuable and will be gathered during the evaluation, but protection comes first once the screen has flagged imminent risk.

Takeaway: An acute positive suicide screen calls for continuous observation and an urgent full safety evaluation before the patient leaves.

Sources: National Institute of Mental Health (NIMH). Ask Suicide-Screening Questions (ASQ) Toolkit: Suicide Risk Screening Tool, Acute positive screen
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191. ASM-1175 (rev 2) · Best answer: B

Two days after a 10th grader died by suicide, a high school social worker reviews the students staff are worried about. One was the student's closest friend, was hospitalized last year after an overdose, and has not come to school yet today. Another sat beside the student in chemistry and keeps crying in class. A third posted a tribute video that friends shared widely. Whom should the social worker contact **FIRST**?

A. The classmate crying in chemistry class
B. The friend who is absent from class
C. The student whose tribute video friends shared

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Open grief in class is common after a peer's death and deserves a check-in today. Visible distress alone, without a close tie or a history of suicidal behavior, places this student behind the absent close friend in the order of outreach.

B **Correct.** After a suicide, outreach starts with the students at highest risk of contagion: close friends and students with prior suicidal behavior or known depression (AFSP & SPRC, 2018). This student has both, plus an unexplained absence, so the social worker should reach the student, or the family, today and assess for suicide risk. It beats A because crying in class is visible but is an expected grief reaction in a student with a looser tie and no known history; the quiet, absent student carries more risk factors.

C **Incorrect** (trap: Right action, wrong time). Widely shared memorial posts raise contagion concerns, and the school should guide safe messaging online. That is a schoolwide task. The student who posted shows no listed individual risk factors, so this contact comes after reaching the highest-risk student.

Takeaway: After a suicide, contact first the students with the most risk factors: close ties to the deceased and prior suicidal behavior, not the most visibly upset.

Sources: AFSP & SPRC (2018). After a Suicide: A Toolkit for Schools (2nd ed.), Crisis Response; Suicide Contagion: identifying other students at possible risk
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192. ASM-1176 (rev 1) · Best answer: D

In a violence risk assessment, which of these risk factors is dynamic and therefore a direct target for a risk-management plan?

A. Two prior assault convictions
B. First arrest at age 15
C. Witnessing violence between parents
D. Current heavy weekend drinking

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Past violence is the strongest single predictor of future violence, which makes it tempting. It is a static, historical factor, though: no plan can change it, so it raises the baseline risk estimate rather than serving as a treatment target.

B **Incorrect** (trap: Near-miss diagnosis). Early onset of offending is a static historical factor. It informs the overall level of risk, but nothing done now can change the age of first arrest.

C **Incorrect** (trap: Near-miss diagnosis). Childhood exposure to violence between parents is a historical risk factor. It shapes the risk formulation and may guide trauma-informed care, but the exposure itself cannot be changed by a plan.

D **Correct.** Dynamic risk factors are changeable conditions, such as current substance use, active symptoms or unstable housing. Static factors are fixed history. Risk-management plans target dynamic factors because reducing them can lower risk (Douglas & Skeem, 2005). Current heavy drinking can be treated and monitored. It beats A because, although prior violence is the strongest predictor, it is static and cannot be changed by any plan.

Takeaway: Static risk factors set baseline risk; dynamic factors such as current substance use are what a violence risk-management plan can change.

Sources: Douglas, K. S., & Skeem, J. L. (2005). Violence risk assessment: Getting specific about being dynamic. Psychology, Public Policy, and Law, 11(3), 347-383, whole article
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193. ASM-1177 (rev 1) · Best answer: A

In an afternoon session, a 58-year-old client with depression tells the social worker they have no thoughts of suicide. That evening, the client's partner, who is named on a signed release, calls to say they found a large supply of sleeping pills hidden in a garage toolbox. The partner adds that the client has been unusually quiet since losing a job last month. What should the social worker do **NEXT**?

A. Contact the client promptly about the partner's call and reassess
B. Ask police to bring the client in for an emergency evaluation
C. Add a session later this week to discuss the partner's call

Why each option is right or wrong

A **Correct.** Collateral information is part of suicide inquiry, and hidden pills suggest preparatory behavior that the earlier denial did not address (SAMHSA SAFE-T, 2009; Posner et al., 2011). The social worker should contact the client promptly, share the partner's report, ask directly about intent and plans for the pills, and set the response by the risk found. It beats B because the client has not yet been asked about the pills; an emergency evaluation becomes the step if the client confirms intent, cannot be reached, or cannot keep safe.

B **Incorrect** (trap: Skips assessment). An emergency evaluation is right when assessment shows intent or imminent danger, or when an at-risk client cannot be reached. Here the client has not yet been asked about the pills, so sending police first skips the reassessment that would show whether this level of response is needed.

C **Incorrect** (trap: Ignores risk). Stockpiled pills after a recent job loss are a warning sign that calls for same-day follow-up. Waiting several days leaves possible preparatory behavior unassessed while the client still has access to the means.

Takeaway: Collateral evidence of preparatory behavior overrides a prior denial: reassess the client promptly before choosing the level of response.

Sources: SAMHSA (2009). SAFE-T: Suicide Assessment Five-step Evaluation and Triage (SMA09-4432), Step 3: Suicide inquiry; Posner, K., et al. (2011). The Columbia-Suicide Severity Rating Scale. American Journal of Psychiatry, 168(12), 1266-1277, suicidal behavior: preparatory acts
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194. ASM-1179 (rev 2) · Best answer: B

For 2 years, a veteran with chronic back pain has described a passive wish not to wake up at most sessions. Today the social worker asks the standard risk questions, and the client again reports no plan and no intent to act. Firearms remain stored with a sibling under an earlier agreement, and the client used the safety plan during a hard night last week. A new team member urges an emergency evaluation. What is the **BEST** response?

A. Arrange an emergency evaluation for the client's safety
B. Review the safety plan together and continue current care
C. Ask the client to sign a no-suicide agreement
D. Reduce sessions to monthly check-ins

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Emergency evaluation fits an acute change: new intent, a plan, new access to means or a recent attempt. None is present. Using it for a stable chronic pattern is more restrictive than needed and may teach the client that honest reports bring a hold.

B **Correct.** Risk is judged against the client's own baseline. Chronic passive ideation without change, with means still secured and a safety plan the client actually uses, calls for continued outpatient care and a reviewed safety plan, the least restrictive effective response (Stanley & Brown, 2012). It beats A because nothing acute has changed: no plan, no intent and no new access to means. Emergency evaluation would become right if any of those changed.

C **Incorrect** (trap: Misuses the technique). No-suicide contracts have no evidence that they prevent suicide and can discourage honest disclosure (Rudd et al., 2006). A collaboratively written safety plan, which this client already uses, is the accepted alternative.

D **Incorrect** (trap: Ignores risk). Stable chronic ideation still carries elevated long-term risk, especially with chronic pain. Reducing contact removes monitoring and support when nothing suggests the client needs less.

Takeaway: Judge suicide risk against the client's own baseline; unchanged chronic ideation with secured means calls for safety planning, not emergency evaluation.

Sources: Stanley, B., & Brown, G. K. (2012). Safety Planning Intervention: A brief intervention to mitigate suicide risk. Cognitive and Behavioral Practice, 19(2), 256-264, whole article; Rudd, M. D., Mandrusiak, M., & Joiner, T. E. (2006). The case against no-suicide contracts. Journal of Clinical Psychology, 62(2), 243-251, whole article
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195. ASM-0008 (rev 1) · Best answer: C

Which part of the mental status examination records answers that wander off and never reach the point?

A. Thought content
B. Perception
C. Thought process

Why each option is right or wrong

- **A** **Incorrect** (trap: Wrong model or concept). Thought content records the themes of thinking, such as delusions, preoccupations, obsessions or suicidal ideation. The question describes how ideas connect, not what they are about.
- **B** **Incorrect** (trap: Wrong model or concept). Perception covers sensory experiences such as hallucinations and illusions. Wandering, loosely connected answers are a feature of thinking rather than perception.
- **C** **Correct.** Thought process describes how thoughts are organized and connected: goal-directed, circumstantial, tangential, or loosely associated (Kaplan & Sadock's Synopsis). Answers that wander through loosely related details and never reach the point describe tangentiality, a process finding. It beats A because thought content covers what the client thinks about, such as delusions, obsessions or suicidal ideas, rather than how ideas link together.

Takeaway: MSE: thought process is how ideas connect (tangential, loose); thought content is what the client thinks about.

Sources: Boland, R., Verduin, M. L., & Ruiz, P. (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.). Wolters Kluwer, mental status examination: thought process
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196. ASM-0114 (rev 1) · Best answer: A

Which mental status exam term describes laughing while recounting a sad event, such as a recent death?

- **A.** Inappropriate affect
- **B.** Labile affect
- **C.** Restricted range of affect
- **D.** Euphoric mood

Why each option is right or wrong

- **A** **Correct.** Affect is the observed emotional expression, and it is called inappropriate when it clashes with the content being discussed, such as laughing while describing a death (Kaplan & Sadock's Synopsis). The finding calls for further assessment, with attention to context and cultural norms, because it can occur in psychotic and other disorders. It beats B because labile affect refers to rapid, abrupt shifts in emotional expression, not a steady mismatch between expression and content.
- **B** **Incorrect** (trap: Wrong model or concept). Labile affect means rapid, abrupt swings in emotional expression, such as moving from tears to laughter within moments. A single expression that conflicts with the topic is better described as inappropriate.
- **C** **Incorrect** (trap: Wrong model or concept). A restricted range of affect means emotional expression varies less than expected across topics. Laughter about a sad event is a mismatch between expression and content, not a narrowing of range.
- **D** **Incorrect** (trap: Wrong model or concept). Mood is the client's own report of their emotional state, while affect is what the interviewer observes. Laughter seen during the interview is recorded under affect, and nothing indicates a euphoric mood.

Takeaway: Affect is observed expression: inappropriate affect clashes with content, labile affect shifts rapidly. Mood is the client's self-report.

Sources: Boland, R., Verduin, M. L., & Ruiz, P. (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.). Wolters Kluwer, mental status examination: affect
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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197. ASM-1061 (rev 1) · Best answer: C

What does the mental status exam contribute to a clinical assessment?

A. A diagnosis that meets DSM-5-TR criteria
B. A chronological history of the presenting problem
C. A snapshot of present thought, mood and behavior
D. A norm-referenced score of symptom severity

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Diagnosis integrates the mental status exam with history, collateral information and course over time. Mental status findings alone, such as flat affect, fit several diagnoses.

B **Incorrect** (trap: Wrong model or concept). A chronological account comes from the history of the present illness and the psychosocial history. The mental status exam records present-moment observations rather than past events.

C **Correct.** The mental status exam (MSE) is a structured, cross-sectional description of how the client appears, behaves, thinks, feels and reasons at the time of the interview. It covers domains such as appearance, mood and affect, thought process and content, perception, cognition, insight and judgment. It beats A because the MSE supplies observations that, combined with history and other data, inform a diagnosis; it does not establish one by itself (Kaplan & Sadock).

D **Incorrect** (trap: Wrong model or concept). Norm-referenced severity scores come from standardized instruments such as the PHQ-9. The mental status exam is a clinician's structured observation, not a scored and normed test.

Takeaway: The MSE is a cross-sectional snapshot of present mental functioning that informs, but does not by itself establish, a diagnosis.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Psychiatric interview: mental status examination
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198. ASM-1062 (rev 1) · Best answer: B

On a mental status exam, a client misperceives a real object, such as seeing a coat on a hook as an intruder. Which finding is this?

A. Visual hallucination
B. Illusion
C. Delusion
D. Derealization

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A visual hallucination is a sensory perception without any external stimulus, such as seeing a person in an empty corner. Here a real coat is being misperceived.

B **Correct.** An illusion is a misperception of a real external stimulus: the coat is really there but is perceived as an intruder. It is recorded under perception and is common with poor lighting, fatigue, delirium or substance effects. It beats A because a hallucination is a perception with no external stimulus at all, so nothing real is being misread (Kaplan & Sadock).

C **Incorrect** (trap: Wrong model or concept). A delusion is a fixed false belief, a disturbance of thought content rather than perception. Misreading a coat in passing is a perceptual error, not a belief held against evidence.

D **Incorrect** (trap: Wrong model or concept). Derealization is a sense that the surroundings are unreal, dreamlike or distant. It changes how the whole environment feels, not the identity of one object.

Takeaway: Illusions misperceive a real stimulus; hallucinations occur without one; delusions are false beliefs, not perceptions.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: perception

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199. ASM-1063 (rev 2) · Best answer: A

An inpatient is asked how the morning went and answers rapidly: 'Grey morning, grey like the sea, I sailed once, sailors steer by the stars, I'm writing a book on astronomy.' The answer continues in the same way for several minutes. Which MSE finding does this answer **MOST** likely show?

A. Flight of ideas
B. Loose associations
C. Circumstantiality
D. Clang associations

Why each option is right or wrong

A **Correct.** Flight of ideas is a thought-process finding in which speech moves rapidly from topic to topic, each shift linked to the one before by an understandable association, as with grey, sea, sailing, stars and astronomy. It is typical of mania and usually comes with pressured speech. It beats B, loose associations, because in loosening of associations the connections between ideas cannot be followed, whereas here each link is clear even though the answer never returns to the question (Kaplan & Sadock).

B **Incorrect** (trap: Wrong model or concept). In loose associations, thoughts shift between topics with no understandable connection, as is often seen in schizophrenia. In this answer each phrase follows from the one before, so the links can be traced.

C **Incorrect** (trap: Wrong model or concept). Circumstantial speech takes a long, detailed detour but eventually returns to answer the question. This answer moves further away with each phrase and never gets back to the morning.

D **Incorrect** (trap: Wrong model or concept). Clang associations link words by sound, such as rhymes or puns, rather than by meaning. Here the links follow meaning, from sea to sailing to stars, not sound.

Takeaway: Rapid topic shifts with traceable links are flight of ideas; untraceable links are loose associations; detours that return are circumstantiality.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: thought process
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200. ASM-1064 (rev 1) · Best answer: C

During a telehealth intake, an adult client smiles steadily while describing a recent job loss and says, 'Honestly, I feel hollow most days.' The social worker is completing the mood section of the mental status exam. What is the **BEST** way to document the client's mood?

A. Record mood as euthymic, consistent with the steady smile
B. Record mood as depressed, based on the worker's impression
C. Record mood using the client's own description, in quotes

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The smile is observed affect, not mood. Recording euthymic mood from outward expression confuses the two domains and contradicts what the client reported.

B **Incorrect** (trap: Misuses the technique). The worker's impression belongs in the affect section and the clinical formulation. Mood should reflect the client's own report, which here is more specific than a general label.

C **Correct.** In the MSE, mood is the client's subjective emotional state and is best recorded in the client's own words, while affect is the worker's observation of outward expression. Quoting the client keeps the record accurate and lets it show the mismatch between the smile (affect) and the reported hollowness (mood), which is itself clinically meaningful. It beats B because substituting the worker's label for the client's report blurs the line between subjective mood and observed affect (Kaplan & Sadock).

Takeaway: Document mood as the client's own report, quoted where possible; outward expression is documented separately under affect.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: mood and affect
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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201. ASM-1065 (rev 1) · Best answer: B

A hospital social worker completes a mental status exam with a patient admitted after a fall. The patient looks down at the floor throughout, answers fully and warmly, and describes a good mood. The patient explains that in their family, looking an elder or professional in the eye is disrespectful. What is the **MOST** appropriate way to record the patient's eye contact?

A. Note the patient's poor eye contact as a sign of low mood
B. Describe the downward gaze and the patient's stated reason
C. Omit eye contact from the record to avoid cultural bias

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Reduced eye contact can accompany depression or anxiety, but this patient reports good mood, engages warmly and gives a cultural reason. Treating the gaze as a symptom pathologizes a norm.

B **Correct.** MSE observations should be described accurately and interpreted in cultural context. Recording the downward gaze together with the patient's explanation gives an accurate observation without pathologizing a culturally rooted norm, and it fits the warm engagement and good mood the patient reports. It beats A because labeling the gaze a sign of low mood reads a cultural norm as a symptom, against the patient's own account (NASW Cultural Competence Standards, Standard 4; DSM-5-TR).

C **Incorrect** (trap: Misuses the technique). Leaving out an observed behavior makes the mental status exam incomplete. Accurate documentation with context, not omission, is how bias is kept out of the record.

Takeaway: Record MSE observations accurately along with the client's cultural context, rather than reading a cultural norm as a symptom.

Sources: NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 4: Cross-Cultural Skills; DSM-5-TR (APA, 2022), Section III: Culture and Psychiatric Diagnosis
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202. ASM-1159 (rev 2) · Best answer: C

During a mental status exam, the social worker asks a client to spell the word 'world' backward. Which area is the social worker assessing?

A. Remote memory
B. Abstract reasoning
C. Attention and concentration

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Remote memory covers events from years past and is tested by asking about personal history. Spelling backward works on information held in mind right now, so it measures attention rather than stored memory.

B **Incorrect** (trap: Wrong model or concept). Abstract reasoning is tested with similarities or proverb meanings, which ask the client to move from specific facts to general ideas. Spelling backward involves no such step.

C **Correct.** Spelling a short word backward requires holding letters in mind and working through them in reverse, so it tests attention and concentration, as does subtracting 7s from 100 (Kaplan & Sadock). Results help show whether attention is impaired, as in delirium. It beats A because memory tasks ask the client to recall past events or a word list after a delay, not to work with information on the spot.

Takeaway: Serial 7s and spelling 'world' backward test attention and concentration on the mental status exam.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: sensorium and cognition

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203. ASM-1160 (rev 1) · Best answer: C

At an outpatient intake, a client describes recurring urges to shout insults during religious services. The client calls them 'my own brain being cruel,' finds them upsetting, and tries to push them away with silent prayer. The client has never acted on them. Which mental status finding is this **MOST** likely to represent?

A. Delusion

B. Thought insertion

C. Obsession

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A delusion is a fixed false belief held despite clear evidence against it. The client holds no false belief here; they see the urges as products of their own mind and find them unwelcome.

B **Incorrect** (trap: Wrong model or concept). Thought insertion is a delusion that someone or something outside has put thoughts into one's head. Calling the urges 'my own brain' shows the client knows they are self-generated.

C **Correct.** An obsession is a recurring thought, urge or image that feels unwanted and distressing, which the person tries to ignore or neutralize with another thought or act (DSM-5-TR). The client recognizes the urges as coming from their own mind and uses prayer to push them away. It beats B because in thought insertion the person believes an outside force has placed thoughts in their mind; this client calls the urges the work of their own brain.

Takeaway: Unwanted thoughts the client recognizes as their own and resists are obsessions; thoughts felt as placed by an outside force are thought insertion.

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Obsessive-Compulsive Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Key Features That Define the Psychotic Disorders (Delusions)

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204. ASM-1161 (rev 1) · Best answer: A

An inpatient with bipolar I disorder who lives alone is being considered for discharge. The team asks the social worker to describe the client's judgment for the mental status exam. Which question serves this purpose?

A. 'What would you do if you ran out of medication at home?'
B. 'Do you believe you have an illness that needs treatment?'
C. 'What does it mean that the early bird catches the worm?'

Why each option is right or wrong

A **Correct.** Judgment is the ability to weigh a situation and choose a sensible course of action, while insight is awareness of having an illness (Kaplan & Sadock). For a client about to live alone, asking how they would handle running out of medication tests practical decision-making in a situation they are likely to face. It beats B because awareness of illness is a separate mental status area; a client may admit being ill yet still make unsafe choices.

B **Incorrect** (trap: Wrong model or concept). This question measures insight, the client's awareness of illness and need for treatment. Insight matters for discharge too, but it is recorded separately and does not show how the client solves real problems.

C **Incorrect** (trap: Wrong model or concept). Proverb interpretation tests abstract thinking, and results depend heavily on education, language and culture. It says little about whether the client can make safe choices at home.

Takeaway: Judgment is assessed by how a client would handle realistic decisions; insight is awareness of illness; proverbs test abstraction.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: judgment and insight
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205. ASM-0115 (rev 1) · Best answer: C

A parent seeks help for a 9-year-old's constant defiance and says the school complains about the child every day. With the parent's written consent to exchange information with the school, the social worker obtains school records. The records show good grades, no discipline reports and positive comments from staff. What should the social worker do **NEXT**?

A. Diagnose oppositional defiant disorder from the parent's report
B. Refer the parent to a behavioral parent training program
C. Discuss the conflicting accounts with the parent and teacher

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A diagnosis built on one informant, after records contradict part of that informant's account, skips the work of reconciling the data. Defiance limited to home can still matter, but the conflicting reports must be understood first.

B **Incorrect** (trap: Right action, wrong time). Parent training may well help if defiance at home is confirmed, and it is an evidence-based option. Recommending it now acts before the social worker understands why the home and school pictures differ.

C **Correct.** Collateral records are integrated with other information, and differences between informants are explored rather than ignored or used to discard one source (De Los Reyes & Kazdin, 2005). The parent describes daily complaints from school, but the records show none, so the next step is to clarify the conflict with both the parent and the teacher. Behavior that appears only at home is itself useful information. It beats B because recommending treatment before reconciling the accounts acts on an incomplete picture.

Takeaway: When collateral records conflict with a client's account, explore the discrepancy with the informants before drawing conclusions or planning treatment.

Sources: De Los Reyes, A., & Kazdin, A. E. (2005). Informant discrepancies in the assessment of childhood psychopathology: A critical review, theoretical framework, and recommendations for further study. Psychological Bulletin, 131(4), 483-509, whole article; Achenbach, T. M., McConaughy, S. H., & Howell, C. T. (1987). Child/adolescent behavioral and emotional problems: Implications of cross-informant correlations for situational specificity. Psychological Bulletin, 101(2), 213-232, whole article

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206. ASM-1601 (rev 1) · Best answer: A

A social worker at a community brain injury program receives a neuropsychological evaluation for a 34-year-old client injured in a fall 8 months ago. The client has signed releases for the program and the evaluator. After reading the report, the social worker understands its summary but not how the processing-speed and memory scores should shape the client's job-training plan. What should the social worker do **NEXT**?

A. Ask the psychologist who did the testing about practical implications
B. Explain to the client what each test score means for work
C. Revise the job-training plan based on the report's summary
D. Request a second neuropsychological evaluation from another provider

Why each option is right or wrong

A **Correct.** Integrating an outside evaluation means applying its findings accurately. Social workers seek colleagues' advice when it serves clients and practice within their competence (NASW Code 2.05(a), 1.04(a)). The social worker grasps the summary but not how specific scores translate into job demands, and the evaluating psychologist can explain that, with releases already signed. It beats C because revising the plan on a partial reading could set training goals that are too hard or too easy; the plan should change once the implications are clear.

B **Incorrect** (trap: Outside the social work role). Interpreting individual neuropsychological scores takes testing expertise that most social workers lack, and a wrong explanation could mislead the client about returning to work. The social worker can share the report's plain-language conclusions and arrange for the evaluator to answer score questions.

C **Incorrect** (trap: Right action, wrong time). Updating the job-training plan is the right later step, once the findings' practical meaning is clear. Changing it now, from a summary the social worker only partly understands, risks goals that do not fit the client's abilities.

D **Incorrect** (trap: Skips assessment). A repeat evaluation duplicates recent testing and costs the client time without any reason to doubt the first report. It fits when a report is outdated or its findings are in serious dispute; here the gap is in understanding, not in the testing.

Takeaway: When an outside evaluation's implications are unclear, ask its author before applying them; don't interpret specialized test scores beyond your training.

Sources: NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 1.04(a)
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207. ASM-1602 (rev 1) · Best answer: B

A 15-year-old enters a residential program after five foster placements. Records from prior providers list ADHD at age 8, oppositional defiant disorder at age 11 and bipolar disorder at age 13. School and child welfare records show a placement change or a new school shortly before each new diagnosis. Staff observe irritability and poor sleep. What is the **MOST** appropriate way for the social worker to use these records?

A. Adopt the most recent diagnosis as the working one
B. Build a timeline linking symptoms, moves and diagnoses
C. Set the records aside and rely on current observation

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). The latest label is not necessarily the most accurate; it was made with the same partial history as the others. Accepting it without reviewing the pattern could lock in a diagnosis that the records themselves call into question.

B **Correct.** Assessment draws on multiple sources of information, and integrating records means comparing them rather than adopting one label (Hepworth et al.). Here each new diagnosis came soon after a move or school change, a pattern visible only when records are arranged over time. A timeline lets the team see whether symptoms track losses and stress, which may point to trauma responses rather than three separate disorders. It beats C because setting the records aside discards the history that reveals this pattern, which current observation cannot show.

C **Incorrect** (trap: Good but not best). Fresh observation matters, but it covers only a few days in a new setting. Ignoring years of records loses the link between moves and new symptoms, the history most needed to understand the teen's irritability and poor sleep.

Takeaway: Integrate records by comparing them over time with current findings; don't adopt or discard prior labels wholesale.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, assessment: sources of information

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208. ASM-1681 (rev 1) · Best answer: B

During intake at a community mental health clinic, a 46-year-old reports 4 months of low mood, tiredness and weight gain. With the client's written consent, the social worker obtains primary care records. They show a thyroid test marked abnormal 5 months ago with a plan to repeat it in 6 weeks, but no repeat test appears. What should the social worker do **FIRST**?

A. Start a treatment plan aimed at the client's low mood

B. Ask the client's physician to follow up on the lab result

C. Tell the client that the thyroid result explains the symptoms

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A plan for the client's mood is a later step and may still be needed. Started first, it skips over an unresolved lab result that could change the formulation and the treatment.

B **Correct.** Records are integrated into the assessment, and a possible medical cause of mood symptoms is addressed first. An underactive thyroid can produce low mood, fatigue and weight gain (Kaplan & Sadock's Synopsis), and these records show an abnormal result whose planned recheck never happened. Using the existing consent, asking the client's physician to follow up brings the result to the professional who can interpret it. It beats A because a depression plan built before the medical question is answered may target the wrong cause; DSM-5-TR directs clinicians to consider a depressive disorder due to another medical condition.

C **Incorrect** (trap: Outside the social work role). Interpreting lab results and naming a medical cause are outside the social work role. The result needs a physician's review; the link is a possibility to raise with the provider, not a conclusion to give the client.

Takeaway: When records show an unresolved medical finding that could explain psychiatric symptoms, coordinate medical follow-up before finalizing a treatment plan.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mood disorders: medical conditions that cause depressive symptoms (hypothyroidism); DSM-5-TR (APA, 2022), Depressive Disorders: Depressive Disorder Due to Another Medical Condition

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.11 Integrating records and evaluations (medical, school, psychological, employment) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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209. ASM-1682 (rev 1) · Best answer: C

A school social worker receives a referral for a 14-year-old whom teachers describe as defiant because the student 'ignores directions.' With a parent's consent, the worker reviews school records. A third-grade evaluation identified a language disorder, and speech-language services stopped after a school transfer in sixth grade, with no reevaluation on file. Teacher notes say most problems occur with multistep spoken instructions. What is the **BEST** action?

A. Interview the student about trouble following directions
B. Start a school behavior plan rewarding prompt compliance
C. Ask the student's school team to revisit the earlier evaluation
D. Refer the student for a psychiatric evaluation for defiance

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Hearing the student's view is a reasonable part of any assessment and may add useful detail. On its own, it leaves the record's strongest lead, an untreated language disorder, unexamined by the team that can evaluate it.

B **Incorrect** (trap: Skips assessment). A reward plan assumes the problem is willful noncompliance. The records point instead to trouble processing spoken directions, which rewards cannot fix and which may leave the student punished for a disability.

C **Correct.** Integrating records means letting prior evaluations shape the current formulation. The file shows a diagnosed language disorder, services that ended without the reevaluation federal rules require before ending eligibility (34 CFR 300.305(e)), and current problems clustered on multistep spoken directions. Together these suggest a language-processing need rather than defiance. Asking the student's school team to revisit the earlier evaluation brings the question to the group responsible for revising services (34 CFR 300.324(b)). It beats A, the strongest competitor, because interviewing the student is useful but cannot settle a language-processing question the team must evaluate.

D **Incorrect** (trap: Hands off too soon). A psychiatric referral for defiance accepts the teachers' label before the documented language disorder is reconsidered. It would fit if a review found no language explanation and serious behavior concerns remained.

Takeaway: Integrate prior evaluations into the current picture; a documented disability in the records can reframe a behavior referral and should prompt team review.

Sources: IDEA Part B regulations, 34 CFR Part 300, 34 CFR 300.305(e)(1); IDEA Part B regulations, 34 CFR Part 300, 34 CFR 300.324(b)(1)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.11 Integrating records and evaluations (medical, school, psychological, employment) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

210. ASM-0110 (rev 1) · Best answer: C

A social worker at a family services agency holds an intake with a client who has limited English proficiency. A trained interpreter is present in the room. The client is seeking help with a child's attendance problems at school. How should the social worker **BEST** conduct the interview?

A. Ask the interpreter to summarize the client's answers
B. Pose questions to the interpreter to relay to the client
C. Speak directly to the client in the first person

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Summaries filter the client's words through the interpreter's judgment, so details, emotion and meaning can be lost. Interpreters should render everything said, completely and accurately, rather than condense it.

B **Incorrect** (trap: Good but not best). Talking about the client in the third person ('Ask what brings them in') treats the interpreter as the person being interviewed. It weakens rapport and can leave the client feeling talked about rather than talked to.

C **Correct.** When working through an interpreter, the social worker speaks directly to the client in the first person ('What brings you in today?'), and the interpreter renders everything said in both directions (NCIHC, 2024). This keeps the helping relationship between the social worker and the client and preserves the client's own words. It beats B because addressing the interpreter in the third person sidelines the client and makes the interpreter the focus of the interview. Using a qualified interpreter also supports the client's understanding (NASW Code 1.03(b)).

Takeaway: With an interpreter, talk directly to the client in the first person and expect everything to be interpreted, not summarized.

Sources: National Council on Interpreting in Health Care (2024). Guide for Partnering with an Interpreter, speak directly to the patient; first-person interpreting; NASW Code of Ethics (2021), 1.03(b)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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211. ASM-1567 (rev 1) · Best answer: A

A client with limited English arrives for intake at an outpatient substance use program. The scheduled interpreter is trained and certified. While the interpreter steps out, the client tells the social worker in basic English, 'That interpreter knows my family. Our community is small.' When the interview resumes, the client gives short answers whenever drinking comes up. What is the **BEST** action?

A. Switch to a remote interpreter with no local ties
B. Remind the client that interpreters keep information private
C. Continue the intake in English using simpler questions
D. Ask the interpreter privately whether they know the client

Why each option is right or wrong

A **Correct.** Interpreted interviews work only if the client can speak freely. Interpreters are bound to confidentiality and impartiality (NCIHC, 2004), yet a client in a small language community may still fear that information will reach family, and here that fear is shutting down disclosure about drinking. A remote interpreter with no ties to the local community keeps professional interpretation while removing the conflict (NASW Cultural Competence Standard 9). It beats B because a reminder about the rules does not resolve the client's concern about a personal connection.

B **Incorrect** (trap: Good but not best). Interpreters are bound to confidentiality, so the reminder is accurate. But the client named a personal link to the interpreter and is already holding back; a reminder leaves the source of discomfort in the room.

C **Incorrect** (trap: Misuses the technique). The client's English is limited, so continuing in English risks misunderstanding on sensitive topics such as substance use. Language access standards call for a qualified interpreter, not a workaround.

D **Incorrect** (trap: Misreads law or ethics). Checking with the interpreter may seem fair, but it reveals what the client shared in confidence and turns the client's concern into a question for the interpreter instead of honoring the client's comfort.

Takeaway: When a client fears an interpreter's community ties, arrange a qualified interpreter from outside that community to protect open disclosure.

Sources: National Council on Interpreting in Health Care (NCIHC) (2004). National Code of Ethics for Interpreters in Health Care, Confidentiality; Impartiality; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication

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212. ASM-1568 (rev 1) · Best answer: C

On an inpatient psychiatric unit, a client tells the social worker during an assessment interview, 'Sometimes I just lose it, and then it's over.' The client offers nothing more and looks at the social worker, waiting. What should the social worker do **FIRST**?

A. Reflect the client's frustration about losing control

B. Ask whether the client hurts anyone when this happens

C. Ask the client to describe a recent time they lost it

D. Suggest the anger management group on the unit

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Reflection builds rapport, but reflecting frustration about losing control assigns a feeling and meaning the client has not stated. Accurate reflection depends on first understanding what the client meant.

B **Incorrect** (trap: Right action, wrong time). Specific risk questions matter on an inpatient unit and may come next. Asked first, this closed question assumes the phrase means aggression and could steer the client toward or away from disclosure.

C **Correct.** Clarifying, or seeking concreteness, turns vague words into specific behavior and meaning (Hepworth et al.). 'Lose it' could mean anger, panic, dissociation, crying or self-harm, and 'then it's over' is just as unclear. Asking for a recent example lets the

client define the experience before the social worker responds to an assumption. It beats B because a closed question about hurting others presumes the meaning is aggression; if the example points to danger, specific risk questions follow.

D **Incorrect** (trap: Skips assessment). An anger group assumes 'losing it' means anger. The phrase could describe panic, dissociation or self-harm, so recommending a service before clarifying risks a poor match and may miss a safety concern.

Takeaway: When a client uses vague or loaded phrases, ask for a concrete example before reflecting, assuming a meaning or recommending services.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Verbal following, exploring, and focusing skills: seeking concreteness
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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213. ASM-1569 (rev 3) · Best answer: B

A child welfare social worker has met monthly for 4 months with a parent working toward reunification, and they have a trusting working relationship. Today the parent says, 'I'm doing everything the plan asks.' Records show the parent missed 4 of the last 6 parenting classes, and a court review is scheduled next month. Earlier in this visit, the parent vented about how long the process is and said, 'At least you understand.' What should the social worker do **NEXT**?

A. Affirm the parent's commitment to getting the children back
B. Point out the gap between the parent's statement and attendance
C. Explore the parent's feelings about the case plan

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Affirmation strengthens motivation, but affirming effort while ignoring the missed classes goes along with an inaccurate picture. It would fit if the parent's account matched the record.

B **Correct.** Confrontation points out a discrepancy between what a client says and does, and it works best within a trusting relationship and after empathic responding (Hepworth et al., confrontation). Here the alliance is solid after 4 months, the parent's frustration has just been heard and acknowledged, and the missed classes threaten reunification. Naming the gap respectfully gives the parent a chance to address it before next month's court review. It beats C because exploring feelings again, right after the parent felt understood, postpones the issue that most affects the children's return.

C **Incorrect** (trap: Good but not best). Exploring feelings is useful early on or when feelings are unclear. The parent has just aired that frustration and felt understood, so more exploration delays the discrepancy that matters most for reunification.

Takeaway: Within an established alliance and after empathic responding, respectfully point out discrepancies between a client's words and actions.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Additive empathy, interpretation, and confrontation

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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214. ASM-1570 (rev 1) · Best answer: B

In a hospice family meeting, an adult child who has cared for a dying parent for 8 months says, 'I feel like a terrible person for wishing this were over.' The social worker replies, 'After months of caring day and night, wanting the strain to end makes sense.' Which interviewing skill is the social worker **MOST** likely using?

A. Reflection of feeling
B. Validation
C. Reassurance
D. Interpretation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Reflection of feeling would name the emotion, for example, 'You feel guilty for wanting this to end.' The reply here does more than mirror guilt; it treats the reaction as understandable in context.

B **Correct.** Validation communicates that a person's feelings or reactions make sense given their circumstances, without judging them (Linehan, 1993). The social worker ties the caregiver's wish to months of exhausting care and says the wish makes sense, which counters the caregiver's harsh self-judgment. It beats A because reflection of feeling mirrors the emotion the client expressed, such as guilt; this response instead affirms that the reaction is legitimate given the situation.

C **Incorrect** (trap: Wrong model or concept). Reassurance offers comfort such as 'You're not a terrible person' or 'It will get easier.' This reply does not contradict the concern or promise relief; it explains why the feeling fits the situation.

D **Incorrect** (trap: Wrong model or concept). Interpretation offers a meaning the client has not recognized, often about underlying motives. The reply stays with the caregiver's stated experience rather than suggesting a hidden explanation.

Takeaway: Validation tells clients their reactions make sense in context; reflection mirrors the feeling; reassurance offers comfort that can dismiss it.

Sources: Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, validation strategies

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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215. ASM-1571 (rev 1) · Best answer: C

A client joins a video session 20 minutes late for the third week in a row. The client apologizes and says, 'I know, I know.' The social worker wants to understand what is getting in the way without sounding critical. Which question is **BEST**?

A. Why do you keep joining late?
B. Is it your work schedule, or are you unsure about coming?
C. What has been making it hard to join on time?

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). This asks about the right topic in the wrong form. 'Why' questions often sound like demands for justification, and a client who is already apologizing may become defensive rather than open.

B **Incorrect** (trap: Misuses the technique). Combining two options in one question limits the answer to the worker's guesses and hints at reluctance. An open question lets the client name the barrier, which may be neither choice.

C **Correct.** Open 'what' questions invite clients to explain in their own terms, while 'why' questions tend to sound accusatory and prompt defensiveness (Hepworth et al., counterproductive communication patterns). The client is already apologizing, so a nonjudgmental open question is most likely to surface the real obstacle, whether scheduling, technology or mixed feelings. It beats B because offering two choices narrows the answer and implies the client may not want to come.

Takeaway: Use open 'what' or 'how' questions to explore problems; avoid 'why' questions and double or leading questions that sound critical.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Avoiding counterproductive communication patterns: 'why' and leading questions
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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216. ASM-1575 (rev 1) · Best answer: B

At a case management intake in a homeless shelter, a client sighs, pushes the forms aside and says, 'The shelter staff, the benefits office, now you. Everybody wants paperwork from me.' The social worker wants to build engagement early in the meeting. Which response is **BEST**?

A. 'So several offices need forms from you right now.'
B. 'It sounds like you're worn out by everyone's demands.'
C. 'I'll keep my part of the paperwork as short as I can.'

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). This paraphrase captures the facts but misses the feeling under them. It is accurate and harmless, yet it tells the client the worker heard the list of offices, not the frustration. Paraphrasing content fits better when the client's message is mainly informational.

B **Correct.** Reflection of feeling names the emotion behind the client's words and shows empathic understanding, which lowers defensiveness and builds engagement (Hepworth et al.). The sigh, pushing the forms away and 'everybody wants' all signal

weariness and frustration, so naming that feeling meets the client where they are. It beats C because a promise about shorter paperwork jumps to problem solving before the client feels understood, and it may be a promise the worker cannot keep.

C **Incorrect** (trap: Good but not best). Offering to lighten the load is kind and may help later, but it skips past the client's feelings and moves straight to fixing. It can also sound like a bargain. Practical adjustments fit once the worker has acknowledged how the client feels.

Takeaway: When a client's words carry strong emotion, reflect the underlying feeling before paraphrasing content or offering solutions.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Empathic communication: reflecting feelings

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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217. ASM-1661 (rev 1) · Best answer: A

During a hospital family meeting, the social worker asks how the patient manages the stairs at home. A trained interpreter and the patient then talk back and forth in the patient's language for about a minute. The interpreter turns to the social worker and says, 'Basically, they're fine with the stairs.' What is the **MOST** appropriate response?

A. Ask the interpreter to interpret everything both people said

B. Accept the summary and move on to the next question

C. Request a different interpreter for the rest of the meeting

Why each option is right or wrong

A **Correct.** Interpreters render each speaker's words completely so that every party knows exactly what was said; national standards for health care interpreters make accuracy central (NCIHC, 2005). A minute-long exchange reduced to 'basically fine' may hide hesitation, conditions or details about falls. Politely asking for a full interpretation of the exchange, and reminding the interpreter to interpret everything going forward, keeps the patient's own words in the assessment. It beats C because a trained interpreter who summarized once can usually correct course after a brief reminder; replacing them is premature.

B **Incorrect** (trap: Misuses the technique). Accepting a summary lets the interpreter filter the patient's answer. The minute of discussion may include worries about stairs that affect discharge safety. Clients are entitled to full communication in their language (NASW Cultural Competence Standard 9).

C **Incorrect** (trap: Hands off too soon). Replacing the interpreter may be needed if summarizing continues after a clear request. As a first response it is premature and would disrupt the meeting when a brief reminder is likely to work.

Takeaway: Interpreters should render everything said; when an interpreter summarizes, ask for a full interpretation of the exchange.

Sources: National Council on Interpreting in Health Care (2005). National Standards of Practice for Interpreters in Health Care, Accuracy standards; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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218. ASM-1662 (rev 2) · Best answer: C

At an outpatient intake, a client referred for poor sleep spends the first 20 minutes describing a long dispute with a neighbor over parking. Each time the social worker reflects the frustration, the client agrees the dispute has been 'eating at' them and adds another detail about the neighbor. About 30 minutes remain. What should the social worker do **NEXT**?

A. Continue reflecting the client's anger at the neighbor
B. Ask a quick series of yes-or-no questions about sleep
C. Link the neighbor stress to sleep and shift the focus there

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Reflection built rapport and showed understanding, and the client confirmed it. Repeating it now adds little and lets the remaining time pass without exploring the sleep problem the client came for.

B **Incorrect** (trap: Misuses the technique). Closed questions gather quick facts, but a rapid string of them after a long open story feels abrupt and yields thin information. Open questions about sleep, tied to what the client shared, will reveal more.

C **Correct.** Focusing is the skill of guiding an interview toward the topics that matter for the client's goals while staying attuned (Hepworth et al.). The client has been heard, has confirmed the reflection, and has limited time left for the sleep problem that brought them in. Connecting the neighbor stress to sleep, for example by asking how the conflict affects nights, honors what the client shared and moves the intake forward. It beats A because more reflection repeats a step the client has already confirmed and uses time the sleep assessment needs.

Takeaway: Once a client feels heard, use focusing to connect their story to the presenting concern and move the interview toward it.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Focusing: selecting topics and maintaining focus
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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219. ASM-1663 (rev 1) · Best answer: B

In a second session, a client is describing a pregnancy loss that happened 2 months ago. Mid-sentence, the client stops, looks down and begins to cry softly. The client has spoken openly so far and has not asked for anything. What response would be **MOST** helpful at this moment?

A. Ask what thoughts are coming up for the client right now
B. Stay quietly present and let the client resume when ready
C. Reassure the client that this kind of grief reaction is normal

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Inviting the client to put thoughts into words is useful a little later. Asked right as the tears start, it interrupts the feeling and moves the client into explaining before they have had a moment with it.

B **Correct.** Purposeful silence gives a client room to feel and to collect thoughts, and attentive silence after strong emotion conveys acceptance (Hepworth et al.). This engaged client is in the middle of grief and has not asked for help, so quiet, attentive presence lets the feeling unfold and leaves the next step to the client. It beats A because an open question is a sound skill, but asked in the first moments of tears it pulls the client into explaining before they are ready. If the silence stretches or the client seems stuck, a gentle prompt then fits.

C **Incorrect** (trap: Good but not best). Normalizing grief can comfort, but offered at once it can feel like an effort to close down the emotion. It also puts the social worker's words in place of the client's experience at a tender moment.

Takeaway: After a client's strong emotion, attentive silence often helps more than questions or reassurance; prompt gently only if the client seems stuck.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Use of silence; attending and empathic communication
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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220. ASM-1582 (rev 1) · Best answer: C

On an inpatient psychiatric unit, the treatment team drafts a discharge plan before meeting with a patient admitted after a manic episode. At the planning meeting, the patient, now calm and organized, says, 'You all already decided everything.' What should the social worker do **FIRST**?

A. Walk the patient through each part of the team's draft plan
B. Explain why the team chose each recommendation in the plan
C. Invite the patient to name the problems that matter most to them

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Reviewing the draft is a needed step, but starting with it confirms the patient's complaint that the plan was made without them. The draft can be reviewed after the patient's own priorities are on the table.

B **Incorrect** (trap: Too directive). Explaining the team's reasoning is useful, but leading with it turns the meeting into persuasion. It treats the patient as someone to convince rather than a partner in defining the problems.

C **Correct.** Clients take part in identifying problems and goals, and plans built on their priorities are more likely to be followed (Hepworth et al.). The patient is now organized enough to participate and has just named feeling shut out, so the first move is to ask what they want the plan to address, honoring self-determination (NASW Code 1.02). Team concerns are then merged in. It beats A because reviewing the draft first repeats the very exclusion the patient described.

Takeaway: Begin intervention planning by asking the client to name the problems and goals that matter to them, then integrate team concerns.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Developing goals collaboratively with clients; NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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221. ASM-1583 (rev 2) · Best answer: B

At an annual planning meeting in a day program, a 40-year-old client with intellectual developmental disorder sits beside a parent who is the client's legal guardian. Each time the social worker asks about goals for the coming year, the parent answers for the client. The client speaks in short sentences and understands pictures well. Which action is **MOST** appropriate?

A. Record the parent's answers as the client's goals for the year
B. Ask the client directly, using plain words and picture choices
C. Ask the parent to leave so the client can speak more freely

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). The guardian's views carry legal weight, but recording them as the client's goals skips the client's own voice. Guardianship does not remove a person's right to be heard about their life and goals.

B **Correct.** Clients take part in planning to the greatest extent they can, and when a substitute decision maker is involved, social workers still inform clients at their level of understanding and safeguard their interests and rights (NASW Code 1.03(c), 1.14). The client communicates in short sentences and understands pictures, so adapting the questions lets the client state goals while the guardian stays involved. It beats C because removing the guardian sidelines a person with legal authority over decisions, when adapting the conversation achieves the same aim (NASW Code 1.02).

C **Incorrect** (trap: Misreads law or ethics). Time alone can help some clients speak, but excluding the legal guardian from a formal planning meeting disregards the guardian's role. Adapting communication with the guardian present respects both people's roles.

Takeaway: When a guardian speaks for a client, adapt communication so the client can state their own goals while the guardian stays involved.

Sources: NASW Code of Ethics (2021), 1.14; NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.03(c)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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222. ASM-1585 (rev 2) · Best answer: A

A child welfare agency holds a family group decision-making meeting for a 7-year-old in foster care. The coordinator has made introductions, and professionals have shared their safety concerns and the resources available. Relatives have asked their questions and seem ready to talk among themselves. What should the social worker do **NEXT**?

A. Step out with the agency staff so the family can plan
B. Present the agency's preferred plan for the family's feedback
C. Stay with the family to guide them through the discussion

Why each option is right or wrong

A **Correct.** In family group decision-making, professionals leave the room after the information-sharing phase so the family can meet privately and develop its own plan (Annie E. Casey Foundation). The family then presents the plan, and the agency accepts it if it addresses safety concerns. Information sharing is finished and relatives are ready, so the next step is private family time. It beats C because guiding the talk keeps professionals in charge and weakens the family's ownership of the plan.

B **Incorrect** (trap: Wrong model or concept). Offering the agency's plan for feedback reflects a professional-led case conference. In this model the family drafts the plan first, and the agency reviews it afterward for safety.

C **Incorrect** (trap: Misuses the technique). Staying to guide the discussion is well meant, but the model's defining feature is that the family deliberates without professionals present. The worker stays available outside the room to answer questions.

Takeaway: In family group decision-making, after information sharing, professionals leave so the family can create its own plan.

Sources: Annie E. Casey Foundation (2013). Four Approaches to Family Team Meetings, Family Group Decision Making: private family time
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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223. ASM-1667 (rev 2) · Best answer: A

A parent whose two children entered foster care meets with a child welfare social worker to build a case plan. The court has ordered substance use treatment and parenting classes. The parent says the top priority is finding a job, because without income they will lose their apartment, and the court also requires stable housing for reunification. What is the **MOST** appropriate way to proceed?

A. Include a job goal in the plan alongside the court-ordered services
B. Focus the plan on the court orders and add employment later
C. Make employment the first goal and begin services once the parent works

Why each option is right or wrong

A **Correct.** Social workers help clients identify and clarify their own goals (NASW Code 1.02), and with involuntary clients, plans pair nonnegotiable requirements with goals the client chooses. This parent's job goal also serves a court requirement, stable housing, so it belongs in the plan. Placing it beside the ordered treatment and classes, and helping the parent schedule all three, respects the parent's priority without dropping required conditions. It beats B because putting off employment ignores the parent's own reasoning and the housing requirement that depends on income (Hepworth et al.).

B **Incorrect** (trap: Good but not best). Court-ordered services must be in the plan, but leaving out the parent's main goal weakens engagement and neglects the housing requirement that rests on income. Required and client-defined goals can work together.

C **Incorrect** (trap: Misreads law or ethics). Honoring the parent's priority matters, but court-ordered services are not optional, and delaying them could stall reunification. The parent's goal should join the plan, not postpone required steps.

Takeaway: In mandated planning, include the client's own priority goals alongside required services rather than choosing between them.

Sources: NASW Code of Ethics (2021), 1.02; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Developing goals and contracts with involuntary clients
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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224. ASM-1668 (rev 1) · Best answer: C

In a second video session, a client with moderate depression agrees that setting goals would help but says, 'I honestly don't know what I want. I just want to feel less stuck.' The client is engaged and willing to keep talking about it. What is the **BEST** way for the social worker to proceed?

A. Offer a list of common goals for people with depression
B. Wait to set goals until the client's mood improves
C. Ask how the client's days would differ if the problem lifted
D. Set a goal of lowering the client's PHQ-9 score

Why each option is right or wrong

A **Incorrect** (trap: Too directive). A menu of goals can help as a last resort, but offering it first steers the client toward the worker's ideas. Goals the client shapes are more meaningful and more likely to be pursued.

B **Incorrect** (trap: Good but not best). Waiting for mood to lift delays help, and goal setting itself can build hope. Depression is a reason to make goals small and concrete, not a reason to put planning off.

C **Correct.** Solution-focused questions such as the miracle question help clients who cannot name goals picture concrete signs that life is better, and those signs can become goals in the client's own words (de Shazer, 1988). This client wants to feel 'less stuck' but cannot yet say what that looks like. Asking how daily life would be different if the problem lifted turns a vague wish into observable changes. It beats A because a list of common goals puts the worker's ideas in place of the client's and can yield goals the client accepts but does not own.

D **Incorrect** (trap: Misuses the technique). Tracking a depression measure is a useful outcome indicator, but choosing it for the client skips the client's own picture of change. It can sit alongside goals the client defines.

Takeaway: When clients cannot name goals, ask solution-focused questions that help them describe life with the problem resolved.

Sources: de Shazer, S. (1988). Clues: Investigating Solutions in Brief Therapy. W. W. Norton, miracle question **Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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225. ASM-0006 (rev 1) · Best answer: C

A client tells the social worker, 'I know my drinking is hurting my relationship, and part of me wants to cut back. But I'm not sure I'm ready to give up my evening drinks.' Which stage of change is the client **MOST** likely in?

A. Precontemplation

B. Preparation

C. Contemplation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Precontemplation means the person does not yet see a problem or is not considering change. This client names the harm and wants to cut back, which places them past precontemplation.

B **Incorrect** (trap: Wrong model or concept). Preparation combines an intention to change soon with concrete steps or a plan. The client voices doubt about readiness and describes no plan, so preparation is premature.

C **Correct.** Contemplation is marked by ambivalence: the person sees reasons to change and reasons not to, without a commitment to act soon (Prochaska & DiClemente, 1983; SAMHSA TIP 35). The client recognizes the harm and wants to cut back but is unsure about giving up drinking. It beats B because preparation involves intending to act soon and taking small steps, such as setting a date, and this client has made no commitment

or plan. Stage-matched work here means exploring and resolving ambivalence (TIP 35, Ch. 5).

Takeaway: Ambivalence, seeing reasons to change without committing to act, marks contemplation; explore and resolve the ambivalence.

Sources: SAMHSA TIP 35 (2019), Ch. 5: From Contemplation to Preparation; Prochaska, J. O., & DiClemente, C. C. (1983). Stages and processes of self-change of smoking: Toward an integrative model of change. Journal of Consulting and Clinical Psychology, 51(3), 390-395, whole article

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226. ASM-0111 (rev 1) · Best answer: B

At a follow-up visit, a client says, 'I stopped drinking 2 months ago. I go to support meetings twice a week and keep no alcohol at home. Weekends are still hard.' Which stage of change **MOST** likely describes this client?

A. Preparation

B. Action

C. Maintenance

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Preparation means intending to change soon and taking small first steps, such as setting a quit date. This client has already stopped drinking and changed their routines, which places them past preparation.

B **Correct.** In the action stage, the person is actively changing the behavior and their surroundings, usually during roughly the first 6 months of change (Prochaska & DiClemente, 1983). This client stopped drinking 2 months ago, attends meetings and keeps alcohol out of the home. It beats C because maintenance refers to sustaining change beyond about 6 months, and this change is still recent and effortful. Stage-matched work means reinforcing progress and problem-solving hard moments such as weekends (SAMHSA TIP 35, Ch. 7).

C **Incorrect** (trap: Wrong model or concept). Maintenance is sustaining a change over time, generally after about 6 months, with a focus on preventing recurrence. At 2 months, with weekends still a struggle, the client is in the active early phase instead.

Takeaway: Action is recent, effortful change (under about 6 months); maintenance is sustaining change beyond that; preparation is planning to act soon.

Sources: SAMHSA TIP 35 (2019), Ch. 7: From Action to Maintenance: Stabilizing Change; Prochaska, J. O., & DiClemente, C. C. (1983). Stages and processes of self-change of smoking: Toward an integrative model of change. Journal of Consulting and Clinical Psychology, 51(3), 390-395, whole article

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227. ASM-1586 (rev 1) · Best answer: C

In a primary care clinic, a client with a slow-healing foot ulcer smokes a pack a day. Asked to rate how important quitting is on a 0-10 scale, the client says 9. Asked how confident they are that they could quit, the client says 2. What should the social worker do **NEXT**?

A. Ask why the client's importance rating is 9 rather than 5
B. Explain how smoking slows the healing of foot wounds
C. Ask what would raise the confidence rating from 2 to 4

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Asking why a rating is not lower draws out reasons for change and works well when importance is moderate. With importance already at 9, it adds little and leaves the real barrier, low confidence, untouched.

B **Incorrect** (trap: Too directive). Health information can raise importance, but this client already rates importance at 9. Giving facts the client does not need can feel like lecturing and does nothing for the client's low sense of ability.

C **Correct.** Readiness rulers separate importance from confidence, and the lower score shows where to focus (SAMHSA TIP 35; Miller & Rollnick, 2023). A 9 on importance and a 2 on confidence mean the client wants to quit but doubts they can, so the barrier is self-efficacy. Asking what would move confidence up identifies supports and past successes to build on. It beats A because eliciting more reasons strengthens importance, which is already high.

Takeaway: When importance is high and confidence low, assess and build self-efficacy rather than adding reasons to change.

Sources: SAMHSA TIP 35 (2019), Ch. 4: From Precontemplation to Contemplation: Building Readiness; Miller & Rollnick (2023). Motivational Interviewing (4th ed.), Importance and confidence rulers
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228. ASM-1587 (rev 2) · Best answer: D

On a smoke-free inpatient psychiatric unit, a patient with schizophrenia who smoked heavily before admission tells the social worker, 'I don't want to start again when I leave. I'm picking a quit date with my doctor.' Discharge is in 5 days. What should the social worker do **FIRST**?

A. Review the pros and cons of smoking with the patient
B. Arrange a quitline referral to begin after discharge
C. Praise the decision and check in after discharge
D. Ask what could make it hard to avoid smoking at home

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Weighing pros and cons helps clients who are still ambivalent. This patient has already decided and is setting a date, so returning to decisional balance mismatches the stage and can stall momentum.

B **Incorrect** (trap: Right action, wrong time). A quitline is a useful support and belongs in the plan, but arranging it before knowing the patient's barriers may miss what the patient most needs. Identify obstacles first, then choose supports that address them.

C **Incorrect** (trap: Good but not best). Affirming the decision is good, but stopping there leaves the patient without a plan for the risks that follow discharge. Encouragement works best alongside concrete preparation.

D **Correct.** A client in preparation intends to act soon, and a key part of building the change plan is to explore and lower barriers to action, letting the client name likely obstacles (SAMHSA TIP 35, Ch. 6). The patient has a decision and a quit date, but leaving a smoke-free unit for home brings new triggers. Asking what could get in the way shapes the rest of the plan. It beats B because a referral chosen before barriers are known may not match what the patient will face.

Takeaway: In the preparation stage, have the client name likely barriers so the change plan and supports address them.

Sources: SAMHSA TIP 35 (2019), Ch. 6: From Preparation to Action: Initiating Change
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229. ASM-1588 (rev 3) · Best answer: B

A client referred by drug court after a third arrest for driving under the influence arrives for an outpatient intake. The client sits with arms crossed and says, 'I'm only here because the judge made me. My drinking is nobody's business.' What should the social worker do **FIRST**?

A. Ask the client to rate readiness to change on a 0-10 scale
B. Acknowledge the client's anger about being sent to treatment
C. Point out that three arrests suggest drinking is a problem

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A readiness ruler is a good way to gauge motivation, and the social worker may use one later in this intake. Asked of an angry client in the first minute, it is likely to feel like a test and draw a defensive '0' rather than useful information.

B **Correct.** Anger at being sent to treatment is common among mandated clients. Meeting it with empathy rather than argument lowers defensiveness and opens the way to honest talk about drinking, and the client's response starts to show where they stand, likely precontemplation here (SAMHSA TIP 35; Prochaska, DiClemente & Norcross, 1992). Acknowledging the frustration does not mean agreeing that drinking is not a problem. It beats A because a readiness ruler works once the client is engaged enough to answer it honestly; asked now, it yields little.

C **Incorrect** (trap: Too directive). The arrest history matters, but confronting a resentful client with it at intake invites argument and more defense of drinking. Motivational approaches avoid labeling and arguing for change, especially before engagement.

Takeaway: With an angry mandated client, acknowledge the frustration first; readiness tools and talk about consequences work better once the client is engaged.

Sources: Prochaska, J. O., DiClemente, C. C., & Norcross, J. C. (1992). In search of how people change: Applications to addictive behaviors. American Psychologist, 47(9), 1102-1114, whole article; SAMHSA TIP 35 (2019), Ch. 4: From Precontemplation to Contemplation: Building Readiness
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230. ASM-1589 (rev 3) · Best answer: C

At an opioid treatment program, a client on methadone for 4 months has had no opioid use and attends every counseling session. The client also uses cannabis daily and says, 'Weed isn't a problem. It's the only thing that helps me sleep.' Program staff want the treatment plan to require stopping cannabis now. What is the **BEST** way to proceed?

A. Make stopping cannabis a required goal in the plan
B. Explain to the client the health risks of using cannabis every day
C. Explore the client's own view of what cannabis gives and costs
D. Leave cannabis out of the plan while opioid goals come first

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Requiring abstinence from a substance the client does not yet see as a problem mismatches the client's stage and can provoke resistance or dropout. It also risks the progress the client has made with opioids.

B **Incorrect** (trap: Too directive). Giving risk information before learning how the client sees cannabis can feel like a lecture and push the client to defend a habit they value for sleep. Information offered with permission can help later, once the client's own view is known.

C **Correct.** Readiness is specific to each behavior: this client is in action for opioids but sees no problem with cannabis, a precontemplation stance toward it (SAMHSA TIP 35; Prochaska et al., 1992). Exploring the client's own view of what cannabis does for and against them, including the sleep problem it is meant to solve, is how motivational work raises ambivalence without provoking resistance, and it keeps goals matched to each behavior's stage. It beats A because requiring an unwanted goal can damage engagement that is clearly working.

D **Incorrect** (trap: Good but not best). Keeping opioid recovery first is sound, but leaving daily cannabis out entirely skips learning how the client sees it and what the sleep problem needs. The plan can hold opioid goals steady while the cannabis conversation begins.

Takeaway: When a client in recovery sees no problem with a second substance, explore their own view of its benefits and costs before setting goals for it.

Sources: SAMHSA TIP 35 (2019), Ch. 4: From Precontemplation to Contemplation: Building Readiness; Prochaska, J. O., DiClemente, C. C., & Norcross, J. C. (1992). In search of how people change: Applications to addictive behaviors. American Psychologist, 47(9), 1102-1114, whole article
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231. ASM-1675 (rev 2) · Best answer: C

A cardiac rehabilitation team describes a 74-year-old patient as 'unmotivated' because the patient rarely does the home exercises. The patient says they want to get stronger. The social worker notices that the patient asks the same questions several times per visit and got lost driving to the clinic last week. What should the social worker do **FIRST**?

A. Use motivational interviewing to build commitment to exercising
B. Give the patient a simplified written home exercise schedule
C. Ask the physician to evaluate the patient's memory problems
D. Ask a family member to supervise the daily exercises

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Motivational interviewing fits ambivalence about change, but this patient already voices a wish to get stronger. The missed exercises track with memory and orientation problems, so building commitment would target the wrong barrier (Miller & Rollnick, 2023).

B **Incorrect** (trap: Right action, wrong time). Simplified written instructions are a sensible support for someone with memory problems and may become part of the plan. Offered first, they skip the evaluation that could find a treatable cause of the decline.

C **Correct.** Assess barriers before concluding that a client lacks motivation. This patient states a wish to improve, yet repeats the same questions within a visit and got lost driving to the clinic. These signs point to a possible cognitive barrier rather than ambivalence. Observed lapses do not establish a cause, and some causes of cognitive decline are treatable, so a medical evaluation comes first (Kaplan & Sadock's Synopsis). It beats B because simplified written steps help most once the team knows what kind of impairment is present; they can support the plan but cannot replace the evaluation.

D **Incorrect** (trap: Good but not best). Family help can support home exercise, but it treats the problem as follow-through alone. It leaves the memory and orientation problems unevaluated and may take over a task the patient wants to manage.

Takeaway: When a client wants change but does not follow through, assess for cognitive or other barriers before labeling the client unmotivated.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Neurocognitive disorders: evaluation of cognitive impairment and reversible causes; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Ambivalence and the spirit of MI

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232. ASM-1676 (rev 1) · Best answer: A

Which instrument is designed to measure how ready a client is to change their drinking?

A. SOCRATES
B. AUDIT
C. CAGE
D. CIWA-Ar

Why each option is right or wrong

A **Correct.** SOCRATES (Stages of Change Readiness and Treatment Eagerness Scale) measures motivation to change drinking. Its scales cover recognizing a problem, ambivalence about it, and steps already taken toward change (Miller & Tonigan, 1996). It tells the worker where the client stands on change, not how much they drink. It beats B, the strongest competitor, because the AUDIT measures hazardous or harmful drinking and likely dependence; a high AUDIT score says nothing about whether the client is ready to act.

B **Incorrect** (trap: Wrong model or concept). The AUDIT is a 10-item World Health Organization screen for hazardous and harmful drinking and likely dependence (Babor et al., 2001). It shows how risky the drinking is, not how ready the client is to change it.

C **Incorrect** (trap: Wrong model or concept). The CAGE is a four-question screen for problem drinking that asks about cutting down, annoyance, guilt and morning drinking (Ewing, 1984). A positive screen prompts further assessment but does not measure readiness for change.

D **Incorrect** (trap: Wrong model or concept). The CIWA-Ar rates the severity of alcohol withdrawal so medical staff can guide withdrawal treatment (Sullivan et al., 1989). It is used during withdrawal management, not to gauge motivation.

Takeaway: Screens such as the AUDIT and CAGE measure drinking risk; readiness instruments such as SOCRATES measure motivation to change.

Sources: Miller, W. R., & Tonigan, J. S. (1996). Assessing drinkers' motivation for change: The Stages of Change Readiness and Treatment Eagerness Scale (SOCRATES). Psychology of Addictive Behaviors, 10, 81-89, scales: recognition, ambivalence, taking steps; Babor, Higgins-Biddle, Saunders & Monteiro (2001). AUDIT: The Alcohol Use Disorders Identification Test, Guidelines for Use in Primary Care (2nd ed.). World Health Organization, purpose of the AUDIT; Ewing, J. A. (1984). Detecting alcoholism: The CAGE questionnaire. JAMA, 252(14), 1905-1907, whole article; Sullivan, J. T., et al. (1989). Assessment of alcohol withdrawal: The revised Clinical Institute Withdrawal Assessment for Alcohol scale (CIWA-Ar). British Journal of Addiction, 84(11), 1353-1357, whole article

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233. ASM-1677 (rev 2) · Best answer: B

A client in outpatient treatment for gambling disorder had not placed a bet for 4 months. After a sports app sent a promotion, the client bet throughout a weekend and lost several hundred dollars. In session, the client says, 'Maybe I don't need to quit completely. I could just bet small amounts.' What should the social worker do **FIRST**?

A. Revise the relapse-prevention plan to block betting app promotions

B. Ask how the client now sees their recovery goals

C. Reassure the client that slips are common and progress remains

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Blocking the trigger that set off the lapse is a sound relapse-prevention step once the client recommits to a goal. Done first, it passes over the client's stated doubt about quitting and assumes an abstinence goal the client may no longer hold.

B **Correct.** Readiness is not fixed; after a lapse, people often cycle back to an earlier stage of change (Prochaska, DiClemente & Norcross, 1992). The client's new talk of

betting small amounts signals that the abstinence goal itself is now in question. Asking how the client views their goals reassesses where they stand before the worker chooses a response. It beats A because revising the relapse-prevention plan assumes the client still holds the abstinence goal; if the goal has shifted, the revision pushes a plan the client no longer owns and invites argument (Miller & Rollnick, 2023).

C **Incorrect** (trap: Good but not best). Normalizing a lapse can ease shame and is often helpful. Here, though, the client is voicing a changed goal rather than shame, so reassurance alone leaves the shift in readiness unexamined.

Takeaway: After a lapse, reassess the client's current goals and stage before resuming relapse-prevention work; readiness can move backward.

Sources: Prochaska, J. O., DiClemente, C. C., & Norcross, J. C. (1992). In search of how people change: Applications to addictive behaviors. American Psychologist, 47(9), 1102-1114, spiral model of the stages of change; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Sustain talk and discord
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234. ASM-1590 (rev 1) · Best answer: B

In Lazarus and Folkman's stress and coping model, what is a person's judgment about whether they have the resources to handle a stressor called?

A. Primary appraisal
B. Secondary appraisal
C. Emotion-focused coping

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Primary appraisal is the person's judgment of what a situation means for them: whether it is irrelevant, harmless, or a threat, harm or challenge. It comes before deciding whether one can cope.

B **Correct.** In Lazarus and Folkman's transactional model, secondary appraisal is the person's judgment of what can be done and whether their resources and options are enough to manage a stressor (Lazarus & Folkman, 1984). Assessing it shows how capable a client feels of coping. It beats A because primary appraisal concerns whether the event is a threat, harm or challenge, not whether the person can handle it.

C **Incorrect** (trap: Wrong model or concept). Emotion-focused coping is a strategy for managing distress, such as seeking comfort or reframing. It is something a person does after appraisal, not the judgment about available resources.

Takeaway: Primary appraisal asks whether a stressor threatens me; secondary appraisal asks whether I have the resources to cope.

Sources: Lazarus, R. S., & Folkman, S. (1984). Stress, Appraisal, and Coping. Springer, cognitive appraisal: primary and secondary appraisal
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.5 Assessing resilience and coping capacity · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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235. ASM-1591 (rev 2) · Best answer: C

A hospital patient had a below-knee amputation after a work injury. The patient jokes with staff about 'saving on socks,' asks detailed questions about prostheses and calls coworkers daily. The social worker's assessment finds that the patient sleeps well, eats meals, works hard in physical therapy and talks about returning to work in a new role. A nurse requests a psychiatric consult because the patient 'must be in denial.' What should the social worker do **NEXT**?

A. Request the psychiatric consult the nurse suggested
B. Encourage the patient to set humor aside and grieve
C. Share the coping findings with the treatment team
D. Explore what the patient's jokes may be covering up

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). A psychiatric consult fits when assessment shows a mental health concern, such as depression or impaired functioning. Here the assessment shows good sleep, appetite and engagement, so a consult would pathologize healthy coping.

B **Incorrect** (trap: Too directive). Pushing the patient to grieve in a particular way imposes the worker's view of adjustment. People cope with loss differently, and this patient is functioning well; the patient may grieve in their own time.

C **Correct.** Coping assessment looks at how a client is actually functioning. Humor is a mature, adaptive defense (Vaillant, 1977), and seeking information and planning a return to work are problem-focused coping (Lazarus & Folkman, 1984). Sleep, appetite and active rehabilitation confirm the patient is adapting. Sharing these findings helps the team support the patient rather than treat strengths as pathology. It beats D because the assessment is already complete; probing for hidden distress treats healthy humor as a symptom.

D **Incorrect** (trap: Good but not best). Gentle exploration suits a patient whose humor seems to mask distress. This assessment already found good functioning and realistic planning, so searching for a hidden problem assumes pathology the facts do not show.

Takeaway: Judge coping by functioning; humor and active problem solving after a loss can be adaptive, not denial.

Sources: Vaillant, G. E. (1977). Adaptation to Life. Little, Brown, mature defenses: humor; Lazarus, R. S., & Folkman, S. (1984). Stress, Appraisal, and Coping. Springer, problem-focused coping
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.5 Assessing resilience and coping capacity · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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236. ASM-1592 (rev 2) · Best answer: A

A 14-year-old in kinship care has lived through a parent's incarceration and three school moves. The teen earns steady grades, plays in the school band and talks about future plans. Since age 8, the same basketball coach has checked in weekly and attended the teen's court hearings. Which factor **MOST** likely accounts for the teen's resilience?

A. A committed relationship with a caring adult
B. Above-average intelligence and school ability
C. Taking part in structured after-school activities

Why each option is right or wrong

A **Correct.** Research on resilience finds that children who do well despite serious hardship have had at least one stable, committed relationship with a supportive adult (National Scientific Council on the Developing Child, 2015). The coach's steady presence for 6 years, including at court hearings, is the clearest protective factor in the case. It beats C because band activity helps, but the coach's long, reliable involvement is the stronger and better-documented support.

B **Incorrect** (trap: Wrong model or concept). Cognitive ability can be protective, but the stem describes steady, not exceptional, grades. Nothing shows above-average intelligence, so this explanation rests on an assumption.

C **Incorrect** (trap: Wrong model or concept). Structured activities such as band can build skills and belonging, and they support resilience. They are less central than a long, committed relationship with a caring adult, which the case documents for 6 years.

Takeaway: A stable, committed relationship with at least one supportive adult is the most consistent protective factor for children facing adversity.

Sources: National Scientific Council on the Developing Child (2015). Supportive Relationships and Active Skill-Building Strengthen the Foundations of Resilience: Working Paper No. 13. Center on the Developing Child at Harvard University, whole paper

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237. ASM-1678 (rev 1) · Best answer: C

A neurologist asks a hospital social worker to assess how a 34-year-old patient newly diagnosed with multiple sclerosis will manage after discharge. The patient is tearful but calm and plans to return home to a demanding job. Which question is **MOST** useful for this assessment?

A. Over the past 2 weeks, how often have you felt hopeless?

B. Would you like information about local MS support groups?

C. How have you gotten through hard times before, and who helped?

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A mood screening question belongs in a full assessment and should be asked if distress deepens. It measures symptoms rather than the coping strategies and supports that this referral asks about.

B **Incorrect** (trap: Right action, wrong time). Support groups may become part of the plan, but offering a resource before assessing the patient's own coping and supports is premature. The answer to option C would show whether a group fits.

C **Correct.** Capacity to cope is judged largely from the resources a person brings: past ways of handling stress and the people who help (Lazarus & Folkman, 1984). Asking how the patient got through earlier hard times, and who helped, gathers both in one open question and starts from strengths (Hepworth et al.). That is what the neurologist requested. It beats A because a mood question measures current depressive symptoms, which matter, but it does not show how the patient handles stress or who will support them after discharge.

Takeaway: Assess coping capacity by asking how the client has handled past hardships and who helped; symptom screens do not measure coping resources.

Sources: Lazarus, R. S., & Folkman, S. (1984). Stress, Appraisal, and Coping. Springer, coping resources; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessing strengths and coping
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238. ASM-0112 (rev 1) · Best answer: A

Which assessment tool **BEST** maps a family's current connections to schools, agencies, work and other outside systems?

A. Ecomap
B. Genogram
C. Culturagram

Why each option is right or wrong

A **Correct.** An ecomap places the family in a central circle and draws lines to outside systems such as school, work, health care and agencies, showing which connections are strong, weak or stressful (Hartman, 1978). It captures the family's present-day person-in-environment picture. It beats B because a genogram maps family structure and patterns across at least three generations, not current ties to outside systems (McGoldrick et al., 2020).

B **Incorrect** (trap: Wrong model or concept). A genogram diagrams family members, relationships and patterns across three or more generations, such as illness, substance use or cutoffs. It is the right tool for intergenerational patterns, not for mapping current outside resources.

C **Incorrect** (trap: Wrong model or concept). A culturagram assesses an immigrant family's cultural context, such as reasons for migration, legal status, language and health beliefs (Congress, 1994). It is useful for cultural assessment but does not chart the family's links to outside systems.

Takeaway: Ecomap shows current ties to outside systems; genogram shows intergenerational family patterns; culturagram shows an immigrant family's cultural context.

Sources: Hartman, A. (1978). Diagrammatic assessment of family relationships. Social Casework, 59(8), 465-476, whole article; McGoldrick, M., Gerson, R., & Petry, S. (2020). Genograms: Assessment and Treatment (4th ed.). W. W. Norton, genogram construction; Congress, E. P. (1994). The use of culturagrams to assess and empower culturally diverse families. Families in Society, 75(9), 531-540, whole article
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239. ASM-1593 (rev 2) · Best answer: B

At a senior center, a 71-year-old client reports forgetting appointments over the past 2 months. The social worker gives a brief cognitive screen, and the score falls in the impaired range. The client mentions starting a new bladder medication about 10 weeks ago and has not seen a doctor since. What should the social worker do **NEXT**?

A. Tell the client the score points to early dementia
B. Arrange a prompt medical evaluation of the memory change
C. Repeat the screen in 6 months to track any change
D. Give the client a second cognitive screen to confirm

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). A screening score does not diagnose dementia, and diagnosing a medical condition is outside the social work role. Labeling the problem could alarm the client when a treatable cause may exist.

B **Correct.** A brief cognitive screen such as the MoCA flags possible impairment but does not diagnose (Nasreddine et al., 2005). The memory problems began around when the client started a bladder medication, and some bladder drugs have anticholinergic effects that can cause confusion in older adults (American Geriatrics Society Beers Criteria, 2023). A medical evaluation can review medications and look for reversible causes. It beats D because a second screen only repeats the finding; the next need is a medical workup.

C **Incorrect** (trap: Ignores risk). Waiting 6 months leaves a possibly reversible, medication-related cause untreated while the client keeps missing appointments. Monitoring fits after a medical evaluation, not instead of one.

D **Incorrect** (trap: Misuses the technique). A second screening tool adds little once one screen is positive and does not explain why. The finding needs evaluation by a medical provider, who can examine medications and other causes.

Takeaway: A positive cognitive screen calls for medical evaluation, including a medication review, not a diagnosis or a wait-and-see plan.

Sources: Nasreddine, Z. S., et al. (2005). The Montreal Cognitive Assessment, MoCA: A brief screening tool for mild cognitive impairment. Journal of the American Geriatrics Society, 53(4), 695-699, whole article; American Geriatrics Society Beers Criteria Update Expert Panel (2023). American Geriatrics Society 2023 updated AGS Beers Criteria for potentially inappropriate medication use in older adults. Journal of the American Geriatrics Society, 71(7), 2052-2081, strongly anticholinergic medications

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240. ASM-1594 (rev 1) · Best answer: A

Which tool measures how well an older adult can handle money, medicines, shopping and meals?

A. Lawton Instrumental Activities of Daily Living Scale
B. Katz Index of Independence in Activities of Daily Living
C. Mini-Cog brief cognitive screen

Why each option is right or wrong

A **Correct.** The Lawton scale measures instrumental activities of daily living, the complex tasks needed to live independently, such as managing money, medications, shopping, cooking and transportation (Lawton & Brody, 1969). Money, medications, shopping and meal preparation are exactly these tasks, and losing them often signals the first loss of independence. It beats B because the Katz Index measures basic self-care, such as bathing, dressing and feeding, and would miss these higher-level tasks.

B **Incorrect** (trap: Wrong model or concept). The Katz Index measures basic activities of daily living: bathing, dressing, toileting, transferring, continence and feeding (Katz et al., 1963). Those basic tasks are often kept long after money, medication and shopping skills decline, so the Katz Index would miss the areas named.

C **Incorrect** (trap: Wrong model or concept). The Mini-Cog screens for cognitive impairment with word recall and clock drawing. It may help explain the decline, but it does not measure the daily living tasks the question asks about.

Takeaway: Basic ADLs cover self-care (Katz); instrumental ADLs cover independent-living tasks such as finances, medications and shopping (Lawton).

Sources: Lawton, M. P., & Brody, E. M. (1969). Assessment of older people: Self-maintaining and instrumental activities of daily living. The Gerontologist, 9(3), 179-186, whole article; Katz, S., Ford, A. B., Moskowitz, R. W., Jackson, B. A., & Jaffe, M. W. (1963). Studies of illness in the aged. The index of ADL. JAMA, 185(12), 914-919, whole article

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241. ASM-1595 (rev 1) · Best answer: C

At intake in a community clinic, a client whose preferred language is Tagalog reports low mood and poor sleep. The clinic routinely gives a standardized depression questionnaire, and its publisher offers versions in many languages, including Tagalog. A trained interpreter is on the call. Which approach is **BEST**?

A. Skip the questionnaire and rely on the clinical interview

B. Have the interpreter read the English version aloud in Tagalog

C. Give the client the published Tagalog version of the questionnaire

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The clinical interview remains essential, but dropping the measure denies this client the standardized screening other clients receive. A properly translated version makes the tool usable, so skipping it is unnecessary.

B **Incorrect** (trap: Misuses the technique). Translating items on the spot changes wording from one administration to the next, so scores may no longer mean what the norms assume. Trained interpreters are essential for the interview, but not as a substitute for a validated translation of a standardized measure.

C **Correct.** Standardized measures give meaningful scores only when given in the form that was developed and tested, and a properly adapted translation preserves that (Standards for Educational and Psychological Testing, Ch. 3). Using the published Tagalog version gives the client equal access to screening in their own language

(NASW Cultural Competence Standard 9). It beats B because translating items on the spot alters their meaning and makes the score hard to interpret.

Takeaway: With clients who prefer another language, use a properly translated and tested version of a standardized measure, not on-the-spot translation.

Sources: Standards for Educational and Psychological Testing (2014). American Educational Research Association, American Psychological Association, & National Council on Measurement in Education, Ch. 3: Fairness in Testing; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication

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242. ASM-1600 (rev 2) · Best answer: C

A hospital social worker meets with a 72-year-old spouse who has cared at home for 3 years for a partner with major neurocognitive disorder. The spouse describes losing sleep, giving up friendships and feeling trapped. The social worker wants a standardized measure of how the caregiving role itself is affecting the spouse. Which instrument is the **BEST** choice?

A. Geriatric Depression Scale
B. Lawton IADL Scale
C. Zarit Burden Interview
D. Ecomap

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The Geriatric Depression Scale screens for depression in older adults. The spouse may well be depressed, but a mood score does not show how much of the distress comes from the caregiving role, which is what the social worker wants to measure.

B **Incorrect** (trap: Wrong model or concept). The Lawton scale rates instrumental activities of daily living such as shopping, finances and medications. It describes the care recipient's functional needs, not the strain the caregiver feels.

C **Correct.** The Zarit Burden Interview is a self-report measure built to capture the burden family caregivers feel, including effects on their health, social life and emotional well-being (Zarit, Reever & Bach-Peterson, 1980). Lost sleep, dropped friendships and feeling trapped are exactly the caregiving strain it measures. It beats A because a depression scale measures mood in general; it cannot show how much of the spouse's distress is tied to the caregiving role, which is the target here.

D **Incorrect** (trap: Wrong model or concept). An ecomap is a drawing of a family's links to outside systems such as friends, agencies and church. It could show the lost friendships, but it is not a standardized measure of how burdened the caregiver feels.

Takeaway: Match the tool to the target: burden scales measure caregiver strain, depression scales measure mood, and IADL scales rate the care recipient's function.

Sources: Zarit, S. H., Reever, K. E., & Bach-Peterson, J. (1980). Relatives of the impaired elderly: Correlates of feelings of burden. The Gerontologist, 20(6), 649-655, whole article

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243. ASM-1679 (rev 1) · Best answer: D

Which tool has a client list the people in their life and rate each one on concrete help, emotional support, closeness, and how often they are in contact?

A. Ecomap
B. Genogram
C. Culturagram
D. Social network map

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). An ecomap diagrams the client or family and their ties to outside systems such as work, school and health care (Hartman, 1978). It shows the nature of those links, not ratings of each person's support.

B **Incorrect** (trap: Wrong model or concept). A genogram maps family members across at least three generations to show patterns such as illness, loss and relationship conflict (McGoldrick, Gerson & Petry, 2020). It does not rate current support from each person.

C **Incorrect** (trap: Wrong model or concept). A culturagram assesses an immigrant family's cultural context, such as reasons for immigrating, language, legal status and health beliefs (Congress, 1994). It addresses culture and migration rather than support from network members.

D **Correct.** The social network map assesses both the structure and the function of a client's personal network (Tracy & Whittaker, 1990). The client names network members and rates each on the kinds of support given, closeness and frequency of contact, which shows who can actually help. It beats A, the strongest competitor, because an ecomap diagrams the family's connections to outside systems such as school, work and agencies, with lines showing strong or stressful ties, rather than rating individual people on specific types of support.

Takeaway: Use a social network map to rate who in a client's network gives concrete and emotional help; an ecomap maps ties to outside systems.

Sources: Tracy, E. M., & Whittaker, J. K. (1990). The social network map: Assessing social support in clinical practice. Families in Society, 71(8), 461-470, whole article; Hartman, A. (1978). Diagrammatic assessment of family relationships. Social Casework, 59(8), 465-476, the ecomap; McGoldrick, Gerson & Petry (2020). Genograms: Assessment and Treatment (4th ed.). W. W. Norton, purpose of the genogram; Congress, E. P. (1994). The use of culturagrams to assess and empower culturally diverse families. Families in Society, 75(9), 531-540, culturagram components

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244. ASM-1680 (rev 2) · Best answer: A

At a primary care visit, a parent of two completes an adverse childhood experiences (ACE) questionnaire and scores 7. The parent describes fears that the score means they will become seriously ill or harm their children, then asks the social worker what the number means. What is the **MOST** appropriate response?

A. Explain that the score estimates risk for groups rather than individuals
B. Recommend trauma-focused therapy because of the high score
C. Encourage the parent to keep talking about these parenting fears

Why each option is right or wrong

A **Correct.** An ACE score counts types of childhood adversity and is linked to risk across populations; its developers stress that it is neither diagnostic nor predictive for an individual (Anda, Porter & Brown, 2020). The parent has voiced specific fears and asked a direct question, so a plain, accurate answer helps most now. Explaining that the score describes group risk, not what will happen to this parent, corrects the misreading and opens talk about the supports that shape outcomes. It beats C because the parent has already described the fears; more exploration would sidestep the question.

B **Incorrect** (trap: Skips assessment). Treatment should follow an assessment of current symptoms and functioning, not a questionnaire score. Recommending therapy on the score alone treats the number as a diagnosis, which the measure's developers caution against.

C **Incorrect** (trap: Good but not best). Inviting more detail is useful early, but the parent has already shared these fears and is now asking for information. Deflecting the direct question could add to the worry the number has caused.

Takeaway: An ACE score shows population-level risk; it does not diagnose or predict an individual's outcome, so explain it accurately to clients.

Sources: Anda, R. F., Porter, L. E., & Brown, D. W. (2020). Inside the adverse childhood experience score: Strengths, limitations, and misapplications. American Journal of Preventive Medicine, 59(2), 293-295, whole article
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245. ASM-1576 (rev 1) · Best answer: C

In a cardiac rehabilitation program, a 68-year-old client mentions having 'a few glasses of wine' most evenings. The social worker needs an accurate picture of the client's drinking to share with the care team. Which question is **BEST**?

A. 'You don't drink much more than that, do you?'
B. 'Would you say your drinking is a problem?'
C. 'On a typical drinking day, how much do you have?'

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). This leading question signals the answer the worker expects and makes it awkward for the client to report heavier drinking. It invites a quick 'no' rather than an accurate amount.

B **Incorrect** (trap: Misuses the technique). Asking the client to judge their own drinking as a 'problem' is a labeling, closed question. Many people who drink at risky levels answer no, so it yields an opinion rather than the amount the team needs.

C **Correct.** Sensitive questions about substance use work best when they are specific, nonjudgmental and assume the behavior may occur. Asking how much the client has on a typical drinking day mirrors the consumption items of the AUDIT screen (Babor et al., WHO) and turns a vague 'a few glasses' into a usable amount. It beats B because asking whether drinking is a 'problem' invites denial and does not measure quantity (Hepworth et al.).

Takeaway: To learn about substance use, ask specific, nonjudgmental quantity and frequency questions; avoid leading or labeling questions.

Sources: Babor, T. F., Higgins-Biddle, J. C., Saunders, J. B., & Monteiro, M. G. (2001). AUDIT: The Alcohol Use Disorders Identification Test: Guidelines for Use in Primary Care (2nd ed.). World Health Organization, AUDIT items 1-3: consumption questions; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Avoiding leading questions

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246. ASM-1577 (rev 2) · Best answer: A

A 15-year-old comes to a pediatric clinic with a parent for a sports physical. The social worker plans a brief psychosocial screen covering substance use, sexual activity and mood. The parent sits beside the teen and starts answering the social worker's first questions. What should the social worker do **FIRST**?

A. Explain confidentiality and its limits, then meet with the teen alone

B. Ask the teen the screening questions in a calm, matter-of-fact way

C. Assure the teen that everything said today will stay private

Why each option is right or wrong

A **Correct.** Adolescents disclose substance use, sexual activity and mood symptoms more honestly in private time with the provider after hearing what stays confidential and what does not, such as serious risk of harm (Hagan et al., Bright Futures). With the parent answering, the teen's own account is lost, so private time comes before the sensitive questions. Explaining the limits honors the duty to discuss confidentiality at the outset (NASW Code 1.07(e)). It beats B because a matter-of-fact tone helps, but asking with the parent present makes honest answers unlikely.

B **Incorrect** (trap: Right action, wrong time). A calm, normalizing tone is good technique for sensitive topics, but asking these questions with the parent beside the teen comes too early. Set up private time and explain confidentiality first, then use this approach.

C **Incorrect** (trap: Misreads law or ethics). Promising complete privacy is inaccurate. Disclosures of abuse or serious risk of harm can require action, so the teen must hear the limits before speaking. A false promise can damage trust later.

Takeaway: Before asking adolescents about sensitive topics, explain confidentiality and its limits and arrange time alone with the teen.

Sources: Hagan, J. F., Shaw, J. S., & Duncan, P. M. (Eds.) (2017). Bright Futures: Guidelines for Health Supervision of Infants, Children, and Adolescents (4th ed.). American Academy of Pediatrics, Adolescence visits: confidentiality and time alone with the adolescent; NASW Code of Ethics (2021), 1.07(e)
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247. ASM-1578 (rev 3) · Best answer: D

During a video intake for depression, the social worker reaches the trauma history section, which asks about sexual abuse and assault. The client is at home. When the client's partner comes up, the client glances off-screen, lowers their voice and begins answering in single words. What should the social worker do **FIRST**?

A. Ask about unwanted sexual experiences in specific terms
B. Skip the trauma section unless the client raises it later
C. Email the trauma questionnaire for the client to finish later
D. Ask whether the client is alone and able to talk freely

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Behaviorally specific wording is the right way to ask about sexual trauma, and it is the next step once privacy is confirmed. Asking now could expose the client to danger if the partner is listening.

B **Incorrect** (trap: Good but not best). Dropping the section leaves out information that shapes diagnosis and treatment, and it signals the topic is off-limits. Trauma questions should be postponed only until the client can speak safely, not abandoned.

C **Incorrect** (trap: Ignores risk). A form sent to a shared home email or device could be seen by the partner, adding risk rather than reducing it. Written screens can help, but only through a channel the client controls.

D **Correct.** Trauma-informed screening calls for a private, safe setting before sensitive questions, because safety comes first (SAMHSA TIP 57; SAMHSA, 2014). The client's glance off-screen and lowered voice at the partner's name suggest someone may be listening, possibly someone the client fears. A brief yes-or-no check lets the client signal danger without explaining. It beats A because specific questions are the right technique only after privacy is confirmed; asked now, they could put the client at risk.

Takeaway: Before asking about sexual trauma, confirm the client can speak privately and safely, especially by video from home.

Sources: SAMHSA TIP 57 (2014), Part 1, Ch. 4: Screening and Assessment; SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Key principles: Safety
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248. ASM-1664 (rev 1) · Best answer: A

At intake to a residential substance use program, a social worker needs to ask a 29-year-old client about any history of sexual violence. The client has heard the limits of confidentiality and agreed to continue with the intake questions. Which question is **MOST** effective for this purpose?

A. 'Has anyone ever forced or pressured you into sexual contact?'
B. 'Have you ever been sexually abused, molested or assaulted?'
C. 'Is there anything about your past that you would like to share?'

Why each option is right or wrong

A **Correct.** Behaviorally specific questions describe the act rather than asking a person to apply a label such as 'abuse' or 'rape.' Many people who experienced unwanted sexual contact do not use those labels, so label-based questions miss them. Research comparing question wording found that behaviorally specific items prompt more people to recall and report such experiences (Fisher, 2009). Asking in plain terms also keeps the tone matter-of-fact. It beats B because the label-based version is direct but depends on the client defining the experience the same way.

B **Incorrect** (trap: Misuses the technique). This question is direct, which helps, but it asks the client to label the experience. People who would not call what happened abuse or assault may truthfully answer no, so real histories go unrecorded.

C **Incorrect** (trap: Good but not best). An open invitation suits many topics, but it is too vague for a sensitive history. It leaves the client to guess what is being asked and makes it even less likely that something hard to name will come up.

Takeaway: Ask about sensitive experiences with behaviorally specific wording that describes acts, not labels the client may not use.

Sources: Fisher, B. S. (2009). Effects of survey question wording on rape estimates: Evidence from a quasi-experimental design. Violence Against Women, 15(2), 133-147, whole article
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249. ASM-1665 (rev 2) · Best answer: C

A school-based health center social worker meets alone with a 15-year-old for a first visit about headaches and missed classes. The visit opened with a review of what stays private and when information must be shared, which the teen restated accurately. The social worker needs to ask about substance use and sexual activity during this visit. What should the social worker do **NEXT**?

A. Ask about substance use and sexual activity right away
B. Wait to cover those topics until the teen raises them
C. Begin with home life and daily activities, then risk topics

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). These questions belong in the visit, but asking them first, before any rapport on easier topics, often yields guarded answers. Starting with home and school makes later honest answers more likely.

B **Incorrect** (trap: Ignores risk). Teens rarely bring up substance use or sexual activity on their own, and waiting can leave real risks unassessed. The social worker raises these topics routinely, after easing in with less sensitive areas.

C **Correct.** Psychosocial interviews with adolescents usually move from important but less threatening topics, such as home, school and activities, toward more personal ones such as drugs and sexuality (Klein, Goldenring & Adelman, 2014). With confidentiality already explained, starting with everyday areas builds comfort and may reveal stresses tied to the headaches and missed classes. The more sensitive questions then follow in the same visit. It beats A because opening with the most personal topics before any rapport tends to produce guarded answers.

Takeaway: With adolescents, explain confidentiality, then move from less threatening topics such as home and school toward drugs and sexuality.

Sources: Klein, D. A., Goldenring, J. M., & Adelman, W. P. (2014). HEEADSSS 3.0: The psychosocial interview for adolescents updated for a new century fueled by media. Contemporary Pediatrics, 31(1), 16-28, interview sequence and confidentiality

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250. ASM-1579 (rev 1) · Best answer: B

A 74-year-old hospital patient had a stroke 5 days ago and now speaks in short, halting phrases. The team plans discharge in 2 days and asks the social worker whether the patient can take part in choosing between home and a rehabilitation facility. The patient's yes and no answers to the social worker's questions have been inconsistent. Which action is **BEST**?

A. Ask the patient simple yes-or-no questions about the options
B. Request a speech-language evaluation of the patient's communication
C. Ask the patient's adult child to choose the discharge setting
D. Note in the record that the patient cannot state a preference

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Simple yes-or-no questions are a common support for people with aphasia, but this patient's answers have already been inconsistent. Without knowing how well the patient understands, the worker cannot tell whether answers reflect real preferences.

B **Correct.** Aphasia can hide a person's competence, so poor speech does not show that someone cannot understand or decide (Kagan, 1998). A speech-language pathologist can assess comprehension and expression and recommend supports, such as written key words or pictures, that let the patient take part. Referral fits when another professional's expertise is needed (NASW Code 1.16(a)). It beats A because the inconsistent answers are exactly the problem; more yes-no questions cannot settle what the patient understands.

C **Incorrect** (trap: Overrides the client's choice). Turning the decision over to family assumes the patient cannot participate before communication has been properly assessed. Family input matters, but a substitute decision maker is considered only after the patient's own ability is evaluated.

D **Incorrect** (trap: Skips assessment). Recording that the patient cannot state a preference treats a communication barrier as an inability to decide. It could shut the patient out of a major decision before anyone has tried supported communication.

Takeaway: When speech or language impairment clouds a client's responses, obtain a communication evaluation before concluding the client cannot participate.

Sources: Kagan, A. (1998). Supported conversation for adults with aphasia: Methods and resources for training conversation partners. Aphasiology, 12(9), 816-830, whole article; NASW Code of Ethics (2021), 1.16(a)
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251. ASM-1580 (rev 1) · Best answer: A

On a home visit, the social worker explains how a client can book rides to dialysis through a new transportation service. The client nods throughout and says, 'Yes, I've got it.' Two ride requests the client tried to make last month were never completed. Which response is **MOST** appropriate?

A. Ask the client to show how they would book the next ride
B. Ask the client whether they have any questions about it
C. Leave a written copy of the booking steps with the client

Why each option is right or wrong

A **Correct.** Teach-back checks understanding by having the person explain or demonstrate the information in their own words, because people often say they understand when they do not (AHRQ, Teach-Back). The client's nodding contrasts with two failed bookings, so the worker needs evidence of understanding, not agreement. Teach-back also shows whether the explanation itself was clear. It beats C because a handout may help later, but it cannot reveal which step is confusing the client.

B **Incorrect** (trap: Misuses the technique). Asking for questions is closed and puts the burden on the client to notice and admit confusion. Many people answer no out of embarrassment or because they believe they understand.

C **Incorrect** (trap: Good but not best). Written steps are a useful reminder, but they assume the client can read and follow them, and they do not show whether the client understood. Give them after teach-back confirms understanding.

Takeaway: Check a client's understanding with teach-back, asking them to explain or demonstrate, rather than accepting nods or 'Any questions?'

Sources: Agency for Healthcare Research and Quality (AHRQ), TeamSTEPPS Curriculum, Communication Tools: Teach-Back
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252. ASM-1581 (rev 1) · Best answer: C

A school social worker receives a referral for a 5-year-old who arrived 2 months ago from a country where English is not spoken. At school the child says almost nothing, yet watches classmates closely, points to request items and joins games. The parent reports that the child chats easily at home in the family's first language. Which explanation is **MOST** likely?

A. Selective mutism linked to school anxiety
B. Language disorder affecting expression
C. Normal silent phase of learning a new language
D. Autism spectrum disorder with language delay

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Speaking at home but not at school fits part of the selective mutism picture. However, the diagnosis does not apply when silence comes from not yet knowing the language spoken in that setting, which is the case for a child here only 2 months (DSM-5-TR).

B **Incorrect** (trap: Near-miss diagnosis). A true language disorder appears in every language a child speaks. This child talks easily at home in the first language, which points to a language difference rather than a disorder.

C **Correct.** Many young children learning a second language go through a nonverbal period, listening and communicating with gestures before speaking the new language (Tabors, 2008). This child arrived only 2 months ago, speaks fluently in the home language and uses pointing and play to communicate. Selective mutism (A) is the strongest competitor, but DSM-5-TR rules it out when the silence reflects limited knowledge of, or comfort with, the language used in that setting.

D **Incorrect** (trap: Near-miss diagnosis). Watching peers, pointing to request and joining games show intact social communication. Combined with fluent home-language speech, these behaviors do not fit autism spectrum disorder.

Takeaway: Silence in a new language is usually a normal nonverbal phase; check the home language before suspecting a disorder.

Sources: Tabors, P. O. (2008). One Child, Two Languages: A Guide for Early Childhood Educators of Children Learning English as a Second Language (2nd ed.). Paul H. Brookes, Developmental sequence: nonverbal period; DSM-5-TR (APA, 2022), Anxiety Disorders: Selective Mutism

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253. ASM-1666 (rev 1) · Best answer: B

At a community clinic, a client returns intake forms with most sections blank. When the social worker points to the consent page, the client says they left their reading glasses at home and asks to take everything home to finish. The client missed two earlier appointments that were scheduled by letter. What should the social worker do **FIRST**?

A. Ask the client to read the first paragraph aloud
B. Offer to review the forms together aloud
C. Let the client take the forms home to finish
D. Refer the client to an adult literacy program

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Asking someone to read aloud amounts to an on-the-spot test. It risks shame and may lead the client to avoid the clinic. Universal precautions favor offering help to everyone over testing individuals.

B **Correct.** Health literacy universal precautions call for assuming that anyone may struggle with written information and offering help without testing or embarrassing the person (AHRQ). Blank forms, a forgotten-glasses explanation and missed appointments scheduled by letter are common signs of limited reading skills. Offering to go over the forms together, reading them aloud as a routine courtesy, gets the needed information and protects the client's dignity. It beats D because a literacy referral may help later if the client wants it, but raising it now singles the client out and leaves today's forms and consent unfinished.

C **Incorrect** (trap: Good but not best). Taking the forms home accepts the client's way of saving face, but the same reading barrier will be there. Missed letters suggest the forms may not come back complete, and consent needs to be understood.

D **Incorrect** (trap: Right action, wrong time). An adult literacy program may be a welcome option later, offered privately if the client raises reading. Suggesting it first labels the client and does nothing for today's forms and consent.

Takeaway: When signs suggest limited literacy, offer routine help with written material, such as reviewing it together aloud, rather than testing the client.

Sources: Agency for Healthcare Research and Quality (AHRQ) (2024). Health Literacy Universal Precautions Toolkit (3rd ed.), Introduction: health literacy universal precautions
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254. ASM-0007 (rev 1) · Best answer: B

A 24-year-old client is brought to a community mental health agency by a sibling. For the past 3 months, the client has heard voices commenting on their actions and has spoken in a disorganized way. There is no history of mood episodes, substance use or medical illness. What is the **MOST** likely diagnosis?

A. Brief psychotic disorder
B. Schizophreniform disorder
C. Schizophrenia
D. Schizoaffective disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Brief psychotic disorder lasts at least 1 day and less than 1 month, with an eventual full return to prior functioning. Three months of symptoms is too long for this diagnosis.

B **Correct.** Schizophreniform disorder has the same core symptom picture as schizophrenia, such as hallucinations and disorganized speech, but the episode lasts at least 1 month and less than 6 months (DSM-5-TR). Three months of symptoms with no mood episodes, substances or medical cause fits it exactly. It beats C because schizophrenia requires continuous signs of the disturbance for at least 6 months. If the symptoms persist past 6 months, the diagnosis would be revised.

C **Incorrect** (trap: Near-miss diagnosis). The symptoms fit schizophrenia, but the duration does not. Schizophrenia requires at least 6 months of continuous signs, counting any prodromal or residual periods.

D **Incorrect** (trap: Near-miss diagnosis). Schizoaffective disorder requires a major mood episode alongside the psychotic symptoms. The stem rules out mood episodes, so this diagnosis does not fit.

Takeaway: Psychosis under 1 month is brief psychotic disorder; 1 to under 6 months is schizophreniform; 6 months or more is schizophrenia.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophreniform Disorder

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255. ASM-0113 (rev 3) · Best answer: C

On an inpatient psychiatric unit, a social worker gathers history for a 22-year-old patient. The patient has heard voices and believed neighbors were spying for the past 4 months. A parent reports that for about 5 months before the voices began, the patient withdrew from friends, stopped attending classes and talked about strangers sending hidden signals. There is no substance use, mood episode or medical cause. What is the **MOST** likely diagnosis?

A. Brief psychotic disorder
B. Schizophreniform disorder
C. Schizophrenia
D. Delusional disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Brief psychotic disorder lasts at least 1 day but less than 1 month, with full return to prior functioning. Two months of active psychosis after a long decline rules it out.

B **Incorrect** (trap: Near-miss diagnosis). Schizophreniform disorder covers a total illness of 1 month to under 6 months. It fits only if the 4 active months are counted alone, but the prodromal period counts toward duration, bringing the total to about 9 months.

C **Correct.** Schizophrenia requires continuous signs of the disturbance for at least 6 months, including at least 1 month of active symptoms, and the 6 months can include prodromal symptoms such as social withdrawal and odd, milder beliefs (DSM-5-TR). Here, about 5 months of prodrome plus 4 months of hallucinations and delusions totals about 9 months, with a clear drop in functioning. It beats B because schizophreniform disorder applies only when the whole episode, prodrome included, lasts less than 6 months; counting only the active phase is the trap.

D **Incorrect** (trap: Near-miss diagnosis). Delusional disorder involves delusions for at least 1 month without prominent hallucinations or the broader decline seen here. Hearing voices, withdrawal and dropping out of classes point elsewhere in the schizophrenia spectrum.

Takeaway: Schizophrenia's 6-month duration counts prodromal and residual periods, not just active psychosis; under 6 months total suggests schizophreniform disorder.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophreniform Disorder
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256. ASM-1000 (rev 1) · Best answer: B

A 54-year-old client with no psychiatric history comes to a community mental health agency after 2 weeks of hearing voices and accusing neighbors of stealing. The client's partner reports new morning headaches. The client also complains of a burning-rubber smell in the apartment that no one else notices. What should the social worker do **FIRST**?

A. Obtain a detailed family psychiatric history
B. Arrange a prompt medical and neurological workup
C. Refer the client to a first-episode psychosis program
D. Begin psychoeducation about psychotic disorders

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Family history belongs in a full assessment and can be gathered soon. It does not come first when late onset, new headaches and an unusual smell point to a possible physical cause that needs prompt evaluation.

B **Correct.** Psychotic symptoms are not attributed to a mental disorder until medical causes are ruled out (DSM-5-TR). Several red flags point to a possible medical or neurological cause: a first psychosis after age 50, no psychiatric history, new headaches and a smell no one else notices (an olfactory hallucination). A brain lesion, seizure disorder or other illness must be checked promptly (Kaplan & Sadock). It beats A because family history adds useful detail but does nothing about a possibly dangerous physical cause; it can be gathered while the workup proceeds.

C **Incorrect** (trap: Hands off too soon). Early psychosis programs serve people with a primary psychotic disorder, usually younger adults. Referring now assumes a psychiatric cause before a medical one is ruled out. It may fit later if the workup is negative.

D **Incorrect** (trap: Skips assessment). Teaching about psychotic disorders assumes the diagnosis. With red flags for a medical cause, education about psychosis is premature and could mislead the client and partner.

Takeaway: New psychosis with late onset, headaches or unusual hallucinations (smell, touch, vision) needs medical evaluation before any psychiatric diagnosis.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Psychotic Disorder Due to Another Medical Condition; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Schizophrenia spectrum chapter: differential diagnosis
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257. ASM-1001 (rev 1) · Best answer: A

A 45-year-old client in private practice says their partner of 20 years is having an affair. For 8 months the client has checked the partner's phone, followed the partner to work and read meaning into a moved car seat. No proof has turned up, and the client's certainty has not changed. The client still runs a small business well and speaks clearly. What is the **MOST** likely diagnosis?

A. Delusional disorder
B. Paranoid personality disorder
C. Schizophrenia

Why each option is right or wrong

A **Correct.** Delusional disorder involves one or more fixed false beliefs lasting at least 1 month, without the broader picture of schizophrenia, and with functioning largely intact apart from the belief's effects (DSM-5-TR). The jealous type centers on a partner's supposed unfaithfulness. Eight months of unshakable certainty despite no evidence, with organized speech and a well-run business, fits. It beats B because paranoid personality disorder is a pervasive distrust of many people that starts by early adulthood, not a single fixed belief that began in midlife.

B **Incorrect** (trap: Near-miss diagnosis). Paranoid personality disorder is an enduring pattern of suspicion toward many people, traceable to adolescence or early adulthood. This belief began 8 months ago and focuses on one person, which points to a delusion rather than a personality pattern.

C **Incorrect** (trap: Near-miss diagnosis). Schizophrenia requires two or more core symptoms, such as hallucinations or disorganized speech, with a decline in functioning and signs lasting at least 6 months. A single belief with clear speech and preserved work does not fit that picture.

Takeaway: One fixed false belief for at least a month, with organized thinking and otherwise intact functioning, suggests delusional disorder.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Delusional Disorder; DSM-5-TR (APA, 2022), Personality Disorders: Paranoid Personality Disorder
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258. ASM-1002 (rev 1) · Best answer: D

During a telehealth session, a 31-year-old client describes 5 days last month of sleeping 3 hours a night without feeling tired. The client talked rapidly, started several work projects and felt 'brilliant.' A coworker commented on the change, but the client kept working and met deadlines. Last year the client was treated for a major depressive episode. What is the **MOST** likely diagnosis?

A. Bipolar I disorder
B. Cyclothymic disorder
C. Major depressive disorder
D. Bipolar II disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Bipolar I requires a manic episode, which causes marked impairment, leads to hospitalization or includes psychosis. A 5-day period in which the client kept working and met deadlines describes hypomania, not mania.

B **Incorrect** (trap: Near-miss diagnosis). Cyclothymic disorder involves at least 2 years of numerous milder highs and lows that never meet full episode criteria. This client had a full hypomanic episode and a major depressive episode, so cyclothymia does not fit.

C **Incorrect** (trap: Near-miss diagnosis). Major depressive disorder applies only when there has never been a manic or hypomanic episode. The recent 5-day high period changes the diagnosis and matters for treatment planning.

D **Correct.** Bipolar II disorder requires at least one hypomanic episode and at least one major depressive episode, with no lifetime manic episode (DSM-5-TR). Five days of reduced need for sleep, rapid speech, increased activity and inflated self-view, noticed by others but without marked impairment, meets hypomania. The past major depressive episode completes the picture. It beats A because bipolar I requires a manic episode: at least 1 week (or any length if hospitalized) with marked impairment, hospitalization or psychosis. This client kept working and met deadlines.

Takeaway: Hypomania (4+ days, noticed by others, no marked impairment) plus a major depressive episode is bipolar II; any manic episode makes it bipolar I.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar II Disorder
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259. ASM-1003 (rev 3) · Best answer: C

At a college counseling center, a 20-year-old client seeks help for 6 weeks of depression. The client mentions that last spring they barely slept for 5 days, felt unstoppable and started several ventures, and roommates said they seemed like a different person. The campus prescriber plans to start an antidepressant at an appointment this afternoon. What should the social worker do **FIRST**?

A. Explore how the energetic period affected the client's friendships
B. Advise the client to delay the prescriber's antidepressant plan
C. Tell the prescriber about the earlier high-energy period

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Learning more about the episode's effects is good assessment and should happen. It is not the first step when a medication decision is hours away and the prescriber does not yet know about a possible hypomanic episode.

B **Incorrect** (trap: Outside the social work role). Advising a client whether to take a medication is outside the social work role. Medication decisions belong to the prescriber, who needs the new history to make them well.

C **Correct.** A bipolar-spectrum illness often first appears as depression, so past hypomania must be identified before an antidepressant is started; treatment with an

antidepressant alone can trigger a switch into mania (DSM-5-TR). Five days of little sleep, high energy and grandiosity, noticed by others, suggests a hypomanic episode. The prescriber makes the medication decision, so the social worker shares the finding before this afternoon's visit, as the Code expects of members of an interdisciplinary team (NASW Code 2.03(a)). It beats A because exploring the episode further is useful but can follow; the time-sensitive step is getting the history to the prescriber.

Takeaway: Possible past hypomania in a depressed client is a bipolar-spectrum warning sign; get that history to the prescriber before an antidepressant starts.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar II Disorder; NASW Code of Ethics (2021), 2.03(a)
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260. ASM-1004 (rev 1) · Best answer: B

In a hospice bereavement program, a client meets with the social worker 18 months after an adult child died. Nearly every day the client longs intensely for the child and says, 'Part of me died too.' The client avoids the child's neighborhood, has stopped seeing friends and feels life has no purpose. The distress far exceeds what the client's faith community expects. What is the **MOST** likely diagnosis?

A. Major depressive disorder
B. Prolonged grief disorder
C. Uncomplicated bereavement

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Major depressive disorder centers on pervasive low mood or loss of interest, with symptoms such as worthlessness and sleep or appetite change. The features here focus on longing for the child and loss of identity, which fit a loss-specific diagnosis better.

B **Correct.** Prolonged grief disorder, added in DSM-5-TR, applies in adults when at least 12 months have passed since the death and, for at least the past month, intense yearning or preoccupation occurs nearly daily with at least three further features, such as identity disruption, avoidance of reminders, trouble re-engaging and a sense of meaninglessness (DSM-5-TR). This client shows all of these, beyond cultural norms. It beats A because the distress centers on the lost child rather than a pervasive low mood with worthlessness; major depression can co-occur but is not the best fit here.

C **Incorrect** (trap: Near-miss diagnosis). Uncomplicated bereavement is an expected response to a death, recorded as a condition for clinical attention rather than a disorder. Daily intense longing 18 months later, with impairment beyond cultural norms, goes beyond it.

Takeaway: Adult yearning for the deceased nearly daily, 12 or more months after a death, with impairment beyond cultural norms, suggests prolonged grief disorder.

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder
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261. ASM-1005 (rev 2) · Best answer: A

On an inpatient unit, a social worker reviews the 4-year history of a 33-year-old patient. The patient has had four manic episodes, each with grandiose delusions and voices, and these episodes have filled most of the illness. Twice, after the mood returned to normal, the patient kept hearing voices that criticized them for about 5 weeks. What is the **MOST** likely diagnosis?

A. Schizoaffective disorder, bipolar type
B. Bipolar I disorder with psychotic features
C. Schizophrenia
D. Delusional disorder

Why each option is right or wrong

A **Correct.** Schizoaffective disorder requires a major mood episode occurring along with schizophrenia's core symptoms (here, delusions and hallucinations), at least 2 weeks of hallucinations or delusions with no mood episode, and mood episodes present for most of the illness (DSM-5-TR). Manic episodes with grandiose delusions and voices, voices lasting about 5 weeks after the mood normalized, and an illness dominated by manic episodes meet all three. The manic episodes make it the bipolar type. It beats B because in bipolar I with psychotic features, psychosis occurs only during mood episodes; here it continued for weeks after the mood returned to normal.

B **Incorrect** (trap: Near-miss diagnosis). Bipolar I with psychotic features fits the manic episodes, but its psychosis happens only during mood episodes. Voices that continue for weeks while mood is normal point to schizoaffective disorder instead.

C **Incorrect** (trap: Near-miss diagnosis). Schizophrenia fits an illness dominated by psychosis, with mood episodes present only briefly. Here manic episodes filled most of the illness, so schizophrenia is not the best fit.

D **Incorrect** (trap: Near-miss diagnosis). Delusional disorder involves fixed beliefs without prominent hallucinations or major mood episodes dominating the course. Repeated mania and critical voices do not fit it.

Takeaway: Psychosis for 2+ weeks without a mood episode, in an illness dominated by mood episodes, points to schizoaffective disorder rather than a mood disorder with psychotic features.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizoaffective Disorder; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder
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262. ASM-1006 (rev 2) · Best answer: C

A patient is admitted to an inpatient psychiatric unit during a first manic episode. On the unit, the patient demands special privileges, insults staff and boasts of being destined for fame. A psychiatry resident suggests adding narcissistic personality disorder to the discharge diagnoses. What is the **BEST** response by the social worker?

A. Support adding it as a provisional diagnosis for follow-up
B. Administer a personality inventory before discharge to confirm it
C. Recommend deferring personality diagnosis until mood stabilizes

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A provisional label sounds cautious, but it still records a personality disorder based only on behavior seen during a manic episode, when grandiosity and entitlement are expected symptoms. Even provisional diagnoses travel with the record and can bias later care. A personality diagnosis can be considered once the mood is stable.

B **Incorrect** (trap: Right action, wrong time). Standardized personality testing can help, but scores taken during acute mania reflect the episode rather than stable traits. Testing belongs after the mood has stabilized.

C **Correct.** A personality disorder is an enduring pattern that begins by early adulthood and is not better explained by another mental disorder (DSM-5-TR). Grandiosity, entitlement and irritability are core features of mania, so traits seen only during a first manic episode cannot show the person's long-term pattern. Personality is best assessed after the mood episode resolves, with history from the patient and others. It beats B because an inventory given during mania would measure the episode, not stable traits; it may be useful later.

Takeaway: Do not diagnose a personality disorder from behavior seen only during an acute mood or psychotic episode; reassess after it resolves.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: General Personality Disorder
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263. ASM-1007 (rev 2) · Best answer: A

At an integrated primary care clinic, a 42-year-old client reports feeling low more days than not for the past 4 years, with little energy and a sense of being a failure. The client has never felt well for more than a few weeks at a time and says, 'This is just who I am.' The low mood began during a divorce that was finalized 4 years ago. The client works full time and cares for two children. What is the **MOST** likely diagnosis?

A. Persistent depressive disorder
B. Major depressive disorder
C. Adjustment disorder with depressed mood

Why each option is right or wrong

A **Correct.** Persistent depressive disorder involves depressed mood most of the day, more days than not, for at least 2 years in adults, with at least two other symptoms such as low energy or low self-esteem, and no symptom-free stretch longer than 2 months (DSM-5-TR). Four years of low mood with low energy and feeling like a failure, never well for more than a few weeks, fits. It beats C because adjustment disorder is not diagnosed when another disorder's criteria are met, and it resolves within 6 months after the stressor and its consequences end; the divorce was finalized 4 years ago.

B **Incorrect** (trap: Near-miss diagnosis). Major depressive disorder needs at least five symptoms nearly every day for 2 weeks. The client describes a chronic, lower-grade pattern with a few symptoms that still allows full-time work and parenting.

C **Incorrect** (trap: Near-miss diagnosis). Adjustment disorder starts within 3 months of a stressor, ends within 6 months after the stressor or its effects end, and is not used when another disorder's criteria are met. Four years of low mood that meets persistent depressive disorder criteria rules it out.

Takeaway: Chronic low mood more days than not for 2+ years in adults, with only brief symptom-free periods, suggests persistent depressive disorder.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Persistent Depressive Disorder; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Adjustment Disorders

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264. ASM-1009 (rev 2) · Best answer: B

A 76-year-old client is evaluated at a memory clinic. Over the past year, the client's alertness has varied widely from day to day, and the client often sees small children in the living room who are not there. Months after the memory changes began, the client developed a shuffling walk and stiff arms. The client's partner says the client now needs help with bills and medications. What is the **MOST** likely diagnosis?

A. Major neurocognitive disorder due to Alzheimer's disease

B. Major neurocognitive disorder with Lewy bodies

C. Delirium

D. Major neurocognitive disorder due to Parkinson's disease

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Alzheimer's disease typically starts with gradual memory loss and steady decline. Marked day-to-day swings in alertness, early well-formed visual hallucinations and later parkinsonism point to Lewy bodies instead.

B **Correct.** Neurocognitive disorder with Lewy bodies is marked by fluctuating cognition with changes in attention and alertness, recurrent well-formed visual hallucinations, and parkinsonism that begins after the cognitive decline (DSM-5-TR). This client shows all three, and needing help with bills and medications makes it major. It beats D because neurocognitive disorder due to Parkinson's disease arises in the setting of already established Parkinson's disease; here the shuffling walk and stiffness began

months after the memory changes. Recognition matters, because people with this condition can react severely to antipsychotic medication.

C **Incorrect** (trap: Near-miss diagnosis). Delirium fluctuates and can cause hallucinations, which makes it tempting. It begins over hours to days and has a medical cause, while this decline has built gradually over a year.

D **Incorrect** (trap: Near-miss diagnosis). This diagnosis also combines cognitive decline, parkinsonism and sometimes hallucinations, which makes it a close neighbor. It applies when cognitive decline develops in someone with established Parkinson's disease. Here the movement signs came months after the memory problems, the sequence that points to Lewy bodies (DSM-5-TR).

Takeaway: Fluctuating alertness, recurrent detailed visual hallucinations and later parkinsonism in a gradual decline suggest neurocognitive disorder with Lewy bodies.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Neurocognitive Disorder With Lewy Bodies

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265. ASM-1010 (rev 1) · Best answer: A

A client is referred to a community agency after an emergency department visit labeled 'panic attack.' The social worker has taken a history: during a heated family argument, the client screamed, shook, felt heat rising to the head and fell to the floor. The client calls it an ataque de nervios and says such episodes run in the family after major conflicts. What should the social worker do **NEXT**?

A. Use the Cultural Formulation Interview to learn the client's view

B. Record panic disorder and refer for exposure-based treatment

C. Transfer the client to a practitioner from the client's culture

Why each option is right or wrong

A **Correct.** DSM-5-TR asks clinicians to understand a client's cultural concept of distress before assigning a diagnosis. Ataque de nervios is a recognized cultural syndrome, often set off by family conflict, that can resemble panic but differs from panic disorder, whose attacks are unexpected and followed by worry about more attacks. The Cultural Formulation Interview gathers the client's own explanation, supports and help-seeking preferences (DSM-5-TR). It beats B because labeling the episode panic disorder skips its cultural meaning and ignores that this episode was triggered, not unexpected.

B **Incorrect** (trap: Near-miss diagnosis). Panic disorder requires recurrent unexpected attacks and at least a month of worry or avoidance. This episode followed a family argument and has a cultural meaning the social worker has not explored, so the diagnosis is premature.

C **Incorrect** (trap: Hands off too soon). Transferring assumes only a same-culture practitioner can help and abandons the assessment. Any social worker can use the Cultural Formulation Interview; a transfer fits only if the client asks for one.

Takeaway: When a client describes distress in cultural terms, use the Cultural Formulation Interview to understand its meaning before applying a DSM diagnosis.

Sources: DSM-5-TR (APA, 2022), Culture and Psychiatric Diagnosis: Cultural Formulation Interview; DSM-5-TR (APA, 2022), Culture and Psychiatric Diagnosis: Cultural Concepts of Distress
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266. ASM-1011 (rev 1) · Best answer: B

An assertive community treatment team visits a client with schizophrenia who has stopped taking medication. The client calmly explains that the voices are real broadcasts from neighbors and that nothing is wrong with their mind. When the team reviews two past hospitalizations, the client agrees they happened but still sees no illness. Which concept **MOST** likely explains the client's view?

A. Denial
B. Anosognosia
C. Avolition

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Denial is a defense that keeps painful facts out of awareness, often with anxiety or avoidance. This client calmly reviews the evidence yet cannot see the illness, which points to a symptom of schizophrenia rather than a defense.

B **Correct.** Anosognosia is an illness-related inability to recognize one's own disorder. DSM-5-TR notes that unawareness of illness in schizophrenia is typically a symptom of the disorder itself rather than a coping strategy, comparable to unawareness of deficits after brain injury, and that it predicts nonadherence and relapse. The client's calm certainty, unchanged by clear evidence, fits. It beats A because denial guards against painful emotion and usually shows distress or avoidance; this client discusses the hospitalizations openly and simply does not register them as illness.

C **Incorrect** (trap: Near-miss diagnosis). Avolition, a negative symptom, is reduced drive to start and sustain activities. It can contribute to stopping medication, but it does not explain the client's firm belief that nothing is wrong.

Takeaway: In schizophrenia, unawareness of illness is often anosognosia, a symptom of the disorder rather than a defense, and it predicts nonadherence.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia (Associated Features)
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267. ASM-1012 (rev 2) · Best answer: C

A social worker in private practice completes an intake with a 38-year-old client who reports 2 months of low mood, fatigue and poor concentration. The client also mentions a 15-pound weight gain, feeling cold all the time and thinning hair. The client has not had a physical exam in 5 years. What should the social worker do **NEXT**?

A. Begin cognitive behavioral therapy for the depression
B. Administer a standardized depression scale to rate severity
C. Refer the client to a primary care provider for a medical evaluation

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Cognitive behavioral therapy may well help later. Starting treatment for depression before checking for a medical cause risks missing a treatable illness that could explain the mood symptoms.

B **Incorrect** (trap: Good but not best). A depression scale measures how severe the symptoms are, but it cannot show whether a physical illness is causing them. The physical signs call for a medical exam first.

C **Correct.** Before a depressive disorder is diagnosed, the social worker considers whether another medical condition could explain the symptoms (DSM-5-TR). Weight gain, constant cold intolerance and thinning hair alongside low mood and fatigue are signs that call for medical evaluation, for example of thyroid function, and the client has had no exam in 5 years. The social worker does not diagnose the medical condition but routes the client to a physician (Kaplan & Sadock). It beats B because a severity score adds detail but cannot tell a medical cause from a primary depression.

Takeaway: Depressive symptoms with physical signs and no recent medical exam call for medical evaluation before a depressive diagnosis or treatment.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Depressive Disorder Due to Another Medical Condition; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Depressive disorders chapter: differential diagnosis (medical conditions)
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268. ASM-1013 (rev 2) · Best answer: A

Which term describes the combination of mutism, holding odd postures for long periods, resisting being moved and repeating other people's words?

A. Catatonia
B. Acute dystonia
C. Negative symptoms

Why each option is right or wrong

A **Correct.** Catatonia is identified when at least 3 of 12 motor and behavioral features are present, including mutism, posturing, negativism (resisting instructions or movement) and echolalia (DSM-5-TR). The description includes all four. Catatonia can occur with psychotic, mood or medical conditions and can become life-threatening, so it calls for prompt medical attention. It beats C because negative symptoms such as reduced speech do not include holding odd postures, actively resisting movement or echoing others' words.

B **Incorrect** (trap: Near-miss diagnosis). Acute dystonia is a sudden, often painful muscle spasm, usually of the neck, jaw or eyes, soon after starting or raising an antipsychotic. It does not explain mutism, resistance to repositioning or echoed speech.

C **Incorrect** (trap: Near-miss diagnosis). Negative symptoms include reduced speech and expression, which can look like mutism. They do not include held postures, active resistance to being moved or repeating another person's words.

Takeaway: Mutism, posturing, negativism and echolalia together signal catatonia, a potentially dangerous syndrome that needs prompt medical evaluation.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Catatonia; DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Medication-Induced Acute Dystonia

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269. ASM-1014 (rev 2) · Best answer: B

In a telehealth intake, a 34-year-old client asks for an ADHD diagnosis to get medication. The client describes poor focus and missed details at work that began 18 months ago, soon after starting rotating night shifts. The client earned good grades in school. Which action is **BEST**?

A. Administer an adult ADHD self-report rating scale

B. Assess childhood symptoms and sleep disruption

C. Tell the client the timeline rules out ADHD

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A rating scale can add useful data, but it screens current symptoms and cannot confirm onset before age 12 or rule out sleep loss. It supplements the history rather than replacing it.

B **Correct.** DSM-5-TR requires several ADHD symptoms before age 12, present in two or more settings, and not better explained by another condition. Problems that began with rotating night shifts raise sleep disruption as a possible cause, and the client's childhood history has not been explored; good grades alone do not settle it. Assessing both tests the request directly against the criteria. It beats A because a self-report scale measures current symptoms but cannot establish childhood onset or separate ADHD from sleep loss.

C **Incorrect** (trap: Skips assessment). Many adults are diagnosed with ADHD whose childhood symptoms went unrecognized, sometimes despite good grades. Ruling it out before taking a childhood history is premature.

Takeaway: ADHD requires several symptoms before age 12; with adult-onset problems, assess childhood history and other causes such as sleep loss before diagnosing.

Sources: DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder
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270. ASM-1015 (rev 3) · Best answer: D

A hospital social worker meets with a 40-year-old patient admitted with sudden weakness in the left leg after a car accident that caused no injury. The medical team's exam findings do not match any known nerve or brain disease, and imaging is normal. The patient is upset about missing work. What is the **MOST** likely diagnosis?

A. Malingering
B. Factitious disorder imposed on self
C. Somatic symptom disorder
D. Functional neurological symptom disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Malingering is intentional faking for an external reward, such as avoiding work or gaining money. Nothing here suggests deception, and the patient wants to get back to work.

B **Incorrect** (trap: Near-miss diagnosis). Factitious disorder involves deliberately falsifying or causing illness to be seen as sick, without an outside reward. The facts give no sign of deception, which this diagnosis requires.

C **Incorrect** (trap: Near-miss diagnosis). Somatic symptom disorder is a close neighbor in the same chapter, but it centers on excessive, persistent thoughts, feelings or behaviors about bodily symptoms, such as constant worry or repeated care-seeking. This patient shows only proportionate concern about missing work. The defining finding is a motor deficit incompatible with known neurological disease.

D **Correct.** Functional neurological symptom disorder (conversion disorder) involves altered movement or sensation, with clinical findings showing the problem is incompatible with recognized neurological or medical disease (DSM-5-TR). Sudden leg weakness with exam findings that fit no nerve or brain disease and normal imaging fits. A stressor such as the accident is common but not required. It beats A because malingering involves intentionally producing symptoms for an outside gain; nothing suggests deliberate production, and the patient is eager to return to work.

Takeaway: Motor or sensory loss inconsistent with known disease, without intentional production, suggests functional neurological symptom disorder.

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Functional Neurological Symptom Disorder (Conversion Disorder)
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271. ASM-1016 (rev 1) · Best answer: C

In the fourth session at a rural agency, a client reports losing hours at a time, finding receipts for purchases the client does not remember and being greeted by strangers using another name. The social worker has no training in dissociative disorders, and no one at the agency does. The clinical director can connect the worker with a regional expert in these conditions. What is the **MOST** appropriate action?

A. Record dissociative amnesia based on the reported gaps
B. Ask the client to describe early trauma in detail
C. Consult a specialist about how to assess the client

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Assigning a dissociative diagnosis without training risks a wrong label. Being called by another name suggests more than amnesia, and the differential needs a clinician competent in dissociative disorders.

B **Incorrect** (trap: Misuses the technique). Trauma history matters, but pressing for detailed early memories before safety and competence are in place can destabilize a client with dissociative symptoms. A trained assessor paces this carefully.

C **Correct.** Social workers practice within their competence and, in an area new to them, seek consultation or supervision from someone competent in it (NASW Code 1.04(b); 2.05(a)). Lost time, unremembered purchases and being called by another name suggest a possible dissociative disorder, such as dissociative identity disorder (DSM-5-TR), a differential the worker is not trained to make, with no in-house expertise. Consulting the regional expert supports an accurate assessment and a referral if needed. It beats B because detailed trauma probing by an untrained worker can overwhelm the client and still would not settle the diagnosis.

Takeaway: When a presentation falls outside your training and no in-house expertise exists, consult a competent specialist before diagnosing.

Sources: NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 2.05(a); DSM-5-TR (APA, 2022), Dissociative Disorders: Dissociative Identity Disorder
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272. ASM-1017 (rev 2) · Best answer: B

A client is brought to an emergency department with hallucinations and disorganized speech. The client cannot give a history, no records or collateral contacts are available, and the duration of symptoms is unknown. Which diagnosis does DSM-5-TR provide for this situation?

A. Other specified schizophrenia spectrum and other psychotic disorder
B. Unspecified schizophrenia spectrum and other psychotic disorder
C. Schizophreniform disorder, provisional

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Other specified is used when the clinician knows why criteria for a named disorder are not met and records that reason, such as persistent hallucinations with no other features. An emergency team with almost no history cannot state any such reason.

B **Correct.** DSM-5-TR uses an unspecified diagnosis when symptoms of a disorder class cause distress or impairment but there is not enough information for a more specific diagnosis, and it names emergency settings as an example. It beats A because other specified requires the clinician to state the specific reason the presentation does not meet a named disorder, which demands information the team lacks. The diagnosis is revised as history, duration and possible substance or medical causes become known (DSM-5-TR).

C **Incorrect** (trap: Near-miss diagnosis). Schizophreniform disorder requires symptoms for at least 1 month, and the provisional label applies while recovery within 6 months is still uncertain. With no history, even the 1-month minimum is unknown.

Takeaway: Use unspecified when information is too limited for a specific diagnosis; use other specified when you can state why criteria are not met.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Unspecified Schizophrenia Spectrum and Other Psychotic Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Other Specified Schizophrenia Spectrum and Other Psychotic Disorder
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273. ASM-1018 (rev 2) · Best answer: A

A probation officer refers a 27-year-old client with a letter listing bipolar disorder from a jail intake screening. The client describes days of feeling invincible with almost no sleep, but only during cocaine binges, followed by several days of crashing. The client has had negative drug tests for the past 6 weeks of court-ordered treatment. What is the social worker's **MOST** appropriate next step?

A. Assess the client's mood since substance use stopped
B. Accept the listed diagnosis and refer for mood stabilizers
C. Request the jail's full screening record before going further

Why each option is right or wrong

A **Correct.** A mood episode does not count toward bipolar disorder when it is attributable to a substance; DSM-5-TR points to an independent disorder when symptoms began before use or persist for a substantial period, such as about a month, after use stops. Highs only during cocaine binges, followed by crashes, fit stimulant intoxication and withdrawal. Six weeks of negative tests give a window to see how mood behaves without cocaine. It beats C because the jail screening is the source of the questionable label; the decisive information is the timing of symptoms relative to use.

B **Incorrect** (trap: Skips assessment). Accepting a screening label without assessment repeats a possible error. Mood symptoms tied only to cocaine use suggest a substance-induced picture, and medication decisions belong to a prescriber after a sound diagnosis.

C **Incorrect** (trap: Good but not best). Records can add context and may help later, with the client's written consent. They do not answer the key question, which is whether mood episodes occur apart from cocaine use.

Takeaway: Before accepting a mood diagnosis, check whether episodes occur only during substance use or withdrawal; substance-induced episodes do not count toward bipolar disorder.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Substance/Medication-Induced Bipolar and Related Disorder
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274. ASM-1019 (rev 1) · Best answer: C

A 26-year-old client at a community agency describes mood swings that shift within hours, often after a partner is slow to answer texts. During these times the client feels intense rage, cuts their arms and fears being left. Between swings the client describes a constant inner emptiness. This pattern has continued since the client's late teens. What is the **MOST** likely diagnosis?

A. Bipolar II disorder
B. Cyclothymic disorder
C. Borderline personality disorder
D. Histrionic personality disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Bipolar II needs distinct hypomanic episodes of at least 4 days plus major depressive episodes. Shifts lasting hours, set off by relationship events, describe affective instability rather than mood episodes.

B **Incorrect** (trap: Near-miss diagnosis). Cyclothymic disorder involves 2 or more years of many milder highs and lows lasting days. It does not explain fear of abandonment, self-harm or chronic emptiness, and mood here changes within hours.

C **Correct.** Borderline personality disorder is a pervasive pattern, beginning by early adulthood, of unstable relationships, self-image and mood, with frantic efforts to avoid abandonment, self-harm, intense anger and chronic emptiness (DSM-5-TR). Mood

shifts lasting hours and set off by a partner's slow replies fit its reactive mood instability. It beats A because bipolar II requires a hypomanic episode lasting at least 4 days; mood changes that come and go within hours in response to relationship events are not mood episodes.

D **Incorrect** (trap: Near-miss diagnosis). Histrionic personality disorder centers on attention seeking and shallow, dramatic emotion. Fear of abandonment, self-harm, rage and chronic emptiness point to borderline personality disorder.

Takeaway: Mood shifts lasting hours, triggered by relationships, with abandonment fears and emptiness suggest borderline personality disorder rather than a bipolar disorder.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Borderline Personality Disorder; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar II Disorder
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275. ASM-1020 (rev 2) · Best answer: B

A school social worker completes an assessment of a 15-year-old student who identifies as nonbinary and uses they/them pronouns. The student has close friends, steady grades and says they feel comfortable with who they are. The student's parent asks the social worker for a diagnosis 'so we know what is wrong.' What is the **MOST** appropriate response?

A. Diagnose gender dysphoria based on the gender incongruence
B. Explain that a diagnosis requires distress or impairment
C. Refer the student to a gender clinic for a diagnosis

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Incongruence between experienced and assigned gender is only part of the picture. Gender dysphoria also requires clinically significant distress or impairment, which this assessment did not find, so the diagnosis would mislabel a student who is doing well.

B **Correct.** Gender dysphoria requires a marked incongruence between experienced and assigned gender lasting at least 6 months, with clinically significant distress or impairment (DSM-5-TR). DSM-5-TR notes that gender nonconformity is not in itself a mental disorder. This student has friends, steady grades and comfort with their identity, so no diagnosis applies, and explaining this to the parent is accurate and respects the student. It beats C because referring for diagnosis implies a disorder the assessment did not find; a gender clinic referral fits if the student wants support or later reports distress.

C **Incorrect** (trap: Hands off too soon). Referring for diagnosis implies the student has a disorder the assessment did not find. Specialized gender services fit if the student wants them or later reports distress.

Takeaway: Gender diversity is not a disorder; gender dysphoria requires clinically significant distress or impairment.

Sources: DSM-5-TR (APA, 2022), Gender Dysphoria: Gender Dysphoria
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276. ASM-1021 (rev 1) · Best answer: A

A child welfare social worker reviews the case of a 9-year-old in kinship foster care. Since age 7, the child has been angry and irritable most of every day. Several times a week, the child has severe outbursts at home and at school, such as screaming and throwing chairs, far beyond what peers do. What is the **MOST** likely diagnosis?

A. Disruptive mood dysregulation disorder
B. Oppositional defiant disorder
C. Bipolar I disorder with rapid cycling
D. Intermittent explosive disorder

Why each option is right or wrong

A **Correct.** Disruptive mood dysregulation disorder involves severe, recurrent temper outbursts, about three or more a week, with irritable or angry mood most of the day between them, lasting at least 12 months in two or more settings, with onset before age 10 (DSM-5-TR). Two years of daily irritability and frequent outbursts at home and school fit. It beats B because when a child meets criteria for both, DSM-5-TR directs that only disruptive mood dysregulation disorder be diagnosed; the severe outbursts and constant irritability go beyond defiance.

B **Incorrect** (trap: Near-miss diagnosis). Oppositional defiant disorder involves angry mood and defiance, but when severe outbursts and constant irritability meet criteria for disruptive mood dysregulation disorder, that diagnosis is made instead.

C **Incorrect** (trap: Near-miss diagnosis). Bipolar I, even with a rapid-cycling pattern, requires distinct manic episodes. Chronic, nonepisodic irritability in a child is the pattern disruptive mood dysregulation disorder was created to describe, partly to curb overdiagnosis of pediatric bipolar disorder.

D **Incorrect** (trap: Near-miss diagnosis). Intermittent explosive disorder involves impulsive outbursts with normal mood between them. This child stays irritable most of every day, which points to disruptive mood dysregulation disorder.

Takeaway: Frequent severe outbursts plus irritable mood most days between them, for 12+ months starting before age 10, suggests disruptive mood dysregulation disorder.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Disruptive Mood Dysregulation Disorder
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277. ASM-1022 (rev 1) · Best answer: C

A social worker at an assisted living facility meets with a 78-year-old resident who has no psychiatric history. Over 6 weeks the resident has stopped eating, sleeps poorly and says, 'I am worthless.' The resident now insists their organs are rotting as punishment for old sins. Staff report that this belief appeared only after the low mood began. What is the **MOST** likely diagnosis?

A. Delusional disorder, somatic type
B. Schizoaffective disorder, depressive type
C. Major depressive disorder with psychotic features
D. Major neurocognitive disorder due to Alzheimer's disease

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Delusional disorder involves fixed beliefs without major mood episodes dominating the picture. The rotting-organs belief arose within a depressive episode with worthlessness and sleep and appetite loss, which points to a mood disorder.

B **Incorrect** (trap: Near-miss diagnosis). Schizoaffective disorder requires at least 2 weeks of delusions or hallucinations with no mood episode. This belief appeared only after the low mood began, so the psychosis is tied to the depression.

C **Correct.** Major depressive disorder with psychotic features applies when delusions or hallucinations occur only during a major depressive episode (DSM-5-TR). Six weeks of poor appetite and sleep and feelings of worthlessness, followed by a belief that the body is rotting as punishment, fit a mood-congruent delusion. Refusing food makes this presentation serious and in need of prompt treatment. It beats B because schizoaffective disorder requires at least 2 weeks of psychosis without a mood episode, and here the belief appeared only after the low mood began.

D **Incorrect** (trap: Near-miss diagnosis). Alzheimer's disease develops gradually with memory and thinking decline. A 6-week change dominated by low mood, worthlessness and a guilt-themed belief points to depression with psychotic features.

Takeaway: Delusions occurring only during a major depressive episode, often with guilt or decay themes, indicate major depressive disorder with psychotic features.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizoaffective Disorder
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278. ASM-1025 (rev 1) · Best answer: B

A 34-year-old client spends about 3 hours a day washing after touching doorknobs or mail. The client is completely sure that skipping the routine will give their children a fatal illness and cannot be talked out of it. Work and family life have narrowed around the washing. Outside of this topic, the client's conversation is organized and goal-directed. Which diagnosis is **MOST** likely?

A. Delusional disorder
B. Obsessive-compulsive disorder
C. Schizophrenia
D. Illness anxiety disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Delusional disorder involves a fixed false belief lasting at least 1 month without the full picture of schizophrenia. When the belief is an obsessional fear that drives compulsions, DSM-5-TR classifies it as OCD with absent insight instead.

B **Correct.** OCD is diagnosed even when the client fully believes the obsessional fear; DSM-5-TR adds the specifier 'with absent insight/delusional beliefs' instead of changing the diagnosis. Contamination fears that drive hours of washing to prevent a dreaded event are the core obsession and compulsion pattern (DSM-5-TR). It beats A because delusional disorder is not diagnosed when the false belief is better explained by OCD: this conviction drives hours of compulsive washing rather than standing alone as a fixed belief.

C **Incorrect** (trap: Near-miss diagnosis). Schizophrenia requires at least two core psychotic symptoms, such as hallucinations or disorganized speech, with signs lasting 6 months. One fixed belief tied to rituals, with organized conversation on other topics, does not fit that picture.

D **Incorrect** (trap: Near-miss diagnosis). Illness anxiety disorder is preoccupation with having or getting a serious illness oneself, usually without hours of rituals. This client fears harming the children through contamination, and the washing is compulsive.

Takeaway: Full conviction about an obsessional fear does not make it psychosis; OCD is diagnosed with the absent-insight specifier.

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Obsessive-Compulsive Disorder **Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

279. ASM-1026 (rev 1) · Best answer: D

At an overnight shelter, staff ask the social worker to see a resident with years of heavy drinking who has not had alcohol for 2 days. Since this morning, the resident has been confused about where they are, walks with a wide, staggering gait and has jerky eye movements. Staff think the resident just needs to sleep it off. What should the social worker do **FIRST**?

A. Complete a brief cognitive screen such as the MoCA
B. Arrange admission to a withdrawal management program
C. Let the resident rest and check on them hourly
D. Send the resident to the hospital right away

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A cognitive screen adds little when sudden confusion and new neurological signs already call for medical care. Screening can follow once the resident is medically stable.

B **Incorrect** (trap: Good but not best). Withdrawal management is a sound plan for heavy drinking, but these new neurological signs need hospital evaluation and treatment first. Many programs cannot treat a possible thiamine-deficiency emergency.

C **Incorrect** (trap: Ignores risk). Assuming intoxication delays care for a possible brain emergency. The resident has not been drinking for 2 days, so sleeping it off does not explain new confusion, gait problems and eye movements.

D **Correct.** Confusion, an unsteady wide-based gait and abnormal eye movements in a person with heavy alcohol use are the classic signs of Wernicke encephalopathy, a thiamine-deficiency brain emergency that can leave lasting memory damage (Korsakoff syndrome) without prompt treatment (Kaplan & Sadock). Alcohol withdrawal around 48 hours can also cause dangerous confusion (DSM-5-TR). Either way, safety comes first and the resident needs medical care now. It beats B because a withdrawal program may not give the immediate hospital treatment these signs require.

Takeaway: Sudden confusion, unsteady gait or abnormal eye movements in heavy alcohol use signal a medical emergency; get medical care before screening or referral.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Alcohol-related disorders: Wernicke encephalopathy and Korsakoff syndrome; DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Alcohol Withdrawal

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280. ASM-1027 (rev 1) · Best answer: A

In private practice, a client seeks help after receiving an eviction notice. The client feels worried and sad, keeps working full time, sleeps well and is actively searching for housing. The client says the worry 'makes sense given the notice.' After a full assessment, the social worker must record the findings, and the client's insurance requires a diagnosis for payment. How should the social worker record the assessment?

A. Record a Z-code condition and no mental disorder
B. Record adjustment disorder because the stressor is identifiable
C. Record unspecified anxiety disorder pending further sessions

Why each option is right or wrong

A **Correct.** A diagnosis must reflect the assessment, and records must be accurate (NASW Code 3.04(a), 4.04). DSM-5-TR lists housing problems among other conditions that may be a focus of clinical attention (Z codes), which name the clinical focus without labeling a mental disorder. The client works, sleeps well and sees the worry as fitting the eviction, so no disorder's criteria are met. It beats B because adjustment disorder needs distress out of proportion to the stressor or real impairment, not just an identifiable stressor; using it to secure payment would misstate the record.

B **Incorrect** (trap: Near-miss diagnosis). An identifiable stressor is necessary but not enough. Adjustment disorder also requires distress out of proportion to the stressor or significant impairment, so recording it here would be inaccurate, whatever the payment pressure.

C **Incorrect** (trap: Misreads law or ethics). Unspecified anxiety disorder still requires anxiety symptoms that cause clinically significant distress or impairment. It is not a placeholder for proportionate worry, and using it this way would make the record inaccurate.

Takeaway: Record only what the assessment supports; use Z codes for a clinical focus when no disorder's criteria are met, even under payment pressure.

Sources: DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Housing Problems; NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 4.04
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281. ASM-1028 (rev 1) · Best answer: C

During a telehealth intake, a 27-year-old client describes 3 years of shifting moods. Every few weeks the client has 2 or 3 days of extra energy, talkativeness and less need for sleep, followed by a week or so of low mood, tiredness and poor concentration. The client has kept the same job throughout and has rarely felt steady for more than a few weeks at a time. Which diagnosis is **MOST** likely?

A. Bipolar II disorder
B. Persistent depressive disorder
C. Cyclothymic disorder
D. Borderline personality disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Bipolar II needs at least one hypomanic episode of 4 or more days and one major depressive episode of 2 or more weeks. These highs and lows are too brief to meet either threshold.

B **Incorrect** (trap: Near-miss diagnosis). Persistent depressive disorder covers chronic low mood for 2 years, but it is not diagnosed when there are recurring periods of hypomanic symptoms. The energetic, talkative stretches point to the bipolar spectrum.

C **Correct.** Cyclothymic disorder involves at least 2 years, in adults, of many periods with hypomanic symptoms and depressive symptoms that never meet full episode criteria, with no symptom-free stretch longer than 2 months (DSM-5-TR). Highs of 2 or 3 days fall short of the 4 days a hypomanic episode needs, and lows of about a week fall short of a major depressive episode. It beats A because bipolar II requires at least one full hypomanic episode and one major depressive episode, which these brief periods do not reach.

D **Incorrect** (trap: Near-miss diagnosis). Borderline personality disorder brings mood shifts lasting hours, usually after interpersonal stress, along with unstable relationships and self-image. Mood periods lasting days, alternating up and down, fit a mood disorder better.

Takeaway: Two or more years of repeated sub-threshold highs and lows, never a full episode, is cyclothymic disorder; full episodes mean bipolar I or II.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Cyclothymic Disorder
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282. ASM-1029 (rev 1) · Best answer: B

At a community mental health intake, a client completes the DSM-5-TR Level 1 Cross-Cutting Symptom Measure. Most domains are rated low, but the client rates the items about sleeping less than usual and starting many more projects than usual as moderate. The social worker reviews these answers with the client. What should the social worker do **NEXT**?

A. Record a provisional bipolar disorder diagnosis
B. Give the matching Level 2 measure for follow-up
C. Repeat the Level 1 measure at the next visit

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). One screen with moderate ratings cannot establish a mood episode. Diagnosing now skips the focused assessment of duration, severity and impairment that any bipolar diagnosis requires.

B **Correct.** In DSM-5-TR, the Level 1 Cross-Cutting Symptom Measure is a broad screen. A domain rated at or above its threshold is followed by a Level 2 measure that examines that area in more depth, along with clinical follow-up. Moderate ratings on sleeping less and starting many projects call for that closer look at possible manic symptoms. It beats C because repeating the broad screen later delays the focused

assessment the positive domain already calls for. Screens point to further assessment; they do not diagnose (DSM-5-TR).

C **Incorrect** (trap: Good but not best). Repeating the broad screen later wastes the signal already present. A domain at or above threshold calls for focused follow-up now, not another general screen.

Takeaway: A positive domain on the Level 1 cross-cutting screen leads to a Level 2 measure and clinical follow-up, not straight to a diagnosis.

Sources: DSM-5-TR (APA, 2022), Section III, Assessment Measures: Level 1 and Level 2 Cross-Cutting Symptom Measures
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283. ASM-1030 (rev 2) · Best answer: A

A jail mental health team is assessing a 31-year-old detainee for frequent rage outbursts, impulsive spending and unstable relationships. A staff member proposes borderline personality disorder. Records show a serious head injury in a motorcycle crash 18 months ago, and family members describe the detainee as easygoing and steady before the crash. What is the social worker's **BEST** recommendation?

A. Request a neuropsychological evaluation of brain functioning
B. Begin dialectical behavior therapy skills for emotion regulation
C. Record borderline personality disorder as a provisional diagnosis
D. Gather more history on personality traits in adolescence

Why each option is right or wrong

A **Correct.** A personality disorder is an enduring pattern traceable to adolescence or early adulthood; a persistent change that follows a brain injury is classified as personality change due to another medical condition (DSM-5-TR). The rage, impulsivity and instability began after a serious head injury in someone described as steady before it. A neuropsychological evaluation clarifies the injury's role and guides treatment. It beats D because family accounts already describe the pre-injury personality, so more history would only delay the evaluation the timing points to.

B **Incorrect** (trap: Right action, wrong time). Emotion-regulation skills may help later, whatever the cause. Starting a borderline-focused treatment now builds on an unconfirmed diagnosis and skips evaluating a likely brain-injury cause.

C **Incorrect** (trap: Near-miss diagnosis). A personality disorder must be an enduring pattern from adolescence or early adulthood. Change that began after a brain injury in a previously steady adult points away from this diagnosis, even as a provisional one.

D **Incorrect** (trap: Right action, wrong time). Premorbid history matters when diagnosing a personality disorder, but family members have already described a steady personality before the crash. More history would add little compared with evaluating the injury.

Takeaway: Personality change that begins after a brain injury points to a medical cause, not a personality disorder; evaluate the injury first.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Personality Change Due to Another Medical Condition
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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284. ASM-1031 (rev 2) · Best answer: D

A client meets criteria for a major depressive episode and, on most days of the episode, also has racing thoughts, pressured speech and increased goal-directed activity. Which DSM-5-TR specifier applies?

A. With anxious distress
B. With rapid cycling
C. With atypical features
D. With mixed features

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Anxious distress means feeling keyed up, restless, worried or afraid something awful may happen. Restlessness can look similar, but racing thoughts, pressured speech and more goal-directed activity are manic symptoms.

B **Incorrect** (trap: Wrong model or concept). Rapid cycling is a specifier for bipolar disorders when 4 or more mood episodes occur within 12 months. It describes how often episodes recur across a year, not manic symptoms occurring within a single depressive episode.

C **Incorrect** (trap: Wrong model or concept). Atypical features involve mood that brightens with good events, oversleeping, increased appetite and sensitivity to rejection. They do not include racing thoughts or pressured speech.

D **Correct.** The mixed-features specifier applies when at least three manic or hypomanic symptoms, such as racing thoughts, pressured speech or increased goal-directed activity, occur on most days of a depressive episode without meeting full manic or hypomanic criteria (DSM-5-TR). It matters because mixed features are a risk factor for later bipolar disorder and shape treatment planning. It beats A because anxious distress describes tension, worry and fear of losing control, not the speeded-up thinking and activity of manic symptoms.

Takeaway: Three or more manic symptoms on most days of a depressive episode call for the 'with mixed features' specifier, a bipolar-risk signal.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Specifiers for Depressive Disorders (With Mixed Features)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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285. ASM-1032 (rev 1) · Best answer: C

A hospice patient, 81, with advanced pancreatic cancer has low mood, poor appetite, weight loss, fatigue and broken sleep. The hospice team asks the social worker whether the patient also has major depression. How can the social worker **BEST** judge whether a depressive disorder is present?

A. Count the appetite, sleep and energy changes as depressive symptoms
B. View the low mood as an expected reaction to dying
C. Focus on hopelessness, worthlessness and loss of pleasure

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). In advanced cancer these physical symptoms are often caused by the illness. DSM-5-TR excludes symptoms clearly due to another medical condition, so counting them would overstate depression.

B **Incorrect** (trap: Good but not best). Sadness and grief are common near death, but major depression is not a normal part of dying. Treating all low mood as expected can leave a treatable disorder, and its suffering, unaddressed.

C **Correct.** DSM-5-TR tells clinicians not to count symptoms clearly caused by another medical condition toward a major depressive episode. In advanced cancer, appetite loss, weight loss, fatigue and poor sleep are often caused by the illness, so they say little about depression. Hopelessness, worthlessness, excessive guilt and loss of pleasure in everything are more telling. It beats B because sadness near death is common, but pervasive hopelessness and worthlessness are not a normal part of dying and deserve treatment.

Takeaway: In serious medical illness, judge depression by psychological symptoms like hopelessness and worthlessness, not by symptoms the illness itself causes.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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286. ASM-1033 (rev 2) · Best answer: B

A partner brings a 57-year-old client to a hospital neurology clinic. Over the past year the client has made crude remarks to strangers, spent savings on impulse purchases, eaten sweets compulsively and shown little concern for family. The changes have come on gradually and steadily. The client remembers recent events and appointments well. Which diagnosis is **MOST** likely?

A. Bipolar I disorder, manic episode
B. Frontotemporal neurocognitive disorder
C. Alzheimer's neurocognitive disorder
D. Antisocial personality disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Mania is episodic: a distinct period of elevated or irritable mood with more energy and less need for sleep. A gradual, steady change in conduct starting at 57 suggests a brain disease, not a first mood episode.

B **Correct.** Frontotemporal neurocognitive disorder, behavioral variant, typically begins in the 50s or 60s with gradual disinhibition, apathy or loss of empathy, compulsive behavior and changes in eating, while memory is relatively spared early (DSM-5-TR). A steady year-long decline in social behavior at 57, with sweets bingeing and intact recall of recent events, fits this pattern. It beats C because Alzheimer's disease usually begins with memory and learning problems, which this client does not show.

C **Incorrect** (trap: Near-miss diagnosis). Alzheimer's disease typically starts with decline in memory and learning. Here memory for recent events is intact while behavior and social conduct have changed, the reverse of the usual Alzheimer's pattern.

D **Incorrect** (trap: Near-miss diagnosis). Antisocial personality disorder requires a pattern from adolescence, with conduct disorder before age 15. New behavior change in midlife does not fit any personality disorder.

Takeaway: New disinhibition, apathy or compulsive eating in midlife with spared memory points to frontotemporal neurocognitive disorder, not a psychiatric disorder.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Frontotemporal Neurocognitive Disorder **Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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287. ASM-1035 (rev 1) · Best answer: C

In a hospital pain clinic, a patient has taken prescribed oxycodone exactly as directed for 8 months after spinal surgery. The patient now needs a higher dose than at first and feels achy and sweaty when a dose is late. A nurse asks the social worker to note opioid use disorder in the chart. The social worker confirms the dosing history with the prescriber. What should the social worker do **NEXT**?

A. Record mild opioid use disorder in the chart

B. Refer the patient to an opioid treatment program

C. Check for criteria beyond tolerance and withdrawal

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Recording a use disorder would rest on tolerance and withdrawal alone, which DSM-5-TR excludes when an opioid is taken as prescribed. The label could also affect the patient's future pain care.

B **Incorrect** (trap: Skips assessment). An opioid treatment program serves people with opioid use disorder. Referring now assumes a diagnosis the facts so far do not support. It may fit if assessment finds other criteria.

C **Correct.** DSM-5-TR does not count tolerance or withdrawal toward opioid use disorder when the medication is taken as prescribed under medical supervision. Needing a higher dose and feeling achy when a dose is late are expected effects of prescribed use. A diagnosis would need other signs, such as craving, use beyond the prescription or giving up activities. It beats A because, once those two expected effects are set aside, nothing yet reaches the 2-criterion threshold.

Takeaway: Tolerance and withdrawal from medication taken as prescribed do not count toward a substance use disorder; look for the other criteria.

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Opioid Use Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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288. ASM-1036 (rev 2) · Best answer: A

At a university diabetes clinic, a 20-year-old client with type 1 diabetes is at a normal weight but has rising blood sugar readings. The client describes eating large amounts of food in a short time about twice a week for 4 months, feeling unable to stop. Afterward, the client skips insulin doses 'to cancel out the calories.' The client says their weight decides how they feel about themselves. Which diagnosis is **MOST** likely?

A. Bulimia nervosa
B. Binge-eating disorder
C. Anorexia nervosa, binge-eating/purging type
D. Other specified feeding or eating disorder

Why each option is right or wrong

A **Correct.** Bulimia nervosa involves recurrent binges with loss of control plus recurrent inappropriate compensatory behavior, both at least weekly for 3 months, with self-worth unduly tied to shape or weight (DSM-5-TR). In people with diabetes, skipping or cutting insulin doses to undo calories is a recognized compensatory behavior (DSM-5-TR). It beats B because binge-eating disorder is diagnosed only when there is no regular compensatory behavior, and this client omits insulin after binges.

B **Incorrect** (trap: Near-miss diagnosis). Binge-eating disorder shares the binges and loss of control, but it excludes regular compensatory behavior. Deliberately skipping insulin to undo calories is compensation, which moves the diagnosis to bulimia nervosa.

C **Incorrect** (trap: Near-miss diagnosis). Anorexia nervosa requires significantly low body weight. This client is at a normal weight, so even with binge and compensation behavior, anorexia does not fit.

D **Incorrect** (trap: Near-miss diagnosis). This category covers serious eating problems that fall short of a full diagnosis, such as bulimia with binges and compensation less often than weekly or for under 3 months. This client binges about twice a week for 4 months and compensates afterward, so the full bulimia nervosa criteria are met (DSM-5-TR).

Takeaway: Insulin omission to undo calories is a compensatory behavior; with weekly binges and weight-based self-worth, it indicates bulimia nervosa.

Sources: DSM-5-TR (APA, 2022), Feeding and Eating Disorders: Bulimia Nervosa
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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289. ASM-1037 (rev 2) · Best answer: B

A school social worker meets with a 17-year-old referred for 'seeing things.' The student describes vivid dreamlike images while drifting off to sleep, falls asleep in class almost daily despite 9 hours of sleep at night, and has had knees buckle suddenly while laughing with friends. These problems began 6 months ago, and grades are dropping. What is the social worker's **BEST** recommendation?

A. Refer the student for a psychiatric evaluation of possible psychosis
B. Refer the student for evaluation by a sleep medicine specialist
C. Screen the student for depression as a cause of sleep problems

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Images that come only while falling asleep are common and are not, by themselves, a sign of psychosis. With sleep attacks and laughter-triggered weakness, a psychosis workup would send care in the wrong direction.

B **Correct.** Daytime sleep attacks despite enough sleep, dreamlike images when falling asleep (hypnagogic hallucinations) and sudden muscle weakness triggered by laughter (cataplexy) together point to narcolepsy, a sleep-wake disorder that often begins in adolescence (DSM-5-TR). A sleep medicine specialist can confirm it, usually with overnight and daytime sleep testing, so this referral fits. It beats C because depression can cause sleepiness but does not explain cataplexy, the most specific clue here.

C **Incorrect** (trap: Good but not best). Depression can cause daytime sleepiness and is worth screening for in a teen with falling grades. It does not explain sudden knee buckling with laughter or dream images at sleep onset, which point to a sleep disorder.

Takeaway: Sleep attacks plus cataplexy or vivid sleep-onset images suggest narcolepsy; refer to sleep medicine rather than treating them as psychosis.

Sources: DSM-5-TR (APA, 2022), Sleep-Wake Disorders: Narcolepsy
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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290. ASM-1038 (rev 1) · Best answer: C

On an inpatient unit, a 24-year-old patient with autism spectrum disorder diagnosed in early childhood is admitted after a family conflict. The team notes flat facial expression, few words, rigid routines and little interest in peers, and a resident suggests adding schizophrenia. The patient's parent says these traits have always been present. What is the **MOST** appropriate input from the social worker?

A. Support adding schizophrenia based on the negative symptoms
B. Gather a detailed developmental history from the parent
C. Explain that prominent delusions or hallucinations are also required

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Negative-like features such as flat expression and few words are part of this patient's lifelong autism. Without prominent delusions or hallucinations for a month, DSM-5-TR does not support adding schizophrenia.

B **Incorrect** (trap: Right action, wrong time). Developmental history is important when autism is uncertain. Here the diagnosis dates to early childhood and the parent confirms the traits are lifelong, so more history would not answer the team's question.

C **Correct.** With a history of autism spectrum disorder, DSM-5-TR allows an added schizophrenia diagnosis only when prominent delusions or hallucinations are present for at least a month, along with the other required symptoms. Flat expression, few words, rigid routines and limited interest in peers are lifelong autism features here and overlap with negative symptoms. It beats B because the parent has already described these traits as lifelong; the decisive question for the team is whether prominent delusions or hallucinations are present.

Takeaway: In autism, add schizophrenia only if prominent delusions or hallucinations last a month; lifelong flat affect and few words don't count.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 3 of 3

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291. ASM-1125 (rev 1) · Best answer: C

A 58-year-old client in an employee assistance program has been meeting with a social worker about work stress. Midway through a session, the client becomes sweaty and short of breath and rubs their left arm, saying the chest pressure is 'just my nerves.' The client has never had a panic attack before. What should the social worker do **FIRST**?

A. Guide the client through slow, paced breathing
B. Ask what thoughts came just before the pressure began
C. Get an ambulance to the office immediately

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Paced breathing helps during a panic attack once a medical cause has been excluded. Using it now treats new chest pressure with arm pain as anxiety and loses time if the heart is involved.

B **Incorrect** (trap: Right action, wrong time). Exploring the thoughts behind an attack is a useful cognitive step in treating panic. It belongs after a medical emergency is ruled out, and asking now delays care for possible cardiac symptoms.

C **Correct.** Possible medical emergencies come before any psychological approach. DSM-5-TR notes that panic-like symptoms that first appear after about age 45, or that look atypical, raise the chance of a medical cause. New chest pressure with left arm pain, sweating and breathlessness in a 58-year-old with no panic history can signal a heart attack, so the social worker gets an ambulance now. It beats A because breathing coaching assumes the episode is panic and wastes minutes that matter if it is cardiac (DSM-5-TR).

Takeaway: When panic-like symptoms first appear in midlife with cardiac red flags, treat them as a medical emergency before considering anxiety.

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Panic Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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292. ASM-1126 (rev 2) · Best answer: D

A 27-year-old is admitted to an inpatient unit after 3 weeks of near-daily methamphetamine use. On admission, the patient believes neighbors are pumping gas through the vents and hears them plotting. With no drug use on the unit, these beliefs fade by day 5, and the patient returns to their usual self. Which diagnosis is **MOST** likely?

A. Brief psychotic disorder
B. Schizophreniform disorder
C. Psychotic disorder due to another medical condition
D. Stimulant-induced psychotic disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Brief psychotic disorder also lasts less than 1 month with a return to baseline, which makes it tempting. It cannot be diagnosed when the symptoms are attributable to a substance, as they are here.

B **Incorrect** (trap: Near-miss diagnosis). Schizophreniform disorder requires at least 1 month of symptoms such as delusions and hallucinations, and it also excludes substance effects. Five days of symptoms that ended with abstinence fall short on both counts.

C **Incorrect** (trap: Near-miss diagnosis). This diagnosis requires evidence that a medical illness, such as a seizure disorder or brain infection, directly causes the psychosis. Here the timing points to the drug: symptoms began during heavy use and cleared with abstinence.

D **Correct.** DSM-5-TR diagnoses a substance/medication-induced psychotic disorder, here a stimulant-induced one, when psychotic symptoms arise during or soon after use of a substance able to cause them and are not better explained by an independent psychotic disorder. The persecutory beliefs and voices began during a methamphetamine binge and cleared within days of abstinence. It beats A because brief psychotic disorder also lasts under a month but is excluded when a substance accounts for the symptoms. Psychosis persisting about a month after stopping would point toward an independent disorder (DSM-5-TR).

Takeaway: Psychosis that starts during stimulant use and clears soon after abstinence points to a substance-induced disorder, not a primary psychotic disorder.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Substance/Medication-Induced Psychotic Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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293. ASM-1127 (rev 1) · Best answer: A

On an inpatient unit, the team plans to diagnose schizophrenia in a Black adult who is guarded, distrusts staff and declines to share personal details. The social worker's interview finds 5 weeks of early waking, slowed movement, poor appetite and intense guilt. The client says past providers 'never listened.' What is the **MOST** appropriate action for the social worker?

A. Press the team for a full evaluation of a depressive episode
B. Explore the client's earlier experiences with mental health providers
C. Support the working diagnosis given the persistent suspicion

Why each option is right or wrong

A **Correct.** A diagnosis must account for all the data. DSM-5-TR warns that schizophrenia is overdiagnosed and mood disorders underrecognized in some racialized groups, including Black Americans, partly because clinicians misread culturally grounded mistrust as paranoia. The client has 5 weeks of classic depressive symptoms, so the team should assess for a major depressive episode, possibly with psychotic features, before settling on schizophrenia. It beats B because understanding past mistreatment helps the alliance but leaves the misdiagnosis risk in place (DSM-5-TR).

B **Incorrect** (trap: Good but not best). Learning how earlier providers treated the client builds trust and may explain the guardedness. It does not, by itself, address the depressive syndrome the team is overlooking, which matters most for the diagnosis and care.

C **Incorrect** (trap: Near-miss diagnosis). Guardedness and distrust after poor past care are not, by themselves, evidence of delusions. Supporting schizophrenia now ignores weeks of depressive symptoms and repeats a documented pattern of misdiagnosis.

Takeaway: Before accepting a psychotic diagnosis, weigh mood symptoms and culturally grounded mistrust, which are often misread as paranoia in racialized clients.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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294. ASM-1128 (rev 1) · Best answer: B

Which DSM-5-TR tool rates how well a person gets around, cares for self, gets along with others and takes part in community life?

A. Global Assessment of Functioning (GAF)
B. WHODAS 2.0
C. Level 1 Cross-Cutting Symptom Measure
D. Cultural Formulation Interview

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The GAF was a single 1-to-100 rating in the old multiaxial system. DSM-5 removed it, partly because it blended symptoms, risk and functioning, and DSM-5-TR uses the WHODAS 2.0 instead.

B **Correct.** The World Health Organization Disability Assessment Schedule 2.0 (WHODAS 2.0) is the functioning measure in DSM-5-TR Section III. It rates six areas: understanding and communicating, getting around, self-care, getting along with people, life activities, and participation in society. Repeating it over time can show whether functioning improves with treatment. It beats A because the GAF scale was dropped along with the multiaxial system in DSM-5 and does not appear in DSM-5-TR (DSM-5-TR, Section III).

C **Incorrect** (trap: Wrong model or concept). The Level 1 Cross-Cutting Symptom Measure screens symptom domains such as mood, anxiety and sleep across diagnoses. It flags symptoms for follow-up rather than rating how well a person functions in daily life.

D **Incorrect** (trap: Wrong model or concept). The Cultural Formulation Interview gathers the client's cultural view of the problem, its causes and help-seeking. It informs diagnosis and planning but does not score daily functioning.

Takeaway: DSM-5-TR rates functioning with the WHODAS 2.0, a Section III measure; the GAF score left with the multiaxial system.

Sources: DSM-5-TR (APA, 2022), Section III, Assessment Measures: WHODAS 2.0
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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295. ASM-1129 (rev 2) · Best answer: A

Group home staff call the social worker about a resident with schizophrenia. For 2 days, the resident has taken only sips of water and almost no food. The resident stands in one spot for hours, holds odd postures, does not answer questions and stiffens when staff try to guide them. What should the social worker do **FIRST**?

A. Get the resident to an emergency department today
B. Rate the resident's negative symptoms with a standard scale
C. Ask the psychiatrist to see the resident at the next clinic visit
D. Have staff push fluids and track intake at the home

Why each option is right or wrong

A **Correct.** Mutism, posturing, rigid resistance to being moved and refusal of food and fluids are signs of catatonia, which DSM-5-TR recognizes in schizophrenia and other conditions. Catatonia with poor intake can lead to dehydration, malnutrition and other life-threatening complications, so it needs urgent medical evaluation today. It beats C because waiting for a routine psychiatry visit leaves a resident who is barely drinking without care, and the cause, including a medical one, must be found quickly (DSM-5-TR).

B **Incorrect** (trap: Near-miss diagnosis). These are not negative symptoms such as low motivation or reduced expression. Mutism, posturing and resistance to movement are catatonic signs, and rating negative symptoms delays urgent care for a resident who is barely drinking.

C **Incorrect** (trap: Ignores risk). Psychiatric follow-up matters, but 2 days of almost no fluids with catatonic signs cannot wait for a routine visit. A prompt psychiatry appointment fits only after medical stability is assured.

D **Incorrect** (trap: Ignores risk). Encouraging fluids and tracking intake is sensible for a resident who is eating poorly for other reasons. A resident with catatonic signs often cannot respond to prompting, and 2 days of minimal fluids already calls for same-day medical evaluation, so home monitoring delays care.

Takeaway: Catatonic signs with poor food or fluid intake are a medical emergency; arrange same-day evaluation rather than routine follow-up.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Catatonia Associated With Another Mental Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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296. ASM-1130 (rev 1) · Best answer: C

A 31-year-old comes to a community agency at a parent's urging. The client has no close friends besides a sibling, dresses in mismatched layers and believes they can sense coworkers' moods through 'vibrations.' The client often thinks strangers' conversations are about them but agrees this may not be true. Unease around coworkers has not faded after 6 years at the same job. Which diagnosis is **MOST** likely?

A. Schizoid personality disorder
B. Schizophrenia
C. Schizotypal personality disorder
D. Avoidant personality disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Schizoid personality disorder also involves few close relationships, but without odd beliefs, magical thinking or eccentric behavior. The 'vibrations' and self-referential thinking point to schizotypal features.

B **Incorrect** (trap: Near-miss diagnosis). Schizophrenia requires fixed delusions, hallucinations or disorganization, usually with a decline in functioning. The client's self-referential beliefs are held loosely, and the client has kept the same job for 6 years.

C **Correct.** Schizotypal personality disorder combines discomfort with close relationships, odd beliefs or magical thinking, ideas of reference, eccentric appearance and social anxiety that does not ease with familiarity. The client shows each one: sensing 'vibrations,' self-referential thoughts held loosely, mismatched dress, and unease after 6 years with the same coworkers. It beats B because the client can doubt the self-referential thoughts, which marks them as ideas of reference rather than fixed delusions (DSM-5-TR).

D **Incorrect** (trap: Near-miss diagnosis). Avoidant personality disorder involves wanting relationships but fearing rejection, with feelings of inadequacy. It does not include magical thinking, ideas of reference or eccentric dress.

Takeaway: Odd beliefs, loosely held self-referential ideas, eccentricity and lasting social anxiety point to schizotypal personality disorder rather than psychosis.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Schizotypal Personality Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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297. ASM-1131 (rev 2) · Best answer: B

A client in private practice was laid off 2 months ago and since then has had poor sleep, tearfulness and trouble concentrating. The client is suing the former employer and asks the social worker to record PTSD because 'that will help the case.' The social worker's assessment supports adjustment disorder with mixed anxiety and depressed mood. What is the **MOST** appropriate response?

A. Record PTSD as a provisional diagnosis while gathering more history
B. Explain why the findings point to adjustment disorder instead
C. Record adjustment disorder and direct legal questions to the client's attorney

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Calling PTSD provisional still puts an unsupported diagnosis in the record to serve a lawsuit. The provisional label applies when criteria are expected to be met, not when the required traumatic exposure is missing.

B **Correct.** Diagnoses must be accurate, and clients deserve a clear account of what their records say. PTSD requires exposure to actual or threatened death, serious injury or sexual violence; a layoff, however painful, does not meet that requirement, and the symptoms fit adjustment disorder. Explaining this openly, including why PTSD does not apply, respects the client while declining a false record. It beats C because handing the question to the attorney treats a clinical question as a legal one and leaves the client to be surprised later, possibly in court (DSM-5-TR; NASW Code 3.04(a), 4.04).

C **Incorrect** (trap: Hands off too soon). The diagnosis is correct, and the social worker should not give legal advice. But the client asked what the record will say, which is a clinical question. Sending it to the attorney sidesteps explaining the findings and leaves the client to learn the diagnosis later.

Takeaway: Record only diagnoses the assessment supports, and explain the findings openly when a client requests a different label.

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder; NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 4.04
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298. ASM-1132 (rev 2) · Best answer: A

Police bring a 23-year-old to the emergency department after finding them directing traffic and announcing a plan to buy the city's stadium. A roommate says the client has barely slept for 3 days, has talked nonstop and has spent their savings online. The client is admitted to the psychiatric unit that night. Which type of mood episode is **MOST** likely?

A. Manic episode
B. Hypomanic episode
C. Major depressive episode with mixed features

Why each option is right or wrong

A **Correct.** A manic episode normally lasts at least 1 week, but DSM-5-TR waives the duration when symptoms are severe enough to require hospitalization. Hypomania needs at least 4 days and, by definition, is never severe enough to need admission. After 3 days of little sleep, nonstop talk, reckless spending and grandiose plans, this client needed admission, so the episode is manic. It beats B because the short duration tempts a hypomania label, yet the need for hospitalization rules hypomania out (DSM-5-TR).

B **Incorrect** (trap: Near-miss diagnosis). Hypomania requires at least 4 consecutive days and must not cause marked impairment or need hospitalization. Three days of symptoms ending in psychiatric admission fails both conditions.

C **Incorrect** (trap: Near-miss diagnosis). Mixed features describe a full major depressive episode with some manic symptoms alongside. This client shows elevated, grandiose mood and increased activity, with no depressive episode described.

Takeaway: Hospitalization makes an elevated-mood episode manic regardless of duration; hypomania needs 4 days and never requires admission.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder
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299. ASM-1133 (rev 3) · Best answer: C

In a hospice bereavement program, a 79-year-old client whose spouse died 4 months ago mentions hearing the spouse say 'good night' most evenings. The client then looks away and asks quietly, 'Does that mean I'm losing my mind?' The client has not told anyone about this before. What should the social worker do **FIRST**?

A. Refer the client for a psychiatric evaluation of hallucinations
B. Tell the client that hearing a late spouse is common in grief
C. Ask what the experience is like and how the client makes sense of it

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). A psychiatric evaluation fits if the voice brings commands, distress, other psychotic signs or a decline in functioning. None of that is known yet, and an immediate referral would confirm the client's fear of being seen as mentally ill.

B **Incorrect** (trap: Right action, wrong time). This is probably true, and normalizing will likely help once the social worker learns that the client knows the spouse died, finds the voice comforting and is functioning well. Given before any exploration, it answers a question whose facts are still unknown and may cut off what the client wanted to say.

C **Correct.** Before answering, the social worker needs to know what the experience is like: whether the client knows the spouse died, whether the voice comforts or frightens, and whether commands, low mood or poor functioning are present. Asking also shows the client the question is safe to raise. Sensing a dead spouse is common in widowhood (Rees, 1971), and DSM-5-TR treats uncomplicated bereavement as a focus of attention, not a disorder, but that judgment rests on these details. It beats B because reassurance offered before the facts are known may be wrong and can close off a frightened client.

Takeaway: When a grieving client reports sensing the deceased and fears it means mental illness, explore the experience before normalizing or referring.

Sources: Rees, W. D. (1971). The hallucinations of widowhood. British Medical Journal, 4(5778), 37-41, whole article; DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Uncomplicated Bereavement

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300. ASM-1134 (rev 1) · Best answer: D

A hospital social worker meets with a 45-year-old patient who has rheumatoid arthritis. The patient's joint pain is persistent and distressing. For the past year, the patient has spent hours each day reading about joint damage, has gone to the emergency room most weeks fearing disability and has stopped working. Reassurance from the rheumatologist brings relief for only a day. Which diagnosis is **MOST** likely?

A. Illness anxiety disorder
B. Psychological factors affecting other medical conditions
C. Generalized anxiety disorder
D. Somatic symptom disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Illness anxiety disorder centers on fear of having or getting a serious illness when bodily symptoms are absent or mild. This patient has a known disease with persistent pain that is itself the focus of distress.

B **Incorrect** (trap: Near-miss diagnosis). This category applies when psychological or behavioral factors worsen the medical condition, such as skipping treatment. Here the problem is an excessive response to the symptom, not behavior that worsens the arthritis.

C **Incorrect** (trap: Near-miss diagnosis). Generalized anxiety disorder involves hard-to-control worry about many areas of life, such as work, money and family. This patient's worry centers on one distressing bodily symptom and drives heavy health-related behavior.

D **Correct.** Somatic symptom disorder involves one or more distressing bodily symptoms plus excessive thoughts, feelings or behaviors about them, usually lasting more than 6 months. A real medical illness does not rule it out. The patient's persistent arthritis pain comes with hours of daily research, weekly emergency visits and leaving work. It beats A because illness anxiety disorder applies when bodily symptoms are absent or mild and the fear of illness is the focus, whereas here a prominent symptom drives the distress (DSM-5-TR).

Takeaway: A real medical illness does not exclude somatic symptom disorder; excessive thoughts and behaviors about a distressing symptom define it.

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Somatic Symptom Disorder
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301. ASM-1135 (rev 2) · Best answer: B

A jail social worker interviews a detainee awaiting sentencing who reports hearing voices 'all day, every day' and asks to move to the mental health unit. The detainee describes the voices vaguely and denies thoughts of harming self or others. Officers' logs describe the detainee sleeping well and playing cards calmly with others. After completing the interview, what should the social worker do **NEXT**?

A. Recommend transfer to the mental health unit for observation
B. Request prior treatment records and reassess over several contacts
C. Document malingering based on the inconsistencies found

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Transfer may be right if corroboration supports the symptoms or if risk emerges. The detainee denies thoughts of harm and appears settled in daily life, so moving now acts on the claim before checking it.

B **Correct.** DSM-5-TR advises strong suspicion of malingering when a legal context and a gap between reported and observed functioning appear together, but suspicion is not a conclusion. With the interview done, the next step is corroboration: prior records, observation by mental health staff and repeat contact. It beats C because labeling malingering now could deny care to someone with a real illness, and the evidence so far is suggestive, not conclusive. The findings then guide the level-of-care decision (DSM-5-TR).

C **Incorrect** (trap: Right action, wrong time). Malingering is recorded only when evidence shows intentional false or exaggerated symptoms for an external gain. Writing it now, after one interview, risks mislabeling a real illness and coloring every later decision.

Takeaway: Suspected malingering calls for corroboration from records and observation before any label or level-of-care change.

Sources: DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Malingering
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302. ASM-1136 (rev 2) · Best answer: B

A mobile crisis team brings an adult to a crisis stabilization unit. The adult is agitated, irritable and tearful, with rapid speech, and cannot give a history. No records or family are available. The symptoms clearly involve mood, but it is unclear whether they belong to a bipolar or a depressive disorder. Which DSM-5-TR diagnosis fits this situation?

A. Unspecified bipolar and related disorder
B. Unspecified mood disorder
C. Unspecified mental disorder
D. Unspecified depressive disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Unspecified bipolar and related disorder fits when symptoms are clearly bipolar-type but criteria or information are incomplete. Here even the bipolar-versus-depressive question cannot be answered.

B **Correct.** DSM-5-TR restored unspecified mood disorder for presentations with clear mood symptoms that do not meet full criteria for any bipolar or depressive disorder, when choosing between unspecified bipolar and unspecified depressive disorder is difficult. Acute agitation in a crisis or emergency setting with no available history is the classic example. It beats C because unspecified mental disorder is for symptoms that cannot be tied to any diagnostic class, and these clearly involve mood. A and D each commit to one chapter, which the limited information cannot support. The diagnosis can be revised as history becomes available (DSM-5-TR).

C **Incorrect** (trap: Near-miss diagnosis). Unspecified mental disorder fits when symptoms cause distress or impairment but cannot be placed in any diagnostic class, sometimes because an emergency setting allows too little information. Here the symptoms clearly involve mood, so the narrower mood category applies.

D **Incorrect** (trap: Near-miss diagnosis). Unspecified depressive disorder fits when the symptoms are clearly depressive but the clinician cannot or does not specify more. Rapid speech and agitation leave open a bipolar picture, so this label commits too early.

Takeaway: When mood symptoms are clear but bipolar versus depressive cannot be determined, DSM-5-TR offers unspecified mood disorder.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Unspecified Mood Disorder; DSM-5-TR (APA, 2022), Other Mental Disorders and Additional Codes: Unspecified Mental Disorder
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303. ASM-1137 (rev 1) · Best answer: A

On a home visit, an adult child reports that their 74-year-old parent with Parkinson's disease has begun seeing small animals in the corners of rooms. The parent knows the animals are not real and is not frightened. A Parkinson's medication dose was raised 3 weeks ago. The adult child wants the parent admitted to a psychiatric hospital. What is the social worker's **BEST** action?

A. Help the family tell the neurologist about the new symptoms
B. Support a voluntary psychiatric admission for a full evaluation
C. Reassure the family that hallucinations are expected with the disease

Why each option is right or wrong

A **Correct.** Psychotic symptoms in a medical illness are first evaluated by the treating physician for medical and medication causes. Visual hallucinations are common in Parkinson's disease, insight is often kept early, and a recent dose increase is a possible trigger the neurologist should review. DSM-5-TR recognizes both psychosis due to another medical condition and medication-induced psychosis. It beats B because the parent is calm, aware the animals are not real and safe at home; admission is more restrictive and bypasses the physician who manages the medications (DSM-5-TR).

B **Incorrect** (trap: Skips assessment). Psychiatric admission may be needed if hallucinations bring danger, severe distress or loss of insight. A calm parent with preserved insight and a possible medical trigger does not need a more restrictive setting first.

C **Incorrect** (trap: Good but not best). Hallucinations are indeed common in Parkinson's disease, but they still warrant review, since a medication change, infection or delirium may be contributing and treatable. Reassurance alone delays that review.

Takeaway: New psychotic symptoms in a medical illness go first to the treating physician to review medical and medication causes.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Psychotic Disorder Due to Another Medical Condition; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Substance/Medication-Induced Psychotic Disorder
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304. ASM-1138 (rev 1) · Best answer: C

A 34-year-old began sertraline for depression 5 weeks ago. Two weeks later, the client stopped sleeping, spent recklessly and announced a plan to run for governor. The prescriber stopped the sertraline at once. Three weeks after it was stopped, the client remains on an inpatient unit with grandiosity, pressured speech and little need for sleep. Which diagnosis is **MOST** likely?

A. Substance/medication-induced bipolar and related disorder
B. Bipolar II disorder
C. Bipolar I disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). This diagnosis applies when mood elevation is attributable to a medication and fades as its effects wear off. Mania lasting weeks after sertraline was stopped goes beyond the drug's effect.

B **Incorrect** (trap: Near-miss diagnosis). Bipolar II disorder requires hypomania and depression with no manic episode ever. A syndrome severe enough for hospitalization and lasting weeks is manic, which makes the diagnosis bipolar I.

C **Correct.** DSM-5-TR counts a full manic episode that emerges during antidepressant treatment as evidence of bipolar I when the syndrome persists beyond the drug's physiological effect. Here mania continues 3 weeks after sertraline was stopped, with grandiosity, pressured speech and decreased need for sleep severe enough for admission. It beats A because a medication-induced diagnosis fits only when symptoms are attributable to the drug, and persistence well after stopping it points to an independent bipolar disorder (DSM-5-TR).

Takeaway: Full mania that begins on an antidepressant and persists beyond the drug's effect supports bipolar I, not a medication-induced disorder.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder
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305. ASM-1139 (rev 1) · Best answer: B

At a pediatric clinic, a parent asks the social worker to start an ADHD diagnosis for their 7-year-old. For 8 months, the child has struggled to finish chores, loses things and interrupts siblings. So far, no information about the child's behavior at school has been gathered. What should the social worker do **FIRST**?

A. Refer the child to the pediatrician to discuss medication
B. Obtain rating scales from the parent and the teacher
C. Start behavioral parent training for the problems at home

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A medication discussion follows an established diagnosis. Referring now skips the multi-setting assessment that ADHD requires and may lead to treating a problem that occurs only at home.

B **Correct.** ADHD requires several symptoms in two or more settings, such as home and school, so information from more than one setting is needed before a diagnosis. The AAP guideline directs clinicians to gather reports from parents and from teachers or other school staff (Wolraich et al., 2019). All information so far comes from home. It beats C because parent training may become part of the plan, but the assessment must first show whether the problem is ADHD, another condition or something limited to home (DSM-5-TR).

C **Incorrect** (trap: Right action, wrong time). Behavioral parent training is an evidence-based treatment for ADHD symptoms and disruptive behavior. Starting it before the assessment is complete puts treatment ahead of knowing what is being treated.

Takeaway: ADHD diagnosis requires symptoms in at least two settings; gather parent and teacher reports before diagnosing or treating.

Sources: DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder; Wolraich, M. L., et al. (2019). Clinical practice guideline for the diagnosis, evaluation, and treatment of attention-deficit/hyperactivity disorder in children and adolescents. Pediatrics, 144(4), e20192528, Key Action Statement 2
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306. ASM-1140 (rev 1) · Best answer: C

At an area agency on aging, an adult child asks for help placing their 71-year-old parent in memory care. Over the past 5 months, the parent has developed a shuffling, unsteady walk, new bladder accidents and growing forgetfulness. The adult child, the main caregiver, says they are exhausted. The parent has had no checkup since the changes began. What should the social worker do **FIRST**?

A. Help the family compare memory care facilities
B. Set up in-home respite care for the tired caregiver
C. Arrange a prompt visit with the parent's physician
D. Screen the parent's memory with a brief cognitive test

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Placement planning may be appropriate later if a progressive, untreatable condition is confirmed. Choosing memory care now assumes dementia and could move the parent from home for a condition that treatment might improve.

B **Incorrect** (trap: Right action, wrong time). Respite is a valuable support for an exhausted caregiver and can be arranged soon. It comes after setting up the physician visit, which could change the diagnosis and the need for placement.

C **Correct.** Potentially reversible causes must be ruled out before planning around dementia. A shuffling gait, bladder incontinence and cognitive decline developing together over months form the classic pattern of normal pressure hydrocephalus, which can improve with treatment. DSM-5-TR places such conditions under neurocognitive disorder due to another medical condition. It beats B because respite eases real caregiver strain but should not delay finding a treatable cause that could change the whole plan (DSM-5-TR; Kaplan & Sadock's Synopsis).

D **Incorrect** (trap: Good but not best). A cognitive screen measures memory but cannot identify the cause. With gait and bladder changes appearing alongside forgetfulness, a medical evaluation is needed, and a screen alone may simply suggest dementia.

Takeaway: New gait problems, incontinence and forgetfulness together suggest a treatable brain condition; arrange medical evaluation before dementia care planning.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Neurocognitive Disorder Due to Another Medical Condition; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Neurocognitive disorders: normal pressure hydrocephalus
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307. ASM-1141 (rev 1) · Best answer: A

At a crisis residential program, a client with borderline personality disorder becomes convinced for an afternoon that staff are plotting to discharge them, after a close peer leaves the program. By evening, the client laughs about the idea. A psychiatry resident proposes adding delusional disorder. Which response by the social worker is **MOST** helpful to the team?

A. Note that brief, stress-linked suspicion fits the existing diagnosis
B. Agree to add the diagnosis and monitor the beliefs closely
C. Suggest psychological testing to clarify the diagnosis first

Why each option is right or wrong

A **Correct.** Borderline personality disorder includes transient, stress-related paranoid ideas or severe dissociation. Delusional disorder requires a fixed delusion lasting at least 1 month. The client's suspicion followed a loss, lasted hours and faded by evening, which fits the existing diagnosis rather than a new psychotic disorder. It beats C because testing adds delay when the course alone shows that delusional disorder criteria are not met (DSM-5-TR).

B **Incorrect** (trap: Near-miss diagnosis). Delusional disorder requires a fixed belief for at least 1 month. An afternoon of suspicion that the client later laughs about falls far short and fits the borderline pattern.

C **Incorrect** (trap: Good but not best). Psychological testing can help when a picture is unclear. Here the brief, stress-linked course already answers the question, and testing would delay an understanding the team can reach now.

Takeaway: Brief, stress-linked paranoid ideas in borderline personality disorder are part of that pattern, not a separate psychotic disorder.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Borderline Personality Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Delusional Disorder
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308. ASM-1142 (rev 1) · Best answer: A

A housing inspector refers a 52-year-old tenant to a community agency. Stacks of newspapers, bags and boxes fill every room, and the tenant sleeps in a chair because the bed is buried. The tenant becomes very upset at the idea of discarding anything, saying each item 'might be needed someday.' Which diagnosis is **MOST** likely?

A. Hoarding disorder
B. Obsessive-compulsive personality disorder
C. Obsessive-compulsive disorder

Why each option is right or wrong

A **Correct.** Hoarding disorder involves persistent difficulty discarding possessions, driven by a perceived need to save them and distress at parting with them, so that clutter blocks the use of living areas. The tenant's distress at discarding, the 'might be needed' reasoning and a buried bed fit this pattern. It beats C because in OCD, saving follows typical obsessions such as fear of harm or contamination, whereas this tenant saves items for their possible usefulness (DSM-5-TR).

B **Incorrect** (trap: Near-miss diagnosis). OCPD can include reluctance to throw out worn-out objects, but it is a broad pattern of perfectionism, rigidity and control. Saving with clutter that fills the home, without that wider pattern, points to hoarding disorder.

C **Incorrect** (trap: Near-miss diagnosis). In OCD, saving happens as part of typical obsessions or compulsions, such as fear that discarding will cause harm. The tenant saves because items may be useful, the reasoning typical of hoarding disorder.

Takeaway: Distress at discarding and saving 'just in case' until living space is unusable define hoarding disorder, separate from OCD and OCPD.

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Hoarding Disorder
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II.C Assessment Practices

309. ASM-1616 (rev 1) · Best answer: A

At a pediatric clinic, a social worker meets the parent of a child who turned 3 last month. The child uses only a few words, and the pediatrician recommends a developmental evaluation and possible services. The family has never received any early childhood services. Which resource is the **BEST** fit for the parent to contact?

A. The local school district's special education office
B. The state's Part C early intervention program
C. A local Head Start program

Why each option is right or wrong

A **Correct.** Under the Individuals with Disabilities Education Act, Part C early intervention serves children under age 3, and public school systems must make special education available from age 3 and find and evaluate children who may need it (34 CFR 303.21; 34 CFR 300.101; 34 CFR 300.111). This child just turned 3, so the school district is responsible for the evaluation and any preschool services, at no cost to the family. It beats B because Part C eligibility ends at the third birthday; that program would have been the right contact a few months earlier.

B **Incorrect** (trap: Wrong model or concept). Part C early intervention is for infants and toddlers under age 3. It was the right resource before the child's birthday, but a child who has turned 3 goes to the school district for evaluation.

C **Incorrect** (trap: Good but not best). Head Start serves many 3- to 5-year-olds and can enroll children with disabilities, but it does not replace the school district's duty to evaluate the child and provide special education. It may be a useful setting once eligibility is known.

Takeaway: Match resources to eligibility: IDEA Part C serves children under 3; from age 3, the public school district evaluates and serves.

Sources: Individuals with Disabilities Education Act regulations, 34 CFR 303.21; 34 CFR 300.101(a); 34 CFR 300.111

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310. ASM-1617 (rev 1) · Best answer: B

A primary care social worker gave a client with panic disorder a list of three low-cost counseling clinics. At follow-up 2 weeks later, the client has not called any of them and explains that phone calls with strangers trigger panic and that the clinic websites were confusing. The client still wants counseling. What should the social worker do **NEXT**?

A. Ask the client what made calling the clinics difficult

B. Call one of the clinics together with the client now

C. Give the client a shorter list with clearer directions

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Asking about barriers is the right step when they are unknown. The client has already named them, panic with strangers on the phone and confusing websites, so asking again delays the connection the client still wants.

B **Correct.** Linking a client to a resource includes navigating access and following through on coordination, not just handing over names (NASW Standards for Social Work Case Management, Standard 6). The client has already explained the barriers: calls with strangers trigger panic and the websites are confusing. A joint call during the visit, a warm handoff, removes both barriers while the client's motivation is high. It beats C because a shorter list still leaves the client to make the frightening call alone.

C **Incorrect** (trap: Good but not best). Clearer directions help with the confusing websites but not with the panic triggered by calling strangers. The client would still face the step that stopped them the first time.

Takeaway: When a client cannot act on a referral list, close the gap directly, such as with a warm handoff, and confirm the connection is made.

Sources: NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring
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311. ASM-1618 (rev 2) · Best answer: B

A social worker at a rural community agency meets a client who wants medication treatment for opioid use disorder. The nearest opioid treatment program is 80 miles away and requires near-daily visits at first. The client works days and has no car. The local primary care clinic offers in-person and telehealth visits. Which resource is **MOST** appropriate to pursue?

A. Daily methadone dosing at the distant opioid treatment program
B. A nearby prescriber of buprenorphine
C. A weekly peer support group in the county
D. A residential treatment program in the nearest city

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Methadone is effective, but for opioid use disorder it is dispensed through certified opioid treatment programs, which usually require near-daily visits at first (SAMHSA TIP 63). Without a car and with a day job, an 80-mile trip each day is unlikely to last.

B **Correct.** Resource selection weighs availability and accessibility, not only the type of service. Since the federal waiver requirement ended in 2023, clinicians with standard DEA registration for Schedule III drugs can prescribe buprenorphine (SAMHSA, MAT Act), so local primary care, in person or by telehealth, can provide medication without daily travel. Methadone for this disorder requires an opioid treatment program (SAMHSA TIP 63). It beats A because an 80-mile daily trip without a car, while working days, is likely to fail despite the medication's effectiveness.

C **Incorrect** (trap: Overrides the client's choice). Peer support can strengthen recovery, but the client asked for medication, the most effective treatment for opioid use disorder. Offering a group instead substitutes the social worker's preference for the client's informed choice.

D **Incorrect** (trap: Good but not best). Residential care is for needs that outpatient treatment cannot safely meet, and nothing here suggests that. It would cost the client the job when medication can be started locally.

Takeaway: Choose resources by accessibility as well as effectiveness; buprenorphine can now be prescribed in ordinary medical and telehealth settings.

Sources: SAMHSA (2023). Waiver Elimination (MAT Act), Consolidated Appropriations Act, 2023, Section 1262; SAMHSA TIP 63 (2021), Part 1: Introduction to Medications for Opioid Use Disorder Treatment
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312. ASM-1690 (rev 2) · Best answer: A

A 31-year-old veteran who completed two combat deployments tells a community agency social worker about nightmares and growing distance from their partner. The veteran refuses to go to a VA hospital, saying it 'feels like being back on base,' and cannot afford private fees. Which referral is **MOST** appropriate?

A. A Vet Center offering individual and couples readjustment counseling
B. The outpatient mental health clinic at the VA medical center
C. A trauma specialist in private practice near the client's home

Why each option is right or wrong

A **Correct.** A fitting resource must be available, acceptable and affordable for this client. Vet Centers are community-based counseling centers located apart from VA medical facilities, offering readjustment counseling at no cost to eligible veterans who served in a combat theater, including couples and family counseling (U.S. Department of Veterans Affairs, 2025). That addresses the nightmares, the strain with the partner, the setting the veteran rejected and the cost barrier. Referral fits when another provider's expertise is needed (NASW Code 1.16(a)). It beats B because the VA clinic is eligible care but is the hospital setting the veteran refused, so follow-through is unlikely.

B **Incorrect** (trap: Overrides the client's choice). The VA clinic offers strong trauma treatment, and the veteran is likely eligible. But the veteran has clearly refused the hospital setting, and pressing that option disregards the client's stated preference.

C **Incorrect** (trap: Good but not best). A trauma specialist in private practice could be clinically suitable, but the veteran has said private fees are unaffordable. A referral the client cannot pay for is unlikely to be used.

Takeaway: Match referrals to what the client will accept and can access; community-based Vet Centers serve eligible combat veterans and families at no cost.

Sources: U.S. Department of Veterans Affairs (2025). Vet Center Services, VA Welcome Kit quick start guide, services, eligibility, location and cost; NASW Code of Ethics (2021), 1.16(a)
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313. ASM-0116 (rev 1) · Best answer: B

At intake, a parent reports being evicted last week and sleeping in a car with 2 young children during freezing nights. The parent also describes low mood and a custody hearing in 6 weeks, and asks first about the hearing. The parent denies suicidal thoughts. What should the social worker address **FIRST**?

A. The parent's depressive symptoms and treatment options
B. Emergency shelter for the family for tonight
C. A legal aid referral for the custody hearing

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Low mood deserves assessment and care, and it belongs in the plan. With no suicidal thoughts reported, it comes after the family's immediate need for safe shelter.

B **Correct.** Treatment and service planning put immediate safety and basic needs first, then the client's other priorities (Maslow, 1943; Hepworth et al.). Young children sleeping in a car on freezing nights face a safety risk tonight, while the hearing is 6 weeks away and the parent denies suicidal thoughts. It beats C because, although starting where the client is matters and the hearing deserves attention soon, it does not outrank an urgent safety need. The social worker can explain this order and plan the hearing next.

C **Incorrect** (trap: Right action, wrong time). The hearing is the parent's stated concern and should be addressed soon, which honors starting where the client is. With 6 weeks until the hearing, it can follow securing shelter for tonight.

Takeaway: In triage, address immediate safety and basic needs first, then the client's other stated priorities, explaining the order collaboratively.

Sources: Maslow, A. H. (1943). A theory of human motivation. Psychological Review, 50(4), 370-396, whole article; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, planning: prioritizing problems and goals
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314. ASM-1603 (rev 1) · Best answer: D

On a busy afternoon, a social worker is the only staff member covering walk-in triage at a community mental health center. Four people have signed in at about the same time and are waiting in the lobby. Which person should the social worker see **FIRST**?

A. A client with long-standing, unchanged passive death wishes, here for routine care
B. A client who ran out of an antidepressant 2 days ago and feels fine
C. A parent who needs a letter for a housing appointment tomorrow morning
D. An older adult whose confusion and fever began this morning

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Passive death wishes deserve attention, and this client should have a risk check today. Because the thoughts are long-standing and unchanged and the visit is routine, they are less urgent than a sudden medical change. New intent or a plan would move this client to the front.

B **Incorrect** (trap: Right action, wrong time). Two days without an antidepressant can bring discontinuation symptoms and calls for a prompt contact with the prescriber. The client feels fine, so the need is urgent but not emergent and can follow the medical concern.

C **Incorrect** (trap: Good but not best). A housing letter matters to the parent's stability and should be done today, but a deadline tomorrow morning leaves time. Administrative needs come after possible medical emergencies in any triage order.

D **Correct.** Triage puts possible medical emergencies ahead of stable concerns. Confusion that began within hours, together with fever, suggests delirium, which develops quickly, usually reflects an underlying medical condition and can be life-threatening without prompt care (DSM-5-TR; Kaplan & Sadock's Synopsis). Seeing this person first lets the social worker confirm the picture and arrange immediate medical evaluation. It beats A because that client's passive death wishes are

long-standing and unchanged and the visit is routine, so a risk check right after the emergency is handled is appropriate.

Takeaway: In triage, sudden changes in mental status or physical condition come before stable, chronic or administrative needs, even serious-sounding ones.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium; Boland, R., Verduin, M. L., & Ruiz, P. (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.). Wolters Kluwer, delirium
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315. ASM-1604 (rev 2) · Best answer: B

A client with bipolar I disorder has spent 12 days on an inpatient unit for mania. The client now sleeps through the night and voices no thoughts of harming anyone, but still has racing thoughts at times while a new medication is adjusted. The client lives with a supportive partner and wants to go home. Which aftercare plan is **BEST**?

- **A.** Continue inpatient care until every symptom resolves
- **B.** Step down to a partial hospitalization program
- **C.** Discharge home with weekly outpatient therapy
- **D.** Transfer to a residential treatment program

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Inpatient care is for imminent danger, grave disability or a need for round-the-clock monitoring. None of these remains, and waiting for every symptom to clear keeps the client in a more restrictive setting than needed.

B **Correct.** Level-of-care decisions aim for the most integrated, least restrictive setting that can still meet the client's needs (Olmstead v. L.C., 1999). This client no longer needs 24-hour care: sleep has returned, there are no thoughts of harm and a supportive partner is at home. Residual racing thoughts during a medication change, though, call for close monitoring several days a week. Partial hospitalization provides structured treatment most of the day while the client sleeps at home. It beats C because weekly therapy is too thin to catch early relapse signs while the medication is still being adjusted.

C **Incorrect** (trap: Ignores risk). Weekly therapy is the right target once mood is stable on the new medication. Stepping down that far now leaves days between contacts while racing thoughts persist, so early signs of relapse could be missed.

D **Incorrect** (trap: Good but not best). Residential treatment adds 24-hour living in a facility, which this client does not need with a supportive partner at home. It is more restrictive than the clinical picture calls for and would delay the return home.

Takeaway: Choose the least restrictive level of care that still meets current clinical needs; step down gradually while symptoms remain during medication changes.

Sources: Olmstead v. L.C., 527 U.S. 581 (1999)
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316. ASM-1605 (rev 1) · Best answer: A

A child welfare social worker meets with a parent whose 4-year-old was placed in foster care after being left alone overnight. The court-approved case plan lists nine tasks, including parenting classes, a substance use assessment, housing applications and a job search. The parent says, 'I can't do all of this at once, and I don't know where to start.' The review hearing is in 5 months. What should the social worker do **FIRST**?

A. Rank the tasks with the parent by their link to safety
B. Help the parent enroll in the nine services this week
C. Ask the court to reduce the number of required tasks
D. Refer the parent to counseling for feeling overwhelmed

Why each option is right or wrong

A **Correct.** When a plan feels unmanageable, partializing breaks it into ordered steps, and priorities follow the concern that brought the family to the agency (Hepworth et al.). The child was left alone overnight, so the tasks most directly tied to safe supervision come first, with others sequenced over the 5 months. Ranking the tasks together also builds the parent's ownership of the plan (NASW Code 1.02). It beats C because asking the court to cut tasks comes after the social worker and parent have tested whether a sequenced plan is workable.

B **Incorrect** (trap: Good but not best). Starting everything at once confirms the parent's fear of being overwhelmed and invites missed appointments that can look like noncompliance to the court. Services can be staggered across the 5 months before the hearing.

C **Incorrect** (trap: Right action, wrong time). Advocating to change the plan is appropriate if, after sequencing, the tasks still cannot be done in time. Going to the court first skips the work of prioritizing with the parent and may prove unnecessary.

D **Incorrect** (trap: Hands off too soon). Feeling overwhelmed by nine tasks is an expected reaction to an unordered plan, not a separate problem that needs referral. Adding another service lengthens the list before the social worker has helped organize it.

Takeaway: Partialize an overwhelming service plan with the client, ordering tasks by their link to the safety concern before seeking plan changes.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, partializing problems and prioritizing goals; NASW Code of Ethics (2021), 1.02
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317. ASM-1606 (rev 1) · Best answer: C

A social worker providing case management at an HIV services agency works with a client who also sees a primary care physician, a psychiatrist and an outpatient substance use program. The client arrives upset after receiving three different instructions about sleep medication timing and appointment days and says, 'Everyone tells me something different.' The client has signed releases for all providers. Which action is **MOST** helpful?

A. Help the client decide which provider's advice to follow
B. Write every instruction on one calendar for the client
C. Convene the providers to agree on one coordinated approach

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Asking the client to choose among clinical instructions places a provider problem on the person least equipped to judge it. The contradictions, which include medication timing, need to be settled by the providers themselves.

B **Incorrect** (trap: Misuses the technique). A single calendar helps with complex schedules and may be useful once the plan is unified. Recording contradictory instructions side by side does not resolve them and leaves the client unsure which to follow.

C **Correct.** Case management coordinates services so that providers work from one plan and care stays continuous (NASW Standards for Social Work Case Management, Standards 6 and 8). The conflicting instructions come from the providers, not the client, and the signed releases allow the social worker to bring them together. It beats B because a calendar organizes the advice but leaves the contradictions in place, so the client would still be following conflicting directions about medication and appointments.

Takeaway: When several providers give conflicting directions, the case manager coordinates them into one plan rather than leaving the client to sort it out.

Sources: NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring; Standard 8: Interdisciplinary and Interorganizational Collaboration
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318. ASM-1607 (rev 2) · Best answer: A

A 16-year-old and a parent come to a family service agency for help. The parent wants the plan to focus on raising the teen's grades. The teen wants the parent to 'stop yelling.' Both say that evenings at home have become tense and that they want more peace. What is the **MOST** appropriate way to begin setting goals?

A. Start with the goal they share: calmer evenings
B. Focus on grades first, since the parent consents to care
C. Start with the teen's goal of less yelling in the evenings

Why each option is right or wrong

A **Correct.** Goals are set collaboratively, and in family work a goal that every member endorses builds the joint commitment that later goals need (Hepworth et al.; NASW Code 1.02). Both people already agree that evenings are tense and want calm, so that shared goal unites them, and work on yelling and on homework routines can fit inside it. It beats C because opening with only the teen's goal aligns the social worker with one side and risks losing the parent, whose cooperation the plan needs.

B **Incorrect** (trap: Misreads law or ethics). A parent's consent to treatment does not make the parent's goal the plan's only focus. Opening with grades sidelines the teen, who must take part for anything to change, and signals that the social worker has taken the parent's side.

C **Incorrect** (trap: Boundary slip or collusion). The teen's concern matters, but leading with it casts the parent as the problem and invites the parent to disengage. Both concerns can be addressed within the goal the two already share.

Takeaway: In family goal setting, begin with a goal all members share to build joint commitment before addressing each person's own goal.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, negotiating goals with families; NASW Code of Ethics (2021), 1.02
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319. ASM-1608 (rev 1) · Best answer: D

The ASAM Criteria (4th edition) rate a person's needs on six dimensions. Which dimension covers how safe and supportive the person's home is, such as roommates who use?

A. Substance use-related risks
B. Person-centered considerations
C. Intoxication, withdrawal, and addiction medications
D. Recovery environment interactions

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Dimension 4 rates the person's own likelihood of continued use and its related harms, which may rise when others at home use. The question asks about the living situation itself, which belongs to a different dimension.

B **Incorrect** (trap: Wrong model or concept). Dimension 6, person-centered considerations, covers the person's preferences, barriers to care and readiness, used in shared decision-making about placement. The safety and support of the home is a separate dimension.

C **Incorrect** (trap: Wrong model or concept). Dimension 1 covers intoxication, withdrawal risk and the need for addiction medications. Substance use by others in the home raises relapse risk but says nothing about the person's current withdrawal state.

D **Correct.** In the ASAM Criteria (4th ed.), Dimension 5, recovery environment interactions, covers how well the person functions in the current living setting, how safe and supportive it is and how the people in it view substance use (ASAM, 2023). Returning to a home where others use is a classic Dimension 5 concern. It beats A

because Dimension 4, substance use-related risks, rates the person's own likelihood of continued use and harm, not the setting itself.

Takeaway: ASAM placement is multidimensional; the safety and support of the home, including others' substance use, fall under the recovery environment dimension.

Sources: ASAM (2023). The ASAM Criteria (4th ed.), Dimension 5: Recovery Environment Interactions
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320. ASM-1683 (rev 1) · Best answer: A

A parent calls an outpatient clinic about a 16-year-old on its 3-week waitlist. The teen has posted goodbye messages online and given away a prized gaming system. When the parent asked directly, the teen said they have been saving pills to take this weekend. The teen is at home now. What should the intake social worker do **FIRST**?

A. Have the parent call emergency services now and stay with the teen
B. Book the teen into the clinic's first urgent opening tomorrow morning
C. Complete a full suicide risk screening with the parent over the phone

Why each option is right or wrong

A **Correct.** Triage sorts by urgency, and a stated plan, gathered means, a time frame and goodbye behavior make this emergent rather than urgent or routine. The parent has already asked directly and the teen confirmed saving pills, so the key risk facts are known. Having the parent call emergency services now, while staying with the teen, matches the response to the danger; crisis systems exist to respond in person to this level of risk (SAMHSA, 2020). It beats C because a longer phone screening would delay help the known facts already call for.

B **Incorrect** (trap: Ignores risk). An urgent next-day appointment fits risk without a plan or means. Here the teen has a plan, saved pills and a weekend time frame, so waiting until tomorrow leaves the danger unaddressed tonight.

C **Incorrect** (trap: Right action, wrong time). Structured screening is the right move when risk is unclear. The parent's direct question has already surfaced a plan, means and timing, so more screening by phone now delays emergency care.

Takeaway: Triage a suicide plan with means and a time frame as emergent: arrange immediate emergency help rather than an expedited appointment.

Sources: SAMHSA (2020) National Guidelines for Behavioral Health Crisis Care: Best Practice Toolkit, Core elements: someone to respond (mobile crisis)
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321. ASM-1684 (rev 1) · Best answer: D

At a first treatment-planning meeting at a community mental health center, a 20-year-old discharged from a psychiatric unit 2 weeks ago says the only goal is to return to college full time next month. The client adds that this 'matters more than anything.' The discharge summary lists steady sleep and medication routines as early priorities. What should the social worker do **FIRST**?

A. Explain that full-time enrollment next month is too soon
B. Record the college goal in the plan exactly as the client stated it
C. Break the goal into steps, such as one course next term
D. Ask what attending college full time would mean to the client

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Feasibility matters, but declaring the goal unrealistic at the first meeting is directive and risks the alliance. Concerns about timing are better raised together once the client's reasons are understood.

B **Incorrect** (trap: Good but not best). Recording the goal as stated honors the client's wishes, but it skips understanding and shaping the goal together. A plan built this way may set the client up for an early setback.

C **Incorrect** (trap: Right action, wrong time). Partializing is likely the right next step and may lead to a single course as a first objective. Proposed before exploring why the goal matters, it can read as the worker overriding the client's aim.

D **Correct.** Goals are negotiated with the client, and understanding what a goal means comes before narrowing or partializing it (Hepworth et al.). The client's strong investment signals that college carries weight, perhaps hope or proof of recovery, that the plan can build on, for example by linking sleep and medication routines to the client's own aim. Asking what attending would mean starts there; recovery planning is person-driven (SAMHSA, 2012). It beats C because breaking the goal into steps is a sound next move, but offered first it can feel like a downgrade before the client feels understood.

Takeaway: In goal setting, first understand what a client's stated goal means to them, then shape and partialize it together.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals: selecting and defining goals with clients; SAMHSA (2012) Working Definition of Recovery, Guiding principles: recovery is person-driven

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322. ASM-1685 (rev 2) · Best answer: B

An emergency department social worker assesses a 38-year-old whose partner brought them in after they said, 'Everyone would be better off without me,' following a job loss. When asked directly, the patient describes passive wishes to be dead but no intent or plan, has no firearms at home, and has never attempted suicide. The partner will stay home this week, and the patient has an established outpatient mental health provider. What is the **BEST** plan?

A. Admit the patient to the inpatient psychiatric unit for observation
B. Discharge home with a written safety plan and a therapy visit within 2 days
C. Keep the patient in the emergency department overnight for reassessment
D. Refer the patient to a partial hospitalization program next week

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Inpatient admission fits intent, a plan, a recent attempt or an inability to stay safe. None is present, so admission would be more restrictive than the risk requires and could disrupt the supports already in place.

B **Correct.** Level of care should be the least restrictive setting that can keep the person safe. The direct assessment shows passive ideation without intent, plan, firearm access or prior attempts, plus a supportive partner and an established outpatient provider. These facts support discharge with a collaborative safety plan, a brief intervention designed for emergency settings, and prompt follow-up (Stanley & Brown, 2012). Without serious, foreseeable and imminent risk, the patient's own choices carry weight (NASW Code 1.02). It beats D because partial hospitalization adds intensity the risk does not require and starts later than the existing provider can.

C **Incorrect** (trap: Good but not best). An overnight hold for reassessment adds little when the direct assessment is complete and protective factors are clear. Extended observation suits intoxication or an unclear history, not this picture.

D **Incorrect** (trap: Good but not best). Partial hospitalization suits people who need structured daily treatment but not 24-hour care. Here risk is lower, an outpatient provider is already in place, and waiting a week delays follow-up.

Takeaway: Match level of care to assessed risk: with no intent or plan and solid supports, outpatient follow-up with a safety plan is least restrictive.

Sources: Stanley, B., & Brown, G. K. (2012). Safety planning intervention: A brief intervention to mitigate suicide risk. Cognitive and Behavioral Practice, 19(2), 256-264, whole article; NASW Code of Ethics (2021), 1.02
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323. ASM-0117 (rev 1) · Best answer: A

A 4-year-old is referred for frequent tantrums and hitting at home and preschool. The caregiver, who benefited from cognitive behavioral therapy for anxiety, asks whether the preschooler could receive the same kind of individual talk therapy. Which modality is **MOST** appropriate for this preschooler?

A. Parent-child interaction therapy
B. Individual cognitive therapy with thought records
C. Weekly social skills group with peers

Why each option is right or wrong

A **Correct.** Modality should match the client's developmental level. A 4-year-old is in Piaget's preoperational stage and lacks the abstract reasoning needed for cognitive restructuring, and disruptive behavior at this age responds best to caregiver-involved behavioral treatment such as parent-child interaction therapy (Kaminski & Claussen, 2017). It beats B because thought records require reading, writing and the ability to examine one's own thoughts, which a preschooler has not developed. The caregiver's success with CBT does not make it fit the child.

B **Incorrect** (trap: Wrong model or concept). Cognitive work with thought records suits older children, adolescents and adults who can reflect on their thinking. A preschooler cannot yet do this kind of abstract, written work, so the approach is mismatched to development.

C **Incorrect** (trap: Good but not best). A social skills group may help some young children, but it leaves out the caregiver, who is central to changing disruptive behavior at this age. Caregiver-involved treatment has the strongest evidence for preschool tantrums and aggression.

Takeaway: Match modality to developmental level: young children with disruptive behavior do best with caregiver-involved behavioral treatment, not cognitive restructuring.

Sources: Kaminski, J. W., & Claussen, A. H. (2017). Evidence base update for psychosocial treatments for disruptive behaviors in children. Journal of Clinical Child & Adolescent Psychology, 46(4), 477-499, whole article; Piaget, J., & Inhelder, B. (1969). The Psychology of the Child. Basic Books, preoperational stage

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324. ASM-1609 (rev 1) · Best answer: B

A school social worker meets with the parents of a 14-year-old recently diagnosed with anorexia nervosa. The pediatrician has found the teen medically stable for outpatient care. The teen lives at home, and the parents are close to the teen and eager to help. The social worker has no training in eating disorder treatment. Which plan is **BEST**?

A. Begin weekly individual counseling with the teen at school

B. Refer the teen to a family-based treatment provider

C. Recommend a residential eating disorder treatment program

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Individual counseling by a social worker without eating disorder training is outside competence, and it leaves out the parents, who are central to adolescent recovery. School support can complement specialized treatment once it is in place.

B **Correct.** Modality is matched to the client's developmental stage and living situation. For adolescents with anorexia nervosa who live with involved parents, family-based treatment, in which parents temporarily take charge of restoring healthy eating, is a recommended approach (APA, 2023; Lock & Le Grange, 2013). The social worker lacks eating disorder training, so referral to a trained provider fits the competence standard (NASW Code 1.04(a)). It beats C because the teen is medically stable for outpatient

care, and residential treatment would remove the teen from the parents who are central to recovery.

C **Incorrect** (trap: Good but not best). Residential care is for medical instability or for illness that has not responded to outpatient treatment. This teen is stable for outpatient care, so residential placement would be more restrictive than needed and would separate the teen from engaged parents.

Takeaway: For adolescents with anorexia nervosa and engaged parents, family-based treatment is a first-line modality; refer when it is outside your training.

Sources: American Psychiatric Association (2023). Practice Guideline for the Treatment of Patients With Eating Disorders, recommendations for adolescents with anorexia nervosa: family-based treatment; Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa: A Family-Based Approach (2nd ed.). Guilford Press, overview and phase 1: parents manage refeeding; NASW Code of Ethics (2021), 1.04(a)
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325. ASM-1610 (rev 1) · Best answer: C

A 28-year-old client with mild intellectual developmental disorder lives in a group home and feels anxious about riding the bus to a new job. The client reads at about a second-grade level, learns best by practicing tasks with staff and asks for help 'feeling less scared.' Which treatment approach is **MOST** appropriate?

A. Standard CBT using the agency's written workbook
B. Insight-oriented therapy exploring the fear's origins
C. CBT adapted with pictures, repetition and a support person

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). The model fits anxiety, but a written workbook assumes reading skills the client does not have. Using it unchanged is likely to frustrate the client and leave the coping skills unpracticed.

B **Incorrect** (trap: Wrong model or concept). Insight-oriented work relies on abstract reflection and extended verbal exploration, a poor fit for this client's learning style. The client's goal is practical, riding the bus with less fear, which calls for concrete, rehearsed skills.

C **Correct.** Modality is matched to the client's cognitive ability. Reviews of therapy for people with intellectual disabilities describe common adaptations: simpler language, repetition, visual materials and involvement of carers (Poku et al., 2026). This client's low reading level and preference for practicing with staff point to a pictured, rehearsal-based approach with a support person who can coach between sessions. It beats A because a text-heavy workbook assumes reading skills the client lacks, so the right model would fail in delivery.

Takeaway: Choose and adapt modalities to cognitive ability: concrete, visual, repeated and supported formats make evidence-based methods usable.

Sources: Poku, A. C., Gray, K. M., Hewitt, O., & Langdon, P. E. (2026). Adapting psychological therapies for individuals with intellectual disabilities: A systematic review. Clinical Psychology & Psychotherapy, 33(1), e70202, whole article

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326. ASM-1611 (rev 2) · Best answer: A

A hospice social worker visits an 86-year-old patient with advanced heart failure. The patient's mood is low, and the patient says, 'I keep going over the past. I want to know it all counted for something before I go.' The patient is alert and has a few months to live. Which approach is **MOST** helpful?

A. Guide a structured life review

B. Begin exposure therapy for death-related anxiety

C. Schedule pleasant activities to lift daily mood

Why each option is right or wrong

A **Correct.** Modality should fit life stage. Butler described the life review as a natural late-life process of revisiting the past that can bring resolution (Butler, 1963), and Erikson framed the final stage as integrity versus despair (Erikson, 1963). This patient is already reviewing the past and wants to find meaning, with only months to live, so a guided life review builds on what the patient is doing. It beats C because activity scheduling targets low mood in general but does not answer the patient's stated need for meaning.

B **Incorrect** (trap: Wrong model or concept). Exposure therapy targets fears that drive avoidance, such as phobias or trauma reminders. The patient is not avoiding thoughts of death but seeking meaning in the past, which calls for a different approach.

C **Incorrect** (trap: Good but not best). Scheduling pleasant activities can lift mood and may help as well, within the limits of advanced heart failure. It does not address the patient's request to make sense of a whole life, the central task at this stage.

Takeaway: Match modality to life stage: older adults near the end of life often benefit from guided life review aimed at integrity and meaning.

Sources: Butler, R. N. (1963). The life review: An interpretation of reminiscence in the aged. Psychiatry, 26(1), 65-76, whole article; Erikson, E. H. (1963). Childhood and Society (2nd ed.). W. W. Norton, eight ages: integrity versus despair

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327. ASM-1612 (rev 1) · Best answer: B

A juvenile court asks a social worker to start a weekly group for eight 14- and 15-year-olds on probation for fighting and theft. The court says a group will serve more youth at lower cost. Most of the youth already spend time together, and several have parents who are willing to take part in services. What is the **BEST** recommendation?

A. Run the group with a structured anger-management curriculum
B. Offer family-based work instead of a peer group
C. Hold individual sessions focused on insight into behavior

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Structure and skill content help, but the format is the problem: placing teens on probation for fighting and theft together can teach and reward problem behavior. A peer group is a better fit for lower-risk youth.

B **Correct.** Development shapes modality. Adolescents are highly responsive to peers, and grouping youth with conduct problems can increase problem behavior through peer reinforcement, often called deviancy training (Dishion, McCord & Poulin, 1999). These youth already spend time together, which raises that risk, and willing parents make family-based work feasible. It beats A because even a structured curriculum still places high-risk teens together, and the cost savings do not offset the risk of worsening behavior.

C **Incorrect** (trap: Good but not best). Individual sessions avoid peer aggregation, but insight-focused talk is a weak fit for adolescent conduct problems and leaves out the parents who are ready to help. Family involvement offers more leverage over daily behavior.

Takeaway: Avoid grouping high-risk adolescents with conduct problems; peer aggregation can worsen behavior, so favor family-based approaches when parents can engage.

Sources: Dishion, T. J., McCord, J., & Poulin, F. (1999). When interventions harm: Peer groups and problem behavior. American Psychologist, 54(9), 755-764, whole article
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328. ASM-1686 (rev 1) · Best answer: A

A 20-month-old was present during several violent incidents between caregivers before the nonoffending caregiver moved with the toddler to a relative's home. Since then, the toddler startles at loud voices, wakes screaming most nights, and clings constantly. The caregiver is also on edge and struggles to soothe the toddler. Which treatment model is the **BEST** fit?

A. Child-parent psychotherapy
B. Parent-child interaction therapy
C. Trauma-focused cognitive behavioral therapy
D. Nondirective individual play therapy

Why each option is right or wrong

A **Correct.** Child-parent psychotherapy is a relationship-based treatment for children from birth to about age 5 who have faced trauma such as family violence (Lieberman & Van Horn, 2008). It treats the toddler and caregiver together, helping the caregiver understand and respond to the child's fear while attending to the caregiver's own trauma. The age, the shared exposure and the caregiver's trouble soothing all point here. It beats B, the strongest competitor, because parent-child interaction therapy coaches parenting skills for disruptive behavior in somewhat older children rather than working through trauma in the relationship (McNeil & Hembree-Kigin, 2010).

B **Incorrect** (trap: Wrong model or concept). This model coaches caregivers live to strengthen the relationship and manage disruptive behavior, usually with preschoolers (McNeil & Hembree-Kigin, 2010). It does not center on trauma or the caregiver's own reactions, and this toddler is younger than its usual range.

C **Incorrect** (trap: Wrong model or concept). Trauma-focused CBT is a strong choice for traumatized children from about age 3 (Cohen, Mannarino & Deblinger, 2017). It relies on coping skills and a trauma narrative that a 20-month-old cannot yet use, even with caregiver involvement.

D **Incorrect** (trap: Wrong model or concept). Individual play therapy works with the child alone and leaves out the caregiver relationship, which is the toddler's main source of safety. At this age, treating the caregiver and child together fits better.

Takeaway: For infants and toddlers exposed to trauma, choose a dyadic treatment such as child-parent psychotherapy that works through the caregiver relationship.

Sources: Lieberman, A. F., & Van Horn, P. (2008). Psychotherapy with Infants and Young Children: Repairing the Effects of Stress and Trauma on Early Attachment. Guilford Press, child-parent psychotherapy: population and approach; McNeil, C. B., & Hembree-Kigin, T. L. (2010). Parent-Child Interaction Therapy (2nd ed.). Springer, overview of PCIT; Cohen, Mannarino & Deblinger (2017). Treating Trauma and Traumatic Grief in Children and Adolescents (2nd ed.). Guilford Press, TF-CBT: appropriate ages

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329. ASM-1687 (rev 1) · Best answer: C

Staff at a nursing facility ask the social worker to begin weekly talk therapy for a resident with moderate Alzheimer's disease. The resident yells at and pushes away aides several times a week, usually during morning bathing. The resident cannot recall conversations from earlier in the day. What is the **MOST** appropriate approach?

A. Hold weekly individual sessions exploring the resident's frustrations

B. Ask the physician to consider medication to calm the resident

C. Help caregivers adjust the routine that sets off the distress

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Reflective sessions depend on remembering and building on earlier conversations. With moderate Alzheimer's disease, any gains would not carry over to the bathing routine where the distress actually happens.

B **Incorrect** (trap: Right action, wrong time). Medication may be considered for severe or dangerous symptoms, but antipsychotics carry a boxed warning of increased

mortality in older adults with dementia-related psychosis (Kaplan & Sadock's Synopsis). Behavioral and environmental changes come first here.

C **Correct.** Modality should match the client's abilities. With moderate memory loss, the resident cannot carry insights from one session to the next, so a caregiver-delivered behavioral approach fits better. Outbursts cluster around morning bathing, and an expert panel recommends describing such patterns and having caregivers adjust routines and surroundings, generally before turning to medication (Kales, Gitlin & Lyketsos, 2014). It beats A because weekly talk sessions rely on recall and reflection the resident no longer has, while aides are present at the moment the distress happens.

Takeaway: Choose interventions that fit cognitive ability; for agitation in moderate dementia, caregiver-delivered changes to triggers come before talk therapy or medication.

Sources: Kales, H. C., Gitlin, L. N., & Lyketsos, C. G. (2014). Management of neuropsychiatric symptoms of dementia in clinical settings: Recommendations from a multidisciplinary expert panel. Journal of the American Geriatrics Society, 62(4), 762-769, DICE approach; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Antipsychotics: boxed warning in older adults with dementia-related psychosis
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330. ASM-1613 (rev 1) · Best answer: C

A 24-year-old client is in treatment for depression at a college counseling center. The client lives with parents and grandparents, as the family's culture expects until marriage, and describes the arrangement as supportive. While reviewing a draft plan, the social worker notes that the center's template lists 'move to independent housing' as a standard goal for clients this age. How should the social worker handle this goal?

A. Keep the goal, since leaving home fits this life stage
B. Ask the client privately whether the family applies pressure
C. Replace it with goals the client defines within the family

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Developmental views that center early adulthood on leaving home reflect individualistic norms. Applied to a client whose culture expects multigenerational living, the goal treats a supportive arrangement as a problem and is unlikely to be owned by the client.

B **Incorrect** (trap: Skips assessment). Checking for family pressure makes sense when a client seems to defer or seems distressed. This client has already described the arrangement as supportive, so probing for coercion treats the family's custom as a likely problem.

C **Correct.** Plans reflect the client's own goals within the client's culture, and social workers recognize the strengths in all cultures (NASW Code 1.05(a), 1.02). This client describes multigenerational living as expected and supportive, so a goal of moving out imposes a norm the client does not share and could add family strain. Goals such as more social contact or steadier study routines can be built within the household. It beats A because independent living is one cultural pattern of adulthood, not a developmental requirement.

Takeaway: Build plan goals from the client's cultural definition of healthy functioning, not from one culture's developmental norm.

Sources: NASW Code of Ethics (2021), 1.05(a); NASW Code of Ethics (2021), 1.02
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331. ASM-1614 (rev 1) · Best answer: B

A client is referred to a mixed-gender trauma recovery group after a sexual assault. She tells the social worker that her faith forbids discussing sexual matters in the presence of men, and she wants trauma treatment. The agency also runs a trauma group for women that has openings. How should the social worker adjust the plan?

A. Encourage her to try the mixed group for a few weeks
B. Switch her referral to the female survivors' group
C. Ask the mixed group to agree to avoid sexual details

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Pressing her to try the mixed group asks her to act against her faith and treats a religious value as an obstacle to overcome. It may lead her to drop out of trauma care altogether.

B **Correct.** Trauma-informed care attends to cultural and gender issues and offers gender-responsive services (SAMHSA, 2014), and social workers provide culturally informed services (NASW Code 1.05(b)). For this client, discussing sexual matters with men present conflicts with her faith, so a mixed group would block the disclosure that trauma work requires. The group for women offers the same treatment in a format that fits her values, and it has openings. It beats C because limiting what members may discuss narrows everyone's recovery work and still leaves the client in a setting her faith forbids.

C **Incorrect** (trap: Misuses the technique). Restricting what other members may discuss changes the group's purpose for everyone and still places the client in a mixed setting. Adjusting her placement respects her value without limiting others' recovery.

Takeaway: When a cultural or religious value conflicts with a service format, adapt the format so effective care stays accessible.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: Cultural, Historical, and Gender Issues; NASW Code of Ethics (2021), 1.05(b)
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332. ASM-1615 (rev 1) · Best answer: C

A client raised in a rural Latin American community is in outpatient treatment for depression. The client believes the illness began with susto, a loss of spirit caused by a frightening event. The client plans to visit a traditional healer for a cleansing and asks the social worker whether that is 'allowed' alongside counseling. What should the social worker do **FIRST**?

A. Refer the client to a provider who shares the client's culture
B. Contact the healer to coordinate care with counseling
C. Ask how the client hopes the healer's work will help
D. Explain that depression is a treatable medical condition

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). A culture-matched provider is not needed to respect the client's beliefs, and transferring would disrupt care that is underway. The current social worker can bring the client's healing practice into the plan.

B **Incorrect** (trap: Right action, wrong time). Coordinating with the healer may help later, with the client's consent. Doing it first skips learning what the practice involves and what the client wants from it, and any contact outside the treatment team needs the client's permission.

C **Correct.** Culturally responsive planning starts with the client's explanatory model and the help the client is seeking, including from traditional healers (DSM-5-TR Cultural Formulation Interview). Asking how the client hopes the healer will help answers the 'allowed' question collaboratively and shows what the practice involves, so it can be woven into the plan. Engaging natural helpers such as folk healers is part of culturally competent practice (NASW Cultural Competence Standard 4). It beats B because coordinating with the healer is a reasonable later step that needs the client's understanding and consent first.

D **Incorrect** (trap: Too directive). Leading with a biomedical explanation dismisses the client's understanding of the illness and may close the conversation. The client's model can coexist with counseling; correcting it is not the goal.

Takeaway: Begin culturally responsive planning by eliciting the client's explanatory model and the help sought, then integrate traditional healing with consent.

Sources: DSM-5-TR (APA, 2022), Section III, Culture and Psychiatric Diagnosis: Cultural Formulation Interview; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 4: Cross-Cultural Skills

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333. ASM-1688 (rev 1) · Best answer: B

An agency is planning a caregiver stress group for adult children caring for parents with dementia. Most of the caregivers who have enrolled speak Cantonese and have limited English. The agency's standard curriculum is written and taught in English. Which planning choice is **MOST** likely to make the group effective?

A. Run the group in English with a trained interpreter present
B. Have a Cantonese-speaking facilitator lead the group in Cantonese
C. Translate handouts for Cantonese-speaking members and teach in English

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A trained interpreter is the right fallback when no bilingual facilitator is available. In a support group, though, it keeps members talking through a third party and gives up the stronger effect of native-language delivery.

B **Correct.** Culturally adapted interventions outperform unadapted ones, and in a meta-analysis, interventions delivered in clients' native language (when it was not English) were about twice as effective as those delivered in English (Griner & Smith, 2006). A facilitator leading the group in Cantonese lets caregivers speak with each other directly, which is central to group support. It beats A because a trained interpreter provides access and is the right fallback when no bilingual facilitator exists, but members would then talk through a third party (NASW Standards for Cultural Competence, Standard 9).

C **Incorrect** (trap: Misuses the technique). Translated handouts help, but teaching in English still leaves caregivers with limited English unable to follow or join the discussion. It adapts the materials without adapting the group itself.

Takeaway: When planning services for clients with limited English, delivering the intervention in their native language improves effectiveness beyond interpretation or translated materials.

Sources: Griner, D., & Smith, T. B. (2006). Culturally adapted mental health interventions: A meta-analytic review. Psychotherapy: Theory, Research, Practice, Training, 43(4), 531-548, abstract: native-language effect; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication

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334. ASM-1689 (rev 1) · Best answer: C

A family that immigrated 4 years ago seeks help because a 15-year-old and the parents argue daily. The teen, now fluent in English, wants to spend evenings with friends and attend parties. The parents see this as rejecting family values and say the teen is 'becoming a stranger.' The teen's grades remain steady. Which intervention plan is **BEST**?

A. Individual sessions to help the teen manage anger with family
B. Parent training in setting consistent limits and consequences
C. Family sessions helping members value skills from both cultures
D. Guidance urging the parents to allow freedoms typical for local teens

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Individual anger work places the problem in the teen alone. The conflict is relational and cultural, and steady grades point away from an individual disorder, so working with the whole family fits better.

B **Incorrect** (trap: Wrong model or concept). Consistent limits help many families, but this approach treats the conflict as a parenting-skills deficit. It does not address the clash in cultural expectations that drives the daily arguments.

C **Correct.** Culture shapes the plan when the conflict itself grows from different rates of acculturation. Here the teen has taken on new norms faster than the parents, and both sides frame the fight in cultural terms. Bicultural Effectiveness Training was developed for this pattern; it helps family members see the skills of each culture as complementary rather than opposed, easing intergenerational conflict (Szapocznik et al., 1984). It beats B, the strongest competitor, because limit-setting training treats the problem as a discipline gap and can sharpen a values clash the family has not yet talked through.

D **Incorrect** (trap: Too directive). Advising the parents to adopt local norms imposes the worker's values and dismisses the family's culture. It would likely deepen mistrust rather than help parents and teen understand each other.

Takeaway: When family conflict stems from an acculturation gap, plan family work that bridges both cultures rather than treating one member or culture as the problem.

Sources: Szapocznik, J., Santisteban, D., Kurtines, W., Perez-Vidal, A., & Hervis, O. (1984). Bicultural Effectiveness Training: A treatment intervention for enhancing intercultural adjustment in Cuban American families. Hispanic Journal of Behavioral Sciences, 6(4), 317-344, whole article

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335. ASM-0009 (rev 1) · Best answer: A

An adult client with bipolar disorder takes lithium. At a home visit, the client reports 2 days of vomiting and diarrhea from a stomach virus. The social worker notices a coarse hand tremor and an unsteady walk that were absent last week. What should the social worker do **FIRST**?

A. Help the client get emergency medical care today

B. Advise the client to skip lithium until the illness passes

C. Recheck the client's symptoms at next week's home visit

Why each option is right or wrong

A **Correct.** Vomiting and diarrhea can dehydrate a person taking lithium and raise the blood level. A new coarse tremor and unsteady gait are warning signs of lithium toxicity, which can progress to confusion, seizures and death (Gitlin, 2016). Suspected toxicity needs same-day medical evaluation, including a blood level. It beats B because the social worker does not direct medication changes. Holding a dose is the prescriber's decision, and it would not treat the toxicity that may already be present.

B **Incorrect** (trap: Outside the social work role). Telling a client to hold a medication is a prescribing decision outside the social work role. It also overlooks that the current symptoms already suggest a dangerous lithium level needing urgent testing.

C **Incorrect** (trap: Ignores risk). Waiting a week ignores signs of possible toxicity, which can worsen quickly. Monitoring suits stable side effects, not new neurological symptoms during dehydration.

Takeaway: Dehydration plus new coarse tremor or unsteadiness in a client on lithium suggests toxicity; get emergency medical care the same day.

Sources: Gitlin, M. (2016). Lithium side effects and toxicity: prevalence and management strategies. International Journal of Bipolar Disorders, 4, 27, toxicity: signs and risk factors

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336. ASM-1066 (rev 1) · Best answer: D

An adult began aripiprazole 10 days ago for psychotic symptoms. At an outpatient visit, the client cannot stay seated, shifts from foot to foot and says, 'I feel like I have to keep moving inside.' The client is distressed by the feeling. Clinic staff call it anxiety about the new treatment. Which explanation is **MOST** likely?

A. Generalized anxiety disorder
B. Tardive dyskinesia
C. Worsening psychotic agitation
D. Akathisia

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Anxiety can cause restlessness, but generalized anxiety disorder requires months of excessive worry. A new urge to move that began within days of an antipsychotic points to a medication effect.

B **Incorrect** (trap: Near-miss diagnosis). Tardive dyskinesia involves involuntary movements, often of the mouth, tongue or face, that develop after months or years of treatment. This restlessness started within 10 days and is felt as an inner urge.

C **Incorrect** (trap: Near-miss diagnosis). Psychotic agitation would come with worsening hallucinations, suspiciousness or disorganization. Here the distress centers on a physical urge to move, a known side effect of the new medicine.

D **Correct.** Akathisia is a medication-induced movement disorder: an inner sense of restlessness with an urge to move, such as pacing or shifting weight, that usually begins within the first weeks of starting or raising an antipsychotic. Aripiprazole is commonly linked to it. It is often mistaken for anxiety or agitation and is distressing enough to raise suicide and nonadherence risk, so the prescriber should be told promptly. It beats A because anxiety does not explain new motor restlessness that began days after the medication (DSM-5-TR).

Takeaway: Inner restlessness and an urge to move soon after starting an antipsychotic suggest akathisia; inform the prescriber promptly.

Sources: DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Medication-Induced Acute Akathisia

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337. ASM-1067 (rev 2) · Best answer: A

A client who has taken sertraline for a year begins tramadol after dental surgery. The next day, at a scheduled clinic visit, the client is agitated and sweating heavily, with diarrhea, a racing heart and jerking muscles in both legs. The client thinks it is a stomach bug. What should the social worker do **FIRST**?

A. Have the client seen in an emergency department now
B. Call the prescriber's office to report the new medicine
C. Advise the client to stop the tramadol and rest at home

Why each option is right or wrong

A **Correct.** Serotonin syndrome can follow when serotonergic drugs are combined, as when tramadol is added to an SSRI. It typically starts within hours to a day and brings agitation, sweating, diarrhea, rapid heart rate and muscle jerking (clonus), especially in the legs. It can become life threatening, so it needs emergency care now. It beats B because a call to the prescriber's office delays treatment of a possible medical emergency; the prescriber can be informed once the client is safe (Boyer & Shannon, 2005).

B **Incorrect** (trap: Ignores risk). The prescriber should learn of the new medication, and calling the office is right for routine side effects. With agitation, heavy sweating, a racing heart and muscle jerking a day after adding tramadol, waiting for a callback is unsafe.

C **Incorrect** (trap: Outside the social work role). Stopping the added drug is likely part of treatment, which makes this tempting. But telling a client to stop a prescribed medicine is a medical decision outside the social work role, and resting at home leaves agitation, a racing heart and muscle jerking unmonitored when the syndrome can worsen quickly.

Takeaway: Agitation, sweating, diarrhea and muscle jerking soon after adding a serotonergic drug suggest serotonin syndrome and need emergency care.

Sources: Boyer, E. W., & Shannon, M. (2005). The serotonin syndrome. New England Journal of Medicine, 352(11), 1112-1120, whole article

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338. ASM-1068 (rev 2) · Best answer: B

A school social worker meets with the parent of a 15-year-old who has taken fluoxetine for 3 months. The teen's mood and attendance have improved, and the teen reports no suicidal thoughts at regular check-ins. The parent just read about the antidepressant warning for youth and plans to stop the medicine tonight. What is the social worker's **BEST** response?

A. Support the parent's decision to stop the medicine tonight
B. Encourage the parent to talk with the prescriber before stopping
C. Assure the parent that the risk is limited to the first weeks

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Parents decide about a minor's treatment, and this concern is valid. Endorsing an abrupt stop without the prescriber, though, risks relapse in a teen who is improving and bypasses medical guidance.

B **Correct.** Antidepressants carry an FDA boxed warning about increased suicidal thinking in children, adolescents and young adults, which calls for close monitoring, especially early in treatment and after dose changes, rather than automatic discontinuation (Hammad et al., 2006; Kaplan & Sadock). Stopping a medicine that is working risks relapse, and medication changes belong with the prescriber. Encouraging a prompt talk with the prescriber respects the parent's authority while keeping the decision informed. It beats C because reassurance about medication risk is medical advice outside the social work role.

C **Incorrect** (trap: Outside the social work role). Risk is highest early in treatment and after dose changes, but offering medical reassurance oversteps the social work role. The prescriber should answer the parent's question about this teen's risk.

Takeaway: A youth antidepressant boxed warning calls for close monitoring; families bring stopping decisions to the prescriber first.

Sources: Hammad, T. A., Laughren, T., & Racoosin, J. (2006). Suicidality in pediatric patients treated with antidepressant drugs. Archives of General Psychiatry, 63(3), 332-339, whole article; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Antidepressants: boxed warning on suicidality in youth
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339. ASM-1069 (rev 2) · Best answer: A

An adult with schizophrenia who has been stable on clozapine moves into a smoke-free group residence and stops smoking 1.5 packs a day all at once. Five days later, staff report that the client is much sleepier, drooling more and walking unsteadily. What is the **MOST** important action for the social worker?

A. Tell the prescriber today that the client quit tobacco
B. Arrange nicotine replacement to ease the client's withdrawal
C. Assess for withdrawal symptoms since the client quit smoking

Why each option is right or wrong

A **Correct.** Tobacco smoke speeds the breakdown of clozapine by inducing the liver enzyme CYP1A2, so stopping smoking suddenly can raise clozapine blood levels sharply within days. Increased sleepiness, drooling and unsteadiness after quitting suggest rising levels, which can progress to seizures. The prescriber needs to know today to check levels and adjust the dose. It beats B because nicotine replacement eases withdrawal but does not affect the enzyme, so it would not lower the clozapine level (Lucas & Martin, 2013).

B **Incorrect** (trap: Good but not best). Nicotine replacement is reasonable support for quitting, but nicotine itself does not induce the enzyme that smoke does. It would not correct a rising clozapine level, the likely cause of these signs.

C **Incorrect** (trap: Near-miss diagnosis). Nicotine withdrawal causes irritability, restlessness, poor sleep and craving, not heavy sleepiness, drooling and unsteady gait. Assessing withdrawal alone delays telling the prescriber about a likely drug-level change.

Takeaway: Abruptly stopping smoking can raise clozapine levels; report quitting and new sedation to the prescriber promptly.

Sources: Lucas, C., & Martin, J. (2013). Smoking and drug interactions. Australian Prescriber, 36(3), 102-104, Antipsychotics: clozapine

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340. ASM-1070 (rev 1) · Best answer: C

A client with depression has taken sertraline for 6 months with partial benefit and also takes an oral contraceptive. In session, the client shows a bottle of St. John's wort bought that morning on a friend's advice and plans to start it tonight. The next prescriber visit is in 3 weeks. What is the social worker's **BEST** response?

A. Support the plan, since herbal remedies are the client's choice

B. Explore the appeal of herbs and revisit at the prescriber visit

C. Urge a call to the prescriber or pharmacist before the first dose

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Clients may choose complementary remedies, and that choice deserves respect. Endorsing it without flagging interaction risks with sertraline and the contraceptive, however, leaves the client deciding without key safety information.

B **Incorrect** (trap: Right action, wrong time). Understanding why herbal remedies appeal is worthwhile and can continue. Waiting for the visit in 3 weeks, though, lets the client start a combination that can raise serotonin dangerously and weaken contraception.

C **Correct.** St. John's wort is a common alternative remedy, but combined with antidepressants such as sertraline it can cause a dangerous rise in serotonin, and it weakens many prescription drugs, including birth control pills (NCCIH, 2017). Because the client plans to start tonight and the prescriber visit is 3 weeks away, the interaction question needs a prescriber or pharmacist now. It beats B because exploring the client's interest, by itself, lets a risky combination begin before anyone with prescribing expertise weighs in. The client still makes the final, informed choice.

Takeaway: Before a client adds an herbal remedy to prescribed medicines, link them promptly with the prescriber or pharmacist to check interactions.

Sources: National Center for Complementary and Integrative Health (2017). St. John's Wort and Depression: In Depth, Safety

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341. ASM-1071 (rev 2) · Best answer: A

In an integrated primary care clinic, an adult taking escitalopram for depression reports feeling much better after 2 months. Pharmacy records show doses are often skipped on weekends. When the social worker asks about the missed doses, the client hesitates and says, 'It's kind of personal.' What should the social worker do **NEXT**?

A. Ask plainly whether sexual side effects play a part
B. Notify the prescriber that the client skips doses
C. Remind the client that daily doses keep the mood stable

Why each option is right or wrong

A **Correct.** Sexual side effects are among the most common reasons people skip or stop SSRIs, and brief weekend breaks from some SSRIs can ease them, which makes weekend-only skipping a telling pattern (Rothschild, 1995). Clients often do not raise the topic unprompted, so after a hesitant 'it's personal,' a plain, nonjudgmental question makes disclosure easier. It beats B because the prescriber can do more once the reason is known, which opens options such as a dose or medication change (Kaplan & Sadock).

B **Incorrect** (trap: Right action, wrong time). The prescriber should hear about missed doses, and this step will likely follow. Reporting before the reason is known gives the prescriber little to act on and skips the client's chance to explain.

C **Incorrect** (trap: Too directive). Education about steady dosing has value, but repeating it before understanding the reason for skipping can feel like a lecture and may close off disclosure of an embarrassing side effect.

Takeaway: When a client hesitates about skipped antidepressant doses, ask plainly about sexual side effects before reporting or educating.

Sources: Rothschild, A. J. (1995). Selective serotonin reuptake inhibitor-induced sexual dysfunction: Efficacy of a drug holiday. American Journal of Psychiatry, 152(10), 1514-1516, whole article; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), SSRIs: adverse effects, sexual dysfunction
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342. ASM-1075 (rev 2) · Best answer: C

In an integrated primary care clinic, a client with depression and chronic back pain reported passive thoughts of death 3 weeks ago. At intake, the client agreed to the clinic's team-based care, in which providers share clinical information. The physician has just started amitriptyline and, to save on copays, prescribed a 90-day supply. The client asks the social worker not to 'make a big deal' of the earlier thoughts. What is the social worker's **BEST** response?

A. Coach the client to keep the pills locked away at home
B. Respect the request and track mood at follow-up
C. Alert the client's doctor to the overdose danger

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Locked storage is sound lethal-means advice and can be part of the plan. By itself, though, it leaves the client in control of 90 days of a drug that can be fatal in overdose. Only the prescriber can shorten the supply or choose another drug.

B **Incorrect** (trap: Ignores risk). The client's wish deserves a respectful hearing, and mood should be tracked. Waiting, however, leaves a large supply of a highly toxic medicine with someone who recently had thoughts of death, a risk the prescriber is not yet weighing.

C **Correct.** Tricyclic antidepressants such as amitriptyline are far more dangerous in overdose than newer antidepressants, mainly because of effects on heart rhythm (Kaplan & Sadock). For a client with recent thoughts of death, a 90-day supply matters. The prescriber can limit the quantity or change the drug; the social worker cannot. The client agreed to team-based care, and HIPAA permits sharing with a treating provider (45 CFR 164.506(c)); the worker still tells the client openly what will be shared and why. It beats A because locked storage still leaves a potentially fatal quantity in the client's hands.

Takeaway: When a client with recent suicidal thoughts receives a large supply of a drug that is lethal in overdose, alert the prescriber.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Tricyclic and tetracyclic antidepressants: overdose and cardiac toxicity; HIPAA Privacy Rule, 45 CFR 164.506(c)
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343. ASM-1076 (rev 1) · Best answer: B

An 81-year-old client with mild memory problems lives with an adult child. During a home visit, the adult child reports that over the past 4 days the client has become confused at night, has trouble passing urine and complains of a dry mouth. Last week the client began taking an over-the-counter nighttime pain reliever that contains diphenhydramine. What is the **MOST** likely cause of the change?

A. Progression of a neurocognitive disorder
B. Anticholinergic effects of the new product
C. Onset of a late-life major depressive episode
D. Normal age-related cognitive decline

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Neurocognitive disorders usually progress gradually over months. A change over 4 days, with urinary and mouth symptoms that began after a new product, points to a reversible drug effect that needs medical review.

B **Correct.** Diphenhydramine, common in over-the-counter sleep and nighttime pain products, is strongly anticholinergic. In older adults it can cause confusion or delirium, urinary retention and dry mouth, and the AGS Beers Criteria advise avoiding it in this age group (American Geriatrics Society, 2023). The timing and the bodily signs fit. It beats A because neurocognitive disorder progresses slowly and would not explain

urinary retention; an abrupt change in attention and awareness suggests delirium, which DSM-5-TR lists as a possible medication effect.

C **Incorrect** (trap: Near-miss diagnosis). Depression in later life can impair concentration and memory. It does not explain trouble passing urine or a dry mouth, and these changes started within days of a new medicine.

D **Incorrect** (trap: Near-miss diagnosis). Normal aging brings mild, slow changes in memory and processing speed. It does not produce nighttime confusion over a few days or urinary retention.

Takeaway: Sudden confusion in an older adult who recently started an anticholinergic product such as diphenhydramine suggests a drug effect needing medical review.

Sources: American Geriatrics Society Beers Criteria Update Expert Panel (2023). American Geriatrics Society 2023 updated AGS Beers Criteria for potentially inappropriate medication use in older adults. Journal of the American Geriatrics Society, 71(7), 2052-2081, First-generation antihistamines (diphenhydramine); DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium (medication-induced)

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344. ASM-1077 (rev 1) · Best answer: A

A 29-year-old client with bipolar I disorder has been stable for 2 years on valproate. At a community clinic visit, the client happily shares a plan with their partner to start trying for a baby next month. The client's next psychiatry appointment is in 4 months. What is the social worker's **BEST** response?

A. Encourage seeing the prescriber before trying to conceive

B. Suggest tapering off valproate before trying to conceive

C. Explore the client's hopes and worries about parenting

Why each option is right or wrong

A **Correct.** Valproate carries a high risk of birth defects, including neural tube defects, and of later cognitive problems in exposed children; ACOG advises avoiding it in pregnancy and recommends planning medication decisions before conception (ACOG, 2023). Exposure happens early, often before pregnancy is known, so the prescriber needs to be involved now rather than in 4 months. Encouraging an earlier visit supports the client's goal instead of questioning it. It beats C because exploring parenting hopes is worthwhile but leaves the medication risk unaddressed while conception is a month away.

B **Incorrect** (trap: Outside the social work role). Stopping or tapering a mood stabilizer is a prescriber's decision. Abrupt changes can trigger relapse of bipolar disorder, and the safest alternative depends on the client's history. The social worker can raise the concern but should not advise a medication change.

C **Incorrect** (trap: Good but not best). Talking about parenting hopes respects the client and can continue. Alone, it misses the urgent issue: a teratogenic medication and a conception plan that comes before the next psychiatry visit.

Takeaway: When a client on a teratogenic medication plans pregnancy, support the goal and link them to the prescriber before conception.

Sources: American College of Obstetricians and Gynecologists (2023). Clinical Practice Guideline No. 5: Treatment and Management of Mental Health Conditions During Pregnancy and Postpartum, Bipolar disorder: valproate; prepregnancy consultation
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345. ASM-1078 (rev 1) · Best answer: D

A psychiatric inpatient with schizophrenia had a haloperidol dose increase 5 days ago. Today the patient is confused and sweating heavily, with a temperature of 103°F. Nursing staff report that the patient's arms are stiff and hard to bend and that the blood pressure keeps swinging up and down. What is the **MOST** likely explanation?

A. Serotonin syndrome
B. Acute dystonic reaction
C. Worsening psychotic symptoms
D. Neuroleptic malignant syndrome

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Serotonin syndrome also brings fever, sweating and confusion. It follows serotonergic drugs, usually starts within hours, and features jerking muscles and overactive reflexes rather than stiff, hard-to-bend limbs after an antipsychotic increase.

B **Incorrect** (trap: Near-miss diagnosis). Acute dystonia is a sudden muscle spasm, such as a twisted neck or upward-rolling eyes, soon after starting an antipsychotic. It does not cause high fever, confusion or unstable blood pressure.

C **Incorrect** (trap: Near-miss diagnosis). Psychosis can worsen, but it does not explain fever, heavy sweating, rigid limbs and swinging blood pressure. Those physical signs point to a medication reaction that is a medical emergency.

D **Correct.** Neuroleptic malignant syndrome is a rare, potentially fatal reaction to dopamine-blocking drugs, often after a new medicine or a dose increase. Its core features are fever, severe muscle rigidity, changed mental status and unstable blood pressure or pulse, developing over days (DSM-5-TR). This patient shows all four after a haloperidol increase. It beats A because serotonin syndrome follows serotonergic drugs, usually begins within hours and features muscle jerking and brisk reflexes rather than rigidity. Either condition needs emergency medical care (Kaplan & Sadock).

Takeaway: Fever, rigidity, confusion and unstable vital signs days after starting or raising an antipsychotic suggest neuroleptic malignant syndrome, a medical emergency.

Sources: DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Neuroleptic Malignant Syndrome; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Medication-induced movement disorders: neuroleptic malignant syndrome
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346. ASM-1079 (rev 1) · Best answer: B

Ten days after starting sertraline for depression, a client on a telehealth visit says the pills 'are not doing anything' and plans to stop them this weekend. The client mentions mild nausea that is fading. The client has not talked with the prescriber about stopping. What is the **MOST** appropriate response from the social worker?

A. Suggest asking the prescriber for a stronger dose
B. Explain the usual delay in benefit and urge a prescriber call
C. Explore the client's general beliefs about taking medication

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Recommending a dose change is outside the social work role, and 10 days is too soon to judge the current dose. The client can raise any concern with the prescriber, who decides on dosing.

B **Correct.** Antidepressants usually take 4 to 8 weeks to work, and NIMH advises against stopping a prescribed medicine without a health care provider's help (NIMH, 2023). The client may simply not know about the delay. Giving general medication information and linking the client to the prescriber fits the social work role and leaves the decision with the client and prescriber. It beats C because exploring beliefs can come later; the client plans to stop within days for a reason that brief education may resolve.

C **Incorrect** (trap: Good but not best). Beliefs about medication shape adherence and are worth exploring. Here the client's reason is specific and fixable: they expect results too soon. Exploration alone leaves the weekend plan to stop in place.

Takeaway: Antidepressants take weeks to work; give general information and route stopping decisions to the prescriber.

Sources: National Institute of Mental Health (2023). Mental Health Medications, Antidepressants; stopping medication
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347. ASM-1162 (rev 1) · Best answer: B

Long-term antipsychotic use can lead to involuntary lip smacking, chewing motions, and tongue movements. Which movement disorder does this describe?

A. Acute dystonia
B. Tardive dyskinesia
C. Akathisia
D. Parkinsonism

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Acute dystonia means sudden, sustained muscle spasms, such as a twisted neck or eyes rolled upward. It usually appears within days of starting an antipsychotic, not after years.

B **Correct.** Tardive dyskinesia is a late-appearing movement disorder linked to long-term use of antipsychotics and other dopamine-blocking drugs. It causes involuntary movements, often of the mouth, lips and tongue, and may persist after the drug is stopped (DSM-5-TR). A social worker who notices these signs alerts the prescriber. It

beats D because parkinsonism brings tremor, stiffness and slowed movement, usually within weeks of starting or raising a dose, rather than repetitive mouth movements after years.

C **Incorrect** (trap: Near-miss diagnosis). Akathisia is an inner restlessness with an urge to keep moving, such as pacing or shifting in a chair. It typically begins within weeks of starting or raising a dose.

D **Incorrect** (trap: Near-miss diagnosis). Drug-induced parkinsonism causes tremor, rigidity, slowed movement and a mask-like face. It tends to develop within weeks of treatment and is not marked by repetitive mouth and tongue movements.

Takeaway: Repetitive involuntary mouth and tongue movements after long-term antipsychotic use suggest tardive dyskinesia; report them to the prescriber.

Sources: DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Tardive Dyskinesia

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348. ASM-1163 (rev 1) · Best answer: C

During a telehealth session, a client who takes phenelzine for depression says they ate aged cheese and dry salami at a party an hour ago. The client now has a pounding headache at the back of the head, a stiff neck, and a racing heart. What should the social worker do **FIRST**?

A. Contact the prescriber and wait for a call back

B. Have the client lie down and recheck the symptoms in an hour

C. Arrange immediate emergency treatment for the client

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). The prescriber needs to know, and that call can follow. Waiting for a call back while blood pressure may be climbing risks a stroke or other serious harm.

B **Incorrect** (trap: Ignores risk). Rest suits an ordinary headache, but these symptoms after tyramine-rich food on an MAOI signal a possible hypertensive crisis. Waiting an hour could allow serious harm.

C **Correct.** Phenelzine is a monoamine oxidase inhibitor (MAOI). Eating foods high in tyramine, such as aged cheese and cured meats, while taking it can set off a hypertensive crisis, a dangerous spike in blood pressure marked by severe headache, stiff neck and palpitations (Kaplan & Sadock). This is a medical emergency, so the social worker helps the client get emergency care at once, confirming the client's location. It beats A because the prescriber should be informed, but waiting for a call back delays treatment that may prevent a stroke.

Takeaway: Severe headache, stiff neck or palpitations after tyramine-rich food on an MAOI signal hypertensive crisis: get emergency care now.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Monoamine oxidase inhibitors: hypertensive crisis and dietary restrictions

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349. ASM-1164 (rev 2) · Best answer: A

A client stopped taking paroxetine 2 days ago after losing insurance coverage. The client now has dizziness, nausea, irritability, and brief 'electric zaps' in the head, and fears the depression is coming back. What is the social worker's **BEST** response?

A. Help the client reach the prescriber today about the stop and cost
B. Explain that these symptoms are harmless and will fade soon
C. Arrange an emergency evaluation for possible serotonin syndrome

Why each option is right or wrong

A **Correct.** Stopping an antidepressant abruptly, especially one that leaves the body quickly such as paroxetine, can cause a discontinuation syndrome with dizziness, nausea, irritability and electric-shock sensations (DSM-5-TR). It is distressing but rarely dangerous, and the prescriber can restart the drug, plan a taper or choose a lower-cost option. Linking the client today addresses both the symptoms and the insurance barrier that caused them. It beats C because serotonin syndrome follows adding or raising a serotonergic drug, not stopping one, so emergency care is not needed here. It beats B because reassurance leaves the medication decision and relapse risk unaddressed.

B **Incorrect** (trap: Good but not best). Discontinuation symptoms often ease within a few weeks, so this is partly accurate. But the client also risks depressive relapse off medication, and judging medical symptoms harmless is not the social worker's call; the prescriber should weigh in.

C **Incorrect** (trap: Skips assessment). Serotonin syndrome follows adding or increasing serotonergic drugs, not stopping one, and it brings fever, agitation and muscle jerks. Emergency care is right for those signs; here a same-day prescriber contact fits.

Takeaway: Symptoms after abruptly stopping an antidepressant call for prompt prescriber contact; address the barrier, such as cost, that caused the stop.

Sources: DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Antidepressant Discontinuation Syndrome
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350. ASM-1165 (rev 1) · Best answer: C

An 82-year-old client has taken lorazepam every night for 10 years. After two falls this month, the client's adult child tells the home-visit social worker they plan to throw the pills away tonight. The client says they would like to stop the pills but fear they will not sleep. What is the **BEST** action?

A. Support removing the pills tonight to prevent another fall
B. Help the family make the home safer against falls
C. Arrange a prescriber review to plan a gradual taper
D. Respect the client's fear and keep the routine as is

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). The goal is right, but stopping a nightly benzodiazepine abruptly after 10 years can cause severe withdrawal, including seizures. Removal should happen through a supervised taper, not overnight.

B **Incorrect** (trap: Good but not best). Home safety changes reduce fall risk and can be part of the plan. They do not address the sedating medication that is a likely cause of the falls.

C **Correct.** Benzodiazepines are on the Beers list of drugs to avoid in older adults because they raise the risk of falls, confusion and fractures (American Geriatrics Society, 2023). After long daily use, stopping suddenly can cause withdrawal, including seizures, so the FDA advises a gradual, individualized taper (FDA, 2020). A prescriber-led taper supports the client's own wish to stop while addressing sleep worries. It beats B because fall-proofing helps but leaves in place the drug most likely driving the falls.

D **Incorrect** (trap: Good but not best). The client's fear deserves attention, but the client has said they want to stop. Keeping the routine leaves a high fall risk in place when a safe taper could honor the client's goal.

Takeaway: Older adults on long-term benzodiazepines who fall need prescriber-guided tapering; abrupt stopping risks dangerous withdrawal.

Sources: American Geriatrics Society Beers Criteria Update Expert Panel (2023). American Geriatrics Society 2023 updated AGS Beers Criteria for potentially inappropriate medication use in older adults. Journal of the American Geriatrics Society, 71(7), 2052-2081, Benzodiazepines; FDA (U.S. Food and Drug Administration) (2020). FDA requiring Boxed Warning updated to improve safe use of benzodiazepine drug class. Drug Safety Communication, September 23, 2020: withdrawal reactions and tapering

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351. ASM-1166 (rev 1) · Best answer: B

In a nursing facility, a resident with major neurocognitive disorder due to Alzheimer's disease has become agitated in the evenings. The physician proposes starting risperidone. The resident's adult child, who is the health care proxy, has read that such drugs can be risky in dementia and asks the social worker what to do. What is the **MOST** helpful response?

A. Advise the proxy to refuse the drug because of its known risks

B. Help the proxy ask the physician about risks and alternatives

C. Reassure the proxy that the physician has weighed the risks

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). The proxy's concern is valid, but advising refusal is a medical judgment outside the social work role. The decision belongs to the proxy after a discussion with the physician.

B **Correct.** Antipsychotics carry a boxed warning for increased death rates in older adults with dementia-related psychosis. The APA practice guideline advises reviewing nondrug approaches and discussing risks and benefits with the patient's surrogate before starting one (Reus et al., 2016). The social worker can confirm that the concern is a real one and help the proxy prepare questions, supporting an informed decision without giving medical advice. It beats C because reassurance closes off a discussion the proxy is entitled to have.

C **Incorrect** (trap: Good but not best). The physician may well have weighed the risks, but the proxy has the right to understand them before consenting. Reassurance discourages the questions informed consent depends on.

Takeaway: Help surrogates weigh antipsychotic risks in dementia by preparing questions for the prescriber; don't advise for or against the drug.

Sources: Reus, V. I., Fochtmann, L. J., Eyler, A. E., et al. (2016). The American Psychiatric Association practice guideline on the use of antipsychotics to treat agitation or psychosis in patients with dementia. American Journal of Psychiatry, 173(5), 543-546, Guideline statements

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352. ASM-0010 (rev 1) · Best answer: B

A client treated for panic disorder has had no panic attacks for 2 months and uses breathing and exposure skills without prompting. At the planned next-to-last session, the client says they feel nervous about ending and asks to keep meeting weekly 'just in case.' What is the social worker's **BEST** response?

A. Continue weekly sessions until the client feels ready to stop

B. Explore the client's worries and review the gains and relapse plan

C. Resume exposure work because the anxiety signals a relapse

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Open-ended weekly sessions after goals are met can foster dependence and confirm the client's fear of coping alone. A planned step-down or booster session could be discussed instead of continuing by default.

B **Correct.** Lasting symptom relief and independent use of skills are signs of readiness for termination. Anxiety about ending is a common, expected reaction rather than a sign of relapse (Hepworth et al.). The best response explores the worry, reviews what the client has achieved, rehearses a relapse-prevention plan and makes clear how to return if needed. It beats A because extending weekly sessions without a clinical need can build dependence and implies the client cannot cope without the social worker. Services end when they are no longer needed (NASW Code 1.17(a)).

C **Incorrect** (trap: Skips assessment). Nervousness about ending is different from a return of panic attacks. Treating it as relapse misreads a normal termination reaction and undercuts the client's confidence in skills already mastered.

Takeaway: Goals met and skills used independently signal readiness to end; anxiety about ending is normal and is processed, not treated as relapse.

Sources: NASW Code of Ethics (2021), 1.17(a); Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, termination: reactions to ending

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353. ASM-0119 (rev 1) · Best answer: B

A client began services 5 months ago for social anxiety, with goals of joining a weekly activity and speaking up in work meetings. At a progress review, the social worker considers several findings. Which finding **MOST** clearly indicates that the client is ready to end services?

A. Says sessions have become the best part of the week
B. Has reached both targets and copes with setbacks on their own
C. Has attended every session and completed each assignment

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Valuing sessions highly can reflect a strong alliance, but it can also signal reliance on the relationship. It says nothing about whether goals are met and may need to be explored as ending approaches.

B **Correct.** Readiness for termination rests on goal attainment and the client's ability to maintain gains independently (Hepworth et al.). This client has reached both treatment goals and manages setbacks without the social worker, which shows the change can last. Services should end when they are no longer needed (NASW Code 1.17(a)). It beats C because perfect attendance and completed homework show strong engagement in the work, not that the work is finished.

C **Incorrect** (trap: Good but not best). Attendance and homework completion show engagement and predict good outcomes, but they describe the process, not the result. A highly engaged client may still have goals left to reach.

Takeaway: Readiness to end services is shown by goals met and gains kept independently, not by attendance or attachment to sessions.

Sources: NASW Code of Ethics (2021), 1.17(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, termination: determining readiness
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354. ASM-1619 (rev 1) · Best answer: C

A school social worker has seen an 8-year-old for separation anxiety since the fall. The child now attends school every day, the teacher reports no morning distress for 6 weeks, and the parent handles drop-offs with the plan they practiced. The child says, 'I like coming here. Can we keep playing?' What is the **BEST** way to proceed?

A. Continue weekly sessions because the child enjoys them
B. End services at the next session to prevent dependence
C. Plan a gradual ending with fewer sessions over several weeks

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Enjoying sessions is not a treatment need. Continuing after goals are met serves the relationship rather than a clinical purpose and can suggest to the child and parent that the child cannot cope without help.

B **Incorrect** (trap: Good but not best). Ending is appropriate, but a sudden stop gives a young child little time to prepare and can feel like a loss. A short planned taper with a review of progress supports lasting gains better than an abrupt close.

C **Correct.** Readiness for termination shows when goals are met, gains hold over time and the client and family use their skills on their own (Hepworth et al.). Daily attendance, 6 weeks without morning distress and a parent who manages drop-offs meet that standard, and services should end when no longer needed (NASW Code 1.17(a)). Spacing out sessions lets the child practice coping with less support and makes ending a success rather than a loss. It beats B because stopping within a week gives a young child little time to prepare.

Takeaway: Once gains hold, taper toward a planned ending; a child's enjoyment of sessions is not a reason to continue, and abrupt stops feel like loss.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, termination: planned endings and readiness; NASW Code of Ethics (2021), 1.17(a)
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355. ASM-1620 (rev 2) · Best answer: A

An adult client in outpatient treatment for PTSD after a car crash agreed in the fifth session to begin processing the crash memory. The next week, the client says, 'I think I'm better now. Let's make this our last session.' Nightmares still occur several times a week, and the client still avoids driving. What should the social worker do **FIRST**?

A. Ask what has prompted the wish to stop now
B. Agree to end and explain how to return to services
C. Pause trauma processing and shift to coping skills
D. Point out that ongoing nightmares mean the goals are unmet

Why each option is right or wrong

A **Correct.** Readiness for termination rests on goals met and gains that hold, not on a request alone (Hepworth et al.). A request to stop right after trauma processing begins may reflect avoidance, a core PTSD symptom, or another reason such as cost or a hard reaction to the last session. Exploring what prompted it, without judgment, comes first because the answer decides what follows: support for the work, a slower pace or a planned ending the client chooses with full information (NASW Code 1.02). It beats D because citing symptoms before hearing the client's reasons can feel like pressure and push an avoidant client out of care.

B **Incorrect** (trap: Right action, wrong time). If the client still chooses to stop once the reasons and progress are discussed, the social worker respects that choice and leaves the door open. Agreeing at once skips learning whether avoidance is driving an early exit from effective treatment.

C **Incorrect** (trap: Good but not best). Slowing the pace of trauma work can help when processing feels overwhelming. Changing course before learning why the client wants to stop guesses at the problem and may not match what the client actually wants.

D **Incorrect** (trap: Too directive). Ongoing nightmares and avoidance are relevant, and reviewing progress against goals will matter. Leading with them before asking why the client wants to stop can feel like an argument and may harden the decision.

Takeaway: When a client asks to end abruptly, especially just after hard work begins, explore the reasons before agreeing, arguing or changing the plan.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, termination: indicators of readiness and premature termination; NASW Code of Ethics (2021), 1.02
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356. ASM-1691 (rev 1) · Best answer: B

After 10 weeks of in-home family services, a parent has met the plan's goals during visits by using a reward chart and calm time-outs with a 6-year-old. The parent's daily log shows the chart is used only on visit days, and the parent calls the social worker during most conflicts between visits. The supervisor suggests closing the case. What is the **BEST** response?

A. Close services, since the plan's goals were met during visits
B. Continue services while fading support to build independent use
C. Refer the parent to a community parenting class and close the case

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). The goals were met in the worker's presence, but readiness means the parent can use the skills alone. Closure is the right step once the log shows steady use between visits.

B **Correct.** A central sign of readiness to end is that the client uses new skills without the worker and keeps gains between contacts (Hepworth et al.). The log shows the chart is used only when the worker visits, and the parent still relies on calls to manage conflicts, so the gains are not yet the parent's own. Continuing while deliberately fading support tests and builds independent use. It beats A because closing now would end services for a family still in need of them, which the Code asks workers to avoid (NASW Code 1.17(b)).

C **Incorrect** (trap: Hands off too soon). A parenting class may help later, but it hands off a family whose gains still depend on the current worker. Closing now with a class referral risks losing progress before it becomes independent.

Takeaway: Close a case once skills appear without the worker present; skills seen only during visits call for fading support first.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Termination: assessing readiness to end; NASW Code of Ethics (2021), 1.17(b)
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357. ASM-1692 (rev 1) · Best answer: C

A couple began counseling 5 months ago for frequent arguments. For the past 2 months, they report using time-outs and calm check-ins on their own, and they recently settled a disagreement about moving without help. They canceled the last two sessions. When the social worker asks about the cancellations, both say things are going well and they feel less need to come. What should the social worker do **NEXT**?

A. Explore whether the cancellations reflect unspoken dissatisfaction
B. Continue biweekly sessions to guard against slipping back
C. Propose planning an ending with a final review session

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Checking whether cancellations signal dissatisfaction is wise, and the worker has already asked. Both partners tied the cancellations to progress, so probing again would second-guess a clear answer.

B **Incorrect** (trap: Good but not best). Ongoing sessions may seem protective, but the couple has shown they keep their gains without help. Extending services past need does not serve them; a return option can be offered at the ending.

C **Correct.** Signs of readiness to end include goals met, skills used independently over time, and gains that hold under real stress (Hepworth et al.). This couple has used their skills alone for 2 months, handled a major decision without help, and, when asked, linked the cancellations to doing well. Proposing a planned ending with a final review consolidates gains and prevents an unplanned drift out of services. It beats B because continuing sessions the couple no longer needs does not serve them, and the Code calls for ending services when they are no longer required (NASW Code 1.17(a)).

Takeaway: When clients use skills independently and gains hold under stress, propose a planned ending rather than continuing or letting services drift.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Termination: indicators of readiness and planned endings; NASW Code of Ethics (2021), 1.17(a)
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